CRESCENT

Policies & Procedures (2026)

For
Crescent Community Care Services Limited

Care Staff

Absenteeism Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To manage absenteeism in such a way as to minimise instances of absenteeism and the effects of unavoidable absenteeism.

Scope

- All employees.

Policy

- Avoidable absenteeism, such as short notice unexplained/un-evidenced sickness, will not be tolerated and may lead to disciplinary action.

- Targets for acceptable absenteeism, such as sickness, will be set (e.g. national average sickness rates) and employees assessed against those targets.

- All absenteeism will be recorded on the personnel file, and the information will be:

  - Reviewed, assessed and discussed during formal supervisions, where acceptable personal targets will be set;
  - Passed to future employers.

- Staff will be interviewed on returning from every instance of absenteeism.

Procedure

- Staff will receive instructions, via the Employee Handbook, about the conduct expected with respect to absenteeism. In general, the organisation should be informed of the intended absence as soon as possible on the first day of absence. If appropriate, the duty manager of the establishment where the employee was expected to work must be informed directly. Failure to do so may lead to disciplinary action.

- An absence form is to be completed for every instance of absenteeism, immediately on return to work. In the case of absence due to sickness, the appropriate form (attached to the Sickness Absence Policy and Procedure) should be used; in the event of absence for any other reason, the form attached below must be used.

- The absence form will be presented to the manager, who will interview the employee regarding the absence and agree with them a strategy for avoiding future absences.

- Unauthorised absences will not be paid for, and may not be converted into holiday pay unless agreed by the manager during contact on the first day of absence.

Reviewed by Marcus Kerridge McColl
December 2025

Purpose

- To comply with statutes, regulations and quality standards.
- To ensure that accidents and incidents are recorded and analysed in order to satisfy health and safety requirements and provide information for the minimisation of risks.
- All persons affected by accidents or incidents.

Policy

- Accidents which cause injury to any person on the organisation's premises, or employees on duty but not on the premises, or Service Users at any time, will be recorded to promote analysis and management of risks.
- The appropriate recording book is "Accident Book B1510", obtainable from good bookshops and Amazon.

Procedure

- In the event of an accident, or an incident (e.g. a "near miss" which did not result in an injury but which may have done so in different circumstances) an accident/incident form should be completed and immediately submitted to the Registered Manager, who, after review of the facts, will take those actions necessary to minimise danger of the same accident/incident in future. These actions should be noted on the form, using the reverse if necessary. The progress of the treatment of any injury must also be recorded, together with any final outcomes evident at the time of completion and transmission of the form. The Registered Manager must sign the form on completion of the investigation in order to denote that they have discharged their responsibility.

- If the accident or incident involves a Service User, the Duty of Candour Policy and Procedure should also be referred to and, if applicable, followed. This policy can be located in the Administration section of your QCS system.

- After the accident/incident investigation and all matters concerned with it are complete, a copy of the signed accident/incident form should be placed in the personal file of any person(s) affected by the accident, and the original placed in the accident book in the department. In the case of employees the record must be kept on their personnel file, and in the case of Service Users, kept on the Care Plan.

- The purpose of the records held in the department is to provide a chronological, ordered record of accidents.

- The intention of the records placed in the personal file(s) is to provide a record of accidents organised by person, and another record organised by accident type.

- Registered Manager is responsible for the recording of accidents in accordance with RIDDOR (Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013), by:

  - Ensuring that this policy and procedure is followed and all accidents are recorded, using the form attached;
  - Ensuring that all accidents which result in absence from work for more than seven days (not including the day of the accident) are reported within 10 days to the Health and Safety Executive;

RIDDOR 2013 reporting requirements

Reportable incidents under RIDDOR 2013 are: Š

- Death and injuries where:

  - The accident is work-related;
  - It results in an injury of a type which is reportable. Š

- The types of reportable injuries are:

  - Death;
  - Specified injuries, which are defined by the HSE as:

    - Fractures, other than to fingers, thumbs and toes.
    - Amputations.

- Any injury likely to lead to permanent loss of sight or reduction in sight.
- Any crush injury to the head or torso causing damage to the brain or internal organs. Œ
- Serious burns (including scalding) which:
  - Cover more than 10% of the body;
  - Cause significant damage to the eyes, respiratory system or other vital organs. Œ
- Any scalping requiring hospital treatment.
- Any loss of consciousness caused by head injury or asphyxia.
- Any other injury arising from working in an enclosed space which:
  - Leads to hypothermia or heat-induced illness;
  - Requires resuscitation or admittance to hospital for more than 24 hours.

- The following occupational diseases are reportable:

  - Carpal tunnel syndrome;
  - Severe cramp of the hand or forearm;
  - Occupational dermatitis;
  - Hand-arm vibration syndrome;
  - Occupational asthma;
  - Tendonitis or tenosynovitis of the hand or forearm;
  - Any occupational cancer;
  - Any disease attributed to an occupational exposure to a biological agent.

- Dangerous occurrences require reporting, for example:

  - The collapse, overturning or failure of load-bearing parts of lifts and lifting equipment;
  - Plant or equipment coming into contact with overhead power lines;
  - Explosions or fires causing work to be stopped for more than 24 hours.

- For full details of reporting requirements, go to the HSE website at: http://www.hse.gov.uk/riddor/reportable-incidents.htm

- Contacts for the HSE, including for online reporting can be found at: http://www.hse.gov.uk/contact/contact.htm

- **Terminology:**

  - An accident is an incident which results in an injury to a person;
  - An incident is similar, but did not on that occasion result in injury, but is required to be recorded and investigated in order to prevent a recurrence and possible injury in the future;
  - An incident could also include an event which led only to property damage;
  - Persons filling in the reporting form should not be concerned about differentiating between an incident and an accident if the allocation is unclear – the Registered Manager will complete the allocation on review of the form.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To maintain safe and effective working conditions.

Scope

- All workers.

Policy

- Crescent Community Care Services Ltd will work with employees and other workers to ensure that their use of either alcohol or drugs does not impair the safe and efficient running of the organisation or the health of its employees.

Procedure

THE WARNING SIGNS!

- Supervisors and managers should be aware that the misuse of drugs or alcohol by employees could come to light in various ways. The following characteristics, especially when arising in combinations, may indicate the presence of an alcohol or drug-related problem.

ABSENCES

- Instances of unauthorised absences.
  Frequent Friday and/or Monday absences.
  Leaving work early.
  Lateness (especially on returning from lunch).
  Excessive level of sickness absence.
  Strange and increasingly suspicious reasons for absence.
  Unusually high level of sickness for colds, flu, and stomach upsets. Š Unscheduled short-term absences, with or without explanation.

HIGH ACCIDENT LEVEL

- At work.
- Elsewhere, i.e. driving, at home.

PERFORMANCE

- Difficulty in concentration.
- Work requires increased effort.
- Depression.
- Irritability.
- General confusion.
- Individual tasks take more time.
- Problems with remembering instructions or own mistakes.

MOOD SWINGS

- Irritability

- Depression
- General confusion.

MISCONDUCT

- An employee's alcohol or drug problem may come to light as a mitigating factor in a disciplinary interview. It should be treated as a mitigating factor for certain "less serious" disciplinary offences, such as poor time-keeping or lateness, provided the person is prepared to undergo treatment.

SELF-REFERRAL

- In some instances, employees may seek help and advice themselves.

INTERVENTION

- Supervisors/managers who feel that an employee's unsatisfactory performance may be drug or alcohol-related should inform their immediate supervisor to discuss the matter. If thought necessary, the department manager should arrange for a meeting with the employee. At the meeting the individual should have their unsatisfactory performance/conduct and "indicators" discussed.
- The department manager should restate the organisation's required standards, making sure the employee understands what is expected of them.
- The department manager should try to establish the cause of the problem (although it must be pointed out that individuals with a drugs or alcohol problem will often go to great lengths to conceal the situation).
- The employee should be reminded or informed of the assistance the organisation is prepared to give employees who are trying to overcome a drink or drugs problem and should be informed of outside agencies at which help can be obtained. The employee should be informed that the organisation requires that their performance be improved to an acceptable standard and that failure to achieve this will result in dismissal.
- The department manager should agree with the employee as to what follow-up action should be taken.
- Where it is established that alcohol or drugs are or could be the problem, an appointment should be arranged with the organisation's Medical Advisor or an independent medical practitioner.
- If the employee denies that either alcohol or drugs are the cause of the problem (whether believed or not), he or she should be treated the same as they would for any other disciplinary/capability problem, whichever is judged as appropriate by the employer. However, if there are strong signs that the employee's unsatisfactory performance is drug or alcohol-related, and they will not admit or acknowledge this, further encouragement should be given at all stages of the disciplinary/capability procedure for the individual to face up to the true, underlying problem(s).

TREATMENT

- Where employees acknowledge that they have a problem and are given help and treatment, this will be on the understanding that:
  - While they are undergoing treatment, they will be on sick leave and will be entitled to the usual Organisation sick pay benefits.
  - Every effort should be made to ensure that on completion of the recovery programme employees are able to return to the same or equivalent work.
- However, where such a return would jeopardise either a satisfactory level of job-performance or the employee's recovery, the department manager will review the full circumstances surrounding the case and agree a course of action to be taken. This may include the offer of suitable alternative employment, the consideration of retirement on the grounds of ill health, or dismissal. (Before a decision on dismissal is made, it should be discussed with the employee and an up-to-date medical opinion should be obtained – see procedure for dealing with ill-health dismissals).

RELAPSE

- Where an employee, having received treatment, suffers a relapse, the Organisation will consider the case on its individual merits. Medical advice will be sought in an attempt to ascertain how much more treatment/rehabilitation time is likely to be required for a full recovery. At the organisation's discretion, more treatment or rehabilitation time may be given in order to help the employee to recover fully.

RECOVERY UNLIKELY

- If, after employees have received treatment, recovery seems unlikely, the organisation may be unable to wait for the employee any longer. In such cases a clear warning will be given to the employee beforehand, and a full medical investigation will have been undertaken.

- In certain cases, strictly at the discretion of management, early retirement may be considered.

SERIOUS MISCONDUCT CAUSED BY ALCOHOL OR DRUGS

Intoxicated employees:

- If an employee is known to be, or is strongly suspected of being, intoxicated by alcohol or drugs during working hours, the department manager will be consulted. Arrangements will be made for the employee to be escorted from the company premises immediately. Disciplinary action will take place when the employee has had time to sober up.

Consumption of alcohol on the premises

- Employees are, under normal circumstances, expressly forbidden to bring or consume alcohol on the premises. Any breach of this rule will result in disciplinary action being taken which is likely to result in summary dismissal. There are, however, certain occasions when the management will make an exception, for example at the annual Christmas party. Such occasions will be clearly notified to employees in advance via the organisation notice boards. At such times, employees will be expected to behave in a responsible manner.

- Drivers are advised, where necessary, to find alternative means of transport home.

DRUG ABUSE ON THE PREMISES

- Employees who take drugs which have not been prescribed on medical grounds will, in the absence of mitigating circumstances, be deemed to be committing an act of gross misconduct and will thus render themselves likely to be summarily dismissed as will any employee believed to be buying or selling drugs, or in possession of unlawful (i.e. non-prescribed) drugs.

TRAINING AND DEVELOPMENT

- Supervisors and managers will be given training in order to develop "early recognition" techniques for identifying any employee who may be misusing alcohol or drugs. Effective interviewing and counselling skills will be developed so that employees can be dealt with promptly, tactfully and firmly.

- All staff will be made aware of the effects of alcohol and drug misuse. They will be encouraged not to cover up for employees with a drink or drug problem but to recognise that collusion represents a false sense of loyalty and will in the longer-term damage those employees. Employees who recognise that they have a drink or drug problem, or that they are at risk of developing one, should be encouraged to come forward for confidential help. They should seek an appointment, in confidence, with the organisation's Medical Advisor.

- Contacts at outside agencies where help can be obtained will be made readily available to all members of staff via the organisation notice boards.

Reviewed by Marcus Kerridge McColl

December 2025

Purpose

- To provide an extended period of leisure time, during which employees can have a break from work, relax and therefore, return to their jobs refreshed.

Scope

- All employees.

Policy

- It is the organisation's policy that employees should be given paid holiday every year, and that this holiday is taken.
- Employees should have the opportunity to take the holiday to which they are entitled.
- Managers are responsible for ensuring that the workload does not prevent this while also seeing that the needs of the organisation are taken into account.
- This policy does not form part of any employee's contract of employment and the organisation may vary or amend it at any time.

Procedure

OBTAINING APPROVAL

- All annual holiday must receive management approval before being taken. When you wish to take part of your annual holiday entitlement you should follow the procedure set out below:
    - All requests for annual holiday should be made in writing on the appropriate form (Holiday Request Form), at the earliest reasonable opportunity and generally no later than 4 weeks before the required start date. They should be passed to the Manager.
    - The Manager will consider the application, having regard for the needs of the organisation and the rules on annual holiday set out below. Where necessary, the Person Registered will be consulted before a final decision is made.
    - A copy of the form marked "approved" or "not approved", with brief reasons for refusal where applicable, should be signed by the Manager and returned to you. Only on receipt of this form, duly approved, should you commit yourself to any holiday plans. If you pay a deposit for the holiday, before the dates are approved, then the organisation is not responsible for any losses you may incur if the dates are refused.
    - In the event you dispute the decision of management, the organisation's Grievances Policy and Procedure should be used. Once approval for annual holiday has been given it will not normally be withdrawn.

CARRYING HOLIDAY OVER

- The organisation's holiday year runs for the period:

  1st April – 31st March

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- Your full annual holiday entitlement for the year must be taken, during the holiday year in which it accrues. No payment in lieu will be made for any untaken holiday, nor will you be allowed to carry it over to the next year in normal circumstances.
- Untaken holiday entitlement can only be carried over into another holiday year:
    - In cases involving long term sickness absence (see below);
    - In cases of maternity, paternity, adoption, parental or shared parental leave;
    - In exceptional circumstances, where your Manager has given permission in writing, or;
    - If otherwise required by law.

ENTITLEMENT

- Unless otherwise stated in an individual contract, you are entitled to

  5.6 weeks

  annual holiday per year (or the pro-rata equivalent if you work on a part time basis).

- 1 “week” is defined as the average working week of the twelve weeks prior to the holiday.

- If your employment starts or finishes part way through the holiday year, your holiday entitlement during that year shall be calculated on a pro-rata basis.

ACCRUAL

- In your first year of employment, and for leave purposes only, you accrue leave entitlement at one twelfth of your annual entitlement for each calendar month in which you work. Accrued entitlement for leave purposes is rounded to the nearest half-day. No service with any other employer counts for these purposes.

- Holidays may not usually be taken in advance of being accrued. If you wish to exceed this limit you must make your request to management at least 3 months in advance. If the holiday is approved, you are reminded that if you leave employment with the organisation having taken holidays in excess of your accrued pay entitlement, the excess will be reclaimed and deducted from your wages. In the event that the excess owed by you exceeds the wages owed, you must refund the difference to the organisation personally.

- For purposes of holiday pay due (or to be repaid) on termination, accrued holiday is calculated pro-rata to the actual days on which you were employed in the holiday year; irrespective of whether this is your first year of employment or not. For pay purposes there is no half-day rounding.

NEW EMPLOYEES

- Every effort will be made to meet the needs of new employees in respect of commitments to holidays already made. If you are a new employee you should inform the organisation of any holiday plans at the earliest opportunity. Where the booked holiday exceeds your annual holiday entitlement, consideration will be given to granting unpaid holiday up to a maximum of 2 weeks.

HOLIDAY RULES

- The following rules apply to all holidays:

  - Staff are unable to book holidays for two weeks over the Christmas and New year period.

- Where there are conflicting holiday requests, priority will normally be given to the employee whose request was received first. Management may exercise some discretion to ensure equitable treatment.

- No more than 2 weeks of annual holiday may normally be taken at one time. If you wish to exceed this limit you must make your request to management at least 3 months in advance.

- No more than 2 weeks holiday may be taken during the two quarters from April to September, and one each on the following two quarters. If you wish to exceed this limit you must make your request to management at least 3 months in advance.

- If you take annual holiday not previously approved you will be deemed to be absent without authorisation and you may be subject to disciplinary action, which could include dismissal.

- The organisation may require you to take (or not to take) holiday on particular dates, including when the business is closed, particularly busy, or during your notice period.

- If you are ill or injured whilst on holiday, to the extent that you would be unable to come to work (when you are meant to be at work) then you may choose to treat the period of incapacity as sick leave and reclaim the affected days of holiday. In these circumstances, you must comply with the organisation's sickness absence reporting requirements as if you were at work. If you are already on sick leave before a pre-arranged period of holiday, you may choose to cancel any days of holiday that coincide with the period of incapacity and treat them as sick leave. You will be entitled to any SSP (not holiday pay) in

relation to any such days of sickness. Any dishonest claims or other abuse of these rules will be treated as misconduct under the organisation's Discipline Policy and Procedure.

- Employees who are on sickness absence, including long term absence, continue to accrue statutory holiday entitlement. There may be limits to this as determined by law.

LATE RETURN

- If, for any reason, you know that you will be late returning from holiday, you must contact the Manager and notify them of your late return as soon as possible. The Manager will then discuss your options with you. Failure to do so will render you liable to disciplinary action for unauthorised absence.

LONG TERM SICKNESS ABSENCE AND HOLIDAY ENTITLEMENT

- Holiday entitlement continues to accrue during periods of sickness absence.

- If you are on a period of sickness absence which spans two holiday years, or if you return to work after sickness absence so close to the end of the holiday year that you cannot reasonably take your remaining holiday, you may carry over any unused holiday entitlement into the following holiday year. Carry over under this rule is limited to the four-week minimum holiday entitlement under EU law (which includes bank holidays), less any leave taken in the holiday year that has just ended. If you have taken four weeks' holiday by the end of the holiday year, you will not be entitled to carry on any leave over into the next holiday year.

- Any holiday that is carried over under this rule but is not taken within 18 months of the end of the holiday year in which it accrued will be lost.

- Alternatively, you can choose to take your paid holiday during your sick leave, in which case you will be paid at your normal rate of pay.

TERMINATION

- On termination of employment you may be required to use any remaining holiday entitlement during your notice period. Alternatively, you will be paid in lieu of any accrued but untaken holiday entitlement for the current holiday year to date. You are entitled to be paid at a rate of 1/260th of your full time equivalent salary for each day of untaken entitlement.

- If, upon termination of your employment, you have taken more annual holiday than you have accrued, the organisation reserves the right to deduct the equivalent of the payments made for such additional days from your final pay (calculated at 1/260th of your full-time equivalent salary for each excess day).

GUIDANCE TO THE SERVICE PROVIDER

- Although this policy refers only to employees, it applies equally to workers, who are also entitled to paid holiday under the Working Time Regulations 1998.

ROLLED UP HOLIDAY PAY

- “Rolled-up” holiday pay agreements with employees and workers (where holiday pay is paid as an identified mark-up on normal pay, rather than being paid when holidays are actually taken) have been challenged in the European courts and the practice is now unlawful.

- Although technically, this poses a risk of employees and workers alleging that they have not been paid for the holidays that they have taken (and therefore seeking to claim their holiday pay twice), some employers choose to take the risk of paying rolled-up holiday pay on the basis that it is simple and convenient.

- Case law suggest that any compensation awarded to individuals who claim is likely to be low or nil, provided that the rolled-up holiday pay arrangement is transparent, reflected in their contracts, and the holiday pay is genuinely in addition to normal hourly pay. If the rolled-up holiday pay is not in addition to the hourly rate of pay (i.e. an identified mark up) employers are at risk of claims under the Working Time Regulations and/or National Minimum Wage legislation.

- If a rolled-up holiday pay system is used, full holiday records should be kept and individuals should be encouraged to take all their holiday entitlement each year.

- The use of rolled-up holiday pay is a decision for the service provider to take, having regard to the fact that it has been held to be unlawful. Quality Compliance Systems Limited accepts not responsibility for claims or complaints arising from a decision to implement or continue a rolled-up holiday pay system.

CALCULATING HOLIDAY PAY FOR CASUAL WORKERS OR WORKERS WITH IRREGULAR HOURS

- The decision to use rolled-up holiday pay is often driven by the need to provide paid holiday entitlement to those whose hours, and sometimes periods of employment, are very irregular.

- The holiday entitlement for these staff, can be calculated as 12.07% of the hours that the work (then added as a mark up on their pay as part of a rolled-up holiday pay arrangement, or added to their accrued entitlement under a traditional, more lawful, holiday pay system).

- The 12.07% is calculated as follows:

  - 5.6 weeks (statutory minimum holiday entitlement) divided by 46.4 weeks (52 week full year minus 5.6 weeks) x 100 = 12.07%.

  - The 5.6 weeks is excluded from the year as the employee would not be present during the 5.6 spent on holiday in order to accrue annual leave.

CARRYING OVER ANNUAL LEAVE

- Where an employee or worker is on maternity, paternity or parental or adoption leave, he/she is entitled to accrue statutory and contractual leave in the normal way. The holiday pay accrued during that absence may be rolled over to the following holiday year.

- A woman on maternity leave must be able to take her paid annual leave during a period other than her maternity leave. This applies to the entire 5.6 weeks of annual leave under UK law. Employers therefore have a duty to allow the worker to take all of her annual leave before she goes on maternity leave, or allow her to carry it over into the next holiday year. ECJ case law suggests that where a period or parental spans two holiday years, the same applies.

- In case of sickness, employees and workers who are ill while on holiday to the extent that they would be unable to come to work (when they are meant to be at work) must comply with sickness absence reporting requirements as if they were at work. If they do this, they will be paid SSP and entitled to re-take their holiday at a later date (and carry it over into the next holiday year if they are unable or unwilling to take it in the current holiday year due to sickness). Carry over under this rule is limited to the four-week minimum holiday entitlement under EU law (which includes bank holidays) and not the 5.6 weeks of annual leave under UK law, less any leave taken in the holiday year that has just ended. If the employee has taken four weeks' holiday by the end of the holiday year, he/she will not be entitled to carry any leave over into the next holiday year.

DEDUCTIONS FROM SALARY

- Where annual holiday taken exceeds what has been accrued at the date of termination, you have the right to deduct the equivalent of the payments made for such additional days from employees' final pay ONLY if it is provided for, in writing and signed, in the employment contract.

Reviewed by Marcus Kerridge McColl
December 2025

Purpose

- To ensure that all employees comply with the Bribery Act 2010 and all its provisions, with special emphasis on (a) not giving bribes; (b) not receiving bribes.
- To clarify our responsibilities as an organisation in preventing bribery within the organisation.
- In particular it is our responsibility, when new employees are recruited, to bring the anti-bribery policy and procedure to their attention and ensure that they sign the anti-bribery declaration. It is our responsibility to explain and ensure that employees understand fully the measures of the law and the organisation's intention to ensure that it is being fully implemented.

Scope

- All directors, employees, contractors, agency staff, volunteers, interns, any agent or other person acting on our specific instructions. If you fall into any one of these descriptions then reference to "you" in this policy refers to you.

Policy Background

- In guidance issued by the Ministry of Justice, a bribe is defined as: "Giving someone a financial or other advantage to encourage that person to perform their functions or activities improperly or to reward that person for having already done so". It goes on to state that "this could cover seeking to influence a decision-maker by giving some kind of extra benefit to that decision maker rather than by what can legitimately be offered as part of a tender process".
- So it includes:
    - Gifts;
    - Hospitality;
    - Facilitation payments;
    - Contributions to political organisations and charities.
- Crescent Community Care Services Limited can be liable under the Bribery Act if you commit a bribery offence. This is because your activities can be attributed to the organisation.
- You can also be punished as an individual by up to ten years imprisonment and a fine. If we as an organisation have taken part in corruption we could face an unlimited fine and considerable damage to our reputation. We therefore take our responsibilities very seriously.

Your Responsibilities

- It is your responsibility to read and be confident that you understand and fully comply with this policy. In any area of doubt refer to the Registered Manager.
- You are required to notify the Registered Manager if you suspect a breach of this policy has occurred.
- You will not suffer detriment as a result of any matter raised, in good faith, under this policy.
- However if you personally breach this policy then we may take disciplinary action (including possibly dismissal for Gross Misconduct) against you.
- If you are not an employee, we may terminate your contract with or without notice as appropriate.

What constitutes a bribe?

- If someone seeks to influence you to act so that you breach trust, lack impartiality or perform in bad faith (i.e. act improperly) then you need to be on your guard. If they seek to do so by offering, promising or giving an advantage to you (such as money, a gift, or a legacy) then that is bribery. Bribery also occurs if you request, agree to receive or actually receive such an advantage. Such exchanges are illegal.
- If you pay, or attempt to pay cash or any other favour or consideration in order to secure business (e.g. a new source of service user), retain business or gain a business advantage for us then that is a bribe. It is a specific offence to bribe or attempt to bribe a public official (such as a CQC Inspector) wherever they may be in the world.
- Reasonable and proportionate gifts as a token of appreciation or as part of facilitating normal business relationships are acceptable; the key is that they must not be an inducement to act improperly.
- If you influence or make decisions on our behalf you are more vulnerable and if you are uncomfortable with any form of influence you should report it to the Registered Manager.

- Offering or accepting gift or hospitality needs caution; in particular such offers and acceptance need to be for items of proportionate value.
- Typical examples of gifts of proportionate value as appreciation would be a bottle of wine, flowers, or chocolates either at Christmas or as a genuine "thank you". These are normally acceptable. However if they are a form of facilitation payment (see below), then they are not.
- All gifts should be declared to the Registered Manager and a record will be kept.
- Hospitality is defined as where we are present at the hospitality itself (say a buffet lunch at an exhibition). Where this level of hospitality falls within our normal business relationship this is acceptable. Hospitality which is disproportionate may be seen to be influencing impartiality and is prohibited under this policy. Please refer any such offers or concerns to the Registered Manager.
- Where the company providing the hospitality is not present, for example if a restaurant voucher is given, then this is a gift, not hospitality. If you are in doubt about the proportionality of any gift you should discuss the matter with the Registered Manager.
- If you are involved in any form of tendering process then gifts and hospitality (even if proportionate) must not be accepted and, if offered, should be politely declined and referred to the Registered Manager.
- Facilitation payments may be experienced where a relative or other agent offers a payment to obtain a level of service for a user that would not normally be provided. Our absolute policy is that such payments are not to be accepted and should be reported to the Registered Manager.
- Requesting a facilitation payment may be regarded as Gross Misconduct and render you liable to summary dismissal.
- If there is the slightest suggestion that you might receive a legacy from a service user then this should be referred to the Registered Manager immediately. Legal advice will be sought.
- You may not solicit gifts or sponsorship for charitable events from residents.
- No donation to a Charity or Political Party/Candidate may made or offered in the name of the organisation without the prior approval of the Registered Manager.
- You should avoid any situation where you might benefit personally from decisions you take or influence on behalf of us. Where a conflict of interest could arise the relevant decision must be referred to the Registered Manager.

Raising Concerns

- You should raise any concerns that you have about wrong-doing under the Whistle Blowing policy, following the procedure there.
- Ultimate responsibility for compliance rests with the Company Secretary.

Guidance to the Service Provider

- An organisation can also be found liable where someone who performs services for it, such as an employee or an agent, pays a bribe in order to secure business, retain business or gain a business advantage for the organisation in question. A defence is in place should this happen, but the organisation needs to be able to prove that it has adequate procedures in place in order to prevent bribery.
- What counts as "adequate" within the defence will depend on the bribery risks you face, e.g. the nature of your business, the size of your organisation, complexity of your business etc. If your business is therefore small, then the procedures you will need to have in place will need to be minimal in order to mitigate those risks, but if you are a large organisation then the risks will be exponentially greater and you should therefore have detailed, extensive procedures in place.
- Larger organisations may be wise to assess their risks carefully and to obtain from employees, particularly those who may be in a position to accept or offer bribes, a signed declaration that they have read, understood and accept the above procedure.
- In any event, you should bring the above policy to the attention of employees. For very small organisations it may be sufficient to do this orally.

Principles

The following six principles will help you to decide what needs to be done for your particular organisation:

- **Proportionality**
    - The action which you decide to take must be proportionate to the risk that you might face and to the size of your business. Consequently you may need to do more if your business is large. Most independent care sector organisations will be operating in a market place where the risk of bribery is small, therefore your actions may not need to be extensive.
- **Top Level Commitment**

- Senior management are in the best position to ensure that the organisation conducts its business without bribery, and should enforce this with a top-down approach. The senior management (especially the Registered Manager and the Registered Provider) will therefore want to actively demonstrate that they have made all staff aware of their intolerance of bribery.

- **Risk Assessment**
  - Think about the people you deal with, especially new business arrangements and public funding bodies, and the inherent risks involved.
- **Due Diligence**
  - Knowing exactly who you are dealing with and employing can help to protect the organisation from less trustworthy individuals. It is therefore especially important that the results of DBS checks are thoroughly checked as part of the recruitment procedure, so that risks are taken into account when making employment offers.
- **Communication**
  - Communication to care staff and representatives of your policies and procedures will assist in enhancing the awareness of how you like to conduct business. In most cases it will be sufficient to draw members of staff's attention to your policy as already discussed.
- **Monitoring and Review**
  - The risks the organisation faces and the effectiveness of its procedures may change over time. You may therefore want to review the latest anti-bribery steps you have taken, so that they keep pace with any changes in the risks that you may face.

Compliance

- It is assumed, within this policy, that employees will seek guidance from the Registered Manager in areas of uncertainty. This is not to suggest that that person will necessarily have the correct answer but responsibility should be placed on him or her or on another specified responsible person to take responsibility for finding the answer or providing appropriate guidance. There is a significant amount of guidance, about proportionality for example, in Government publications on the Internet and in other reliable web sources.
- It is important that one individual takes responsibility for day-to-day compliance. In most cases this would naturally fall to the Registered Manager but ultimate responsibility will fall on the Company Secretary, Owner or an equivalent position.

Hospitality

- Genuine hospitality or other similar business expenditure that is **reasonable** and **proportionate**, is not prohibited under the Act. All hospitality must however, be *bona fide*.
- Should hospitality be considered a cover for bribery, then the authorities will look at the value of the hospitality, the level provided, the way in which it was provided and the influence of the person receiving it on a business decision.
- The guidance from the Ministry of Justice does state that a business can continue to provide genuine hospitality, such as taking clients to sporting events, dinners, offering gifts to clients as a reflection of your good relations or paying reasonable travel expenses. For example, it may be acceptable to accompany a potential client to a sports event and to then provide dinner. However to provide them with tickets to go on their own, or with others unconnected to your business, and to then also pay for their dinner and travel expenses could be seen as a gift and, if disproportionate, as a bribe.
- No gifts or hospitality should be given or accepted during a tender process or during contractual negotiations if there is any realistic risk that such gifts or entertainment could influence the outcome of the tender process or contractual negotiations.

Facilitation Payments

- Facilitation payments, which are payments to induce officials to perform, or speed up the performance of routine functions that they are otherwise obligated to perform, are bribes. An example here would be attempting to bribe a CQC inspector. There is no exemption for such payments under the law.

Political Contributions

- It is unwise to make donations, whether in cash or kind, in support of a political party or candidate, as this can be perceived as an attempt to gain an improper business advantage.

Charitable Contributions

- Charitable support and donations are acceptable under law whether of in-kind services, knowledge, time, or direct financial contributions. However, service providers must be careful to ensure that charitable contributions are not used as a cover to conceal bribery. Only make charitable donations that are legal and ethical.

Reviewed Marcus Kerridge-McColl
September 2025

Anti-Bullying Policy and Procedure
Crescent Community Care Services Limited

Scope

- All workers and Service Users.

Policy

- We are committed to providing a caring, friendly and safe environment for all of our workers and Service Users so they can work and live in a relaxed and secure atmosphere. Bullying of any kind is unacceptable, and if bullying does occur, all incidents will be dealt with promptly and effectively.

- The attention of all employees is drawn to the:

  - The Social Care Workers Codes of Practice, which can be found for download on the Social Care Institute of Excellence (SCIE) website at http://www.scie.org.uk/workforce/files/CodesofPracticeforSocialCareWorkers.pdf?res=truea copy of which is given to all employees;
  - Discipline Policy and Procedure, a copy of which is shown to all employees on recruitment, is referred to in the Staff Handbook that is issued to all employees, and is available in the Policy and Procedure Manual in the staff room/administration office. This policy sets out the disciplinary action which may be taken in the event of bullying activity;
  - Whistleblowing Policy and Procedure, a copy of which is shown to all employees on recruitment, is referred to in the Staff Handbook that is issued to all employees, and is available in the Policy and Procedure Manual in the staff room/administration office;
  - Sexual and Racial Harassment Policy and Procedure, a copy of which is shown to all employees on recruitment, is referred to in the Staff Handbook that is issued to all employees, and is available in the Policy and Procedure Manual in the staff room/administration office.

- What is bullying?

  - Bullying is the use of aggression with the intention of hurting another person physically or emotionally.

  - Bullying results in pain and distress to the victim.

  - Bullying can be:

    - Emotional; being unfriendly, excluding and ignoring, tormenting (e.g. hiding or interfering with belongings, threatening gestures, threatening language);
    - Physical; pushing, kicking, hitting, punching or any use of violence;
    - Racist; racial taunts, graffiti, gestures;
    - Sexual; unwanted physical contact or sexually explicit comments;
    - Homophobic; because of, or focusing on the issue of sexuality;
    - Verbal; name-calling, criticising in public, sarcasm, spreading rumours, teasing;
    - Cyber; All areas of the internet, such as email and internet chat room misuse;
    - Mobile; threats by text messaging and calls, misuse of associated technology i.e. camera and video facilities.

- Signs and symptoms.

  - A person may indicate, by signs or behaviour, that he or she is being bullied. Supervisors should be aware of these possible signs and that they should investigate if a person:

    - Is reluctant to work without a specific person accompanying them; Œ
    - Asks that a particular member of staff does not care for them;
    - Changes their usual routine;
    - Is unwilling to come to work;
    - Begins to take unannounced days off;

- Becomes withdrawn, anxious, or lacking in confidence;
- Feels ill in the morning;
- Begins to work to a lower standard;
- Has possessions which are damaged or 'go missing';
- Has unexplained cuts or bruises;
- Becomes aggressive, disruptive or unreasonable;
- Is bullying other people;
- Is reluctant to discuss their behaviour;
- Gives improbable excuses for any of the above;
- Is nervous and jumpy when a cyber message is received.

- These signs and behaviours could indicate other problems, but bullying should be considered a possibility and should be investigated.

Procedure

- Report bullying incidents to your supervisor.
- Record cases of serious bullying.
- If necessary and appropriate, police will be consulted.
- The bullying behaviour or threats of bullying must be investigated and the bullying stopped quickly.
- An attempt will be made to help the bully/bullies change their behaviour.
- Use the supervision process to formally record bullying behaviour, and agree a plan of action to stop it.
- Clearly spell out the potential disciplinary action which could be taken in the event of appropriate corrective action not being followed by a person who is exhibiting bullying behaviour, or if serious incidents occur.
- Refer to the Discipline Policy and Procedure in serious and/or intransigent cases.
- Use the abuse procedures in the case of bullying of a Service User.
- Consider the use of advocacy in the case of a Service User bullying a worker, and refer the Service User and/or their advocate to the behavioural standards set out in the Service User's Handbook and Agreement.

Reviewed by Marcus Kerridge McColl
December 2025

Anxiety Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To provide individualised, safe and supportive care for people with a diagnosis of anxiety or people who may live with anxiety.

Scope

- All workers working with Service Users who have been given a diagnosis of anxiety disorder or who may be living with an anxiety disorder.

Definition

- Anxiety is a term in very common usage. It is usually described as a feeling of fear that is not rational. The symptoms may be physical, such as sweating, becoming dry mouthed, butterflies in the stomach, and others. There may be symptoms in the persons thinking, poor concentration, or feeling that something awful will happen.

- These symptoms may, for some, just last a short time, but for others much longer. The psychiatric classification system, Diagnostic and Statistical Manual of Mental Disorder volume four (DSM-IV) is widely used in mental health services in the United Kingdom, and defines anxiety by identifying seven types:

    1. **Panic disorder with or without agoraphobia -** The main characteristic of panic disorder is the occurrence of a panic attack linked with the fear of another attack.

    2. **Phobias -** Specific phobias and social phobia are both included under this heading. A phobia is described as an intense and irrational fear of a specific object or situation that is so intense it can cause the individual to be compelled to go to great lengths to avoid it. Social phobia can include a fear of being judged, scrutinised or humiliated in some way.

    3. **Obsessive Compulsive Disorder** is characterised by unwanted, intrusive, persistent or repetitive behaviours. These will reflect a person's attempts to control them and the anxiety caused by them.

    4. **Stress disorders -** This includes post-traumatic stress disorder (PTSD) and acute stress disorders. These are categorised as being a symptomatic response to a traumatic experience that the individual has had.

    5. **Generalised Anxiety Disorder (GAD)** is the most commonly diagnosed anxiety disorder and usually affects young adults.

    6. **Anxiety disorders due to known physical cause -** This includes medical conditions and symptoms caused by drug misuse.

    7. **Anxiety disorder not otherwise specified -** This is not a separate type of disorder but included in order to cover symptoms that do not meet the criteria for any of the other anxiety disorders.

- Diagnosis of anxiety is undertaken by medical practitioners. It is regarded as a common health problem, in that the symptoms of anxiety disorders may be a common experience amongst the general population. Care and support staff may be working with someone who they feel has symptoms of an anxiety disorder. Consideration should be given to referring that person to their General Practitioner. Signs and symptoms that care and support staff should be alert to include the following:

<u>Social</u>

- Taking extreme actions to avoid situations (like never going outside).
- Being fearful of meeting people.

<u>Physical Impact</u>

- Not sleeping properly.
- Physical feelings of anxiety such as dry mouth or stomach cramps.

Psychological

- Panic attacks, unpredictable and sudden attacks of anxiety.
- Poor concentration.
- Irritability.
- Feeling worried a lot of the time.

Description

- As with many psychiatric diagnoses, the term anxiety can be a controversial one. Anxiety can be a perfectly usual response to a number of situations, particularly in a modern busy stress-laden society. However, where the symptoms of anxiety become so disabling for someone that it affects their daily living then this suggests the person is need of support to manage this.

Policy

- There are a number of good practice principles that care and support staff should be aware of whilst providing care and support for people with anxiety disorder:
    - Being aware that a mental health diagnosis can be stigmatising, and that people may not want to admit to this;
    - Being able to identify other sources of support, including self-help groups, support groups and other local and national resources;
    - Awareness of the principles of the Mental Capacity Act 2005 (MCA). QCS policies on the MCA will provide further information when working with people who lack mental capacity to make decisions about their care*;*
    - Being aware that there are different types of anxiety, and recovery is possible.

Procedure

- The National Institute for Health and Clinical Excellence (NICE) has produced guidelines for treating and managing certain types of anxiety. It covers psychological treatments, medication and the services that should be made available in hospital and in the community. These guidelines are reviewed regularly and are downloadable from the NICE website at www.nice.org.uk. Specifically, there are NICE guidelines published on the following types of anxiety disorder:
    - Generalised Anxiety Disorder (CG113);
    - Social Anxiety (CG159);
    - Obsessive Compulsive Disorder (CG31);
    - Post-Traumatic Stress Disorder (CG26).
- For the majority of people who are diagnosed with an anxiety disorder, interventions identified in the NICE guidelines will be delivered by their GP and primary care team. For people with the most severe and disabling disorders, those interventions are likely to be carried out by specialist mental health services. However there is still a crucial role for staff working in care home and community settings in supporting these interventions. Some of these ways are described in NICE guidelines, and the ways that care staff can support people with anxiety disorders could include the following:
    - Build a relationship and work in an open, engaging and non-judgemental manner;
    - Explore the person's worries in order to jointly understand the impact of their anxiety;
    - Explore treatment options collaboratively with the person;
    - Ensure that discussion takes place in settings in which confidentiality, privacy and dignity are respected;

- Provide information appropriate to the person's level of understanding about the nature of their disorder, or sources of further information including self-help groups and support groups, in particular where they can talk to others with similar experiences;

- When families and carers are involved in supporting a person with anxiety, provide information on opportunities to seek a carer's assessment of their caring, physical and mental health needs and information about carer support groups.

- Specialist interventions that might be offered to someone with anxiety disorder will include:

Self-help

- Self-help groups have provided a lot of support for people with anxiety, and may offer support using principles of Cognitive Behavioural Therapy. Organisations such as Anxiety UK can offer information on local groups. GP surgeries are another source for further information on local support groups.

Talking treatments

- These might include counselling and Cognitive Behavioural Therapy (CBT) to understand why you think the way you do. These treatments would be arranged via someone's General Practitioner, and in England may be provided by the NHS Improving Access to Psychological Therapies programme.

Medication

- There are different types of anti-anxiety medication. These can include tranquilisers and anti-depressant medication. Some of these may be addictive, and some may have side effects and may take some time before they help with symptoms. The person's GP would advise on the prescription of these medications.

Crisis intervention

- Where there is need for urgent intervention, particularly where someone is expressing suicidal intentions, then in such situations the involvement of Crisis Resolution and Home Treatment (CRHT) teams may be required and access to crisis intervention can be made through referral by the person's GP.

Frameworks for delivering care and support

- All treatments and interventions should be offered as part of a care planning process based on an individual's needs. Tools in the QCS Care Management System allow for compilation of assessment of mental health needs and a care plan format that should be reviewed on a regular basis. The majority of people receiving treatment for anxiety will receive this from their General Practitioner. For those whose mental health needs are complex and severe, who receive care and treatment from mental health services, this should be delivered as part of the Care Programme Approach framework for delivering mental health care. The support and interventions by care staff should be included in the overall Care Programme Approach care plan, and these staff may be invited to contribute to CPA care planning meetings. The person who is the subject of the CPA care plan should have their own copy. Where care staff are aware that someone is in receipt of professional mental health services, they should be aware of how the care plan can be accessed, and who the person's care co-ordinator is. If not they should contact the appropriate mental health team. The contents of a CPA care plan should include:

    - Details of who the care co-ordinator is (mental health professional);

    - Identify the interventions and anticipated outcomes;

    - The date of the next planned review;

    - Crisis and contingency arrangements;

    - Arrangements for care and support;

    - Arrangements for the management of risk to the Service User and to others.

- Crisis and contingency planning would be conducted by the person's care co-ordinator, but care staff should be aware of these plans. A crisis plan should identify responses to crisis situations that might arise for that person, such as a worsening of their anxiety disorder that cannot be managed with usual coping strategies. A contingency plan will outline a planned course of action that should follow some breakdown or failure in the person's care plan. Contact names and numbers of key professionals should form part of these plans, but a list of names and numbers on their own would not be regarded as sufficient. For crisis and contingency plans to be effective and workable, the Service User should be involved in drawing up their own plan when they are experiencing a period of wellness.

Risk Management

- If you believe someone you are working with is at risk of harm you should share that information with others you work with, and where appropriate, the person's GP. Assessment and management of people at risk is a specialist area of work that should be undertaken by health professionals, but care staff and the person themselves should be able to contribute to that. Risk management should not be seen as elimination of risk. There are risks for everyone in all walks of life, and to try and eliminate them would result in a loss of independence and choice. So risk management should be seen in the context of positive risk taking. Current policy thinking in relating to people with mental health problems can be found in the Department of Health's Best Practice in Managing Risk (2007). The Best Practice document also describes the importance of a collaborative approach to managing risk, involving the person and the whole of the care team, so that trusting relationships are developed that aid communication.

Legal Frameworks

- The framework for the compulsory care and treatment of people with mental disorder is governed by the Mental Health Act 1983 (which was amended in 2007). The Mental Health Act Code of Practice gives guidance on the definition of mental disorder for the purposes of the Act. The Code includes the following as examples of conditions which could fall under the definition of a mental disorder:

    - *Neurotic, stress-related and somatoform disorders, such as anxiety, phobic disorders, obsessive compulsive disorders, post-traumatic stress disorder and hypochondriacal disorders.*

- Where a person lacks capacity as a result of an impairment of, or a disturbance in the functioning of, the mind or brain, to make decisions, the principles involved in making decisions for someone in their best interests are governed by the Mental Capacity Act 2005. The Mental Capacity Act 2005 also includes provision for the appointment of lasting powers of attorney, regarding financial and welfare decisions if the person should lose the mental capacity to make these decisions.

- Policy and procedures surrounding the Mental Health Act 1983 and the Mental Capacity Act 2005 are contained within the QCS manuals.

Reviewed by Marcus Kerridge McColl
November 2025

Appearance Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To maintain a professional impression

Scope

- All staff representing our company

Policy

- All staff members are required to be clean and tidy in appearance.

- Staff members own personal care must be to a high level as you are working closely with both Service Users and other Staff members.

Procedure

- During the Policies and Procedures training the Manager will outline the standards of personal care and standards of dress which apply to staff as follows
    - Care Workers are to wear the appropriate Uniform at all times
    - Should have good personal care themselves
    - Clothing must be clean, odour free and in good condition
    - Hair must be kept clean and tidy
    - Staff must keep their hair tied back so not to cause a Health & Safety issue.
- Jewellery / Badges etc – Crescent Community Care Services Limited reserves the right to require that employees do not wear jewellery including wrist watches, badges or clothing that contains text or images which it believes may cause offence to the Service User or other members of staff
- Should any employee have difficulties in maintaining these standards they should discuss the matter with their immediate superior
- Employees who do not comply with these standards will be subject to the organisations normal disciplinary procedure. In serious cases, where an employees appearance is, in the organisations view, unacceptable, the employee will be required to return home to change.
- Employees have the right to lodge an appeal against a decision on the acceptability of their appearance, using the organisations normal grievance procedure.
- The organisation reserves the right to change the uniform as necessary in order that it conforms with the Corporate Image.
- Failure to wear the organisations uniform, or to keep it in the proper condition, will be treated as an issue of misconduct and dealt with under the normal Disciplinary Procedure.
- PPE (Personal Protective Clothing & Equipment)is essential and to be used with the uniform
    - For care staff this includes
        - Aprons Where appropriate
        - Gloves at all times
        - Flat & closed in shoes at all times
- The PPE here is specified as a minimum. Where risks can be reduced by reasonable and additional protection, further PPE may be required.
- Where PPE is provided, this must be worn at all times. Any failure to do so places an individuals safety at risk and may result in disciplinary action. All members of staff that are issued PPE should keep it maintained and only use it for the companies purposes.
- All uniforms must be returned on leaving and where uniforms are not returned in reasonable condition a charge will be made to cover the cost of replacement.

Reviewed as Marcus Kerridge-McColl
December 2025
Blood Spillage Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To prevent cross contamination from Exposure to blood

Scope

- All Spillages of blood

Policy

- Blood spillage from any source will be cleaned in a manner which protects the person cleaning the spillage from the possibility of cross contamination

Procedure

- Materials for dealing with all blood spillage

- 1 disposable plastic apron
- 1 pair of disposable gloves
- 1 yellow bag
- Paper towels
- Bowl
- A sodium Hydrochlorite 1% solution (i.e Milton Fluid or the same)

- All blood spillages / splashes must be dealt with immediately
- Careful dilutions of bleach water or Milton Fluid
- Pour the cleaning solution into the bowl for cleaning
- Wear disposable gloves and apron, wipe over the area with paper towels until all of the blood has been removed and then place towels into the yellow disposal bag
- Wash the are with paper towels and the cleaning solution and then place into the yellow disposal bags
- Wipe over the area to ensure that the area is clean
- Wash away the solution and ensure the sink is cleaned out fully with cleaning fluid
- Secure the yellow disposal bag and inform for collection correctly

NOTE

For the most, Care workers should not clean up blood spillages unless in a real emergency where the above procedure is carried out.

Reviewed Marcus Kerridge-McColl
October 2025

Care and Daily Living Policy and Procedure
Crescent Community Care Services Limited

Purpose

- The daily living experience of Service Users will be enhanced by the skilled and competent delivery of Care.

Scope

- All Care activities.

Policy

- Care training to best practice standards will be carried out.

Procedure

- Assessments of the Service User's health and daily living needs will be carried out by the person nominated by the Manager for that purpose.

- An evaluation and review of the Service User's personal health status will be carried out by the person nominated by the Manager for that purpose.

- An evaluation of the Service User's personal evaluation of their own wellbeing will be carried out by the person nominated by the Manager for that purpose.

- Service User satisfaction surveys which incorporate Service Users' opinions of their Care will be carried out.

- Audit and review of the Service User Care Plans and achievement of Service Users' outcomes detailed in the Daily Record documentation will be carried out, together with personal interviews with Service Users.

- Staff tasked with any of the above will be appropriately skilled and qualified.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To support the career development of adult social care workers who are not members of regulated professions.

Scope

- All adult social care workers who are not members of regulated professions.

Policy

- The Care Certificate replaces previous standards for induction training and development for adult social care workers.
- As an induction standard, the implication is that all workers should be benchmarked against the Care Certificate standards, and where skills and knowledge are deficient, topped up by training and development. In this manner, the whole workforce will be working to common standards of competence in the areas covered by the Care Certificate, with substantial benefits to the quality of the services offered to Service Users.
- Because of the potential benefits to staff competence and consistency of working, which in turn will significantly improve quality of services to Service Users, QCS recommends that:
  - All newly recruited adult social care workers, who are not members of regulated professions, complete the Care Certificate.
  - All existing workers' competencies and skills are assessed against the Care Certificate standards, and where any deficiencies are identified, those deficiencies are added to the personal development plan for that individual worker. Note that it may be organisationally more convenient for the Care Certificate development to be offered to all workers. Some workers may prefer to be seen to be up to date with their certified training for either personal or professional reasons.
- Crescent Community Care Services Ltd will support all employees to complete a Care Certificate, providing access to either internal or external resources in order to do so, as best suits the organisation and the individual.
- Note that the Skills for Care documentation uses the word "worker" throughout, which indicates that, in line with the QCS training approach, all employees are subject to the induction standards, and not only those who are direct carers. This approach recognises the team, approach to quality care, the need for the motivation of all workers including indirect workers, and the fact that indirect workers do have an impact on perceived care quality.

Background

- The Care Certificate is a set of knowledge and skill standards which is designed to apply to workers in adult social care who are not a member of regulated profession. This group includes:
  - Activities worker;
  - Day Care Assistant, Day Care Officer;
  - Domiciliary care worker, Home care worker;
  - Nursing Assistant (in a nursing home or a hospice);
  - Personal Assistants, Reablement Assistant;
  - Residential Care Worker, Senior Home Care Worker;
  - Support Worker.
  - Other roles may be included where achievement of all of the standards is possible.
- Quoted from Skills for Care: (http://www.skillsforcare.org.uk/Standards/Care-Certificate/Care-Certificate.aspx)

- Following the Francis Inquiry the Secretary of State has issued recommendations on the recruitment, learning and development, management and support of healthcare assistants and social care support workers.

- The Care Certificate is the start of the career journey for these staff groups and is only one element of the training and education that will make them ready to practice within their specific sector. Although the Care Certificate is designed for new staff, new to care and offers this group of staff their first step on their career ladder, it is also offers opportunities for existing staff to refresh or improve their knowledge.

- The Care Certificate sets out explicitly the learning outcomes, competences and standards of care that will be expected in both sectors, ensuring that the HCSW/ASCW is caring, compassionate and provides quality care.

THE STANDARDS

1. **Understand Your Role**

   - The QCS recruitment process, job descriptions and the recommended availability of all policies and procedures to all employees supports this element of the standards.

2. **Your Personal Development**

   - Use of the QCS Training Policy and Procedure approach, with the development of personal development plans supports this element of the standards.

3. **Duty of Care**

   - The recommended availability of all QCS policies and procedures to all employees supports this element of the standards.

4. **Equality and Diversity**

   - The recommended availability of all QCS policies and procedures to all employees supports this element of the standards.

5. **Work in a Person Centred Way**

   - The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure, and the Service User Handbook, to all employees supports this element of the standards.

6. **Communication**

   - The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure, and the Service User Handbook, to all employees supports this element of the standards.

7. **Privacy and Dignity**

   - The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure and the Service User Handbook, to all employees supports this element of the standards.

8. **Fluids and Nutrition**

   - The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure and the Nutrition Policy and Procedure, to all employees supports this element of the standards.

9. **Awareness of Mental Health, Dementia and Learning Disability**

   - The recommended availability of all QCS policies and procedures, particularly the Care Planning Policy and Procedure, and the Service User Handbook, to all employees supports this element of the standards.

10. **Safeguarding Adults**

    - The recommended availability of all QCS policies and procedures, particularly the Safeguarding Policy and Procedure, the Care Planning Policy and Procedure and the Service User Handbook, to all employees supports this element of the standards.

11. **Safeguarding Children**

    - The QCS Safeguarding Policy and Procedure supports this element of the standards.

12. **Basic Life Support**

    - To meet this element of the standards, it is necessary for employees to successfully complete practical Basic Life Support Training that meets the UK Resuscitation Council guidelines. The guidelines can be found at https://www.resus.org.uk/pages/guide.htm. While training in this area is an important element of the skills and knowledge of an adult social care worker, and may come to be expected by Service Users and their families, it is a specialised area and staff development should be through recognised and certified trainers, with formal certification of competence at the end of the training, and regular re-certification.

13. **Health and Safety**

    - The recommended availability of all QCS policies and procedures, particularly the Health and Safety Policy and Procedure, to all employees supports this element of the standards.

14. **Handling Information**

    - The recommended availability of all QCS policies and procedures, particularly the Communication Policy and Procedure, the Confidentiality Policy and Procedure and the Care Planning Policy and Procedure, to all employees supports this element of the standards.

15. **Infection Prevention and Control**

    - The recommended availability of all QCS policies and procedures, particularly the Infection Control Policy and Procedure, to all employees supports this element of the standards.

Procedure

- New recruits

    - Make it clear that new recruits will be expected, as a condition of their employment, to complete the Care Certificate.

    - In cooperation with the new recruit, assess the method of development which best suits them and the organisation, for instance, self-study, or external courses.

    - Support the new recruit to complete the Care Certificate.

- Existing employees

    - Encourage all existing employees to work to obtain a Care Certificate.

    - In cooperation with the employee, assess the method of development which best suits them and the organisation, for instance self study, or external courses.

    - Support the employee to complete the Care Certificate.

- Enabling Care Certificates for all employees:

    - Decide as an organisation how you can best support development of the workforce towards Care Certificates. While the Care Certificate is ideally suited to the in-house development method, which is proven to be the most effective model for most employees, the support for that in-house development must be designed, planned and implemented for results to be tangible. Implementation methods may include the training of in-house trainers and assessors (usually regarded as the most effective method), or combining in-house assessors with external teaching aids such as eLearning sources, or the use of external
    assessors and external teaching aids.

    - Skills for Care provide a range of materials, including workbooks, which effectively support an in-house development model. These, and other resources, are available at: http://www.skillsforcare.org.uk/Standards/Care-Certificate/Care-Certificate-Materials.aspx

- The CQC's approach to Care Certificates is set out at: http://www.cqc.org.uk/sites/default/files/20250318_one_page_cqc_position_care_certificate.pdf

Reviewed by Marcus Kerridge McColl
December 2025

Care Communication and Information Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To support the values of The Agency.

Scope

- All Care procedures.

Policy

- Continuity of Care delivery will be based on the unique, holistic needs of each Service User, enabled by the effective recording of information.
- Service Users will have access to their own Service User Care Plan.

Procedure

- All information related to the Service Users' health and social wellbeing is documented in the Service User Care Plan.
- Service User Care Plans are current and appropriate and easily understood.
- All relevant information is readily available and accessible.
- All staff will have read the Service User Care Plans before writing the record of daily Care.

Residential Setting

- For all staff working in a residential setting, any changes to Service Users are monitored and documented in the communication book and the Service User Care Plan.
- All staff must read the communication book before commencing duties, reading all entries made since the last time they were on duty.
- The attached Care Communication and Action Form will be used to communicate with Care administration, the Care supervisor or the Registered Manager.

Community Setting

- For all staff working in a community setting, any changes to Service Users are monitored and communicated to the administration office. This action should be noted by the staff member in the record of daily care, and in the Service Users Care Plan.
- All staff must read the record of daily Care before commencing duties, reading all entries made since the last time they were on duty.

- On receipt of the changes, the Administration staff must complete the attached Care Communication and Action Form.

Reviewed by Marcus Kerridge McColl
December 2025

Care Management Protocol
Crescent Community Care Services Limited

Purpose

- To define accountability, responsibility and lines of communication and authority.

Scope

- All workers.

Policy

- The day-to-day management of Care within The Agency, including compliance with the organisation's policy, legislation and best practice, is the responsibility of the Registered Manager. The Registered Manager is responsible to the Registered Provider for the standard of the Care in The Agency.

- The Registered Manager will carry out day-to-day quality control of care, ensuring that all organisational policies, statutory requirements and best practices are complied with, and that changes to promote continuous improvement are supported.

- The Registered Manager will carry out regular Care Audits and report the results to the Management Meeting, together with:

  - Inspection reports;
  - Accident reports;
  - Customer feedback and complaints.

- No changes to the Care system, service, equipment, staffing, suppliers or supplies specification may be made without consultation with, and the agreement of, the Registered Provider.

- The Registered Manager will have 2 years experience in a senior management capacity in a relevant Care setting within the past 5 years, and a qualification to at least QCF Diploma Level 4 or equivalent, or, in the case of a nursing home, be a Registered Nurse and have a relevant management qualification.

- The Registered Manager will only be responsible for one registered establishment.

Reviewed by Marcus Kerridge McColl
December 2025

Purpose

- To guide practice in the use and management of indwelling catheters.

Scope

- Proposed use, and use of indwelling catheters.

Policy

- Note - This policy is provided as a generic policy and procedure for use when no other more applicable guidelines are available or mandated. Local institutional or regional guidelines, professional guidelines, and statutory body guidelines should, if they exist, be used in preference to this policy.
- Catheter insertion and management will only be performed by staff with the appropriate level of training, skills and authorisation.
- Indwelling catheters will be actively managed in order to maintain the comfort, privacy and dignity, and infection- free status of the Service User, while maintaining staff protection and overall infection control.
- Any Service User requiring, or who may require, an indwelling catheter will be risk assessed and an individual plan of care prepared which is specific to their personal care and medical needs, physical and medical condition, physical environment, and personal wishes.
- Changes of catheter management status, such as initial use, catheter change, issues met and changed management regimes will be communicated to all staff on duty at the time of the change, and the attention of staff drawn to the need to read the Care Plan during the next staff hand-over.
- Before carrying out any operations on catheter systems, perform hand hygiene and don clean gloves, and a plastic apron if local procedure dictates.

Procedure Catheter Insertion

- Refer to current clinical guidelines, such as the Royal Marsden Manual of Clinical Nursing Procedure.

Catheter Maintenance

- Monitor the catheter system at the intervals specified in the Care Plan, checking that tubing and drainage bag are secure and in the proper position to maintain efficient flow while preserving privacy and dignity, and that no blockages are evident.

- Assess urine for clarity, colour, odour or the presence of blood by visually inspecting the urine in the tube, bag or at the sample port or when emptying the drain collection system.

- Maintain a closed drainage system by utilising pre-connected, sealed catheter-tubing junctions.

- Maintain unobstructed urine flow and keep the catheter and collection tube free from kinking.

- Keep the collection bag below the level of the bladder or hips at all times (Note: Exercise particular care to keep bag off the floor).

- Empty the collection bag regularly using a separate, clean collection container for each Service User.

- Replace the collection bag according to the detailed instructions of the system manufacturer and according to training, and of authorised to do so by the Registered Manager. If in doubt ask the Registered Manager to allocate the task to a person with the required skills and authority.

- Provide daily peri-care.

- Perform hand hygiene before and after any contact with the catheter system.

Emptying the Catheter

- Empty the collection system at regular intervals as specified in the plan of care.

- Perform hand hygiene and don clean gloves and a plastic apron.
- Provide a clinical waste disposal container adjacent to the working area.
- Disengage the outlet tube from housing, direct the outlet tube into a collection container, and release the clamp to empty urine from the collection system. Avoid splashing.
- Re-engage the clamp and reinsert the outlet tube into the housing.
- Place gloves and apron in the disposable container and dispose of according to local clinical waste disposal procedures.
- Perform hand hygiene.

Urine Sampling

- This procedure may only be performed by staff with the relevant training and skills, and authorisation in writing by the Registered Manager.
- Request that the procedure be carried out by a suitably skilled and authorised person if in doubt.
- Check the Service User's Care Plan for any indication of issues, including the distress of the Service User, on previous sampling occasions.
- If delivering the sample to the GP surgery for onward transmission to the laboratory, check the latest available deposit time at the surgery and the availability of transport to the surgery. Arrange the transport and plan the sample taking to meet any deadlines.
- Inform the Service User of the proposed sampling, and the reason(s), and reassure them if necessary.
- Perform hand hygiene and don clean gloves and a plastic apron.
- Provide a clinical waste disposal container adjacent to the working area.
- Occlude or kink the drainage tubing a minimum of three inches below the sample port until urine is visible under the access site.
- When urine is visible under the sample port, swab the surface of the sampling port with an antiseptic wipe.
- Using aseptic technique, use a disposable syringe (body only) to slowly draw off a sample through the access port.
- Transfer the specimen into a specimen container, label and place in a transport pouch.
- Replace any clamps moved or removed and unkink all tubes. Check the safe and comfortable operation of the catheter system before any other procedures. Check that the catheter bag and tubes are positioned in such a way as to preserve the privacy and dignity of the Service User.
- Send specimen to the laboratory immediately or refrigerate until transported.
- Place used syringe, gloves and apron in the disposable container and dispose of according to local waste disposal procedures.
- Perform hand hygiene.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To comply with the Data Protection Act 1998 Š
- Taking into account:
  - The CCTV Code Of Practice produced by the Information Commissioner;
  - The Human Rights Act 1998;
    The Regulation Of Investigatory Powers Act 2000;
    Caldicott Report 1997.

Scope

This policy will cover all employees of Crescent Community Care Services Ltd, persons providing a service (voluntary or paid) to the organisation, Service Users, visitors and all other persons whose image(s) may be captured by the system at Crescent Community Care Services Ltd's premises.

Guidance to the Service Provider

This policy is for the guidance of the Service Provider.

Policy

- Crescent Community Care Services Ltd must, before deploying any CCTV system, modifying an existing system, or on first introduction of this Policy, conduct a written impact assessment which must include:
  - An examination of the effects which the deployment has on the privacy of employees of Crescent Community Care Services Ltd; persons providing a service (voluntary or paid) to the organisation; Service Users; visitors and all other persons whose image(s) may be captured by the system.
  - Who will be using the CCTV images? Who will take legal responsibility under the Data Protection Act?
  - What is the organisation's purpose for using CCTV? What are the problems it is meant to address? These could include:
    - Prevention or detection of crime or disorder;
    - Apprehension and prosecution of offenders (including use of images as evidence in criminal proceedings);
    - Interest of public and employee Health and Safety;
    - Protection of public health;
    - Protection of property and assets;
    - To provide evidence in support of misconduct.
  - What are the benefits to be gained from its use?
  - Are the images to be used concurrently i.e. the monitor manned permanently in order to identify and deal with incidents such as unauthorised or dangerous ingress or egress of people, or are images to be made and stored only for review if past incidents are to be reviewed?
  - Can CCTV technology realistically deliver these benefits? Can less privacy-intrusive solutions, such as improved lighting, achieve the same objectives?
  - Do you need images of identifiable individuals, or could the scheme use other images not capable of identifying the individual?
  - Will the particular equipment/system of work being considered deliver the desired benefits now and remain suitable in the future?
  - What future demands may arise for wider use of images and how will you address these?
  - What are the views of those who will be under surveillance?

- What could you do to minimise intrusion for those that may be monitored, particularly if specific concerns have been expressed?

- Where the system will be operated by or on behalf of a public authority, the authority will also need to consider wider human rights issues and in particular the implications of the European Convention on Human Rights, Article 8 (the right to respect for private and family life). This will include:

  - Is the proposed system established on a proper legal basis and operated in accordance with the law?

  - Is it necessary to address a pressing need, such as public safety, crime prevention or national security?

  - Is it justified in the circumstances?

  - Is it proportionate to the problem that it is designed to deal with?

If this is not the case then it would not be appropriate to use CCTV.

- The impact assessment must be written, and stored for future reference, including auditing the system to ensure that it meets the assessed parameters.

Policy application

- Crescent Community Care Services Ltd must designate a Data Controller to take the legal responsibility for the operation and compliance of the CCTV system and resulting data. If the organisation is a legal entity e.g. a limited company, then the Data Controller can be the limited company, it does not need to be a person.

Procedure

Siting the cameras

- It is essential that the location of the equipment be carefully considered, because the way in which images are captured will need to comply with the Data Protection Act.

- All cameras will be located in prominent positions within Service User, public, and staff view and do not infringe on living, circulating (corridors etc), clinical and treatment areas.

- All CCTV surveillance will be automatically recorded and any breach of these Codes of Practice will be detected via controlled access to the system and auditing of the system.

- Signs need to be erected on all entrance points to Crescent Community Care Services Ltd premises and throughout the site to ensure staff and visitors are aware they are entering an area that is covered by CCTV surveillance equipment. The signs must include details on the purpose, organisation and contact details.

- Use of Covert CCTV (Directed) surveillance if required should be considered only with consultation with the Police, whose advice and requirements must be met.

Quality of the images
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- It is important that the images produced by the equipment are as clear as possible in order that they are effective for the purpose(s) for which they are intended. This is why it is essential that the purpose of the scheme be clearly identified. For example, if a system has been installed to prevent and detect crime, then it is essential that the images are adequate for that purpose.

- All camera installations and service contracts should be undertaken by NACOSS approved security companies. Upon installation, all equipment is tested to ensure that only the designated areas are monitored and high quality pictures are available in live and play back mode. All CCTV equipment should be serviced and maintained on an annual basis by a NACOSS approved company.

- The system can consist of cameras recording to digital recorders. These recorders must be located in secure locations, with access control at two levels (a) access to the location available only to authorised persons (b) access to images must be password protected and limited to persons authorised to view them.

Processing the images

- Images, which are not required for the purpose(s) for which the equipment is being used, should not be retained for longer than is necessary. While images are retained, it is essential that their integrity be maintained, whether it is to ensure their evidential value or to protect the rights of people whose images may have been recorded. It is therefore important that access to and security of the images is controlled in accordance with the requirements of the 1998 Act.

- All images that are digitally recorded must be stored securely within the system's hard drives for up to 30 days when they are then automatically erased. Located within the Crescent Community Care Services Ltd's premises there may be sub monitors which display images of public areas, however these must be located within enclosed areas and only accessible to Crescent Community Care Services Ltd 's authorised employees.

- Where the images are required for evidential purposes in legal or disciplinary proceedings, a DVD disc recording is made and placed in a sealed envelope signed and dated and held by the Registered Manager until completion of the investigation. Viewing of images must be controlled by the Data Controller or a person nominated to act on their behalf. Only persons trained in the use of the equipment and authorised by the Data Controller can access data.

Access to and disclosure of images to third parties

- It is important that access to, and disclosure of, the images recorded by CCTV and similar surveillance equipment is restricted and carefully controlled. This will ensure that the rights of individuals are preserved, but also to ensure that the continuity of evidence remains intact should the images be required for evidential purposes e.g. a Police inquiry or an investigation being undertaken as part of the disciplinary procedure.

- Access to the medium on which the images are displayed and recorded must be restricted to such parties and persons as the Data Controller may decide, subject to the restrictions of legislation and good practice.

- Access and disclosure to images is permitted only if it supports the purpose of the scheme. Under these conditions, the CCTV images record book and the appropriate view / release form (see Appendix) must be completed.

Access to images by individuals

- Section 7 of the 1998 Data Protection Act gives any individual the right to request access to CCTV images.

- Individuals who request access to images must be issued an access request form (see appendix). Upon receipt of the completed form, the Registered Manager and Provider's representative will determine whether disclosure is appropriate and whether there is a duty of care to protect the images of any third parties. If the duty of care cannot be discharged then the request can be refused.

- A written response must be made to the individual, giving the decision (and if the request has been refused, giving reasons) within 40 days of receipt of the enquiry. If disclosure is appropriate a payment in advance of £10.00 may be required.

Enforcement

- The Information Commissioner has the power to issue Enforcement Notices where they consider that there has been a breach of one or more of the Data Protection Principles. An Enforcement Notice will set out the remedial action that the Commissioner requires of the Trust to ensure future compliance with the requirements of the Act. The ICO should be notified prior to installation of any CCTV. When informing the ICO you will be required to identify what the data you are recording will be used for.

Reviewed by Marcus Kerridge McColl
December 2025

Challenging Behaviour Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with current best practice in managing challenging behaviour.

Scope

- This policy applies to all staff who have contact with Service Users, and particularly those Service Users who demonstrate challenging behaviour.

Policy

Underpinning Values

- Crescent Community Care Services Ltd adopts the position that challenging behaviour has a purpose in that it meets a particular need for the individual. These needs may be demonstrated through a variety of emotions and behaviours.

- Therefore, it is important that staff can:

  - Develop an understanding of these needs;
  - Anticipate the needs;
  - Soothe situations before they occur;
  - Provide the most appropriate responses to the emotions/behaviours displayed.

- When working with individuals with challenging behaviour staff will be expected to:

  - Recognise the individual's rights to take risks;
  - Ensure that the risk assessment places the Service User central to the purpose;
  - Support the individual through difficulties;
  - Preserve the dignity of the individual;
  - Ensure that rights and choices are promoted;
  - Create an environment in which individuals feel safe and secure;
  - Try to ensure that reflection after an incident places it in the context of the individual's life, and that this can be used to bring about positive outcomes for the individual.

Advance Statements

- Service Users identified as being at risk of disturbed/violent behaviour will be given the opportunity to have their views and wishes recorded, in the form of an advance statement. Advance statements are non-legally binding, but give an opportunity to identify Service Users' preferences about interventions and care.

- Service Users will be encouraged to identify as clearly as possible what interventions they would and would not wish to be used. They must be encouraged to review their wishes with staff, and changes must be clearly documented within Care Plans and other documentation.
- If the professional to whom the wish is being expressed forms the opinion that the Service User lacks capacity to understand the wish they are expressing, the professional must record their opinion, and their reasons for it, alongside the record of the Service User's wish. An advocate or carer should be invited to support a Service User who may lack capacity to understand the wish they are expressing. (Mental Health Act Code of Practice 9.21.)

Service and Profession-Specific Interventions

- A range of interventions, specifically based on the needs of users of particular services, should be provided within the Service to support the prevention and management of violence and aggressive behaviours. These will include the use of specific challenging behaviour models, person-centred Care approaches and speciality programmes in, as appropriate, Learning Disability, Children and Older People Services. Training for those service-specific interventions will be provided.

- In addition, there may be specialist interventions based on professional models, e.g. in clinical psychology. Training would be based within that professional group.

Employee Support

- The Service will ensure, through pre-employment and regular performance appraisal processes, that all employees have the necessary knowledge, skills and physical capability to manage the potential level of violence and aggression in their employment role. Employees will need to be capable of participation in the level of training required for their role.

- All employees who have been involved in an incident of violence or aggression will be offered timely support based upon their individual needs or preferences. Support methods can include:

  - Practical help with transport or accessing medical help;
  - Signposting to specific victim support or individual counselling;
  - Critical incident analysis/guided reflection;
  - Counselling or psychological therapies;
  - Information and assistance in relation to criminal justice procedures;
  - Access to Occupational Health Services;

- Training updates;
- Additional training as required;
- Access to staff representation and buddy systems, where appropriate;
- Professional and line management supervision which must be documented.

Record Keeping

- All employees must adhere to the relevant policies, in order to ensure that all information that will promote the proactive prevention and management of challenging behaviour is taken into account and acted upon. These include incident reporting, record keeping and maintenance of contemporaneous multi-professional progress recording.

Definitions

Challenging Behaviours

- "Severely challenging behaviour refers to culturally abnormal behaviour of such intensity, frequency of duration, that the physical safety of the person or others is likely to be placed in jeopardy, or behaviour that is likely to seriously limit or delay access to and use of ordinary community facilities." (Emerson, 1995).

- It is important that the term 'challenging behaviour' is utilised by all services as an all-encompassing concept and not to categorise or diagnose, and therefore assume, specific treatment rationales. For the purpose of the policy challenging behaviour is perceived as being expressed on a continuum from very low level behaviour to severe and extreme behaviours.

- There are a range of definitions of challenging behaviour; Emerson's definition can be used Service-wide.

- To ensure that people with less severely challenging behaviour receive appropriate care and support, Qureshi's (1994) definition is also provided.
- "People are said to display challenging behaviour if they:

  - Had, at some time, caused more than minor injuries to themselves or others, or destroyed their immediate living environment, or;
  - Showed behaviour, at least weekly, that required intervention by more than one member of staff to control or remove them from physical danger, or caused damage which could not be rectified by immediate Care staff or caused at least one hour's disruption, or;
  - Caused more than a few minutes disruption at least daily".

- Behaviours that could be described as challenging include:

  - Physical aggression;
  - Verbal aggression;
  - Self-injury;
  - Property destruction;
  - Non-compliance;
  - Anti-social nuisance behaviour.

- It is accepted that the definition of any given behaviour as challenging is subjective and relative. Therefore it is always necessary to precisely describe the behaviour that is being labelled as challenging in terms of its effects on the person, on their lifestyle, and on other people.

De-escalation

- This is defined as the purposeful use of a complex range of communication and therapeutic intervention skills, based on a knowledgeable understanding of the causes of violence and aggression, which is aimed at the prevention, reduction or management of the probability of violent or aggressive behaviour.

- Refer to the De-escalation Policy and Procedure.

Disengagement Physical Intervention (Breakaway)

- Breakaway interventions are those techniques used in 1:1 defence and disengagement from an attacker in a physical assault.

Restrictive Physical Intervention (Control and Restraint)

- Physical intervention is defined "as any method of responding to challenging behaviour which involves some degree of direct physical force to limit or restrict movement or mobility." (Harris et al, 1996).

- Physical intervention "involves the use of force to control a person's behaviour and can be employed using bodily contact, mechanical devices, or changes to the person's environment." (Department of Health, 2002).

- The purpose of restrictive interventions:
  - "Is to take immediate control of a dangerous situation where there is a real possibility of harm to the person or others if no action is undertaken. And
  - End or reduce significantly the danger to the patient or others". (Mental Health Act 1983 Code of Practice 2025 26.36.)
- Refer to Restraint Policy and Procedure.

Mechanical Restraint

- This is defined as the use of splints, arm cuffs, weighted blankets or any other device that is used to prevent injury by restricting movement or mobility which has been considered within an individual's risk during physical interventions; this would be detailed in the plan of Care for individuals. Protocols and training issues regarding the use of mechanical devices in physical interventions would be detailed in the relevant service areas documentation and policy.
- Refer to Restraint Policy and Procedure.

Consent

- "Consent" is a Service User's agreement for a Care or health professional to provide Care. Service Users may indicate consent non-verbally (for example, by presenting their arm for their pulse to be taken), orally, or in writing. For the consent to be valid, the Service User must:
  - Be competent to take the particular decision;
  - Have received sufficient information to take it;
  - Not be acting under duress.

Capacity

- Having mental capacity means that a person is able to make their own decisions. The Mental Capacity Act 2005 is designed to cover situations whereby someone is unable to make decisions because of an impairment of, or a disturbance in the functioning of, their mind or brain. The Act says that a person is unable to make a particular decision if, due to an impairment of, or a disturbance in the functioning of, their mind or brain they cannot:
  - Understand information about the decision to be made (the relevant information);
  - Retain the information in their mind;
  - Use or weigh that information as part of the decision-making process;
  - Communicate their decision.
- See Paragraph 4.14 Mental Capacity Act 2005 Code of Practice.

Care Interventions

- A proactive recovery-focused approach aimed at preventing the likelihood of challenging behaviour occurring. The gathering of information from Service Users, their carers and families, colleagues and other agencies will lead to a formulation of the risk issues.
- That assessment will identify preventative strategies, both to promote a positive environment and to minimise the risks. Specific management guidelines will be based on individual planning of Care.
- Interventions may include the promotion of alternative coping skills and activities, environmental change, de- escalation, use of rapid tranquilisation, physical control and restraint, and seclusion.
- Minimum level of interventions must be used and Service Users supported in developing their own positive coping and risk management skills. Exceptions to these principles may be requested through an individual’s advance request. Further

functional analysis of behaviours, near misses and incidents must be undertaken to inform the clinical and management strategies.

PROCEDURE

Incident Reporting

- All incidents of challenging behaviour, violence and aggression, physical or non-physical, and near misses to incidents will be reported using the Record of Challenging Behaviour form in this document, or the HS01 Accident and Incident Reporting Policy and Procedure - where appropriate.

- Employees will be supported in the incident reporting procedure and advised to complete requests as soon after the incident or near miss as possible.

Post Incident Review

- The purpose of a post-incident debrief is "to identify and address physical harm to Service Users or staff, ongoing risks and the emotional impact on Service Users and staff, including witnesses" and to "determine the factors that contributed to an incident that led to a restrictive intervention, identify any factors that can be addressed quickly to reduce the likelihood of a further incident and amend risk and care plans accordingly." (National Institute for Health and Care Excellence: NG10, *Violence and Aggression: Short-term management in Mental Health, Health and Community settings 2025.)*

- "The aim of a post incident review will be to seek to learn lessons, support staff and Service Users and encourage the therapeutic relationship between staff, Service Users and their carers." (NICE 2005).

- A post incident review will take place as soon as possible and within at least 72 hours of an incident ending. If possible, a person not directly involved in the incident will lead the review. The review will address:

  - What happened during the incident;
  - Any trigger factors;
  - Each person's role in the incident;
  - Their feelings at the time of the incident, at the review and how they may feel in the near future;
  - What can be done to address their concerns.

- A documented record of this must be drawn up, especially in relation to restraint.
  If a Service User is restrained, this must be discussed and the reasons why this way deemed necessary shared. Collaborative planning to prevent recurrence is essential.

- Service Users and visitors who witness restraint must also be given an explanation of the need for forceful intervention to ensure understanding and allow any anxieties or fears to be voiced.

Lone Working

- Many mental health and learning disability employees work as lone practitioners either in Service premises or in community settings. The systems and procedures to be used to prevent, minimise and manage risk for lone workers are outlined in the lone working policies.

Contacting the Police in the Management of Violence and Aggression

- All Service employees are advised to contact the local police force, using the emergency 999 call number, if they are involved in a challenging behaviour or violent or aggressive situation that cannot be de-escalated or managed by the range of clinical procedures and interventions available to clinical practitioners.

This is particularly important if a violent incident is provoked or initiated by those who are not in receipt of clinical services and are not known to the clinical practitioner.

Engagement and Observation

Engagement and observation are therapeutic risk management interventions that can be utilised to support the management of challenging behaviour.

Rapid Tranquilisation

There is a range of interventions where medicines are used to assist in the prevention, reduction and management of violence and aggression. Specific use of prescribed medication to ensure rapid tranquilisation of Service Users who are becoming increasingly disturbed, violent or aggressive, or where their challenging behaviours are escalating, may be used according to individual and pre-prescribed principles. Specific training is mandatory for employees to use these approaches.

Primary Prevention

When used as a primary prevention, medication must only be used where a Service User has a mental health or co-morbid mental health problem that is alleviated by medication. A physical health and pharmacology assessment will be completed prior to administration of as required medication, and before the use of a restrictive physical intervention.

Secondary Prevention Reaction

When considering the use of medication as a secondary prevention, or reactive strategy, the Service advocates evidence-based approaches less dependent on medication and requires compliance with the guidance given in the Guidance for Restrictive Physical Interventions (Department of Health, 2002, p18).

De-escalation Techniques

- NICE guidelines indicate that health and social care provider organisations should give staff training in de- escalation that enables them to: recognise the early signs of aggression and be able to respond appropriately. These guidelines also stress the general principle that staff should:
    - Establish a close working relationship with Service Users that will allow them to be aware of changes in mood. (National Institute for Health and Care Excellence: NG10, *Violence and Aggression: Short-term management in Mental Health, Health and Community settings* 2025.)
    - Refer to De-escalation Policy and Procedure.

Disengagement Physical Intervention (Breakaway)

- The Service supports the use of Breakaway interventions that are taught in order to enable the staff to disengage from an attacker in a physical assault. The Service acknowledges that the use of reasonable force may also be necessary. Wherever possible, and from the formulation derived from the clinical risk assessment, a plan of Care prescribing the use of such interventions, or other preventative strategies, would be compiled.

Reasonable Force

- There is no legal definition of 'reasonable force'. It is therefore not possible to comprehensively set out when it is reasonable to use force, or the degree of force that may reasonably be used. It will always depend upon the circumstances of the case.
- There are two relevant considerations:
    - The use of force can be regarded as reasonable only if the circumstances of the particular incident warrant it. The use of any degree of force is unlawful if the particular circumstances do not warrant the use of physical force. Therefore physical force could not be justified to prevent an individual from committing a trivial misdemeanour, or in a situation that clearly could be resolved without force.
    - The degree of force employed must be in proportion to the circumstances of the incident and seriousness of the behaviour or the consequences it is intended to prevent. Any force used must always be the minimum needed to achieve the desired result.
- Reasonable force may be used for Service Users who are detained under the Mental Health Act, where it is "therapeutically necessary" to be medicated, restrained, fed or secluded. (See Section 63, part 4 of the Mental Health Act 1983 concerning medical treatment for mental disorder not requiring the consent of the patient.) For medical treatment for physical disorders, this cannot be administered without the consent of the person. For someone who lacks the mental capacity to consent, reasonable force may be used where the treatment is in the person's best interest. (Section 6 of the Mental Capacity Act 2005.)

Restrictive Physical interventions

- When violent, aggressive or challenging behaviour cannot be de-escalated and it is assessed that there are significant risks to the safety and security of the perpetrator and/or the potential victim(s), employees may need to use restrictive physical interventions (control and restraint). Those using these interventions must demonstrate competency through assessment.

- The training for the use of physical intervention is generic, but specific interventions for specialist needs can be identified for employees during training or from requested consultation.

- For the purpose of this policy, physical intervention, as defined, does not involve the use of mechanical devices. The Service supports the use of restrictive physical intervention techniques that are taught to enable staff to respond to challenging behaviour using direct physical contact to limit or restrict the movements of another, or disengage as a last resort when a Service User is presenting a high risk of harm to themselves or others (see Protocol for Good Practice in Restrictive Physical Intervention).

Deprivation of Liberty

- Where restrictions or restraint on Service Users would constitute a deprivation of the person's liberty, this would be in contravention of the person's human rights, unless authorisation for the deprivation of liberty was sought. For further guidance on this, reference should be made to the QCS Policy and Procedure on the Deprivation of Liberty Safeguards.

IMPLEMENTATION

- Awareness of this policy will also be disseminated to all Service employees through a line management briefing, cascaded through the operational and corporate service structures, supported by a standardised briefing.

- The Registered Manager will designate a person to assess the environmental risks in premises within their areas of responsibility.

- An overall service specific risk assessment will be completed with the involvement of all staff in order to determine the level of risks posed, and the training requirements necessary to manage those risks.

- The outcome of the risk assessments will be assessed, reviewed and agreed through the service specific governance systems.

- A register of environmental risks is to be maintained.

- Mandatory training will be provided in line with the organisational training needs analysis, with attendance monitored via human resources and reports sent to local managers to ensure mandatory training needs are met.

- For training that the Service has not classified as mandatory, managers should consider the necessary competencies for the specific work required and arrange for these needs to be met through the appraisal and personal development plan process. Access to support to meet these needs is outlined in the training policy.

- Employees will be expected to complete a self-assessment of physical health in relation to undertaking training, rehearsal and practice in the delivery of breakaway and physical interventions. Any concerns regarding physical capability will be referred to the Registered Manager.

Training Levels

- The Service will ensure that the appropriate training and education is available to implement this policy. All employees will be expected to participate in the training and education programmes as determined by the organisational training needs analysis and identified as required by their role. Their competencies will be monitored through the Service appraisal, supervision systems and Human Resources Systems. Appropriate recording will be maintained in staff's personal files.

- Different forms of training are available:

- **Theory** - Following the Safety, Management and Security standard national curriculum on the introduction to violent and aggressive behaviour, as well as promoting personal safety competency outcomes of applied knowledge, as assessed by an appraiser in a work setting**.**

- **De-escalation and Breakaway training** - All of the content of the theory component, plus positive communication and the use of functional analysis in the prevention and management of violence and aggression, de-escalation interventions, preventative and breakaway physical interventions. Competency outcomes will be assessed during training.

- **Bespoke Breakaway training** - All of the content of the theory component, plus positive communication and use of functional analysis in the prevention and management of violence and aggression, De-escalation interventions, preventative and breakaway physical interventions including a seated restraint technique. Competency outcomes are assessed during training.

- **Physical intervention** - All the content of the theory component and de-escalation and breakaway, plus physical interventions used in the management of violence and aggression and positional asphyxia. Competency outcomes are assessed during training.

PROTOCOL FOR CHALLENGING BEHAVIOUR ASSESSMENT AND MANAGEMENT

- This protocol outlines specific practice underpinning the Care of those designated as having challenging behaviours.

- For each individual:

  - An assessment for capacity to consent must be carried out, and if capacity is present, permission must be gained for any assessment or intervention prior to it being carried out. If they are deemed to lack capacity to consent, or they are refusing treatment, professionals are to follow the 'best interest' rule.
  - A comprehensive assessment and formulation must be completed, incorporating: Œ

    - Comprehensive assessment;
    - A risk assessment tool;
    - Functional assessment.

  - All interventions (including physical interventions) must be carried out and documented.
  - Other interventions must match well with the assessment.
  - The Service User must have been involved in the development of the intervention plan.
  - Primary and Secondary preventions and agreed Reactive strategies (including any restrictive physical interventions) must be written into the Service User's Care plan.
  - Any restrictive physical interventions must be carried out in accordance with the current Guidance for Restrictive Physical Interventions from the National Institute for Health and Care Excellence, General Services Association and the British Institute for Learning Disabilities.
  - Following any incident involving challenging behaviour (excluding those behaviours which are pervasive, in occurrence more than twice daily and do not cause significant physical harm), the following must be carried out:

    - A post-incidence record must be completed on the case notes;
    - A post-incidence analysis must be carried out and any recommendations carried out (e.g. amendments to Care Plan);
    - Where appropriate, a debrief must be carried out with the person who has presented the challenging behaviour to check their physical, mental and emotional well-being;
    - A debrief must be given to any staff members involved in the incident and a range of resources for support will be offered.

GOOD PRACTICE IN RESTRICTIVE PHYSICAL INTERVENTIONS

Good Practices During Restraint

Introduction

- Restraint techniques must only be used as a last resort, when all primary and secondary interventions have failed and the level of risk is such that more intensive interventions are required in order to maintain safety. Service Users with a history of challenging behaviour/violence and aggression must, as soon as practicable, have a relevant care/treatment plan drawn up, in collaboration with the multi-disciplinary team, Service User, carers, family, and/or advocates. Where possible this must describe and set out a broader strategy for addressing the Service User's needs and challenging behaviour.
- Gender, age, size and build, and in addition physical and sensory impairment issues, must be taken into account (e.g. pregnancy, children, younger persons and older persons).

- Wherever possible the Service User must be approached and an agreement sought to stop the behaviour, or co- operate with a request. Before any attempt is made to restrain an individual a visual check must be made for weapons. If there is any doubt, the Service User must not be approached and police summoned to assist. If weapons are visible then the Service User must be asked to put them down in a neutral area and not to hand them over.

Responsibilities

- The person controlling the individual's head will assume control of the intervention and will manage the situation. Other individuals, present at the situation, will take direction from the head person. Compliance with this procedure ensures minimisation of risk to staff and Service Users.

- There must be continual verbal de-escalation between the individual in control of the incident/situation, and the Service User. Communication between the rest of the team must be minimal.

- Staff will demonstrate at all times respect dignity and privacy and ensure that there is communication between the Service User and themselves.

- During any physical restraint intervention the Service User's physical condition must be monitored in order to reduce physical risks e.g. positional asphyxia. Where mechanical restraint is employed during and after physical intervention, guidance in the Mental Health Act Code of Practice says this should be regularly reviewed by a registered medical practitioner. This policy should be read in conjunction with Chapter 26 of the Code of Practice titled 'Safe and Therapeutic responses to disturbed behaviour'.

- When possible, other Service Users and staff not involved with restraint must be relocated to another area.

- Restraint must be used as a last resort. At the earliest opportunity the Service User will be disengaged from the restrictive physical intervention when it is deemed safe and practicable to do so.

- The Service does not support the deliberate application of pain being applied to Service Users to gain compliance. It is acknowledged "that there may, however, be high risk situations where an element of pain - for both the individual and staff - may be unavoidable if the emergency is to be safely resolved". (Guidance from the National Control and Restraint General Services Association at http://ncrgsa.com.)

- The Service training needs analysis describes who must attend which form of Management of Violence and Aggression training, and be deemed to be competent before engaging in these practices.

- Where physical restraint is used, staff must record the decision and the reasons for it and document and review every episode of physical restraint, which must include a detailed account of the restraint and record the post- incident review.

- Staff must use debriefing and post-incident review following all episodes of restrictive physical interventions.

Reviewed by Marcus Kerridge McColl
November 2025

Changes in Health State Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To respond appropriately to changes in the Health Status of Service Users

Scope

- All Service users

Policy

- Staff are vigilant to the day to day good health of Service users and promptly report any changes or concerns. Service Users are assisted to seek help and advice from their GP when feeling unwell, or staff ensure any health concern is assessed as soon as possible by a medical practitioner
- Information on the Health status of the Service User and details of any known health issues and contact arrangements in the event of illness are recorded in the Service user Guide and also on the computer system in the office

Procedure

- Care Plans show a thorough assessment of the Health issues of the Service user and a current Health Action Plan
- Details of any signs and symptoms that may denote that a Service user is unwell are given in Care Plans
- Any drug reactions / interactions are noted in the Care Plans and medicine administration records along with the contradictions of medicines
- Service Users are helped to express their state of health and encouraged to communicate in an appropriate way any pain or ache or discomfort. This might include the use of a sign, gesture, symbol or object of reference for example.
- The Use of analgesia is clearly defined in the care plan. Staff should observe any instruction for administration, including method and timing ensuring that combined medicines (particularly those containing paracetamol) are carefully managed. This may include cough medicines, cold powders and similar products.
- In the event of an unplanned medical appointment staff will inform relevant family members or emergency contacts.
- Any health changes care changes or medical changes need to be reported to the team and relevant care plans updated

Reviewed Marcus Kerridge-McColl – November 2025

Changes to Contracted Hours Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that staff changes are managed sufficiently as to not affect the Service to the Clients of Crescent Community Care Services Limited

Scope

- To all staff members of Crescent Community Care Services Limited

Policy

- To ensure that there is adequate cover for all calls to clients of Crescent Community Care Services Limited
- To maintain staffing levels at all times

Procedure

- All staff members are to give at least 2 weeks notice for permanent changes to their contracted working hours. This is to ensure that the changes can be planned effectively and the Service users calls a managed efficiently.

Reviewed Marcus Kerridge-McColl
November 2025

Purpose

- To comply with good practice in child protection in situations where employees may have unsupervised access to children while working, for instance where a child lives in a Service User's own home.
- To comply with safe practice where children visit Service Users in registered premises, or in the Service User's own home.

Scope

- Situations whereby employees have unsupervised access to children while working.
- Situations where children visit Service Users in registered premises, or in the Service User's own home.

Policy

- We at Crescent Community Care Services limited are committed to practices which protect children from harm. Staff in this organisation accept and recognise our responsibility to develop awareness of issues that can cause children and young people harm.
- We will endeavour to safeguard children and young people by:
    - Adopting child protection guidelines through a code of behaviour for staff and volunteers;
    - Sharing information about child protection and good practice with children, parents and staff;
    - Sharing information about concerns with agencies who need to know, and involving parents and children appropriately;
    - Following carefully the procedures for recruitment and selection of staff;
    - Providing effective management for staff through supervision, support and training;
    - Providing a safe environment for children.

Procedure

CODE OF BEHAVIOUR

Statement of Intent

- It is the policy of Crescent Community Care Services Limited to safeguard the welfare of all children and young people by protecting them from physical danger, and all forms of abuse, including physical, emotional and sexual harm.
- Crescent Community Care Services Limited is committed to creating an environment in which young people can feel safe and comfortable and secure while in contact with any of Crescent Community Care Services Limited's employees. Personnel should at all times show respect and understanding for individuals' rights, safety and welfare, and conduct themselves in a way that reflects the ethos and principles of Crescent Community Care Services Limited.

Guidelines for all Crescent Community Care Services Limited employees

Attitudes

- Staff should be committed to:
    - Treating children and young people with respect and dignity;
    - Always listening to what a child or young person is saying;
    - Valuing each child and young person;
    - Recognising the unique contribution each individual can make;
    - Encouraging and praising each child or young person.

By Example

- Staff should endeavour to:
    - Provide an example, which we would wish others to follow;
    - Use appropriate language with children and young people and challenge any inappropriate language used by a young

person or child, or an adult working with young people;

  - Respect a young person's right to privacy.

One-to-one Contact

- Staff should not spend excessive amounts of time alone with children, away from others.
- In the unlikely event of having to meet with an individual child or young person staff should make every effort to keep this meeting as open as possible.
- If privacy is needed, ensure that other staff are informed of the meeting and its whereabouts.

Physical Contact

- Staff should never:
  - Engage in sexually provocative or rough physical games, including horseplay;
  - Do things of a personal nature for a child or a young person that they can do for themselves. If such an incident arises, for example, where a child or young person has limited mobility Crescent Community Care Services limited staff should seek a member of their family to deal with such an incident;
  - Allow, or engage in, inappropriate touching of any kind.

General

- Staff should:
  - Be aware that someone might misinterpret our actions no matter how well intentioned;
  - Never draw any conclusions about others without checking the facts;
  - Never allow themselves to be drawn into inappropriate attention-seeking situations such as tantrums or crushes;
  - Never exaggerate or trivialise child abuse issues or make suggestive remarks or gestures about such issues, especially to a child or young person, even in jest.

Risk Assessments

- Where children are admitted to a premises, or are resident on the premises, all risk assessments carried out on the environment, equipment, substances and anything with which the child may come into contact must specifically assess the risks to and from children. Until this range has been checked, extended where necessary, and all advised actions carried out, children should not be admitted on the premises.

SHARING INFORMATION ABOUT CHILD PROTECTION AND GOOD PRACTICE WITH CHILDREN, STAFF AND VOLUNTEERS

- Good communication is essential in any organisation. In Crescent Community Care Services Limited, every effort will be made to ensure that, should individuals have concerns, they will be listened to and taken seriously.
- It is the responsibility of the management to ensure that information is available to, and exchanged between, all those involved in this organisation and its activities. Some information is confidential and should only be shared on a strictly need-to-know basis.

Children and Young People

- Children and young people have a right to information, especially any information that could make life better and safer for them. Crescent Community Care Services Limited will act to ensure they have information about how, and with whom, they can share their concerns, complaints and anxieties.
- When sharing information, Crescent Community Care Services Limited personnel will be sensitive to the level of understanding and maturity, as well as to the level of responsibility, of the people with whom they are sharing.

Parents

- Parents / persons with parental responsibility are ultimately responsible for their children's welfare at all times, and they should be assured that their children are in contact with staff employed by a credible organisation.
- Where the children are not the recipient of the Service, but are simply visiting a Service User Parents should also closely supervise at all times and accept responsibility for their children when they are visiting a Service User at home, whether that is the Service User's own home or a registered premises such as a hospital or care home. It is the responsibility of the parent(s) to ensure that their children:
  - Are made aware of the need for the privacy of the Service Users;
  - Are made aware of the need to not disturb other Service Users in the vicinity;

- Are made aware that a registered premises, and often a Service User's own home, may hold materials which may be harmful to children. The parents must be vigilant for such dangers, and children aware that they should not touch or consume anything without checking with their parents first;
- Are made aware that a registered premises, and often a Service User's own home, may contain equipment which may be harmful to children. The parents must be vigilant for such dangers, and children aware that they should not touch anything without checking with their parents first.

Staff

- As an organisation whose employees may occasionally be in contact with children and young people in the workplace, it is imperative that each staff member of the Crescent Community Care Services Limited staff in this situation is aware of their responsibilities under the Child Protection legislation and has a working knowledge of The companies procedures.
- Staff will report to their line manager if children or young people enter their workplace.
- The line manager will ensure that all staff who do or may have contact with children or young people receive direct supervision with regard to the contents of this policy, and conduct regular checks on the workplace.

Other Bodies

- A copy of our Child Protection Policy will be made available to any other appropriate body.

SHARING INFORMATION ABOUT CONCERNS WITH AGENCIES WHO NEED TO KNOW, AND INVOLVING PARENTS AND CHILDREN APPROPRIATELY - PROCEDURE FOR REPORTING ALLEGATIONS OR SUSPICIONS OF ABUSE

- In any case where an allegation is made, or someone in Crescent Community Care Services Limited has concerns, a record should be made. As far as is practically possible, details must include:
  - Name of child or young person;
  - Age;
  - The Agency address (if known);
  - Date of birth (if known);
  - Name(s) and address(es) of parent(s), or person(s) with parental responsibility;
  - Telephone numbers if available;
  - Is the person making the report expressing their own concerns, or passing on those of somebody else? If so, record details;
  - What has prompted the concerns;
  - Include dates and times of any specific incidents;
  - Whether the child or young person has been spoken to;
  - If so, what was said;
  - Whether anybody has been alleged to be the abuser;
  - If so, record details;
  - Who has this been passed on to, in order that appropriate action is taken? e.g. school, designated officer, social services etc;
  - Whether anyone else has been consulted? If so, record details;
  - **ACTION TAKEN.**

Record Keeping

- All records, information and confidential notes should be kept in separate files in a locked drawer or filing cabinet.
- Only designated persons will have access to these files.

Disclosure

- Never guarantee absolute confidentiality, as Child Protection will always have precedence over any other issues.
- Listen to the child, rather than question him or her directly.
- Offer him / her reassurance without making promises, and take what the child says seriously.
- Allow the child to speak without interruption.
- Accept what is said – it is not your role to investigate or question.
- Do not overreact.

- Alleviate feelings of guilt and isolation, while passing no judgement.
- Advise that you will try to offer support, but that you must pass the information on.
- Explain what you have to do and whom you have to tell.
- Record the discussion accurately, as soon as possible after the event.
- Use the child's words or explanations – do not translate into your own words, in case you have misconstrued what the child was trying to say.
- Contact one of the Crescent Community Care Services Limited's Directors or Registered Manager for advice / guidance.
- The Registered Manager may then discuss the concern / suspicion with the relevant organisation, and, if appropriate, make a direct referral.
- If either Registered Manager is not available, or it is inappropriate to approach them, the member of staff with the concern should make direct contact with the relevant organisation themselves.
- Record any discussions or actions taken within 24 hours.

Reviewed Marcus Kerridge-McColl
September 2025

Choice of Carer Gender Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To support the choice of the Service User.

Scope

- All Service Users.

Policy

- Service Users will be asked to express any preference they have for a Carer of a particular gender.
- Where it is not possible for the Service User to indicate a choice, family preferences will be followed.
- Where it is not possible for the Service User to indicate a choice, and there is no family or other relevant advocate, Care will always be provided by carers of the same gender as the Service User.

Procedure

- The Service User, family member, or relevant advocate will be asked if they have a preference for the gender of the carer.
- Their choice will be recorded on the Service User Care Plan.
- Staff of the appropriate gender will be arranged according to identified Service User needs.
- Where a male Carer is assigned to provide personal Care for a female Service User, the Service User will be offered a female chaperone.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To establish criteria for the handling and disposal of waste products resulting from Crescent Community Care Services Ltd activities, as well as a set of procedures to guide staff in terms of legislation and best practice.

Scope

- All employees.

Policy

- **Crescent Community Care Services Ltd** is committed to the protection of the environment through the implementation of an effective waste management programme that meets or exceeds all legislative and regulatory requirements placed upon it.
- **Crescent Community Care Services Ltd** will comply with:
    - The Controlled Waste Regulations;
    - The Environmental Protection Act including the Duty of Care Regulations;
    - The Carriage of Dangerous Goods Regulations;
    - The Hazardous Waste Regulations.
- Principles for consignments of clinical waste:
    - It is **Crescent Community Care Services Ltd** policy to dispose of normal clinical waste in sealed bags to a registered clinical waste disposal contractor, provided it does not contain pharmaceutical products.
    - Alternative disposal methods may be considered where the producer confirms that no such waste needs to be incinerated, which is confirmed by regular local audits.
    - Where the waste is known to be non-infectious, then it may be consigned as such. Otherwise it should be consigned as hazardous by infection; even if only very small amounts of infectious materials are (or may be) mixed with otherwise non-infectious waste. Any waste contaminated with, or containing cytotoxic or cytostatic medicines must be consigned separately to the above.
    - Controlled drugs are to be denatured prior to disposal.
    - Waste sharps must be place in an approved container and ideally separately consigned or added to and consigned with the cytotoxic waste.
    - Materials which are not hazardous and therefore do not need to meet the clinical waste disposal regulations should not be disposed of in hygiene bags. Disposal of clinical waste is very costly compared to non-hazardous waste disposal.

Procedure

- **Duty of Care Regulations:**
    - The Duty of Care Regulations (See Appendix attached to this policy) must be brought to the attention of all employees who are producing, handling, or storing all types of controlled waste. Breach of the Duty of Care Regulations is a criminal offence.
    - The waste producer (Crescent Community Care Services Ltd) is responsible for complying with the clinical waste disposal regulations. This responsibility cannot be assumed to be taken up by a third party, such as the waste removal contractor.
    - Therefore, the waste producer must at all times satisfy themselves that the waste disposal contractor is meeting the regulations on the producer's behalf.
- **Clinical waste disposal procedure:**
    - The procedure to be used for the disposal of clinical waste is as follows:
    - Waste is accurately identified as being "clinical";
    - Waste is appropriately packaged by type;
    - Waste is stored correctly;
    - The waste is stored securely prior to disposal;

- Clinical waste transfer note has been completed.

- **Identification of clinical waste:**
  - Responsibility for identifying clinical waste lies strictly with the producer. It is important to remember for identification purposes that all waste contaminated with clinical waste is considered to be clinical waste itself.
- **Materials identified as clinical waste must be disposed of through a licensed contractor:**
  - Under no circumstances must clinical waste be allowed to enter the general waste stream.
  - Clinical waste must be carefully segregated from other waste whilst in production and storage, and must never be placed in internal or external general waste bins.
- **Storage and packaging of Clinical waste:**
  - Under the Duty of Care Regulations (section 34 Environmental Protection Act 1990), all waste must be kept safe from:
    - Corrosion and wear of waste containers;
    - Accidental spilling or leaking;
    - Accident or weather breaking contained waste open and allowing it to escape;
    - Waste blowing away whilst stored or transported, scavenging of waste by vandals, thieves, children, trespassers or animals.
- **Types of Packaging:**
  - Requirements for packaging vary depending on, for example, the category of the waste, whether it is transported off-site, and the method of final disposal.

Waste Group	Type of Clinical Waste
Group A	Includes the following items: identifiable human tissue*, blood animal carcasses and tissue from veterinary centres, hospitals, or laboratories. Soiled surgical dressings, swabs and other similar soiled waste. Waste materials, for example from infectious disease cases, excluding any in groups B-E.
Group B	Discarded syringe needles, cartridges, broken glass and other contaminated disposable sharp instruments or items.
Group C	Microbiological cultures and potentially infected waste from pathology departments and other clinical or research laboratories.
Group D	Drugs or other pharmaceutical products.
Group E	Items used to dispose of urine, faeces and other bodily fluid secretions or excretions, which do not fall within group A. This includes used disposal bed pans or bed pan liners, incontinence pads, stoma bags and urine containers.

Group A:

- Group A clinical waste (waste that is contaminated with body fluids, human and animal tissue) should be placed in designated bins lined with yellow clinical waste sacks only. The sacks need to comply with the requirements of CDGRR[1] / ADR .This means that they need to be design-tested and certified (usually referred to as UN type-approved) for transportation off site.
- Containers should be no more than three-quarters filled or a maximum of 4KG weight securely tied (not stapled).
- For ease of storage and handling, it is advised that tissue waste is kept packaged in clinical waste sacks frozen in a dedicated freezer.

Group B:

- Clinical waste sharps should only be stored in sharps containers that comply with British standard BS 7320 and UN 3291.
- When three-quarters full, the container should be securely closed – on no account should attempts be made to reopen a sealed sharps container.
- Sharps containers should not be placed in yellow clinical waste sacks, but kept separate for collection.

Group C:

- Any group C waste which is not sterilised before leaving site has to be packaged in accordance with approved requirements

(i.e. UN type-approved container).

Group D:

- Group D clinical waste must be transported in appropriate UN type-approved packaging. The use of clinical waste sacks is not acceptable.

Group E:

- Group E wastes are not considered dangerous for transport by road under CDGRR and do not have to be in UN type-approved containers. However, risk assessments under CoSHH may indicate the need for precautions to ensure the health and safety of people handling the waste. Producers need to take this, and the possibility of causing offence, into account when making decisions about packaging.

Labelling:

- Packaging for clinical waste needs to be labelled to show clearly the type of waste. If the waste is transported off-site for final disposal, labels must meet the requirements of CDGRR. These regulations prohibit consigning the waste for transport unless the following information is clearly shown:
    - The appropriate designation of the waste. For example, the designation of unspecified infectious waste is "CLINICAL WASTE, UNSPECIFIED, N.O.S";
    - The appropriate UN Number. In the case of CLINICAL WASTE UNSPECIFIED, N.O.S this would be UN 3921 (The number must always be preceded by the letters UN;
    - The appropriate danger sign. The danger sign for infectious substances, which is needed for waste classified as UN 3921, is shown below.

Dealing with accidents incidents and spillages:

- Employers at all points of the waste chain need written procedures for dealing with accidents or incidents, which address:
- Immediate first aid measures;
- Immediate reporting to a responsible, designated person;
- Recording of the accident/incident;
- Investigating the incident and implementing remedial action. Initial investigation should preferably take place before any damaged container is removed;
- Retaining, if possible, the item and information about its source to help identify possible infection risks;
- Attendance by any injured person to an A&E department and, if an employee, with subsequent referral to a General Practitioner as soon as possible;
- In the cases of sharps injuries, procedures also need to cover arrangements for suitable medical advice and counselling.

Spillages:

- Clear written procedures need to be in place specifying the following reporting and investigation procedures:
    - Use of a safe system of work for clearing up clinical waste;
    - Appropriate requirements for decontamination;

- Protective clothing.

- Spillage kits should contain the following:
  - Disposable gloves;
  - A disposable apron;
  - Clinical waste sack;
  - Paper towels;
  - Disposable cloths;
  - Disinfectant recommended (by, for example, local control);
  - Means of collecting sharps.

Spillage Kits:

- Disinfectants containing 10,000-ppm available chlorine are recommended for spillages. The use of sodium dichloroisocyanurate (NaDCC) granules is also generally recommended because made-up solutions lose activity with time and require regular replacement. Suitable inert, absorbent materials may be used to deal with liquid spillages after disinfecting.
- The use of certain disinfectants themselves may present a health risk in confined spaces. COSHH applies to such risks and appropriate precautions must be specified based on risk assessments. Glutaraldehyde must not be used.
- Appropriate equipment needs to be provided for collecting spilled waste and placing it in new containers. Sharps must not be picked up by hand. Spilled waste and any absorbent materials must be placed in a clinical waste container for disposal.

Appendix - Statutory Instrument 1991 No. 2839

The Environmental Protection (Duty of Care) Regulations 1991

STATUTORY INSTRUMENTS

1991 No. 2839

ENVIRONMENTAL PROTECTION

The Environmental Protection (Duty of Care) Regulations 1991

Made: 16th December 1991

Laid before Parliament 17th December 1991

Coming into force 1st April 1992: 1st April 1992

The Secretary of State for the Environment as respects England, the Secretary of State for Wales as respects Wales and the Secretary of State for Scotland as respects Scotland, in exercise of the powers conferred on them by section 34(5) of the Environmental Protection Act 1990[1] and of all other powers enabling them in that behalf, hereby make the following Regulations:

Citation, commencement and interpretation

1. -

(1) These Regulations may be cited as the Environmental Protection (Duty of Care) Regulations 1991 and shall come into force on 1st April 1992.

(2) In these Regulations -

“the 1990 Act” means the Environmental Protection Act 1990;

“transferor” and “transferee” mean respectively, in relation to a transfer of controlled waste by a person who is subject to the duty imposed by section 34(1) of the 1990 Act, the person who in compliance with that section transfers a written description of the waste and the person who receives that description.

Transfer notes

2. -

(1) The transferor and the transferee shall, at the same time as the written description of the waste is transferred, ensure that such a document as is described in paragraph (2) (“a transfer note”) is completed and signed on their behalf.

(2) A transfer note shall -

(a) identify the waste to which it relates and state -

(i) its quantity and whether on transfer it is loose or in a container;

(ii) if in a container, the kind of container; and

(iii) the time and place of transfer;

(b) give the name and address of the transferor and the transferee;

(c) state whether or not the transferor is the producer or importer of the waste and, if so, which;

(d) if the transfer is to a person for authorised transport purposes, specify which of those purposes; and

(e) state as respects the transferor and the transferee which, if any, of the categories of person shown in column 1 of the following Table describes him and provide any relevant additional information specified in column 2 of the Table. (Please turn over).

Category of Person	Additional Information
An authority which is a waste collection authority for the purposes of Part II of the 1990 Act.	
A person who is the holder of a waste management licence under section 35 of the 1990 Act or of a disposal licence under section 5 of the Control of Pollution Act 1974[2].	If the waste is to be kept, treated or disposed of by that person, the relevant licence number and the name of the licensing authority.
A person to whom section 33(1) of the 1990 Act does not apply by virtue of regulations under subsection (3) of that section.	
A person registered as a carrier of controlled waste under section 2 of the Control of Pollution (Amendment) Act 1989[3].	The name of the waste regulation authority with whom he is registered and his registration number.
A person who is not required to be so registered by virtue of regulations under section 1(3) of that Act.	
A waste disposal authority in Scotland.	

Duty to keep copies of written descriptions of waste and transfer notes

3. The transferor and the transferee shall each keep the written description of the waste and the transfer note or copies thereof for a period of two years from the transfer of the controlled waste.

Duty to furnish documents

4. A person who has been served by a waste regulation authority with a notice in writing specifying or describing any document and requiring its production shall, if the document is one which at that time he is under a duty to keep under regulation 3, furnish the authority with a copy of it at the authority's office specified in the notice and within the period (not being less than 7 days) so specified.

NB – SOME LOCAL AUTHORITIES DO NOT HAVE A WASTE DISPOSAL SERVICE AND SO THE DISPOSAL OF PADS HAVE TO BE RUN IN ACCORDANCE WITH THE LOCAL AUTHORITY POLICY

Reviewed Marcus Kerridge-McColl
November 2025

Purpose

- The purpose of this policy is to promote focus on Service User requirements, working efficiency, high productivity and a harmonious working environment by means of effective communications between all members of staff and Service users

Scope

- All Service Users and employees.

Policy

- Communication systems will be set up and maintained to facilitate the flow of information throughout the organization – This will be determined in differing manners according to the requirement of communication

Service User Information

- Regular meetings of Service Users and families will be organised, supported and the outcomes recorded and communicated to all Service Users and families.
- Memos will be distributed to employees and will be produced on at least an ad-hoc basis and if possible to a known schedule, and all employees are encouraged to read and understand and if they feel that information should be passed over then to contribute material to the memo.
- Service Users are informed before any announced statutory visits.
- Information for or about Service Users received from outside the organisation, e.g. by telephone calls, will be noted on their computerized file and a copy kept on their paper file

Job and Work Related Information

- Job description and/or statement of key tasks.
- Standards of working, including relevant QCF Diploma standards.
- Supervision sessions.
- Appraisal.
- Notices and other written communications.
- Staff Meetings/Team briefings.
- Verbal instructions.
- Custom and practice.

Policies and Procedures

- Crescent Community Care Services Limited is charged with keeping these up to date and for distributing amendments as and when necessary.
- Team leaders/supervisors are responsible for explaining procedures to be followed when operating equipment, completing documentation etc to all other members of staff
- General guidance on health and safety matters will be provided to employees by their manager who is also responsible for supplying information to employees on safe working practices and particular hazards in their own departments. This will either be by updates on training or self study with documentation sent with the memos.
- Any queries employees may have relating to the work they are required to do should be raised with their supervisor or manager.
- Regular departmental meetings will be held to discuss issues connected with the smooth operation of the department. Arrangements will be made for as many staff to be on duty for such meetings as is possible; staff who attend out of their normal working hours will not be paid for the attendance unless agreed in advance at the manager's discretion.
- Team members are encouraged to make suggestions which they believe may assist in improving efficiency and promoting job satisfaction.
- There will be a Quality Meeting, at least once per quarter in line with our ISO9001 Registration. Any changes will be cascaded down to staff to ensure best practice
- Most areas of work are subject to QCF Diploma standards of working, and where this is the case, these are the standard of performance required. Arrangements will be made for staff in these areas to become knowledgeable about and be trained to

these standards.

- Notices will be used where information is not critically important.
- Where the controlled giving and receiving of information is important, a meeting will be used. Meeting times will be issued by managers who will ensure that all members of their teams are read the information in the briefing word for word, and face to face. Feedback is encouraged.
- Overall, where possible, verbal communication is preferred, as it allows a two way communication process, which is far more effective. Where the information is intended to be acted on over a period of time, verbal communication should always be confirmed with a permanent written record.

Quality Related Information

- Information about the overall performance of the establishment, or individual work areas if this level of information is available, will be distributed to all staff reasonably soon after it becomes available. Examples include:
    - Internal audit reports.
    - Responsible Individual reports (where applicable).
    - Periodic reports from the CQC
    - ISO9001 Inspection reports.
    - External audit reports.
- The Quality Policy and Procedure, Quality Meetings Policy and Procedure and the Management Meeting Policy and Procedure each describe the processes by which staff can feed back on the contents of the various quality reporting in the home or agency, and become involved in the continuous improvement programme covering all services provided by the organisation.

Organisation News

- Regular meetings will be held at which the Registered Provider will brief Registered Managers on the organisation's current position and future prospects.
- In turn, Registered Managers have a duty to pass on appropriate information to their team members in the way most suited to their operational requirements, e.g. general meeting, team briefing, etc.
- Suggestions and comments raised by employees should be fully considered and passed to higher authority where appropriate.
- While there is no ideal substitute for providing and receiving information in a face-to-face context, back-up is provided by the organisation in the form of newsletters and other material (see below).
- A memo, distributed to employees, will be produced on an ad-hoc basis, and all employees are encouraged to contribute material to the memo where necessary.
- Important news items about the organisation which are of immediate interest to all members of staff will be issued via a team briefing note, which will be communicated to all team members verbally within a specified time.
- All information given out by the organisation and its managers should be regarded as commercially sensitive, and only material clearly indicated as non-confidential may be disclosed to members of the public.
- Any queries regarding the information contained in any communications should be raised in the first instance with the departmental manager.
- All employees are reminded that, except for information freely available to members of the general public, no details of the organisation's operations should be disclosed to outsiders without the prior consent of the manager or Registered Provider.

Media Queries

- Any query received from the media must be passed immediately to the nominated manager or the Directors. Employees must not attempt to deal with the query themselves.

Note

- A European directive gives employees in the UK rights to information and consultation. The directive gives employees the right to be informed about the businesses' economic situation and to be informed and consulted about employment prospects and about decisions which may lead to substantial changes in work organisation or contractual relations, including redundancies and transfers. The directive applies to businesses with 50 or more employees. The directive does not apply to businesses with fewer than 50 employees.

Reviewed by Marcus Kerridge-McColl

September 2025

Complaints, Suggestions and Compliments Policy and Procedure
Crescent Community Care Services limited

Purpose

- The Registered Provider operates an effective mechanism for the receipt, recording, investigation and resolution of all complaints, in order to comply with the regulations.
- The arrangements for investigation of complaints are fair and transparent.
- Complaints and suggestions from Service Users or their relatives are a valued source of information regarding the quality of our service, and are a primary source of information regarding possible abuse.
- Care Quality Commission and Local Government Ombudsman guidelines are adhered to.

Scope

- Service Users.
- Relatives.
- Other professionals outside agencies.
- All employees.

Policy

- In all cases complaints and concerns shall be treated seriously in a sensitive and confidential manner.
- Complaints and suggestions must be handled in such a way as to first of all reach a satisfactory outcome with the complainant, and to turn a potentially difficult and damaging problem into a source of quality improvement.
- A copy of this complaints procedure will be given to all Service Users and their representatives at the beginning of the service, and copies will also be made available throughout the service.
- All formal or serious complaints will be investigated by a person not related to the immediate source of the complaint.
- The recording of complaints will not be confined to “serious” or “substantial” complaints. The existence of records for complaints of an apparently minor nature is an indication of the effectiveness of the procedure, the openness of the culture of the organisation and its employees, and their vigilance in the area of abuse.
- Complaints will be recorded on Service Users’ files in order to identify any pattern of complaints relating to an individual, including Care or service provision in order to update and review the Care Planning process.
- Complaints will be recorded centrally in order to identify any pattern of complaint relating to all or a group of Service Users. This record will contain minor complaints in addition to serious complaints, and will be accessible to all members of staff where appropriate, unless this is a safeguarding issue. In order for this to be established, members of staff are to make appropriate entries, in a timely fashion, to Care Planning or risk assessments.
- The central information, with regards to complaints, suggestions and compliments, will be regularly reviewed and analysed. The summary will be regularly considered by the Management Meeting for quality assurance purposes.
- Compliments will be recorded centrally and made available for all parties to read, also on the personnel file of any member of staff individually complimented.
- Employees who are the subject of a complaint should not communicate directly with the complainant unless accompanied by a senior member of staff, unless requested directly to do so by the complainant.
- Where the complaint gives rise to concerns regarding the wellbeing of one or more Service Users, serious consideration must be given to suspension of the person or persons complained about, and an investigation must be initiated immediately in order to identify any risk to the health and welfare of the Service User involved.

Procedure

Standard Arrangements

- There are several distinct levels of dealing with a complaint, and it is important for the speedy and effective resolution that

each level is followed.

- The principles applied are:
    - The nearer the person dealing with the complaint is to direct service delivery, the better the likely outcome of the complaint. That person has a better detailed knowledge of the service and can react quickly and appropriately. An exception to this principle will be made in the case of a complaint which alleges abuse, in which case the complaint will be immediately and directly reported to a senior manager. At this point the safeguarding policy should be followed as per local authority advice and the necessary notification made to the CQC.
    - Accepting that personalities can be a factor in complaints, the multiple stages allow this problem to be avoided.
    - The complaints process will only be regarded as "completed" when the complainant or their representative has indicated, in writing if possible, that they are satisfied with the outcome of the complaint procedure.
    - Complaints and suggestions will in all cases be taken seriously, recorded, their practicality/usefulness investigated, and the instigator informed of the decided outcome.
- Appoint a complaints manager within the business that will be responsible for dealing with all complaints. It could be the Registered Manager, a Care Manager or a HR Manager. Within smaller businesses, it is very likely that the service provider him/herself will be the designated complaints manager. Whoever is appointed, they must be in a senior position within the business.
- The Agency must be able to respond to complaints both by email and in writing. Telephone conversations are not sufficient in themselves, as they do not constitute a proper record.
- It is helpful to have a dedicated email address and inbox for managing the complaints and ensure that someone, preferably the complaints manager, reviews the inbox on a daily basis.
- Update the complainant in writing about your progress during the investigation.
- The complaint procedure must be publicly available. It must be:
    - On your website;
    - Clearly visible in public areas of your registered address;
    - Sent out with all contracts for Care, and;
    - Included within all Service User Guides.
- Investigations and outcomes will be recorded on the complaints form, adding additional sheets as required.
- All employees are warned that written complaints recording rules must be complied with, and those records held where they are freely available to supervisors and managers. Any attempt to conceal a complaint may give rise to formal disciplinary action.
- The complainant will be requested to examine the written records of the complaint and sign to indicate agreement with the outcome.
- In the event of a continued disagreement which cannot be resolved internally, the complainant will be advised to approach an appropriate external authority, such as the CQC, funding authorities such as Social Services or NHS, an independent advocacy service, or the local government Ombudsman.
- For privately funded Service Users a range of advocacy services are available. The Registered Manager should support the Service User to contact an appropriate independent advocate if the Service User shows any signs of being unable to fully make, or further pursue, the complaint.
- The completed complaints form will then be handed to the Registered Manager for permanent filing, centrally and on the Service User file.
- The Management Meeting will periodically (recommended every three months) review all complaints since the previous review in order to identify trends and matters which may have appeared to be relatively minor at the time, but which indicate a deeper problem.
- The services action plan should be updated to include all actions to be taken to resolve any requirements or recommendations made following any investigation.

Written Procedure

- A complaint can be made: by telephone; in writing; by email; or in person. All responses will be made/followed up in writing (preferably email).
- Complaints can be made to:

  Crescent Community Care Services Limited

3 Festing Buildings, Highland Road, Southsea, Hampshire, PO4 9DA
02392 738398

info@crescentcare.co.uk

- Complainants must receive an acknowledgment within 24 hours. In this acknowledgment let them know who will be investigating the complaint.
- Set a time limit for the acceptance and investigation of complaints. 12 months is appropriate. Once established you can state that:
  - A complaint must be made no more than 12 months after:
    - The date the event occurred, or if later,
    - The date the event came to the notice of the complainant.
  - The time limit will not apply if Crescent Community Care Services Limited is satisfied that:
    - The complainant can give a good reason for not making the complaint within that time limit, and;
    - Despite the delay, it is still possible to investigate the complaint effectively and fairly.
- State also how you will deal with anonymous complaints.
- All complaints will be dealt with by a senior person within the Care organisation.
- Complainants will receive (as far as reasonably practical):
  - Assistance to help them understand the complaints procedure; and
  - Advice on where they may obtain such assistance.
- The notice board at your registered address (or other prominent public place) should include a display of advocacy services with contact details. It needs to be regularly reviewed, to ensure that advocacy services contact details are up to date.
- Crescent Community Care Services Limited will only accept complaints from a third party under certain conditions:
  - Either:
    - Where you know that the Service User has consented, either verbally or in writing, or;
    - Where then Service User cannot complain unaided and cannot give consent because they lack capacity within the meaning of the Mental Capacity Act 2005; and
    - The representative is acting in the Service User's best interests – For example, where the matter complained about, if true, would be detrimental to the Service User.
- The procedure must be available, upon request, in other languages and formats. You only need to prepare a special format on request to keep costs low.
- You should state your expected turnaround/resolution time. This should be 28 days, but be realistic advising longer where necessary.
- All complaint investigations should be completed within 6 months at the latest, unless a different time period has been agreed. This should only be done when there is a good reason for it.
- Complainants have the right to refer their complaint to the Local Government Ombudsman if they are unhappy with the outcome of the investigation. The complainant also has the right to alert the Care Quality Commission.
- Suggested wording:

  "Once your complaint has been fully dealt with by Crescent Community Care Services Limited, if you are not satisfied with the outcome you can complain to the Local Government Ombudsman (LGO). The LGO provides a free, independent service. You can contact the LGO Advice Team for information and advice, or to register your complaint:

  T: 0300 061 0614

  E: advice@lgo.org.uk

  W: www.lgo.org.uk

  The LGO will not usually investigate a complaint until the provider has had an opportunity to respond and resolve matters.

  Our service is registered with and regulated by the CQC. The CQC cannot get involved in individual complaints about providers, but is happy to receive information about our services at any time. You can contact the CQC at:

  Care Quality Commission (CQC)
  National Correspondence
  Citygate, Gallowgate

Newcastle upon Tyne NE1 4PA
Tel: 03000 616161
Fax: 03000 616171

03000 616161

Monitoring

- Record the following information on a complaints log:
  - Each complaint received;
  - The subject matter and outcome of each complaint;
  - Details of the reasons for delay where an investigation took longer than the agreed response period agreed; and
  - The date the report of the outcome of the investigation was sent to the complainant.

Annual Reports

- Each provider must prepare an annual report for each year in which it must:
  - Specify the number of complaints received.
  - Specify the number of complaints that the provider decided were well-founded, partly or fully.
  - Specify the number of complaints that the provider has been informed have been referred to other bodies.
  - Give the subject matter of complaints received.
  - Summarise any matters of importance in those complaints themselves or in the way that the complaints were handled.
  - Summarise any matters where action has been or is to be taken to improve services as a consequence of those complaints.
  - A "year" means a period of 12 months ending on 31st March.
  - Providers must ensure that its annual complaint report is available to anyone on request.

Acknowledgement Letters

- The acknowledgement letter should include an offer to discuss the complaint with the complainant at a mutually agreed time, to go over:
  - The manner in which the complaint is to be handled; and
  - The period (response period) within which the investigation of the complaint is likely to be completed.
  - If the complainant does not accept the offer of a discussion, the provider must determine the response period (which should always be within 6 months) and notify the complainant in writing of that period.
  - The acknowledgement can be sent by letter or email.
  - Take account of the complainant's preferences when communicating with him or her.

Final Response Letter

- The final response letter must include a report giving:
  - A detailed explanation of how the complaint has been considered;
  - The conclusions reached, including any remedial action needed; and confirmation that any action needed has either already been taken or, if not yet taken, the proposed timescale when such action will be completed.
  - The letter must inform complainants of their right to take their complaint to the LGO if they are not happy with the outcome.
  - The final letters should be signed by the "responsible individual" or sent by email in their name.
- If the response is not ready within 6 months, you should:
  - Notify the complainant in writing accordingly and explain the reason why; and
  - Send the complainant in writing a response in accordance with the above as soon as reasonably practicable after 6 months.

General Information for Staff Managing Complaints

People who can complain

- A complaint can be made by:

- Someone who receives or has received Care services;
- Someone who is affected (or likely to be affected) by the action, omission or decision of the provider who is the subject of the complaint; or
- A representative of either of these, under certain conditions.
- If a provider is not satisfied that the representative is acting with the Service User's consent or in their best interests, the provider must notify the representative in writing, and state the reason for its decision.

Complaints you do not have to investigate

- Crescent Community Care Services Limited is not required to investigate the following complaints:
  - A complaint by an employee relating to their employment (you should handle this in a different way, for example through your grievance procedure);
  - A complaint that was made in person or by telephone and is resolved to the complainant's satisfaction no later than the next working day after the day the complaint was made; and
  - A complaint that has already been investigated and resolved.
  - In these circumstances, the provider will, as soon as is reasonably practicable, notify the complainant in writing of its decision to not investigate the complaint and the reasons why. It would be best practice to have a standard letter for this purpose.

Duty to co-operate

- If a complaint involves more than one provider/commissioner of services there is a duty on local authorities and the NHS to co-operate and provide a single response. Every provider must work with CCGs or local authorities to provide single response to complaints.

Key Contacts:

Care Quality Commission (CQC)
National Correspondence
Citygate, Gallowgate
Newcastle upon Tyne NE1 4PA
Tel: 03000 616161
Fax: 03000 616171

Social services (for Service Users funded by social services)
0300 555 1386

Portsmouth City Council ASC Department
Tel: 02392 822251

The Local Government Ombudsman
PO Box 4771
Coventry. CV4 0EH
Tel: 0845 602 1983 or 024 7682 1960
Fax: 024 7682 0001
advice@lgo.org.uk

Independent advocacy services

Choices Advocacy,

The Warren Centre Plus,

Warren Crescent,

Shirley Warren,

Southampton S016 6AY

Reviewed Marcus Kerridge-McColl

December 2025

Confidentiality Policy and Procedure

Purpose

- To ensure that all information is collected, recorded, stored, shared and disposed of in the best interests of the Service User, with regard for their human rights and in line with legislation.

- To ensure the Service User is aware of the organisation's confidentiality policy and procedure.

Scope

- All workers and third parties involved in the care of Service Users.

Policy

- The Service User's rights to confidentiality must be safeguarded in accordance with the common law duty of confidentiality and Article 8 of the Human Rights Act 1998. The Care service employee will use information only in the best interests of the Service User and where possible with their express consent. All information, verbal, written, electronic or photographic / video / audio recording is managed in line with the Data Protection Act 1998 and The Access to Health Records Act 1990.

Procedure

Data Collection and Storage

- Staff must ensure that any information collected and stored is limited to that which is essential for the care and safeguarding of the Service User. This relates to personal data and health information essential to establish individual needs and provide staff with the means to plan care and support. Staff must retain information and data securely in locked storage when not in use. Electronic data should be password protected and access limited to essential users only. Photographs and video or sound recordings should not be shared without the express consent of Service Users or their legal advocates.

Disclosure

- Data and information should only be shared with those identified as eligible to access it. This must only be shared for the purposes of ensuring individual needs are met and for the safe delivery of care. So information passed to a care worker may be shared with members of the care team where they are concerned with the care and treatment of the Service User. Staff must be advised of the requirement to confirm the identity and right of access of any third party seeking information about a Service User or The Agency in writing, by telephone or in person before any disclosure is made.

- Consent to disclose information with family members should be obtained from the Service User. There are many good reasons why informal carers should be informed and involved with the professional care of their loved one. If family members wish to pass on their concerns or views to a member of care staff this does not constitute a breach of confidentiality by the care staff.

Consent

- As far as reasonably practicable, written consent to the use of personal information should be gained from each Service User or their advocate. Inform the subject when gaining consent of the specific details of the information/action to be shared and with whom. This should form part of the information provided at the outset of admission to the service.

- If the Service User lacks the mental capacity to consent to information being shared, staff may share information if it is in the Service User's best interests (in accordance with the principles of the Mental Capacity Act 2005). Staff should still clearly explain to the Service User the reasons for the decision to share information and accurately record these.

- Where a Service User has appointed a Lasting Power of Attorney under the Mental Capacity Act 2005 that person should be consulted where information is to be shared in someone's best interests.

- If there are any concerns in these areas you should seek advice from your supervisor.

- Access to data and records must be managed in line with legislation. Permission to access health information may be sought by Service Users or their advocates in writing and access must be managed appropriately. It is good practice to involve Service Users with the collection and recording of personal data as this ensures focus on the needs and rights of each individual. This paragraph should be read in conjunction with the Access to Information Policy and Procedure.

- The use of social media should be avoided unless strict management of information is assured to protect the rights of all individuals in our care. Staff must not discuss confidential information about Service Users in any place where third parties might access it, such as on an outing.

- Staff must ensure that all data and information held pertains only to that individual. Care must be taken to limit recording to identify only the Service User whose record it is, and not to include sensitive identifiable data about others in Care Plans and record entries.

- Confidentiality with respect to a Service User may only be breached if:

  - Where information is required by statute or court order.

  - Information suggests the Service User is at risk of harm to themselves or from others, or others may be at risk of harm from the Service User, and where the passing on of information would be in the person's interest, or the wider public interest.

  - Information, if withheld, could put others at risk; and where the passing on of information would be in the public interest.

  - Where the Service User may have broken the criminal law and where there is justification in the public interest to breach confidentiality, Section 115 of the Crime and Disorder Act 1998 gives a power (but not an automatic duty) to organisations to disclose information to the police 'for the prevention, detection and reduction of crime'. This applies in England and Wales and in Scotland (where the Crime and Disorder Act is amended by the Criminal Justice (Scotland) Act 2003).

Disposal of Confidential Information

- In the event of a Service User's death or leaving the service, records should be removed from circulation and retained in secure archive storage. There is a requirement to retain care and clinical records for not less than 3 years in line with Part 3 section 17 of The Care Homes Regulations 2001. It is good practice to retain records for up to 20 years to meet insurance requirements.

- When confidential information no longer needs to be stored, destruction of paper records must be by shredding to ensure no risk of third parties accessing sensitive data. Destruction of electronic records by deletion must be undertaken by means that ensure no future retrieval is possible.

Reviewed by Marcus Kerridge McColl
November 2025

CQC Fundamental Standards Policy and Procedure
Crescent Community Care Services

Purpose

- To support and inform compliance with the 11 Fundamental Standards set out in the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014.

Scope

- All activities of the Service.

Policy

- Crescent Community Care Services Ltd will, by following the Quality Compliance Systems Ltd (QCS) compliance management system, meet the requirements of the Regulation, and in particular the 11 Fundamental Standards of care contained in the Regulations.

Relationships between the Regulations, Fundamental Standards, Key Questions and Key Lines of Enquiry.

- The 11 Fundamental Standards are listed in Appendix A. These are the legal requirements placed on all Providers in England.

- While the CQC registration checks and inspections are intended to check the compliance of the Service with the Fundamental Standards, the methodology of doing so is not to inspect against the Standards, but carry out the check by asking 5 Key Questions.

- The 5 Key Questions are expanded into the detail required for inspection through the Key Lines of Enquiry (KLOEs).

- The KLOEs are in turn expanded into Prompts, which guide the CQC inspector in detail during the inspection.

- The complete set of KLOEs flowing from the 5 Key Questions contain all of the requirements set out in the 11 Fundamental Standards in the Regulations.

- Therefore meeting the requirements of the KLOEs also satisfies the need to meet the Fundamental Standards.

- The structure of the cross-referencing between Fundamental Standards, Key Questions and KLOEs is tabulated in Appendix B.

Because the KLOEs resulting from the 5 Key Questions exactly map the Fundamental Standards, and the CQC base inspections on the Key Questions, QCS has structured its compliance support on the 5 Key Questions.

The structure of the QCS compliance support policies is:

- One CQC Fundamental Standards Policy and Procedure (this policy), setting out the overall legal requirements, and how these are met by the QCS compliance management system.

- Five Key Questions Policies, setting out the statement of the standard to be met in relation to that question, listing the KLOEs which support the standard, and the QCS policies which define the activity which supports the KLOEs.

- One Policy for each KLOE, listing the definition of the KLOE, the QCS policies which have a direct relationship to meeting the KLOE (the Primary Policies), and the QCS policies which have a supporting relationship to the KLOE.

- Each KLOE policy also includes an explanation of the policies, documents, activities and direct observational evidence which the CQC inspector may examine in order to determine if the KLOE is being met.

- Each KLOE policy also includes a checklist of evidence to support the Service to check if it is meeting that particular KLOE.

Procedure

QCS recommends that Crescent Community Care Services Ltd carries out training with all employees to make them aware of their responsibilities in meeting the Fundamental Standards. The training should include:

- A brief explanation of the structure of the new requirements as laid out in the Relationships section above should be used to demonstrate that successful compliance with the Standards will be achieved by fully carrying out all of the policies contained in the QCS compliance management system.

- Use of the extensive lists of QCS policies in each KLOE policy to demonstrate to employees, including the Provider and Senior Management, the interdependence of the full suite of QCS policies. Confirm that cherry picking certain policies and ignoring others, for whatever reason, will place Crescent Community Care Services Ltd in danger of being found to be not meeting some of the legal requirements.

Appendix A.

The Health and Social Care Act 2008 (Regulated Activities) Regulations 2014.

Sections 4 to 14, the "Fundamental Standards"

General

3. A registered person must, in so far as they are applicable to each regulated activity, comply with regulations 4 to 14 in carrying on the regulated activities in respect of which they are registered.

SECTION 2

Fundamental standards

Person-centred care

4.—(1) The care and treatment of service users must reflect their needs and preferences.

(2) Paragraph (1) applies—

(a) subject to paragraph (4), and

(b) in so far as it is in accordance with generally accepted professional standards, practices and principles for the care and treatment to reflect such needs and preferences.

(3) The things which a registered person is required to do to comply with paragraph (1) include—

(a) carrying out an assessment of the needs and preferences for care and treatment of the service user;

(b) designing care or treatment with a view to ensuring service users' welfare;

(c) so far as is reasonably practicable, permitting and encouraging service users to make, or participate in making, decisions relating to their care or treatment to the maximum extent possible;

(d) where the service user is unable to express their preferences because they lack capacity, determining and acting in accordance with the best interests of the service user;

(e) where applicable, providing for the making of reasonable adjustments to meet the service user's individual needs; and

(f) have sufficient quantities of suitably accessible equipment and medicines to ensure the safety of service users and to meet their assessed needs.

(4) Where Part 4 or 4A of the Mental Health Act 1983(17) applies to a service user, care and treatment must be provided in accordance with the provisions of that Act.

(5) For the purposes of paragraph (3)(d)—

(a) section 2 of the 2005 Act (people who lack capacity) applies for the purposes of determining whether a service user lacks capacity, and

(b) section 4 of the 2005 Act (best interests) applies as it applies for the purposes of that Act.

Dignity and respect

5.—(1) Service users must be treated with dignity and respect.

(2) The things which a registered person is required to do to comply with paragraph (1) include—

(a) promoting the privacy, autonomy, independence and involvement in the community of the service user; and

(b) having due regard to any protected characteristics (as laid down in section 4 of the Equality Act 2010(18)) of the service user.

Need for consent

6.—(1) Care and treatment must only be provided with, and in accordance with, the consent of the relevant person.

(2) Where the relevant person is unable to give such consent because they lack capacity, the registered person must determine and act in accordance with the best interests of the service user.

(3) Where Part 4 or 4A of the Mental Health Act 1983 applies to a service user, paragraph (1) is subject to the provisions of that Act.

(4) For the purposes of paragraph (2)—

(a) section 2 of the 2005 Act applies for the purposes of determining whether a service user lacks capacity, and

(b) section 4 of the 2005 Act (best interests) applies as it applies for the purposes of that Act.

Safe and appropriate care and treatment

7.—(1) All care and treatment provided to service users must be appropriate and safe.

(2) The things which a registered person is required to do to comply with paragraph (1) include—

(a) taking appropriate steps to mitigate the risks to service users of receiving care or treatment that is inappropriate or unsafe;

(b) taking appropriate steps for the proper and safe management of medicines;

(c) establishing and operating effectively systems designed to assess the risk of, and to prevent, detect and control the spread of, infections, including those that are health care associated; and

(d) where responsibility for the care and treatment of service users is shared with, or transferred to, other persons, working in cooperation with such other persons to ensure that appropriate care planning takes place to ensure the health, safety and welfare of the service users.

(3) For the purposes of paragraph (1), care or treatment will not be appropriate if it is not provided in accordance with generally accepted professional standards, practices and principles or if it includes—

(a) unlawful discrimination against service users, or

(b) control or restraint which is—

(i) unlawful, or

(ii) not necessary to prevent, or not proportionate to, the risk posed if the service user was not subject to control or restraint.

Safeguarding service users from abuse

8.—(1) Service users must not be subject to abuse. (2) Appropriate steps must be taken—

(i) to prevent abuse before it occurs, and

(ii) to respond to any allegation of abuse.

(3) For the purposes of this regulation, “abuse”, in relation to a service user, includes—

(a) sexual abuse,

(b) physical or psychological ill-treatment,

(c) theft, misuse or misappropriation of money or property, or

(d) neglect and acts of omission which cause harm to a service user or place the service user at risk of harm.

Meeting nutritional needs

9.—(1) The nutritional needs of service users must be met.

(2) In this regulation, “nutritional needs” means adequate nutrition to sustain life and good health and includes—

(a) those needs which can be met wholly or in part by the provision of food or drink,

(b) parenteral nutrition and dietary supplements when prescribed by a healthcare professional,

(c) any reasonable requirements arising from a service user's religious or cultural background, and

(d) where necessary, support to eat and drink.

Cleanliness, safety and suitability of premises and equipment

10.—(1) All premises and equipment used by the registered provider must, in accordance with generally accepted professional standards, practices and principles, be—

(a) safe, clean and secure,

(b) suitable for the purpose for which they are being used, and (c) properly used and maintained.

(2) The registered provider must, in relation to such premises and equipment, maintain standards of hygiene appropriate for the purpose for which they are being used.

(3) For the purposes of this regulation "premises" does not include a service user's own home

Receiving and acting on complaints

11.—(1) Any complaint received must be appropriately investigated and appropriate action must be taken in response to the complaint.

(2) The registered person must establish and operate effectively a system for identifying, receiving, recording, handling and responding appropriately to complaints, and requests for action to be taken, by service users and other persons in relation to the carrying on of the regulated activity.

(3) The registered person must send to the Commission, when requested to do so, a summary of—

(a) complaints made under such complaints system,

(b) responses made by the registered person to such complaints and any further correspondence with the complainants in relation to such complaints, and

(c) any other relevant information in relation to such complaints as the Commission may request.

Good governance

12.—(1) Systems or processes must be established and operated effectively to ensure compliance with the requirements in this Part.

(2) Such systems or processes must enable the registered person, in particular, to—

(a) assess, monitor and improve the quality of the services provided (including the quality of the experience of service users in receiving those services);

(b) assess, monitor and mitigate the risks relating to the health, safety and welfare of service users and others who may be at risk which arise from the carrying on of the regulated activity;

(c) maintain an accurate, complete and contemporaneous record in respect of each service user, including a record of the care and treatment provided to the service user;

(d) maintain such other records as are appropriate in relation to—

(i) persons employed, and

(ii) the management of the regulated activity;

(e) seek and act on feedback from service users and other persons on the services provided in the carrying on of the regulated activity, for the purposes of continually evaluating and improving such services; and

(f) evaluate and improve their practice in respect of the processing of the information referred to in sub- paragraphs (a) to (e).

Staffing

13.—(1) The registered person must deploy sufficient numbers of suitably qualified, skilled and experienced persons in order to meet the requirements of this Part.

(2) The registered person must ensure that persons employed by the registered provider in the provision of a regulated activity—

(a) receive appropriate support, training, professional development, supervision and appraisal;

(b) are enabled where appropriate, from time to time, to obtain further qualifications appropriate to the work they perform; and

(c) where such persons are healthcare professionals, or social workers or other professionals registered with the Health and Care Professions Council, are enabled to provide evidence to their relevant professional body demonstrating, where it is possible to do so, that they continue to meet the professional standards which are a condition of their ability to practice.

Fit and proper persons employed

14.—(1) Persons employed by the registered person must—

(a) be of good character,

(b) have the qualifications, skills and experience which are necessary for the work to be performed, and (c) be physically and mentally capable, after reasonable adjustments are made, of properly performing tasks which are intrinsic to the work for which they are employed.

(2) Recruitment procedures must be established and operated effectively to ensure that persons employed meet the conditions in paragraph (1).

(3) The information specified in the Schedule and such other information as is appropriate must be available in relation to each such person employed.

(4) Persons employed must be registered with the relevant professional body where such registration is required by, or under, any enactment in relation to—

(a) the work that the person is to perform, or

(b) the title that the person takes or uses.

(5) Appropriate steps must be taken in relation to a person employed by the registered person who no longer meets the criteria in paragraph (1), including—

(a) where the person is a healthcare professional, informing the body responsible for regulation of the health care profession in question, or

(b) where the person is a social worker or other professional registered with the Health and Care Professions Council, informing the Council.

Reviewed by Marcus Kerridge McColl
November 2025

Cross Contamination between Care and Food Preparation Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards. Š
- To prevent cross-contamination.

Scope

- All workers.

Policy

- Clothing and personal hygiene will be managed in order to minimise the risk of cross-contamination between Care and food preparation activities.

Procedure

- Care staff who are carrying out activities which may lead to contamination of food must wear barrier clothing (e.g. disposable apron) when carrying out care, and that clothing must be removed before entering the food preparation area.
- Care staff may enter food preparation areas for food preparation purposes only.

Reviewed by Marcus Kerridge McColl
December 2025

Data Security Breach Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To identify potential data security breach risks, specify safeguards, and state information handling in the event of a breach.
- To meet the seventh data protection principle of the Information Commissioner's Office (ICO), which is:
    - "Appropriate technical and organisational measures shall be taken against unauthorised or unlawful processing of personal data and against accidental loss or destruction of, or damage to, personal data."

Scope

- All data relating to Crescent Community Care Services Limited, its Service Users and employees held within the organisation or its associates, however stored.
- For obligations under the Data Protection Act also refer also Access to Information Policy and Procedure.

Policy

- Crescent Community Care Services Limited, as an organisation which processes personal data, must take appropriate measures against unauthorised or unlawful processing and against accidental loss, destruction of or damage to personal data.
- A data security breach can happen for a number of reasons:
    - Loss or theft of data or equipment on which data is stored;
    - Inappropriate access controls allowing unauthorised use;
    - Equipment failure;
    - Human error;
    - Unforeseen circumstances such as a fire or flood;

- Hacking attack;
- 'Blagging' offences where information is obtained by deceiving the organisation who holds it.

Procedure

- If breach has occurred, there are four important elements to any breach management plan:
  - Containment and recovery;
  - Assessment of ongoing risk;
  - Notification of breach;
  - Evaluation and response.

Containment and Recovery

- Data security breaches will require not just an initial response to investigate and contain the situation but also a recovery plan including, where necessary, damage limitation. This will often involve input from specialists across the business such as IT, HR and legal and in some cases contact with external stakeholders and suppliers. Consider the following:
  - Decide on who should take the lead on investigating the breach and ensure they have the appropriate resources.
  - Establish who needs to be made aware of the breach and inform them of what they are expected to do to assist in the containment exercise. This could be isolating or closing a compromised section of the network, finding a lost piece of equipment or simply changing the access codes at the front door.
  - Establish whether there is anything you can do to recover any losses and limit the damage the breach can cause. As well as the physical recovery of equipment, this could involve the use of backup media to restore lost or damaged data or ensuring that staff recognise when someone tries to use stolen data to access accounts.
  - Where appropriate, inform the police.

Assessing the Risks

- Some data security breaches will not lead to risks beyond possible inconvenience to those who need the data to do their job. An example might be where a laptop is irreparably damaged but its files were backed up and can be recovered, albeit at some cost to the business. While these types of incidents can still have significant consequences the risks are very different from those posed by, for example, the theft of an employee database, the data on which may be used to commit identity fraud. Before deciding on what steps are necessary, further to immediate containment, assess the risks which may be associated with the breach. Perhaps most important is an assessment of potential adverse consequences for individuals, how serious or substantial these are and how likely they are to happen.
- The following points are also likely to be helpful in making this assessment:
  - What type of data is involved?
  - How sensitive is it? Remember that some data is sensitive because of its very personal nature (health records) while other data types are sensitive because of what might happen if it is misused (bank account details).
  - If data has been lost or stolen, are there any protections in place such as encryption?
  - What has happened to the data? If data has been stolen, it could be used for purposes which are harmful to the individuals to whom the data relate; if it has been damaged, this poses a different type and level of risk.
  - Regardless of what has happened to the data, what could the data tell a third party about the individual? Sensitive data could mean very little to an opportunistic laptop thief while the loss of apparently trivial snippets of information could help a determined fraudster build up a detailed picture of other people.
  - How many individuals' personal data are affected by the breach? It is not necessarily the case that the bigger risks will accrue from the loss of large amounts of data but is certainly an important determining factor in the overall risk assessment
  - Who are the individuals whose data has been breached? Whether they are staff, customers, clients or suppliers, for example, will to some extent determine the level of risk posed by the breach and, therefore, your actions in attempting to mitigate those risks.
  - What harm can come to those individuals? Are there risks to physical safety or reputation, of financial loss or a combination of these and other aspects of their life?
  - Are there wider consequences to consider such as a risk to public health or loss of public confidence in an important service you provide?
  - If individuals' bank details have been lost, consider contacting the banks themselves for advice on anything they can do to help you prevent fraudulent use.

Notification of Breaches

- Informing people and organisations that you have experienced a data security breach can be an important element in your breach management strategy. However, informing people about a breach is not an end in itself.

- Notification should have a clear purpose, whether this is to enable individuals who may have been affected to take steps to protect themselves or to allow the appropriate regulatory bodies to perform their functions, provide advice and deal with complaints.
- From 26 May 2011 certain organisations (service providers) have a requirement to notify the Information Commissioner (ICO), and in some cases individuals themselves, of personal data security breaches.
- For more information about the specific breach notification requirements for service providers see ICO on the internet
- Answering the following questions will assist other types of organisations in deciding whether to notify:
    - Are there any legal or contractual requirements? Service providers have an obligation to notify the Commissioner in certain circumstances. Health and Social Care providers will have a legal responsibility to notify their Regulator of breaches, and may have a contractual obligation to notify commissioners of services, such as Social Services or NHS.
    - Can notification help you meet your security obligations with regard to the seventh data protection principle? This is "Appropriate technical and organisational measures shall be taken against unauthorised or unlawful processing of personal data and against accidental loss or destruction of, or damage to, personal data."
    - Can notification help the individual? Bearing in mind the potential effects of the breach, could individuals act on the information you provide to mitigate risks, for example by cancelling a credit card or changing a password?
    - If a large number of people are affected, or there are very serious consequences, you should inform the ICO.
    - Consider how notification can be made appropriate for particular groups of individuals, for example, if you are notifying children or vulnerable adults.
    - Have you considered the dangers of 'over notifying'. Not every incident will warrant notification and notifying a whole customer base of an issue affecting only one customer may well cause disproportionate enquiries and work.
- You also need to consider who to notify, what you are going to tell them and how you are going to communicate the message. This will depend to a large extent on the nature of the breach but the following points may be relevant to your decision:
    - Make sure you notify the appropriate regulatory body. A sector specific regulator may require you to notify them of any type of breach but the ICO should only be notified when the breach involves personal data.
    - There are a number of different ways to notify those affected so consider using the most appropriate one. Always bear in mind the security of the medium as well as the urgency of the situation.
    - Your notification should at the very least include a description of how and when the breach occurred and what data was involved. Include details of what you have already done to respond to the risks posed by the breach.
    - When notifying individuals give specific and clear advice on the steps they can take to protect themselves and also what you are willing to do to help them.
    - Provide a way in which they can contact you for further information or to ask you questions about what has occurred – this could be a helpline number or a web page, for example.
    - When notifying the ICO you should also include details of the security measures in place such as encryption and, where appropriate, details of the security procedures you had in place at the time the breach occurred. You should also inform us if the media are aware of the breach so that we can manage any increase in enquiries from the public. When informing the media, it is useful to inform them whether you have contacted the ICO and what action is being taken. ICO will not normally tell the media or other third parties about a breach notified to us, but we may advise you to do so.
- The ICO has produced guidance for organisations on the information we expect to receive as part of a breach notification and on what organisations can expect from us on receipt of their notification.
    - This guidance is available on their website
    - You might also need to consider notifying third parties such as the police, insurers, professional bodies, bank or credit card companies who can assist in reducing the risk of financial loss to individuals, and trade unions.

Evaluation and Response

- It is important not only to investigate the causes of the breach but also to evaluate the effectiveness of your response to it. Clearly, if the breach was caused, even in part, by systemic and ongoing problems, then simply containing the breach and continuing 'business as usual' is not acceptable; similarly, if your response was hampered by inadequate policies or a lack of a clear allocation of responsibility then it is important to review and update these policies and outline responsibility in the light of experience.
- You may find that existing procedures could lead to another breach and you will need to identify where improvements can be made.

- The following points will assist you:
    - Make sure you know what personal data is held and where and how it is stored. Dealing with a data security breach is much easier if you know which data are involved. Your notification with the Information Commissioner will be a useful starting point.
    - Establish where the biggest risks lie. For example, how much sensitive personal data do you hold? Do you store data across the business or is it concentrated in one location?
    - Risks will arise when sharing with or disclosing to others. You should make sure not only that the method of transmission is secure but also that you only share or disclose the minimum amount of data necessary. By doing this, even if a breach occurs, the risks are reduced.
    - Identify weak points in your existing security measures such as the use of portable storage devices or access to public networks.
    - Monitor staff awareness of security issues and look to fill any gaps through training or tailored advice.
    - Consider whether you need to establish a group of technical and nontechnical staff who discuss 'what if' scenarios – this would highlight risks and weaknesses as well as giving staff at different levels the opportunity to suggest solutions.
    - If you have completed the Business Continuity Plan for dealing with serious incidents, consider implementing a similar plan for data security breaches, or incorporating security breaches into the overall Business Continuity Plan. Breach of data security could in some circumstances be serious enough to endanger the business.
    - It is recommended that at the very least you identify a group of people responsible for reacting to reported breaches of security.

Other information

- Additional guidance is also available if you need further information on data security breaches

Reviewed Marcus Kerridge-McColl

September 2025

DBS/Disclosure Policy and Procedure
Crescent Community Care Services Limited

Scope

- All workers who have contact with Service Users, in whatever capacity.

Policy

General principles

- As an organisation we use the DBS service to help assess the suitability of applicants for positions of trust. We comply fully with the DBS Code of Practice regarding the correct handling, use, storage, retention and disposal of Disclosures and information. We also comply fully with its obligations under the Data Protection Act and other relevant legislation pertaining to the safe handling, use, storage, retention and disposal of Disclosure information and have a written policy on these matters. This is available to those who wish to see it on request.
    - Employees covered by the Police Act 1997 are defined as "Care Workers". The official definition is:
    - Definition of a care worker, from the Health and Social Care Act 2008. Care Worker means:
        - An individual who is or has been employed in a position which is such as to enable them to have regular contact in the course of their duties with adults to whom accommodation is provided at a care home or agency;
        - An individual who is or has been employed in a position which is such as to enable them to have regular contact in the course of their duties with adults to whom prescribed services are provided by an independent hospital, an independent clinic, an independent medical agency or a national health service body;
        - An individual who is or has been employed in a position which is concerned with the provision of personal Care in their own homes for persons who by reason of illness, infirmity or disability are unable to provide it for themselves without assistance.
- Care Workers must not be employed unless they have been confirmed as not listed on the DBS Register.

- Care Workers must not have their employment confirmed until a satisfactory Disclosure is received.

- Care Workers must not be employed unless a check of the Nursing and Midwifery Council Register in the case of Registered Nurses proves positive.

- While employees' DBS checks are in progress, the employee will be supervised at all times, and will not have unsupervised access to Service Users. **BEWARE: Some CQC areas insist that new employees are not employed in contact with Service Users until the check has been received and assessed.**

- Disclosure information is never kept on an applicant's personnel file and is always kept separately and securely, in lockable, non-portable, storage containers with access strictly controlled and limited to those who are entitled to see it as part of their duties.

- In accordance with section 124 of the Police Act 1997, Disclosure information is only passed to those who are authorised to receive it in the course of their duties. We maintain a record of all those to whom Disclosures or Disclosure information has been revealed and we recognise that it is a criminal offence to pass this information to anyone who is not entitled to receive it.

- Disclosure information is only used for the specific purpose for which it was requested and for which the applicant's full consent has been given.

- Once a recruitment (or other relevant) decision has been made, we do not keep Disclosure information for any longer than is absolutely necessary. This is generally for a period of up to six months, or immediately following the next **CQC** inspection, whichever is the later, to allow for the consideration and resolution of any disputes or complaints. If, in very exceptional circumstances, it is considered necessary to keep Disclosure information for longer than six-months, we will consult the DBS about this and will give full consideration to the Data Protection and Human Rights of the individual subject before doing so. Throughout this time, the usual conditions regarding safe storage and strictly controlled access will prevail.

- Once the retention period has elapsed, we will ensure that any Disclosure information is immediately suitably destroyed by secure means, i.e. by shredding, pulping or burning. While awaiting destruction, Disclosure information will not be kept in any insecure receptacle (e.g. waste bin or confidential waste sack). We will not keep any photocopy or other image of the Disclosure or any copy or representation of the contents of a Disclosure. However, notwithstanding the above, we may keep a record of the date of issue of a Disclosure, the name of the subject, the type of Disclosure requested, the position for which the Disclosure was requested, the unique reference number of the Disclosure and the details of the recruitment decision taken.

- Disclosure information may not be passed on to a third party, for instance as part of a reference to a subsequent employer.

Procedure

- Employees must register with the DBS direct, or through an "umbrella body". Contact the DBS for details.

- All employees offered a position:

  - Must not be offered a post until a satisfactory DBS check has been received, either together with the Disclosure or the DBS First procedure;
  - If a DBS First check has been received and is satisfactory, the employee may be offered the post subject to confirmation which is subject to receipt of a satisfactory Disclosure DBS Check.
  - The check will be an Enhanced level for those employees having direct access to Service Users, and Standard level for those who do not have direct access. Best practice dictates that Enhanced Level checks are used in all instances, due to the practical difficulties of ensuring that a person subject to only a Standard check never has unsupervised access to Service Users.
  - Be the subject of a Disclosure Application Form to the DBS;
  - Be informed that will they be supervised at all times, and will not have unsupervised access to Service Users. (Or, in those areas where the Care Quality Commission so insist, informed that they may not be employed where they will be in contact with Service Users until the check has been received);
  - Have a member of staff, who has satisfactorily completed the DBS check process attached to them on each shift as a supervisor, and who ensures that the unchecked employee does not have lone one-to-one contact with any vulnerable person, until the check process has been satisfactorily completed.

- On receipt of the Disclosure, the Registered Manager, in consultation with the Registered Provider if necessary, must assess the information and make a decision as to whether or not to confirm the employment. Refer to the section on employment of offenders, below.

  - Note 1: The Registered Provider must always be consulted in the event of a conviction or relevant caution being revealed on a Disclosure, and where it may be proposed to employ the applicant. The decision to employ must rest with the Registered Provider and be explicitly recorded as such.
  - Note 2: The Protection of Freedoms Act 2012 makes provision in the future (date to be announced) for Disclosure to be sent only to the applicant and not the proposed employer, thereby requiring the applicant to supply the document to the Employer. The Employer will have no right to receive the Disclosure directly.

- Other documents and policies which refer, and which must be followed in order to meet the standard, are as follows:

  - DBS Policies and documents attached to this policy;
  - Recruitment – Individual Candidate Application;
  - Induction Policy and Procedure.

- The Registered Provider and Registered Manager will be DBS Checked at the Enhanced Disclosure level via the Care Quality Commission.

- On the 17th June 2013 the DBS launched the "Update Service". By paying an annual subscription applicants can register their Disclosure online and give permission to future employers to check this information online rather than applying for a new Disclosure.

- With permission from the applicant an employer can carry out a free status check providing the check required is of the same level and type.

- Given that the status check does not reveal any new information regarding the applicant since the original Disclosure was registered, the Disclosure can be accepted and used.

- The Update Service allows DBS Disclosures to be portable from employer to employer as well as allowing an existing Disclosure to be checked, with permission, as frequently as an employer deems appropriate.

- More information on the Update Service can be found at: https://www.gov.uk/government/publications/dbs-update-service-employer-guide

Reviewed by Marcus Kerridge McColl
December 2025

Dementia Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To provide person centred and safe Care to those living with dementia.

Scope

- All workers.

- All processes within the service.

Definition

- Dementia is a loss of mental ability severe enough to interfere with normal activities of daily living, lasting more than six months, not present since birth, and not associated with a loss or alteration of consciousness.

Description

- Dementia is a group of symptoms caused by gradual death of brain cells. The loss of cognitive abilities that occurs with dementia leads to impairments in memory, reasoning, planning and behaviour. While the overwhelming number of people with dementia are elderly, dementia is not an inevitable part of ageing; instead, dementia is caused by specific brain diseases. Alzheimer's disease (AD) is the most common cause, followed by vascular or multi-infarct dementia.

- The prevalence of dementia is difficult to determine, partly because of differences in definition among different studies and partly because there is some normal decline in functional ability with age. The prevalence of dementia roughly doubles for every five years of age beginning at age 60. Dementia affects about 1% of people between ages 60 and 64, 5-8% of all people between ages 65 and 74, up to 20% of those between 75 and 84, and between 30% and 50% of those age 85 and older. About 60% of nursing home patients have dementia.

- The cost of dementia can be considerable. While most people with dementia are retired and are not affected by income losses from their disease, the cost of Care is often enormous. Financial burdens include lost wages for family caregivers, medical supplies and drugs, and home modifications to ensure safety. The psychological cost is not as easily quantifiable but can be even more profound. The person with dementia loses control of many of the essential features of his/her life and personality, and loved ones lose a family member even as they continue to cope with the burdens of increasing dependence and unpredictability.

Policy

The service will adhere to the following principles while providing Care for people with dementia:

»Care Planning and delivery will be person centred.

»Care delivery will be by staff who have specialist training in dementia care, and who have access to specialist support.

»Care delivery will focus on meeting needs and aspirations.

»The Agency will promote dignity and respect and maintaining human rights.

»Closely coordinated between different professionals and services across health, social Care and housing.

Procedure

Techniques and environmental changes supporting good quality dementia care:

Risk Assessment

Tools are available throughout the QCS system for personal and environmental risk assessment, and these should be used comprehensively and assiduously when providing dementia Care.

- Within Care Planning, very detailed risk assessment should be carried out in relation to the Service User with dementia and their physical environment. In addition, risk assessment of the potential conflicts between Service Users consequent on their individual behavioural characteristics. For example, a Service User whose wandering behaviour causes them to invade the personal space, or even the belongings, of other Service Users.

- In general, Service Users with dementia are at raised risk of:

  » Abuse;
  » Violent behaviour;
  » Disruptive behaviour;
  » Isolation;
  » Falls;

» Malnutrition;
» Security;
» Tissue viability;
» Fast changing condition;
» Inability to give informed consent;
» Inability to participate in Care Planning; » Inability to express wishes;
» Communication difficulties;
» Depression;
» Accidents.

Life History

- The use of life history (LH) research and recording is especially important in the Care of persons with dementia. A full LH record, made available to all staff providing support to the Service User, enables the carer to more fully understand behaviours of the Service User, and suggest strategies for their management.

- The life history can be obtained from the Service User and their family. It doesn't have to be taken all at once. After you have the initial conversation, they are likely to think of more details to add. It's particularly helpful to provide a form to the family members to take away and complete, that way they can provide thoughtful responses. The information sought after should assist in conversation with the Service User in the months and years to come as well as assist in facilitating activities.

  » Friends and Family - It is important to record the names and details of the Service Users grandparents, parents, brothers and sisters, cousins, children, grandchildren, great grandchildren, nieces and nephews, etc. Focus on details of each that will bring back happy memories for the Service User. Record the ages of children and grandchildren.

  »School and Education.
  »Religious activities.
  »Hobbies.
  »Working Life.

- The QCS Care Plan has a Life History section.

Memory Box

- Accompanying the life history it is useful to ask the family to put together a personal memory box for the individual. The box could contain photos, newspaper cuttings, books, ornaments. If the person was a gardener they could include favourite tools (if not a safety risk). If they were a chef or cooking fanatic they could include cooking utensils, etc. Memories relating to brothers and sisters are also very important. It is often the unexpected that brings pleasure and peace to people who's memories are fading.

ABC Charting

- This is a technique that is widely used. The approach can be defined as:

  » "**A**" - What are the antecedents or triggers of the challenging behaviour? The idea is that all behaviours are triggered by something; they are not random. The trigger could be an environmental issue (too hot, too cold, too noisy?), an unmet need (want the WC, hungry, thirsty?) or a disease (pain, headache, not-well feeling?)

  » "**B**" - Is the challenging behaviour which is causing the problem.

  » "**C**" - Is the consequence of the behaviour, or the reaction of those affected by the behaviour. The way that we react to challenging behaviours can have a large impact on whether that behaviour is more or less likely to re-occur.

  » "**D**" - For De-escalate and Decide. Once the situation has been diffused and calmed down, decide what you can do to prevent a similar situation in the future.

Communication

- Communication techniques are important to the successful outcome of the process. Some tips on effective (and non-effective) communication techniques follow.

Communication Do's:

- Position yourself to maintain eye contact and be at the person's eye level or lower;
- Look at the person directly and make sure that you have their attention before you speak. Always identify yourself first and tell them what you are intending to do;
- Ensure that the tone of voice used is one which conveys respect and dignity. Think about how you communicate, don't just react;
- Use visual clues wherever possible;
- Make sure your expectations of them are realistic. For instance, ask for only one action at a time;
- Watch the person's body language and non-verbal communication and try to interpret it;
- Use a calm and reassuring tone of voice and wherever possible, paraphrase what you just said;
- Always speak slowly, but not patronisingly so, and enunciate your words clearly. If the person is hearing-impaired, manage your communication to overcome or alleviate that;
- Talk about things which are familiar to the person;
- Use touch if that is appropriate.

Communication Don'ts:

- Don't talk to the person as if they were a child or use baby talk;
- Don't use complex words or phrases or long sentences;
- Don't glare at the person you are speaking to or otherwise visually challenge them;
- Don't try to compete with a distracting environment; change the environment or move;
- Don't start to speak without having first said who you are;
- Don't break eye contact while speaking, for instance by going off and doing something in the room while carrying on speaking;
- Don't cause more confusion and confrontation by asking for unrealistic things, such as asking the person to do more than one thing;
- Don't ignore your own body language – be aware of it and manage it positively;
- Don't ramble – keep to the point;
- Don't interrupt the person unless it is absolutely necessary;
- Don't attempt to touch the person, or invade their personal space if they are showing any fear or aggression.

Some techniques for communicating with a person with dementia:

- Ensure that you communicate only in a quiet place that is free from distraction;
- Be aware of the person's language and culture (a consequence of good Care Planning and Life History recording) and take these into account in your communication behaviour;

- Be aware of the person's perception capability, attention span, intellectual level and degree of understanding (again a consequence of good Care Planning), and take that into account when communicating. Stay within the person's perception and understanding boundaries;

- Ensure that the communication is open and conveys respect and trust. Patronising speech or talking to the person with dementia in a child-like way may either foster a sense of helplessness and dependency or trigger an angry and defensive response;

- Pause often, making sure that the person has an opportunity to respond;

- Make sure that sensory aids (hearing aids, spectacles) are appropriately utilised and sensory impairments (wax, cataracts) are treated.

How to make sure you are heard and seen:

- Check that their hearing aid is on and working (if applicable);

- Stand in front of the person where they can see you;

- Face the person directly so they can see your facial expression and mouth; Š

- Place yourself at eye level or lower when talking or listening;

- Identify yourself by name;

- Use the person's name.

How to make contact with the person:

- Keep yourself and those around you calm and relaxed;

- Touch the person gently, if they like to be touched (Care Planning again);

- Smile and use humour.

How to make communication easy to understand:

- Wherever you can use gestures, pictures and/or signs to explain or express things;

- Always avoid talking over/across/about the person;

- Always speak gently and clearly at an even pace - avoid shouting;

- Always ask just one question at a time;

- Always use specific names of people and places instead of pronouns: e.g.; Jim, our neighbour, or Sally, our dog, not "him" or "it";

- Wherever you can, use a statement rather than ask a question;

- Always wait for a response after you speak;

- Always explain what you are going to do and what you are doing;

- Always repeat or rephrase your message if there is no response.

Some other ideas:

- Always allow for the time a damaged brain takes to process messages;

- Always show your concern with reassurance and acceptance;
- Always give praise when it's appropriate;
- Always respond to the feelings expressed by the person;
- When talking in a group, place the person so that the conversation is around them and they won't feel 'left out';
- Make it easy to join in conversation by asking questions that only need a 'yes' or 'no' answer;
- Always avoid arguments over mistaken ideas: e.g.; If the person insists they have seen a TV program a million times before even though it is a first run say: "Oh well, I don't think I've seen it before. It's interesting isn't it?";
- Remember that touching enhances feeling of security, especially if the person is upset. Unless they respond aggressively.

Specialist Support

- The Care Planning process should pay particular attention to the identification and involvement of external specialists, such as the CPN, in the process in order to incorporate their specialist knowledge into the service delivery.

Challenging Behaviour or Distressed Reaction

- Ensure that challenging behaviour or distressed reaction is recorded, and reviewed as a part of the Care Planning cycle. Use the recording chart in the QCS Challenging Behaviour Policy and Procedure.
- A checklist can be useful in helping you step through the process of changing challenging behaviour.

Reviewed by Marcus Kerridge McColl
November 2025

Depression Policy and Procedure
Crescent Community Care Services Limited

Purpose
To provide individualised safe and supportive care for people with a diagnosis of depression or people who may be depressed.

Scope

All workers working with Service Users who have been given a diagnosis of depression or who may be depressed.

Definition

- Depression is a term in very common usage. It is a very common experience, for some lasting a short time but for others much longer. Because it is such an everyday term we need to be aware that 'something getting you down' can be a long way from the experience of living with depression. The diagnostic definition of depression includes lowness of mood and lack of pleasure, and there is a wide range of severity of symptoms of depression. The psychiatric classification system, Diagnostic and Statistical Manual of Mental Disorder volume four (DSM-IV) is widely used in mental health services in the United Kingdom, and defines depression as follows:

  » Persistent sadness or low mood and/or;
  » Marked loss of interests or pleasure;
  » At least one of these, most days, most of the time for at least 2 weeks.

- And then categorises severity as ranging through the following states depending on the number of symptoms and their impact:

» Mild depression;
» Moderate depression;
» Severe depression.

- Diagnosis of depression is undertaken by medical practitioners. Care and support staff may be working with someone who they feel is depressed. Consideration should be given to referring that person to their General Practitioner. Signs and symptoms that care and support staff should be alert to include the following:

  - **Social**

» Withdrawing from friends and family;
» Losing interest in things they used to enjoy;
» Not looking after themselves properly.

  - **Physical Impact**

» Losing appetite, or eating more than usual; Š
» Not sleeping properly;
» Feeling anxious.

  - **Psychological**

» Losing confidence in themselves;
» Lacking concentration;
» Expressing thoughts of worthlessness, and even suicide.

Description

- As with many psychiatric diagnoses, the term depression can be a controversial one, and its interpretation can differ depending on the person's cultural background. Diagnosis of depression is more widespread in Western societies. However it is universally accepted that people can experience great feelings of sadness that can be debilitating and lead to loss of normal functioning. There are different views about what treatments or interventions work best for someone who is depressed. Depression is usually characterised by a number of factors or symptoms, which include social impact, physical impact and psychological impact or a mix of all of these. There are different types of depression which are usually more specific in their cause, such as post-natal depression, seasonal affective disorder, and bipolar affective disorder. Other people experiencing symptoms of depression may well be experiencing grief and loss.
- There can be some obvious explanations as to why someone becomes depressed, perhaps due to distressing circumstances in their life, or perhaps there are no obvious explanations, and the causes may then be thought to be biological or genetic. The incidence of depression is very high. At least one person in five will experience depression at some point in their life. It appears to be more common in women than men. This may be because women are more willing to report symptoms of depression to their doctor.

Policy

- There are a number of good practice principles that care and support staff should be aware of whilst providing care and support for people with depression:

» Being aware that a diagnosis of depression can be stigmatising, and that people may not want to admit to this;

» Being able to identify other sources of support including self-help groups, support groups and other local and national resources;

» Awareness of the principles of the Mental Capacity Act 2005. QCS policies on the MCA will provide further information when working with people who lack mental capacity to make decisions about their care*;*

» Being aware that there are different types of depression and recovery from depression is possible.

Procedure

- The National Institute for Health and Clinical Excellence (NICE) has produced guidelines for treating and managing depression. It covers psychological treatments, medication and the services that should be made available in hospital and

in the community. These guidelines are downloadable from the NICE website at www.nice.org.uk. For the majority of people who are diagnosed with depression those interventions will be delivered by their GP and primary care team. For people with the severest of depressions those interventions are likely to be carried out by specialist mental health services. However there is still a crucial role for staff working in care home and community settings in supporting these interventions. The interventions that are likely to be offered will fall under one or more of these categories:

- **Self-Help**

Š » Self-help groups have provided a lot of support for people with depression, and self-help group users report that shared experiences have helped them develop coping strategies. GP surgeries are a usual source for further information on local groups.

- **Talking Treatments**

» These might include counselling and Cognitive Behavioural Therapy (CBT) for depression. These would be arranged via someone's General Practitioner, and in England may be provided by the NHS Improving Access to Psychological Therapies programme.

- **Anti-depressant Tablets**

» There are different types of anti-depressant medication. Some of these have side effects and may take some time before they help with symptoms. The person's GP would advise on the nature of anti-depressant medication.

- People with the severest depressions may need urgent intervention, particularly where someone is expressing suicidal intentions and in such situations the involvement of Crisis Resolution and Home Treatment (CRHT) teams may be required and access to crisis intervention can be made through referral by the person's GP.

Frameworks for Delivering Care and Support

- All treatments and interventions should be offered as part of a care planning process based on an individual's needs. Tools in the QCS Care Management System allow for compilation of assessment of mental health needs and a care plan format to that should be reviewed on a regular basis. The majority of people receiving treatment for depression will receive this from their General Practitioner. For those with severest depression who receive care and treatment from mental health services this should be delivered as part of the Care Programme Approach framework for delivering mental health care. The support and interventions by care staff should be included in the overall Care Programme Approach care plan, and these staff may be invited to contribute to CPA care planning meetings. The person who is the subject of the CPA care plan should have their own copy. Where care staff are aware that someone is in receipt of professional mental health services they should be aware of how the care plan can be accessed, and who the person's care co-ordinator is. If not they should contact the appropriate mental health team. The contents of a CPA care plan should include:

  » Details of who the care co-ordinator is (mental health professional);
  » Identify the interventions and anticipated outcomes;
  » The date of the next planned review;
  » Crisis and contingency arrangements;
  » Arrangements for care and support;
  » Arrangements for the management of risk to the Service User and to others.

- Crisis and contingency planning would be conducted by the person's care co-ordinator, but care staff should be aware of these plans. A crisis plan should identify responses to crisis situations that might arise for that person such as a worsening of their depression that cannot be managed with usual coping strategies. A contingency plan will outline a planned course of action that should follow some breakdown or failure in the person's care plan. Contact names and numbers of key professionals should form part of these plans, but a list of names and numbers on their own would not be regarded as sufficient. For crisis and contingency plans to be effective and workable, the Service User should be involved in drawing up their own plan when they are experiencing a period of wellness.

Risk Management

- At certain times, particularly at times of crisis, people with depression may experience risk factors that might expose them to:

  » Risk of harm to themselves, including self-neglect during periods of depression, actual self-harm, or suicide;

  » Risk of harm and abuse by others, particularly financial or sexual exploitation as a result of feelings of worthlessness.

- If you believe someone you are working with is at risk of these you should share that information with others you work with, and where appropriate, the person's GP. Assessment and management of people at risk is a specialist area of work that should be undertaken by health professionals, but care staff and the person themselves should be able to contribute to that. Risk management should not be seen as elimination of risk. There are risks for everyone in all walks of life, and to try and eliminate them would result in a loss of independence and choice. So risk management should be seen in the context of positive risk taking. Current policy thinking in relating to people with mental health problems can be found in the Department of Health's Best Practice in Managing Risk (2007). The Best Practice document also describes the importance of a collaborative approach to managing risk, involving the person and the whole of the care team, so that trusting relationships are developed that aid communication.

Importance of Good Communication

- Good communication is critical in offering care and support to someone with depression. As with all communication this should be based on principles of effective communication:

  » Good communication – providing any information in a clear and helpful way, including written and verbal information;
  » Acknowledging the importance of developing a trusting relationship with people you work with;
  » Discussion with the person will be conducted in a way that respects confidentiality, privacy and respect but being clear when you might need to share information. You should respect the person's confidentiality but principles of risk management may call on you to share information with other members of the care team;
  » Being aware that the symptoms of depression may result in barriers to effective communication. Here are some examples:
  - During periods of low mood the person may not respond or be very slow to respond;
  - The persons concentration may be very limited.

- Consider specialist support where there are additional barriers to communication.

Other Support

- There are other ways of working that care staff and support workers can employ in offering support to someone with depression:

  » Encouraging the person to talk about how they feel;
  » Where possible, offer opportunities for physical exercise or activity;
  » Offering a healthy diet of food and drink. Be aware that alcohol makes depression worse;
  » Encouraging the person to write down what is troubling them and whether they can tackle it;
  » Maintain hope. There are many people who have had depression and emerged from it strongly;
  » Being alert to changes in patterns of behaviour and thinking, such as the person becoming more withdrawn, and discussing these changes with the person;
  » Encouraging the person to be alert to their own mood, by writing things down they think and experience and note any changes.

Legal Frameworks

- The framework for the compulsory care and treatment of people with mental disorder is governed by the Mental Health Act 1983 (which was amended in 2007).
- Where a person lacks capacity as a result of an impairment of, or a disturbance in the functioning of, the mind or brain, to make decisions, the principles involved in making decisions for someone in their best interests are governed by the Mental

Capacity Act 2005. The Mental Capacity Act 2005 also includes provision for the appointment of lasting powers of attorney regarding financial and welfare decisions if the person should lose the mental capacity to make these decisions.

- Policy and procedures surrounding the Mental Health Act 1983 and the Mental Capacity Act 2005 are contained within the QCS manuals.

Teamworking

- Working with people with depression is most effective when undertaken as a team approach, involving the person, their family and friends, primary care services, mental health professionals and paid carers and support workers.
- There are a number of principles in effective team working that should be recognised:

  » Case recording, particularly of important decisions;
  » Staff support and supervision;
  » Communication and information sharing;
  » Learning opportunities with members of the wider care team;
  » Awareness of policies and procedures.
- Other sections of the QCS manuals give further guidance on all of these. All of these may be identified by care staff as development and training needs.

Reviewed Marcus Kerridge-McColl
November 2025

Development Appraisal Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To assist employees in performing their jobs to the best of their abilities, maximising their levels of job satisfaction and their contributions to the organisation's objectives.

- To identify individual employee's training and development needs required to meet the organisation's goals.

- To highlight the potential of each individual employee to develop within their current position, or into another.

- To ensure that employees are aware of the contribution they make to the achievement of the objectives of the organisation.

Scope

- All employees.

Policy

- Each employee will be appraised annually, and an exchange of views will take place between supervisors and their immediate subordinates.

- The summary of this interview and the attached Action Plan is intended to be a fair representation of the discussion, and is referred to as a working document throughout the year.

- Personal appraisal documentation must be stored in the individual employee's personnel file, and subject to the normal security and privacy measures applying to that file.

Procedure

- A clear difference will be established between supervision and appraisals:

  - Supervisions are backward looking; the assumption is made that the employer has competently appointed the person being supervised, and they have the skills to do their job. Therefore the supervision interview is about how well, or otherwise, the person has used their skills in achieving the standards required, and identifying where they need to lift

their performance and use the skills they have. Supervision is clearly identified as a line employee exerting their authority and responsibility to maintain their subordinate's standards of performance. While supervision is primarily intended to identify performance shortcomings, this should be used as an opportunity to discuss support requirements needed to improve performance. Supervision should also be used to provide positive feedback where performance is adequate or exceeding the required standard.

- Appraisals are forward looking; they are about stating what the business objectives are for the next year, as set out in the Business Plan, and identifying and clarifying the appraisee's part in achieving those objectives. Following on from that, the discussion focuses around what new skills, capabilities or resources the appraisee needs in order to meet the personal objectives which have just been agreed. If reference is made to past performance, it is only done when it is required to identify the current level of the appraisee's performance. This would be done in order to decide what development is required to move to the level of performance required by the new objectives. Appraisals are more collaborative than supervisions, the intention being for the appraiser and appraisee to co-operate on deciding how they are jointly to achieve the future objectives of their mutual employer.

- Appraisal interviews will take place on an annual basis during a two month period which is announced well in advance. The scheduled appraisal period will follow shortly after the finalisation and publication of the annual business plan review.

- The organisation will ensure that the business plan is available before the schedule begins, that those to be appraised have received at least that portion of the plan which relates to them, and that each employee's contribution to the achievement of that plan has been identified.

- Appraisal interviews should be carried out by the appraisee's immediate line employee on a one-to-one basis.

- Training or coaching will be provided to all appraisers prior to conducting their first appraisal interview.

- New employees will be appraised in their sixth month of employment, and thereafter on an annual basis that coincides with the overall schedule.

- Prior to appraisal, appraisers should inform the appraisee that their interview is due, and give them the appraisal preparation form together with the current Job Description and (if any) task list. The appraisee should be shown the appraisal form to ensure that they are aware of the types of questions they will be asked and can prepare for them. The appraiser will gather appropriate information relevant to the performance review, such as:

  - Supervision records;
  - Line manager's, or other manager's opinions;
  - Training records;
  - Discreet enquiries to Service Users;
  - Discreet enquiries to co-workers.

    - By "discreet" is meant that such information gathering must be carried out in a manner which does not undermine the authority or dignity of the person being appraised, and does not give rise to a general discussion of their merits and demerits.

- The appraisal interviews will take place from the most senior person in the organisation downwards in order to ensure that the communication of corporate objectives is made more effectively.

- The appraisal interview should take place in private, in comfortable surroundings with no distractions, not overlooked by other people, and arrangements made to have no interruptions.

- As the appraisal proceeds, any matters requiring action must be listed on the Action Plan located at the end of this pack, noting all agreed actions, together with agreed resources and target dates.

- Appraisal forms will be completed by the appraiser shortly after the interview takes place, with a short follow-up meeting to review, agree and sign the form. A copy of the completed appraisal form should be given to the appraisee.

- The completed appraisal form and Action Plan must be viewed as working documents and as such be continually referred to and reviewed throughout the year.

- Any items on the Action Plan requiring the actions of others, or for others to be informed, should be added to the appropriate individual's or the general Action Plan.

- Progress against the Action Plan should be reviewed by the appraisee and their line manager at each supervision following the appraisal, and at other performance review meetings.

Reviewed by Marcus Kerridge McColl
December 2025

Disabled Workers Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with the Equality Act 2010 and good employment practice.

Scope

- Job applicants, employees, workers, those on work experience, ex-employees. It also includes agency staff.

Policy

- The objective of the policy is to ensure that individuals are selected, allocated work and otherwise treated solely on the basis of their relevant aptitudes, skills and abilities.

- The organisation recognises that it has a clear legal and moral obligation towards its employees and the community at large to ensure that people with disabilities are offered equal opportunities to enter employment and progress within the organisation.

- In order to be protected, and for us to meet our responsibilities, you need to make us aware of any disability unless it is one that is apparent.

- In addition to complying with the Equality Act 2010 the organisation will follow procedures designed to provide for fair consideration and selection of disabled applicants and to identify / meet (satisfy) their training and career development needs.

- If you become disabled during your employment steps will be taken to accommodate your disability by making reasonable adjustments to your employment situation. Adjustments will be made in consultation with you and may include redeployment to another post.

- Appropriate relevant training will be provided to facilitate a smooth transition to any new role.

- All employees have a responsibility to *avoid* discriminating against disabled workers in any of the following ways:

  - Directly: e.g. by not appointing someone because of their hearing
  - Indirectly: e.g. by unnecessarily specifying a height restriction for the job
  - Association: e.g. because a partner has a disability
  - Perception: e.g. because an employee is thought to be disabled even though they are not »
  - Harassment: e.g. intimidating an employee because of their disability
  - Victimisation: e.g. treating someone adversely because they have complained in the past

Guidance to the service provider STATUTORY REQUIREMENTS

- The law requires employers to make reasonable adjustments to ensure that people within the scope of this policy (above) are not placed at a disadvantage by reason of a disability.

- Disability has a specific definition in the Equality Act in that a person is disabled if they have (or have had) a physical or mental impairment that has (or has had) a ‘substantial’ and ‘long-term’ negative effect on their ability to do normal daily activities.

- Long term generally means 12 months or more.

- However certain conditions, namely HIV infection, cancer or multiple sclerosis, are included from the point of diagnosis, whether or not they affect day-to-day activities at the time.

- Normal day-to-day activities are ones that may involve any of the following: » mobility;

  - doing something with hands;
  - physical coordination;
  - continence (controlling bladder and bowels);
  - ability to lift, carry or move everyday objects;
  - speech, hearing or eyesight;
  - memory or ability to concentrate, learn or understand; or » perception of the risk of physical danger.

RECRUITMENT AND SELECTION

- You should seek to ensure that:

  - All recruitment advertisements, job descriptions and person specifications are drafted in such a way as to clearly reflect the essential and desirable requirements of the post.
  - Consideration is given to making reasonable adjustments to application forms and other documentation to accommodate that needs of disabled people.
  - Applicants have a clear opportunity to ask for any reasonable adjustments that they may need during the selection process. This will enable the organisation to proactively prepare for any potential disabled employee and to include structural and or other adjustments to its development plans.
  - The selection process does not in any way place disabled candidates at a disadvantage.
  - Assistive technology, such as induction loops and Braille, is available during the recruitment process.
  - That documents relating to recruitment and selection are available in an accessible format to ensure equality of opportunity for disabled candidates.
  - The physical environment for conducting interviews is user friendly with disabled access.
  - You make reasonable adjustments to accommodate the needs of successful disabled applicants after all necessary recruitment, health and disability checks have been completed (with specific reference to intrinsic functions of the job) and an employment offer has been accepted.
  - Where job roles or tasks are deemed to be suitable for people with particular disabilities, such people are given preference.
  - When a disabled person is arriving for interview they are offered any help which may be required.
  - The usual probationary period applies to people who have disabilities unless there a cogent reasons for making an adjustment.
  - Where adaptation of premises may be required to enable a particular disabled person to take up employment, the matter is fully investigated with relevant and appropriate advisory services and authorities.
  - Where advice is needed on a medical condition or on the safety implications of recruiting a person with a particular disability, the relevant advisory service is contacted for guidance.

DURING EMPLOYMENT

- It is important that all employees show consideration towards disabled colleagues. Where special arrangements need to be made to ensure health and safety, you should consider occupational health and other advice.

- You should ensure the employees' colleagues are briefed, as may be appropriate.

- Training and promotion opportunities need to be made available to all, regardless of disability. You may need to monitor the application of the organisation's equal opportunity policy throughout a disabled employee's employment.

- An extended trial period may be offered to a disabled employee on transfer or promotion where it is reasonably considered appropriate.

DISABILITIES ARISING DURING EMPLOYMENT

- If an employee becomes incapable of carrying out his or her normal duties because of a disability, consideration must be given to reasonable adjustments to the job role, working conditions, redeployment and retraining.

- Where appropriate, you should arrange through an Occupational Health Service for an employee's capabilities to be assessed with a view to identifying suitable alternative work for that individual.

TERMINATION OF EMPLOYMENT

- **Capability:** where an employee's performance falls to an unacceptably low standard through deterioration in his or her disability, you need to:

  - Explore the possibility of restructuring the job to allow continuation in the role;
  - Consider alternative job roles/employment;
  - Consider transfer to a "holding register" until an appropriate vacancy is available. If having considered the above options, it is apparent that the employee cannot realistically continue in employment with the organisation, a decision to dismiss should be taken only by a director of the organisation following consultation with the employee, his or her representative, the occupational health advisor/disablement resettlement officer and a senior HR advisor.
  - Generally speaking the employer's responsibility is to make reasonable adjustments so a disabled person is not at a disadvantage. Provided reasonable adjustments have been made the normal capability procedure can, and needs to be followed.

- **Misconduct:** When a disabled employee's conduct (for reasons unconnected with the disability) is unacceptable, the organisation's normal disciplinary procedures need to be followed.

- **Redundancy:** In a redundancy situation the particular circumstances of disabled employees can be given particular consideration. Able bodied employees do not have a specific right to protect them from an employer discriminating in favour of an employee who has a disability.

- **Appeals:** Disabled employees who wish to appeal against a disciplinary sanction or a dismissal decision must be required to use the organisation's normal appeals procedure.

EMPLOYEE REPRESENTATIVES

- Employee representatives, alongside management, have responsibilities for ensuring that the organisation's policy is fairly and consistently applied.

- The organisation's policy and procedures concerning the employment of disabled people will be reviewed periodically and discussed with other parties as appropriate.

Reviewed by Marcus Kerridge McColl
December 2025

Discipline Policy and Procedure
Crescent Community Care Services Limited

Purpose

- Crescent Community Care Services Ltd believes that we offer our Service Users a high standard of care, arising from the cooperation of our workforce who maintain high standards of conduct, attendance and job performance.

- The purpose of this procedure is to ensure the safe and effective operation of the business and the fair and equal treatment of all its employees to ensure continuing high standards.

- Crescent Community Care Services Ltd views the disciplinary procedure as a positive contribution to the success of the business for both the organisation and its employees.

Scope

- All employees.

Policy

- No disciplinary action will be taken against an employee until the case has been fully investigated. All stages of the procedure will be implemented without undue delay.

- At every stage in the procedure the employee will be advised of the nature of the complaint against them and will be given the opportunity to state their case before any decisions are made.

- The employee will have the right when the 'formal procedure' is being invoked, to be accompanied by a work colleague or a Trade Union representative (as defined in the Employment Relations Act 1999) during a disciplinary interview.

- The employee will always be informed in writing of any disciplinary action to be taken and the reason for it, indicating the specific areas of improvement required within a specific period.

- No employee will be dismissed for a first breach of discipline except in the case of gross misconduct when the penalty will be dismissal without notice and without payment in lieu of notice.

- The employee will have the right to appeal and should follow the Appeal Procedure.

Procedure

- ***Employee*** *- This procedure is not part of your contract of employment; however, we will not depart from it without good reason.*

- ***Employer*** *– consult your legal expenses insurers before beginning this process.*

- All formal disciplinary action will be conducted by the Registered Manager or a person nominated by them. Appeals will normally be conducted by the Registered Person. We reserve the right to suspend individuals during formal disciplinary action. Any suspension will be on a paid basis and does not mean that we have prejudged the issue.

- Before any formal disciplinary action is taken, the relevant person will carry out a full investigation to establish the facts. The investigation will normally include a meeting with you. Investigatory meetings are not disciplinary meetings and you will not necessarily be offered the right to be accompanied.

- Even in the most serious allegations of gross misconduct (see below), a full investigation will be held. In any alleged case of gross misconduct you are likely to be suspended pending the outcome of the investigation.

- Before any *disciplinary* meeting, you will be:

  - Told in writing of the allegations/complaints against you, and the basis of those allegations;
  - Given a reasonable opportunity to consider your response to that information;
  - Offered the opportunity to be accompanied by a work colleague or a trade union representative (see right to be accompanied, below);
  - You must take all reasonable steps to attend the meeting. At the meeting, you will be given a full opportunity to comment on the allegations, to put forward any defence or arguments you want, and to comment on what disciplinary sanction (if any) is appropriate.

Informal Warning

- After establishing the facts, we may consider that there is no need to resort to the formal procedure, and that it is sufficient to talk the matter over with you. A note of the informal warning may be kept on your personnel file; however, they are there for background and would not normally be taken into account in the event of subsequent disciplinary procedures.

- The purpose of an informal warning is to provide an opportunity for improvement or for the matter to be corrected without the necessity for formal disciplinary procedures to be enacted.

Formal Disciplinary Process Right to be accompanied

- You have the right to be accompanied at any disciplinary hearing by a single companion who is either: -

  - A work colleague; or,
  - A full-time official employed by a trade union; or,

- A lay official, so long as they have been certified in writing by their union as having experience of, or as having received training in, acting as a worker's companion at disciplinary or grievance hearings.

- Your representative has the right to explain and sum up your case, and to respond to any views expressed at the hearing. They may not answer questions on your behalf. If your representative cannot attend on the date we have set for the interview, we will always postpone the interview for up to five days and may (at our discretion) postpone it for longer.

Stage 1: Written Warning

- If it is decided that your conduct or performance is unsatisfactory your manager may give you a Written Warning.

- This will state the nature of the complaint, the required standards that must be met and, where appropriate, a time limit for improvement. It will also state that further disciplinary action will follow if the required standards are not met or if there is further misconduct.

- You will be informed of your right of appeal, and how and where this should be made. A record of the warning and related discussions will then be placed on your personnel file. It will normally cease to have effect after 6 months.

- If your conduct is sufficiently serious, we may omit stage 1, and proceed straight to stage 2.

Stage 2: Final Written Warning

- For more serious matters, or where you have failed to meet the required standards after being warned, you may be given a Final Written Warning. This will state the nature of the complaint, the required standards that must be met and, where appropriate, a time limit for improvement. It will also state that you will be dismissed if the standards are not met or if there is further misconduct.

- Again, you have the right to appeal. A record of the warning and a note of all related discussions will be placed on your personnel file. A final written warning will normally cease to have effect after 12 months.

Stage 3: Dismissal

- Where there has been gross misconduct (in which case, stages 1, and 2 may be omitted) or where you have failed to meet the required standards after due warnings have been given, you may be dismissed. In extenuating circumstances we may apply another sanction such as disciplinary transfer, disciplinary suspension without pay or demotion.

- This will be confirmed in writing and will include details of the appeals procedure.

- In cases of gross misconduct, the dismissal will be without notice (or pay in lieu of notice).

Dismissal without notice ('Summary Dismissal')

- We regard certain issues as serious enough to warrant 'summary dismissal' without prior warning. These issues would constitute gross misconduct, which is the action would break the contract between us. Matters that may justify summary dismissal, include, but are not limited to:

  - Actions which may harm the well-being of a Service User ("abuse");
  - Acts of dishonesty where your conduct affects your ability or suitability for continued employment with us; for example, theft, fraud, the deliberate falsification of records or expenses, a relevant criminal warning or conviction, or inclusion on the DBS register;
  - Serious insubordination or rudeness to customers or suppliers; deliberate damage to property;
  - A serious breach of Health & Safety policies;
  - Physical violence or aggressive behaviour;
  - Indecent or immoral acts;
  - Being under the influence of, or possessing, alcohol or illegal drugs during employment hours (unless you have been authorised to, and have been entertaining clients, in which case a reasonable amount of alcohol consumption is permitted);
  - Bringing Crescent Community Care Services Ltd into serious disrepute;
  - Any breaches of confidentiality requirements in your contract of employment, other than minor breaches;
  - Harassment or bullying, other than minor breaches;
  - Breaches of our Equal Opportunities Policy, other than minor breaches;
  - Wilful misrepresentation at the time of appointment, including:

- Previous positions held;
- Qualifications held;
- Falsification of date of birth;
- Declaration of health;
- Failure to disclose a criminal conviction/caution within the provision of the Rehabilitation of Offenders Act.

- Abuse of the protected disclosure provisions;
- Serious failure to abide by the professional code of conduct which applies to your work, a copy of which was issued to you on your engagement, a revised copy of which will have been issued to you if it has changed since your engagement, and a copy of which is also available for inspection in the office;
- Deliberate disclosure of privileged confidential information to unauthorised people;
  Negligent or deliberate failure to comply with the legal requirement for The Agency's policy & procedure concerning medicines;
- Working whilst contravening an enactment or working in such a way that is in breach of rules laid down by statutory bodies, e.g. erasure from the register of the Nursing & Midwifery Council.

- Except in the most serious cases of gross misconduct (see below), a full investigation will be held and, if necessary, you will be suspended pending the outcome of the investigation.

- In the event of gross misconduct, disciplinary action could take the form of a demotion, or a downgrading, with subsequent financial loss as well as loss of status, rather than a dismissal. This decision will be taken at our discretion.

Other Disciplinary Matters

- Matters which may justify invoking the disciplinary procedure but which do not amount to gross misconduct include, but are not limited to, the following:

  - Refusal to obey a legitimate instruction;
  - Refusal to answer a question during the course of a properly constituted investigation;
  - Absence without permission, or persistent absence;
  - Poor timekeeping;
  - Failure to report damage to our property;
  - Failure to carry out your duties adequately;
  - Breach of our policies, procedures and practices;
  - Continued poor work performance, or a persistent failure to keep up-to-date with technical developments;
  - Improper use of our equipment;
  - Harassment or bullying (in ways that are not deemed serious enough to constitute gross misconduct);
  - Breaches of our equal opportunities policy (in ways that are not deemed serious enough to constitute gross misconduct)
  - Actions bringing Crescent Community Care Ltd into disrepute (in ways that are not deemed serious enough to constitute gross misconduct).

What you can expect during the Disciplinary Process

- At each stage of the disciplinary process:

  - You will be told of the expected standard of performance/behaviour, and the nature of the shortfall in expected standards will be identified;
  - You will be given the opportunity to reply to any allegations made against you, and to outline any mitigating circumstances you wish to be considered;
  - All the facts will then be considered. Occasionally we may ask you back for a further interview;
  - You will be told of the disciplinary sanction being imposed (although this may be done in writing). This will include details of any timescale within which improvement is to be achieved and the likely consequences if there is no improvement; and
  - You will be advised of the right to appeal.

Right to appeal

- If you are not satisfied with a disciplinary decision, you may appeal, in writing, within five working days. Arrangements to hear the appeal will normally be made within five working days of receiving your written request. If the decision you are appealing against was a decision to dismiss you, the appeal may be heard after the dismissal has taken place.

  - All appeals must set out the grounds on which you are making the appeal.

- We will invite you to an appeal hearing, and remind you of your right to be accompanied.
- The outcome of the appeal will be confirmed to you in writing and will take one of three forms:
    - The original decision will be upheld, in which case any disciplinary sanction will be confirmed;
    - The original decision will be overruled, in which case any disciplinary sanction will be rescinded;
    - The original decision will be substantially confirmed but a less severe sanction will be substituted for that originally imposed (usually in cases of appeals based on extenuating circumstances).

- There is no further right of appeal.

Guidance for the Service Provider

This guidance here is not intended to supplant the ACAS Guide, which is more extensive, but it should nevertheless assist you if you are undertaking such processes for the first time or are under-confident about what may be required. It is, however, general guidance and is not a substitute for professional advice. The latter can be obtained via QCS.

1: Recommended Procedure for Disciplinary Hearing Conducting a disciplinary hearing

- The information here is for guidance, in order to assist in a fair hearing. It cannot guarantee fairness; that will depend on how you handle or view the information that comes forward. If, elsewhere, you have more detailed or radically different procedural guidance then it may be more appropriate to follow those procedures. Public sector employees, even if brought into a private sector organisation, may have additional rights to those indicated here.

Setting the scene

- The person whom you have invited to a disciplinary hearing is likely to be nervous. Therefore it is important to be as empathetic as you reasonably can be.

- Introduce the people who are present and explain the reason for their presence or role. Disciplinary hearings should not be conducted alone. Typically the chair of the meeting will have an HR Adviser with them and perhaps a note taker.

- The employee has the right to be accompanied by a Trade Union representative or work colleague. Indeed the latter in particular should be encouraged as it provides for better balance. At the outset it is beneficial to make sure that the employee has been made aware of that right and, if it has been declined, to make a note to that effect.

- Accompanier's have the right to address the hearing but not to answer questions on the employee's behalf.

- Outline how you intend to conduct the hearing. The employee needs to know that they will get the opportunity to put their side of the story and at what point. Usually it is best to be firm about putting the detail of the charge(s) first and asking the employee to note any points he or she may want to challenge. They can make their points later rather than interrupt you. Make clear how you see the accompanier's role. They have the right to have the opportunity to address the meeting but not to answer on the employee's behalf.

Getting started

- The chair should then outline the reason(s) for the hearing and make sure that the employee has received any relevant paperwork or the investigation report, as appropriate.

- If dismissal is a potential sanction this should have been flagged up previously but it is good practice for the employee to know what the possible outcome(s) could be. Without being intimidating it is valuable to clarify these possible outcomes – which must include no action being taken at all, if that should prove appropriate.

The heart of the hearing

- Then proceed to detail the charge(s) levelled against the employee. This may mean going through the investigation report. Alternatively it may mean going through evidence, such as performance reports.

- Once you have completed this stage allow the employee to challenge any assertions in the allegations or the report. Accompaniers will often wish to challenge the allegations, content of the report or other purported facts or evidence. You should be prepared for this. It is debatable whether you can refuse but, in practice, accompaniers may assert that this is part of their right to address the hearing.

- Apart from the opportunity to challenge the report or allegations the employee should also have the opportunity to offer any explanation for their behaviour. Indeed they should be encouraged as it is better to hear any explanation now than later in a public hearing. Blame may be assigned to others by the employee and you will have to decide whether this needs to be followed up. However, all misconduct should be fully investigated first and all being well potential claims of others to blame or "framing" will have been covered in the investigation.

- Always be prepared to adjourn if new information comes to light that might have a bearing on the fairness of any decision.

- In addition you should seek out any mitigating circumstances. Employees will not necessarily recognise they are relevant or may be reluctant to share them. Divorce, family illness, bereavement and other life crises can all impact on employee behaviour. If they exist then you need to know about them if possible. You don't have to consider them as relevant (if they are not) but, again, they are best in the open now, not in a subsequent Tribunal hearing.

- Ask the employee if he or she has anything to add or indeed whether they have a different "story".

- Getting everything into the open is important to sound decision making. So listening and questioning are valuable skills.

- It is good practice to give the accompanier the opportunity to have a final say on behalf of the employee.

The next step

- Adjourn the meeting. This is not always essential but it is good practice.

- It is also the point where a colleague, preferably an HR Adviser, can aid the deliberations. If there is a lot of material then take time.

Communicating the decision

- You may want to advise the employee that you will let them know the decision when it has been made rather than keeping them waiting. If it is not to be at the meeting you might want to ask them how they would prefer to be informed of the decision once it is made.

- It may be that you will wish to seek further advice and/or reflect on the meeting before making a decision.

- But once a decision has been taken the employee must be informed of the decision and, if the decision is adverse, informed of their right of appeal, to whom that appeal should be lodged and by what timescale.

- Written confirmation of the decision should be sent to both parties within 5 working days of the hearing.

A note on notes

- Notes of disciplinary hearings are essential to being able to defend a decision if it is challenged. Contemporaneous notes are ones taken at the time of the hearing. If the outcome may be serious, damaging to a person's career or would be a dismissal, then a separate trained note-taker is advisable to make such notes. This may mean three people on one side but make clear that one is just a note-taker.

- A compromise to having a third person is to halt the proceedings as necessary to note key points. You then have some points on which to rest a case. Other notes made immediately after the meeting can be used to flesh the contemporaneous ones out provided they are made while the hearing is still fresh in the mind. Notes written up later carry much less weight.

- The employee need not be shown the full notes; although they could be required to be disclosed, by a subsequent court order. Showing full notes, when emotions may be running high, is best avoided and, indeed, the notes might not need to be disclosed at all.

- It would be good practice, however, to summarise the key points of the discussion on any subsequent warning form.

2: Recommended Procedure for Appeals

Conducting an appeal hearing

- The information here is for guidance, in order to assist in a fair appeal. It cannot guarantee fairness; that will depend on how you handle or view the information that comes forward. If, elsewhere, you have more detailed or radically different procedural guidance then it may be more appropriate to follow those procedures. Public sector employees, even if brought into a private sector organisation, may have additional rights to those indicated here.

- Generally, the appeal will be heard by someone not involved in the preceding procedural stages, if possible, and usually a more senior manager.

Setting the scene

- Even though the employee or ex-employee (the appellant) has instigated this meeting, they are likely to be nervous. Therefore it is important to be as empathetic as you reasonably can be.

- Introduce the people who are present and explain the reason for their presence or role. Appeal hearings should not be conducted alone. Typically the chair of the meeting will have an HR Adviser with them and perhaps a note taker.

- The employee has the right to be accompanied by a Trade Union representative or work colleague. Indeed the latter in particular should be encouraged as it provides for better balance. At the outset it is beneficial to make sure that the employee has been made aware of that right and, if it has been declined, to make a note to that effect.

- Accompanier's have the right to address the hearing but not to answer questions on the employee's behalf.

- Outline how you intend to conduct the appeal hearing. The employee needs to have the opportunity to put their reasons for appeal and provide evidence to support the reasons. They should be able to make their points without interruption. Clarify the accompanier's role. The accompanier has the right to address the meeting but not to answer questions on the employee's behalf.

Getting started

- The chair should make sure that the employee has received any relevant paperwork, as may be appropriate.

- It is good practice for the employee to know what the possible outcome(s) could be. Without being intimidating it is valuable to clarify these possible outcomes – which must include no action being taken at all, if that should prove appropriate.

The heart of the appeal hearing

- The chair should then invite the employee to put forward the reasons for their appeal. They may need help and encouragement to put their case, especially if they are not accompanied. It is in your interests to give them a full hearing.

- Once you have completed this stage you may well want to query and question the employees reasons and assertions. This should be gentle cross-examination. If there are points that the employee has failed to appreciate this may be your opportunity to help them see your viewpoint.

- Sometimes you may feel more investigation is required and, if so, you will need to adjourn.

- Always be prepared to adjourn if new information comes to light that might have a bearing on the fairness of any decision. An appeal should not be a re-hearing but, on occasion, that will be necessary; and especially so if the original hearing was flawed on procedure.

- As with a disciplinary hearing, getting everything into the open is important to sound decision making. So listening and questioning are valuable skills.

- It is good practice to give the accompanier the opportunity to have a final say on behalf of the employee.

The next step

- Adjourn the meeting. This is not always essential but it is good practice.

- It is also the point where a colleague, preferably an HR Adviser, can aid the deliberations. If there is a lot of material then take time.

Communicating the decision

- You may want to advise the employee that you will let them know the decision when it has been made rather than keeping them waiting. If it is not to be at the meeting you might want to ask them how they would prefer to be informed of the decision once it is made.

- It may be that you will wish to seek further advice and/or reflect on the meeting before making a decision.

- But once a decision has been taken the employee must be informed of the decision and, unless otherwise is the case, informed that this is the final stage of the procedure and there is no further right of appeal.

- Written confirmation of the decision should be sent to both parties within 5 working days of the hearing.

A note on notes

- Notes on appeal hearings are essential to being able to defend a decision if it is challenged. Contemporaneous notes are ones taken at the time of the hearing. If the outcome of the disciplinary hearing was damaging to a person's career or was a dismissal, then a separate trained note-taker is advisable to make such notes. This may mean three people on one side but make clear that one is just a note-taker.

- A compromise to having a third person is to halt the proceedings as necessary to note key points. You then have some points on which to rest a case. Other notes made immediately after the meeting can be used to flesh the contemporaneous ones out provided they are made while the hearing is still fresh in the mind. Notes written up later carry much less weight.

- The employee need not be shown the full notes; although they may be required, by a subsequent court order, to be disclosed. Showing full notes, when emotions may be running high, may be best avoided and, indeed, they might not need to be disclosed at all.

3: Recommended Procedure for Investigations

What if abuse is suspected?

- Where matters of Service User abuse are suspected then there may be a requirement to involve local social services by contacting the Adult Protection Team or Safeguarding Officer, or in certain cases, the police. Indeed there may be a condition that you do so if you receive public funding. It might be, in those cases, that the whole investigation is taken out of your hands.

- This guidance is intended for situations which can remain under your control.

So what is an investigation and to what degree should it be conducted?

- Essentially investigation is about establishing all facts that might be relevant to the case, so far as is reasonably possible.

- There are circumstances where the degree of investigation can be limited, particularly if the situation is not too serious. For example, perhaps someone has accepted responsibility for a careless error but nevertheless a formal warning is justified.

- On the other hand if a serious, careless error is not admitted, or contested, and dismissal for gross misconduct is being contemplated, then thorough investigation is essential. The facts might not be as they first appear to be. Having such an investigation carried out by an experienced person can assist in establishing fairness. In any case it is preferable that the investigating officer and any subsequent disciplinary chairperson are different people.

The prime task is to gather evidence.

- Evidence can come in a number of forms:
    - Witness testimony
        - This is frequently the main source of evidence. You really want such testimony from more than one source. It is also important that you are confident that each witness is independent. If they all report to the same person this may give cause for doubt. It is not unusual for employees to claim that they have been "framed". You don't want the first time you hear of this to be when you are in a Tribunal. So be alert to this possibility and eliminate conspiracies if you can.

- Employees are entitled to call witnesses to a disciplinary hearing. It may therefore be wise to speak to any witnesses that the employee intends to put forward in their defence – but NOT to intimidate them!! Be careful, so you are not accused of doing so.
- Make detailed notes of what the witnesses tell you and challenge them and what they say. You need to flush out any doubts, conspiracy or false testimony.

- Investigatory meeting
  - Employees should be given advance warning of an investigatory meeting. It is up to you to decide whether allowing accompaniment would hinder or help the investigation. Be clear with the employee about the status of the meeting – it is an investigatory meeting, not a disciplinary meeting or a disciplinary hearing.
  - The purpose of the meeting is to establish facts, or at least the facts as the employee sees them. It is not the place for defence or explanation; that may come later if a disciplinary meeting is still required.
  - Similarly you might not wish to put other evidence that you have gathered before him or her. That can come later. Nonetheless in some cases doing so can be helpful. It is a matter of judgement in each set of circumstances.
- Signed statements
  - If signed statements are presented to an employee then then that may reduce the likelihood of him or her calling witnesses to a disciplinary meeting and thus simplify the process.
- Unsigned statements
  - Sometimes there is the fear, real or imagined, of the witnesses being subject to harassment, intimidation or violence from the employee against whom they are testifying. it is crucial that the disciplining officer also talks to the witnesses and challenges their statements, as discussed above. If it comes to a Tribunal then it will be the disciplining (or dismissing) officer who has to stand up in court and justify his or her decision.
- Other evidence
  - Photographs and even Smartphone videos can provide corroborative evidence.
- CCTV footage (including targeted footage) may also be used as corroborative evidence but blanket CCTV coverage must not be used as a means of monitoring employees. That could infringe human rights.
  - Tests of sobriety are also sometimes valuable though obviously they need to be used at the immediate time. For example, can the apparently drunk employee read a simple passage of writing?

Was it the employee's responsibility and were they trained for that responsibility?

- An employee will often assert that a particular failing was not their responsibility and that therefore they should not carry the can.
- An organisation chart, job description and training records all help to establish who is responsible for what and to justify a decision to dismiss if appropriate.

Have actions, that have been taken already, suggest how serious the misdemeanour might be?

- If first line management treats the situation as minor then this can undermine the whole case. If gross misconduct is suspected then this should normally lead to suspension pending investigation. Such suspension should be "neutral", paid and appropriate. If in doubt, seek advice.

Did the employee "know the rules"?

- This is a secondary, but nevertheless important, point.
- A starting point is the written particulars or written contract of employment. It is important to show the employee has received these. It is better still if he or she has signed them to say they have been read and understood.

- Disciplinary rules are also important. These should be readily accessible to the employee. An actual copy in the employee's hand is not essential but it is important that the employee knows they exist and where they can be accessed.

- A record of attendance at an induction can also be valuable; especially if disciplinary rules are covered at that induction.

- Previous warnings, even if they are expired, can also underline the fact that the employee knew the rules.

- Investigating officers should produce a report which the employee can see prior to the hearing. This will form an important part of the disciplinary hearing as will any defence or counter arguments presented by the employee.

Reviewed by Marcus Kerridge McColl

December 2025
Diversity and Equality Policy and Procedure
Crescent Community Care Services Limited

Purpose

- The purpose of the Equality and Diversity Policy and Procedure is to ensure that the principles of the Equality Act 2010 are understood and embedded into practice within Crescent Community Care Services Limited. In addition the policy will promote and formalise how the Crescent Community Care Services Limited will ensure that our practice treats people fairly, and will ensure that dignity and respect is at the heart of what we do.

The Aims of the Equality and Diversity Policy are:

- To encourage, promote and celebrate diversity in all our activities and services;
- To ensure equal access to jobs and volunteer opportunities;
- To create environments free from harassment and discrimination;
- To confront and challenge discrimination where and whenever it arises, whether it is between colleagues, or in any other area relating to Crescent Community Care Services Limited's work;
- To ensure acceptance and implementation of this policy is mandatory for all positions in Crescent Community Care Services Limited;
- To ensure, through positive action and as far as is practicable, that all Crescent Community Care Services Limited's premises and services are accessible to all;
- To ensure that employment and advancement within the organisation is not restricted because of any protected characteristics or for any other discriminatory reason.

Scope

- All employees, job applicants, volunteers, trustees, members and Service Users.

See next page for contents.

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- Our understanding of Diversity and Equal Opportunities

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- Expectations
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Policy

Statement

- Crescent Community Care Services Limited strives for high standards both as an employer and as a provider of services. We recognise the need to promote diversity and wholeheartedly support a policy of equal opportunities and uphold human rights in all areas of our work and responsibilities.
- This policy will define and formalise our approach to Equality and Diversity and will recognise and publicise our commitment and our responsibilities under the Equality Act 2010.
- We will address these principles in all areas and particularly the following:
    - Communicating our expectations;
    - Recruitment and promotion;
    - Interviews and selection;
    - Training, including induction training;
    - Benefits and terms and conditions of employment;
    - Service User delivery;
    - Furthermore, the organisation will monitor the composition of its workforce and Service User base and take appropriate action if it appears that this policy is not fully effective in all areas of its operation.
- This policy provides clear guidelines for all who work with or for Crescent Community Care Services Limited to understand their responsibilities with regard to Equality and Diversity.
- Failure to follow the policy and associated procedures will lead to disciplinary or other appropriate action.
- Crescent Community Care Services Limited’s aims and objectives as set out in this policy, will be achieved through training, effective monitoring, and a willingness to promote Equality and Diversity, addressing issues where they arise.
- Our training, publications, interaction with stakeholders, and other activities, Crescent Community Care Services Limited will ensure that those we work with know our statements of policy and understand our commitment to promote Equality and Diversity.
- Crescent Community Care Services Limited will review, on an annual basis the content and implementation of its Equality and Diversity Policy and strategy. Where evidence is found of ineffectiveness, lessons will be learnt and action taken to ensure improvements are made.
- Definition of protected characteristics and what we understand by discrimination.
- The Equality Act 2010 makes it unlawful to discriminate against people with a 'protected characteristic'. These are:
    - Race, including colour, nationality, ethnic or national origin;
    - Sex;

- Pregnancy or Maternity;
- Disability, including physical, sensory, HIV, cancer, mental health or learning disability;
- Age;
- Religion or belief;
- Sexual orientation;
- Gender re-assignment;
- Married or civil partnership status.

- In addition, if Crescent Community Care Services Limited provides services for Service User that are commissioned by the Local Authority or other Public Body there is a requirement under the Equality Act 2010 that there is an additional 'Public Sector Equality Duty'. This means that {Client_Name_Official) must foster good relations between people with protected characteristics and those that do not, have due regard to eliminate discriminatory practice, promote Equality and eliminate unlawful discrimination. The Crescent Community Care Services Limited will ensure these duties are met.
- Crescent Community Care Services Limited understand Discrimination can take place in a number of ways:
  - Directly; e.g. by not appointing someone because of their skin colour;
  - Indirectly; e.g. by requiring all male staff to be clean shaven (which discriminates against some beliefs);
  - Association; e.g. because an employee's partner has a re-assigned gender;
  - Perception; e.g. because an individual is thought to be disabled when not;
  - Harassment; e.g. Causing distress to a person on the basis of their religion;
  - Victimisation; e.g. treating adversely someone who has complained in the past, such as a whistleblower;
- Crescent Community Care Services Limited's Equal Opportunities ensure that policies, procedures and practice within Crescent Community Care Services Limited do not discriminate against the people within it and those who come into contact with it. It is about treating people fairly and equally.
- Diversity means that all people are valued as individuals and are able to maximise their potential and contribution to Crescent Community Care Services Limited and to the community. It recognises that people from different backgrounds, can bring fresh ideas, and a different approach to the services we provide.
- Positive Action refers to measures taken in order to assist employees or learners who have been under-represented in specific areas, to reach a level of workplace knowledge and competencies that is comparable with 'representative' employees.
- We recognise that part-time workers and workers on zero hours contracts are to be treated no less favourably than full time workers.
- We also recognise that agency workers acquire basic rights after 12 weeks employment, meaning they are entitled to comparable terms and conditions as comparable "permanent" workers, and the same rights relating to unfair dismissal and redundancy.
- Where employees are being recruited to or promoted to any position that is exempt from the Rehabilitation of Offenders Act we may ask the following question of applicants:
  - "Do you have any convictions, cautions, reprimands or final warnings that are not protected as defined by the Rehabilitation of Offenders Act 1974 (Exceptions) Order 1975 (as amended in 2013)?"
- Applicants to exempted positions will be subject to a Disclosure and Barring Service (DBS) check. Risk assessments will be undertaken where relevant information is revealed either at interview or from a subsequent check.
- Crescent Community Care Services Limited recognise that it is unlawful to discriminate against anyone because they are either a member of a trade union or religious group.

Implementation

Expectations

- The Registered Provider has responsibility for implementing and developing the policy. The overall coordinating responsibility for equal opportunities and management of diversity is delegated to the Registered Manager (Mrs Louise Gould).
- However, Crescent Community Care Services Limited believes that all who work with or for the organisation have an individual responsibility to understand and use the policy and ensure a personal involvement in its application and to promote Equality and Diversity.
- Therefore, Crescent Community Care Services Limited expects individuals:

- To have formally recorded that they have read and understood the policy.
- To have undertaken training in Equality and Diversity and to understand their individual responsibilities as defined by this policy and the Equality Act 2010.
- To cooperate with measures introduced by Crescent Community Care Services Limited to ensure equality of opportunity, diversity, non-discrimination and positive reinforcement.
- Not to harass, abuse or intimidate any other employee, job applicant, volunteer, trustee, member, Service User, contractor/agency, stakeholder or participant in relation to any of the characteristics described in the Definition section above or for any reason whatsoever.
- To inform the Registered Manager if they suspect discrimination is taking place in any shape or form. Or if they suspect the Registered Manager of discrimination in any shape or form they should inform the line manager of the Registered Manager.
- To raise matters through the grievance procedure if they are the subject of discrimination.
- To follow the Whistleblowing Policy and Procedure if they have a reasonable belief that the matter relates to (and only if it relates to) the public interest No employee or other person covered by the scope of this policy will suffer detriment where matters are raised in good faith or reasonable belief.

- Crescent Community Care Services Limited expects Line Managers:
  - To ensure that proper records of employment decisions are maintained that are consistent with this policy and regular reviews of employment practices are carried out.
  - To ensure that grievances are dealt with in a fair and consistent manner and in line with our Grievance Policy and Procedure.
  - To ensure that within their area of responsibility individuals are aware of their legal obligations under the Equality Act 2010, and of the organisation's Equality and Diversity Policy and Procedure.
  - To ensure that the highest standards of Equality of Opportunities practice are observed in the delivery of Crescent Community Care Services Limited services and to undertake training and development opportunities to ensure that their competence is maintained at all times.
  - To provide or arrange training for staff that they are directly responsible for in Equality and Diversity and to monitor the understanding of staff, and if shortfalls in understanding are identified additional training to be provided.
- The person with responsibility for Equality and Diversity will:
  - Actively promote the benefits of employee and participant diversity, in employment, services, training and all other activities associated with the Crescent Community Care Services Limited.
  - Maintain a full awareness and understanding of the legal framework for Equalities and Diversity and ensure that all staff are aware of the implication of any changes on policies, procedures and practice.
  - Seek the views and opinions of employees, volunteers, customers and clients on the operation of the policy in his/her locality/area of responsibility, in particular to meet the diverse needs of the users of the service.
  - Offer advice and guidance to members of staff, volunteers and organisations in Crescent Community Care Services Limited's equality and diversity policy and procedures.
  - Ensure that Managers are supported in their roles with regard to the Equality and Diversity Policy and Procedure.
  - Ensure that Managers, including Registered Managers, are appraised regularly on the understanding and implementation of equal opportunities and diversity within Crescent Community Care Services Limited and the public domain.
  - Ensure that the Equality and Diversity Policy and associated documents are reviewed on an annual basis and any amendments or additions are cascaded to all employees, agencies and those who have a business relationship with Crescent Community Care Services Limited.
  - Review and approve policies, procedures and practices that impact on equal opportunities and diversity in practice.
  - Coordinate the delivery of an equality and diversity strategy and action plan to be monitored and reviewed on a regular basis.
  - Facilitate training and open discussion on equal opportunities and diversity issues as appropriate.
  - Ensure reasonable adjustments are made to working practices, equipment and premises and offer, where appropriate, additional support to trustees, staff and volunteers to ensure that they are able to play a full and active part in Crescent Community Care Services Limited's work.
  - Where new processes are planned, ensure that an Equality Impact Assessment is carried out to establish if any discrimination is likely to occur as a result of the change, and to plan any remedial actions.

Recruitment and Publicity

- Crescent Community Care Services Limited strives to ensure that all our employees, job applicants, volunteers, trustees, members and Service Users reflect the wider community.
- We will provide accessible and accurate information on vacant posts through advertisement, covering job descriptions, person specifications and interview arrangements. Wherever appropriate, vacancies will be advertised sufficiently to reach the widest possible range of candidates, either internal and/or external with positive action used where needed.
- Applicants will be informed through all recruitment material of Crescent Community Care Services Limited's commitment to equal opportunities and diversity and the existence of this policy.
- Care will be taken to ensure that 'essential' and 'desirable' requirements in Person Specifications are not discriminatory.
- Job titles will be non-discriminatory.

Interviews and Selection

- So far as reasonably possible, short-listing and interview panels will reflect the diverse makeup of Crescent Community Care Services Limited.
- Staff involved in recruitment will have appropriate equality and diversity training to ensure that they understand the implications for recruitment.

Training

- In line with this policy, Crescent Community Care Services Limited will not discriminate in the provision of training courses, promotion, mentoring, secondment or other opportunities.
- Appropriate training will be provided to enable staff and volunteers to perform their jobs effectively. The training offered will take into account the needs of all people and their 'protected characteristics'.
- Briefing on this policy will form part of the first day Induction Procedure for all staff, including senior staff and volunteers.
- Staff and volunteers are encouraged to discuss their development and training needs, through a process of regular support and annual appraisals, to include an annual skills audit of employees.

Services Users

- Crescent Community Care Services Limited will endeavour to deliver services in a way that genuinely recognise the importance of an inclusive society that brings opportunities and access, not barriers and disempowerment, to individuals.
- Crescent Community Care Services Limited will strive to ensure that our Service Users' reflect the community which Crescent Community Care Services Limited serves.
- The Crescent Community Care Services Limited contract for the delivery of the service will ensure Service Users are aware of their responsibilities to comply with the equality and diversity policy.
- All advertising or information directed to Crescent Community Care Services Limited will not imply any preferred group, unless a genuine qualification exists limiting a vacancy, such as a condition of registration or the specialism of the service or Service User Guide.
- All promotional material and information of Crescent Community Care Services Limited's will display text relating to Crescent Community Care Services Limited's commitment to equality and diversity.
- Guidelines for admission are not discriminatory and are in line with the Equality and Diversity Policy and Procedure.

Grievances

- Any staff member or volunteer who feels that they have been a victim of unlawful discrimination or unfairly treated in a way contrary to the intention of this policy should raise the issue through Crescent Community Care Services Limited's established Grievance Policy and Procedure.
- Any Service User who feels that they have been unfairly treated in a way contrary to the intention of this policy should make a complaint through Crescent Community Care Services Limited's Registered Manager, who must report any such complaint to the Registered Provider. If the complaint is about the Registered Manager, this should be made directly to the Registered Provider.
- All incidents of discrimination by Service Users should be raised with the Registered Manager, and in the event of a failure to agree satisfactory remedies, will be dealt with under the terms of the Service User agreement/contract.
- Any job applicant who believes that they have been treated unfairly and contrary to the intention of this policy should raise the issue with the Registered Manager or the Registered Provider.

Breach of Policy

- Where it appears that there may have been, or there is, a breach of the policy, the Manager will investigate the

circumstances and action will be taken.

- If the breach involves the Registered Manager, the Registered Provider will carry out the investigation. This may be a delegated responsibility (delete as appropriate).
- Any member of staff found to be in breach of this policy will be subject to disciplinary action.
- Any volunteer found to be in breach of this policy will be counselled on their actions and may, where necessary, be removed from Crescent Community Care Services Limited's volunteer register.
- Any Service User found in breach of this policy will, where appropriate, be counselled on their actions and may, where necessary, be refused future services from Crescent Community Care Services Limited.

Monitoring

- Crescent Community Care Services Limited recognises the need for a continuing commitment to genuine equal opportunities and diversity within the organisation. The effectiveness of the policy's aims and objectives can only be judged by how the policy operates in practice.
- If the Crescent Community Care Services Limited provides services for Service User that are commissioned by the Local authority or other public body there is an expectation under the Equality Act 2010 'Public Sector Equality Duty' {Client_Name_Official) should be transparent and share information about how well Equality is being promoted.
- As such the {Client_Name_Official) will establish objectives with regard to Equality and will monitor the success of these objectives and establish plans to improve the delivery of these objectives.
- The collection/analysis of data is vital in informing change and improving performance. Where appropriate, statistics on Crescent Community Care Services Limited's services will be collected and analysed in relation to Equality and Diversity.
- We will review employee turnover and seek information on reasons for leaving through exit interviews. Local and national data or statistics will be used to benchmark our performance.
- The Registered Provider and Registered Manager will annually review equality of opportunity relating to Crescent Community Care Services Limited services. Recruitment and selection procedures will be monitored and reviewed annually by the relevant Manager who will report to the Registered Provider. All aspects of personnel policies and procedures shall be kept under review in order to ensure that they align with this policy.
- In order to determine the impact of this policy it is important that a monitoring system is developed, which will measure commitment, progress and effectiveness and good practice. The Equality and Diversity Policy will be monitored and reviewed as follows:
    - The policy will be an annual agenda item at Crescent Community Care Services Limited's quality team meetings.
    - The relevant manager will undertake an annual policy review for Crescent Community Care Services Limited. All relevant parties will be encouraged to submit comments for consideration.
- If it is found that the policy is excluding or discouraging the development of staff or volunteers or restricting Service Users, the Registered Manager will take steps to amend the policy.

Reviewed Marcus Kerridge-McColl

December 2025

Double Up Home Care Policy and Procedure
Crescent Community Care Services

Purpose

- To ensure that services are delivered to contract, and in a safe and personalised manner**.**
- All care delivery requiring two or more carers in attendance.

Policy

- The need for double up service will be subject to normal assessment and management processes, with the objective of ensuring that the service is robust, safe, and meets the Service User's needs.

Procedure

- **Management:**

  - A competent assessor will always carry out the assessment of the need for Double up service, which will be as a result of an opportunity or need, and carried out within the normal assessment and review procedures of The Agency.
  - Double up will normally be proposed in response to moving and handling process needs, but other issues may give rise to a proposal, such as security. The objective of the double up service must be clearly identified in the Care Plan.
  - The Service management must, following the creation of a double up requirement in the Care Plan, identify and allocate the second carer using the same processes to match skills, knowledge, personality, gender and reliability which are used for primary/single carers.
  - The Service User will be fully involved with the assessment which gives rise to the need for double up and will consent to the Care Plan.
  - The care service coordinator will organise the logistics of providing a double up service, and communicate the service design including the risk factors, to all staff concerned, and the Service User and their family/advocate.
  - The service design will be fully annotated within the Care Plan for the individual Service User with the objective of ensuring that the service is stable, consistent and independent of individual service managers on duty.
  - The care service coordinator will, on notification that only one carer is in attendance on a particular visit, discuss the immediate needs of the Service User with the carer and instruct them regarding the limits of what processes they can safely carry out, and make alternative arrangements for any processes which must be carried out for the safety and comfort of the Service User.

- **Communication and familiarisation:**

  - New carers must be introduced to the Service User at the beginning of their first attendance. The lead carer will brief any second carer who has not previously attended that Service User before the introduction, preferably before entry to the premises.
  - The lead carer has responsibility for all recording of services delivered within the Care Plan documentation, communication with service management, communication with other health professionals, and family/advocate communications.
  - The lead carer will attend all service reviews.
  - Both double up carers must ensure that they communicate primarily with the Service User during their visit, involving them in the conversation at all times, and never talking over and excluding them while discussing about matters not related to the processes in hand.

- **Lead carer:**

  - In each double up situation, one person will be formally designated by the service management as the lead carer for that Service User. The lead person will not always be the most senior carer, but will typically be the carer who is normally permanently allocated to that Service User and who therefore best knows their detailed preferences such as preferred mode of address.
  - A lead carer is also required to ensure that moving and handling processes are not subject to misunderstandings and therefore increased risk, and that processes are agreed before use and enforced by the lead carer.

- **Security and safety:**

  - The lead carer retains responsibility for the security of the Service User, and the premises, at all times.
  - Each carer will notify the service office immediately if the second carer unexpectedly does not attend.
  - Carers will not, under any circumstances, attempt identified double handling processes without the second carer being present and participating. In the event of non-attendance by a second carer the carer in attendance will contact the service management to discuss alternative courses of action for double handling, if any.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To meet the requirements of Section 20 of the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014.
- To define the obligations of Crescent Community Care Services Limited to act in an open and transparent way in relation to Service User care and treatment.

Scope

- Any notifiable incident which occurs during the provision of a regulated activity.

Policy

- The Provider, individual or organisation, and their representatives, must make public commitments to transparency, openness and fairness; demonstrate their compliance with this standard of behaviour in daily practice and communication; support employees at all levels to follow the commitment; and not undermine their efforts to do so.
- The publication and dissemination of this policy can be regarded as a statement of commitment to transparency and openness by Crescent Community Care Services Limited.
- The Provider, individual or organisation, and their representatives have the primary responsibility for the effective execution of this policy. In normal practice, the Registered Manager may carry out the procedure set out below, unless the Provider considers that the nature of the incident means that the investigation may be compromised if the Registered Manager carries it out, in which case the Provider should delegate a representative of the Provider to manage the procedures.
- Service Users and their representatives must be informed as soon as reasonably practical of any notifiable incident.
- A notifiable incident is defined as any unintended/unexpected incident in respect of the Service User during the provision of a regulated activity, which in the reasonable opinion of a health care professional could have or has resulted in:
    - The death of a Service User or;
    - Severe harm, moderate harm or prolonged psychological harm:
        - **Prolonged psychological harm** can be defined as psychological harm which a Service User has or is likely to experience for a continuous period of at least 28 days.
        - **Moderate harm** can be defined as harm that requires a moderate increase in treatment, including re-admission, prolongation of care, admission to hospital, referral to hospital as an outpatient, cancelling of treatment that is otherwise needed, or transfer to another specialist facility or treatment area.
            - Moderate harm also includes significant but not permanent harm.
        - **Severe harm** can be defined as a permanent reduction of bodily, sensory, motor, psychological or intellectual functions, including procedures carried out on the wrong person, or wrong area of the body of the right person.

Notifications should include:

- Notification that the incident has occurred.
- An apology – this is different to an admission of liability. Whilst it is appropriate to express sympathy or regret, the apology should not include any admissions of fault.
    - Note that the Complaints, Suggestions and Compliments Policy and Procedure already states that open and responsive communications must be established with a person who complains (in the context of this policy the complainant is the service itself in the first instance, because it is the service which has identified its own incident), and that an apology is the best way of maintaining open communication and taking heat out of the situation in order that it can be approached professionally, rationally and logically. Therefore, if the Complaints, Suggestions and Compliments Policy and Procedure is fully and effectively implemented, there should not be a reluctance to acknowledge a problem and apologise for its occurrence, without defining blame.
- What further enquiries will be taking place – if an internal investigation is to take place, who will be interviewed and whether external input will be obtained and when enquiries might be completed. Note that good quality assurance practice dictates that an internal investigation will always take place when an untoward incident (not just an incident as defined under this regulation) occurs, in order to provide quality improvement data for the management process. Refer to the Quality Assurance, Quality Meetings and Management Meeting Policy and Procedure documents within the QCS system.
- A factual account of the incident – it is important that the account is purely factual and should avoid expressing an opinion as to the cause of the incident or admitting any blame.
- Confirmation of when an update will be provided – the further notification should include an apology and details of the outcome of any further investigations.
- Where for any reason the Service User cannot be contacted, or after contact declines to communicate with the service, a written record of all attempts to contact them must be kept.

Reporting a possible breach of candour:

- If any individual considers that a breach of candour has taken place, i.e. a notifiable incident has not been reported as outlined above, they

must report their allegation to the Registered Manager:

  - The Registered Manager will conduct an investigation into the allegations and report the findings to the Provider for action if appropriate.
  - If the allegation concerns the Registered Manager, the individual must report the matter to the Provider directly, who will carry out the investigation and take any action which may be required.
  - If the allegation concerns the actions of the Provider, the individual must inform the Provider, and if action is not seen to be taken, the matter must be reported to the Care Quality Commission. In this event the Provider must keep the reporting individual informed of progress, and carry out corrective action, if they wish to avoid referral to the Care Quality Commission.

- If a breach of candour is found to have occurred following investigation, and that this breach was by a professionally registered person, then that person should be reported to their professional registration body for further consideration. The same action should be taken if, during the investigation, it is found that a professionally registered person had obstructed another person in their professional duty of candour.

Procedure

Awareness

- All staff must be made aware of their personal responsibility to report incidents which may be covered by this regulation, through exposure to this policy and procedure with adequate time to read and understand it. Group implementation sessions would be a useful means of achieving this understanding, supported by inclusion of the matter in the next employee supervision session to test their retained understanding:
  - Remind staff of the Accident and Incident Reporting Policy and Procedure, which overlaps to a significant extent with this policy. If the Accident and Incident Reporting Policy and Procedure is fully and effectively implemented, staff will already be aware of the need for vigilance and the need to report not just accidents or incidents, but also "near misses" i.e. incidents which may have been stopped or ceased before any harm was done, but which if not stopped in time may have caused harm to an employee or Service User. As such the Accident and Incident Reporting Policy and Procedure already goes beyond this policy scope, but does not include the requirement to notify the Service User or their representative.
  - Note that the Serious Incident Notification Policy and Procedure also overlaps with this policy, within the scope of notification to the CQC.
  - Remind staff that attempts by other staff to prevent them from reporting incidents is bullying and/or harassment, and that they should report this immediately to their manager, or if the pressure is from their manager, a representative of Crescent Community Care Services Limited.
  - Remind staff that should the incident appear at first to be on the borderline of reportable and non-reportable. It should be reported anyway.

Reporting, Investigation and Communication

- All staff must report incidents defined in this policy in a way which generates an understandable and permanent record, for instance using the attached forms even if a verbal report has been already been made. The report must be made to the person on duty and in charge of the service at the time of the incident, who in turn must formally report it to the Registered Manager if they are not the same person, as soon as is practical, with regard to the seriousness of the incident. For example, a serious incident, may require immediate notification to the Registered Manager or their delegate even if they are not on duty at the time. A less serious incident may only require to be brought to their attention when they are next on duty. If in any doubt, report immediately.
- The Registered Manager or delegate should:
  - Carry out an initial assessment of whether the report does include details of a notifiable incident under the regulations. If the conclusion is yes, or borderline, continue with this procedure. If it is considered that the report does not constitute a notifiable incident, discuss that conclusion with the Service User and the staff member making the report. If they agree with the conclusion, follow normal incident reporting procedures as set out in the Accident and Incident Reporting Policy and Procedure. If they do not agree and continue, after consultation, explanation and negotiation, to hold the opinion that the incident is reportable under the regulations, then proceed with this procedure.
  - Inform the Provider or their representative of the incident report, and agree with them who is the most appropriate person to continue the procedure. If the Provider takes over the role, the Provider should continue the process using the following procedure.
  - Prepare and deliver as soon as is practical a notification of the incident (or borderline incident which may not yet have been fully defined) to the Service User and their representatives. While this can be done verbally, and may need to be done in that way if circumstances dictate, such as the incident being very obvious at the time and needing to be acknowledged on the spot, good practice would be to gather at least outline information and record it, and quickly develop a notification statement with some thought and consideration behind it. This could be done in rough on the notification record attached simply in order to clarify matters and ensure all available data has been considered, then transferred to a more formally formatted copy later. It is important to retain all records permanently, including rough notes, written notes on verbal exchanges, and written notes on discussions with the Service User and representatives. The latter could usefully be recorded on the Family and Service User discussion record sheet from the Care Plan.
  - Include an apology that the incident occurred, without apportioning blame in any way.
  - Inform the Service User and representative of sources of support and information which may be of assistance to them. Offer them support from within the organisation if possible, for instance by allocating to them staff other than those in whom they may have lost confidence as a result of the incident. External resources such as advocacy services should also be considered and offered if appropriate.
  - Prepare a list of people to be interviewed in order to gather data statements; also any other information which may assist an

investigation, and include that information in the notification.

- Inform the Service User and representative of the likely progress of the investigation and the estimated date of the final conclusion, or any proposed interim stage. If this is done separately to the initial informing of the incident, include the apology made in that document.
- Identify and inform any other notification requirements, for instance CQC, Health and Safety Executive, GPs, Care Commissioners, Pharmacist, Adult Abuse Team, DBS, insurers (who will require notification as soon as possible and who may become involved with investigations), Environmental Health, and/or regional or national management if appropriate to the organisation.
- Prepare a list of people to be interviewed in order to gather data statements; also any other information which may assist an investigation.
- Carry out the interviewing and data gathering identified, adding any further sources which may be identified as the investigation proceeds.
- Having gathered all the evidence you think you need, carry out an investigation and reach a conclusion. Record your deliberations and how the various factors were weighed and balanced, and keep the record. Seek outside or independent advice if you feel that you are lack the skills or knowledge to effectively weigh the factors and reach a conclusion you are confident of.
- During the process this far, if possible avoid identifying individual person involved by name. The purposes of the investigation are to establish if the incident actually happened, define its nature, gather facts about the processes around the incident, and identify causes where possible. At this stage no blame can be attached to any person or process because the facts are not yet known, therefore to identify individuals by name may be unfair to them.
- Prepare a statement to be given to the Service User and representative stating the outcome of the investigation. Remember that the underlying objective of the regulation is that the service be open, honest, fair, and responsive in its response to the incident. The final statement should include a more specific apology than the generic one issued with the first notification, because the causes of the incident should have been established. However, it is very advisable to discuss this area with your insurers, who will have competing concerns to your own obligations under the legislation, will not wish you to admit liability. If a conflict does arise between the obligations under legislation and the insurer's views, the legislation must be followed.

If a Breach of Candour is found to have taken place

- Follow the policy set out above. Use the accident/incident reporting form attached to make the report in a full and accountable manner.

Reviewed Marcus Kerridge-McColl

October 2025

Emergency Communication Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that there is effective communication in an emergency situation

Scope

- All employees and Service users / families

Policy

- The Registered Manager has a 24 hour on call responsibility, except for where the is a specific arrangement made to the contrary and is clearly communicated to all members of staff and Service users
- The On Call Care Manager, or the Duty Manager may be contacted out of hours and they will have access to the Registered Manager or Directors if guidance is required.

Procedure

- If advice is required then contact the On Call or the office
- In the event of any emergency including carer illness, nonattendance of work or for Service Users and their families the Office or the On Call must be informed immediately
- Any care worker unable to access a Service users property must call the office or On Call immediately

- The On Call will always have a Duty Manager that they can gain advice from
- If the Duty Manager is unable to offer advice or support then they may contact the Registered Manager and or the Directors
- All relevant telephone numbers are kept in the office and by the On Call Care Manager
- The On Call Care Manager telephone number for out of hours is 07958 789359
- The On Call Care Manager and the office also have a list of other emergency numbers including the Emergency Services, Duty Social Workers and a list of all the Service users GP's

Reviewed Marcus Kerridge-McColl
September 2025

Entry to Service Users' Premises and Safekeeping of Keys Policy and Procedure
Crescent Community Care Services Limited

Purpose

To comply with best practice safety requirements and protect from security problems.

Scope

All premises entered by Crescent Community Care Ltd staff members, or for which Crescent Community Care Ltd holds keys.

Policy

- The security of access to Service Users' homes is of paramount importance, and should never be compromised.
- Strict procedures must be followed by all agency staff having access to Service Users' premises, their keys or their access codes.
- Employees of The Agency must not take keys directly from a Service User without the knowledge of the office.
- Keys should never carry a home address or telephone number; this is for the safety of Service Users.
- If keys are lost, the loss must be reported to the manager immediately.

Procedure

- All visits to Service Users' homes will be planned in advance, and clearly logged on the on-site attendance documentation and notified to the administration office.
- Any changes or exceptions to the above policy will be notified in advance and recorded as specified.
- Carers will not hold keys to Service Users' homes unless by prior arrangement with the Registered Manager.
- Holding of Service Users' keys will be on an individual basis, not a matter of regular practice, and supported by a needs assessment which will be kept in both the on-site records and the service administration office.
- A receipt will be issued for keys held.
- Keys will be kept secure in a key safe in the service administration office, and will be issued to the Carer attending the Service User's home as required, and returned when not required. Issue and return will be recorded.
- The address will not be fixed to the keys. Use a code, such as the Service User file reference number, or a specific code held in that file.
- On cessation of service, or change of circumstances which no longer require keys to be held, the keys will be returned and a receipt obtained.
- Carers attending a Service User's home will at all times wear their identification badge.
- Carers attending a Service User's home will ring the doorbell or knock so as to announce their presence on opening the door.
- If the Service User's home has an entry code, all reasonable Care must be taken to ensure that the code is kept confidential, and the administration officer informed immediately if there are any concerns about the security of the door entry procedure, whereupon administration must make immediate arrangements for a change of code.
- Entry codes will be changed at random intervals not less frequently then every three months, and in every case where there is a change of Carer.

- If Carers are unable to gain access to the home, or access is given but treatment is refused, they should contact the administration office immediately to inform their line manager and leave a note that they have attended. The line manager will contact the family to inform them of the issue of access and the registered manager will decide if any further action is required, including the need to notify social services or the emergency service, based on risk assessment and knowledge of the family. This may be referred to as a Non-Access Policy by some authorities.
- Time for Care visits must be strictly observed and if the Carer is delayed for any reason the family must be notified.

Reviewed by Marcus Kerridge McColl
November 2025

Fair Access, Diversity and Inclusion Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To promote equality of opportunity, diversity, inclusion in employment and the delivery of goods and services regardless of sex, marital or civil partnership status, gender reassignment, race, colour, nationality, ethnic or national origins, sexual orientation, disability, religion or belief, political affiliation and age and to overcome discrimination due to visible and non-visible differences. The purpose is also about respecting and valuing other people.

Scope
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- The Fair Access, Diversity and Inclusion Policy applies throughout Crescent Community Care Services Ltd including:
  - Recruitment and Employment practices;
  - Training and development;
  - Service and product procurement (contractor / supplier standards).
- For employees, compliance with this policy is a condition of employment within Crescent Community Care Services Ltd services.
- People who use or seek Crescent Community Care Services Ltd services shall be protected by the policy. Equally they shall be expected to respect the rights of others under the terms of the policy.

Policy

- Crescent Community Care Services Ltd upholds the right of people who use its services, its employees and volunteers to be treated fairly and without discrimination at all times. Crescent Community Care Services Ltd also further upholds the right of those people to express their concerns or complaints and for them to be heard and dealt with appropriately.
- Crescent Community Care Services Ltd as an employer and service provider shall take all reasonable and practical measures to ensure its practices are free from discrimination and inclusive and accessible to all.
- Crescent Community Care Services Ltd wishes to set minimum standards in accordance with the law and the requirements of its regulators and funders, and will ensure that service delivery, employment and the supply of goods and services will be carried out in line with these standards. Those standards are set out in the accompanying sections of this policy.
- In summary, standards shall:
  - Promote ‘equality of opportunity’;
  - Address and tackle discrimination in proactive and positive ways;
  - As far as is practically possible, actions under this policy will be proactive rather than reactive in nature.

Aims of policy Service Users

- There shall be a clearly worded statement in an accessible format or with an easy read version (delete as appropriate) which shall be made available and posted clearly in all services and which provides a summary of the Policy Statement.

Learning and development

- This policy sets out the basis upon which the employees shall be expected to demonstrate their learning and development.

Public reference

- Crescent Community Care Services Ltd is required to show publicly that it has adopted a policy covering Fair Access, Diversity and Inclusion, more commonly known as 'equal opportunities'. This policy document and its summary demonstrates Crescent Community Care Services Ltd's public commitment.

Responsibilities
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- **Marcus Kerridge McColl** is responsible for promoting Equal Opportunities both in the home and in the community settings.

- Individual members of the **Crescent Community Care Services Ltd** Management Team are responsible for the detailed implementation of actions under this policy within their areas of responsibility and for ensuring that the review of this policy is carried out in a timely manner in consultation with relevant experts in the field.

- **Marcus Kerridge McColl** is the lead member of the **Crescent Community Care Services Ltd** Management Team for the implementation of this policy.

- **Marcus Kerridge McColl** is specifically responsible for ensuring that monitoring and performance data, relating to the recruitment, employment and development of **Crescent Community Care Services Ltd** employees, is updated and made available at quarterly intervals to senior managers, kept/stored securely and disseminated only in accordance with laid down guidelines and the Data Protection Act.

- All managers of **Crescent Community Care Services Ltd** who are responsible for press, publicity and marketing strategy will ensure that any actions arising from the policy are addressed in external statements and will reflect as one of the key tenets of Crescent Community Care Services Ltd services.

- All managers will:

  - Embody best practices at all times in promoting Fair Access, Diversity and Inclusion within **Crescent Community Care Services Ltd** services and workplace settings;
  - Ensure that new employees during the first day of induction are made aware of this policy and its application within their role, day-to-day work and contact with clients, Service Users and all stakeholders;
  - Consider the need for Personal Development Planning and regular briefings to address any matters under this policy;
  - Actively address with individual staff members (through formal and informal supervision and appraisals) as well as teams, any issues arising from this policy.

- All Staff must at all times abide by this policy. Breach of this policy may result in serious consequences.

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Legal and regulatory context

- Key groups of people and some types of discrimination that are dealt with under statute include:

  - Equal pay;
  - Gender Issues;
  - Race (includes culture and language);
  - Disability (physical and mental health, learning disability, sensory impairment etc);
  - Religious beliefs or non-belief;
  - Sexual orientation;
  - Gender Realignment (Recognition of);
  - Age;
  - Mental Incapacity;
  - Housing (accessibility).

Shortened policy statement
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- An abbreviated and easy to read summary of this policy is produced in the Appendices and shall be displayed wherever **Crescent Community Care Services Ltd** is responsible for delivering services.

- It is important that Service Users, employees and volunteers are reminded of their rights and responsibilities, in their contact with or for **Crescent Community Care Services Ltd**, and that breaches of this policy will be taken very seriously. Swift action will be taken to address any breaches.

- Employees must note that only Shortened Statements which have received **Crescent Community Care Services Ltd** senior management approval may be displayed.

Standards

Individual and personal behaviour

- The culture of **Crescent Community Care Services Ltd** is demonstrated through the standard of behaviour and attitudes shown by employees towards others. The culture of any organisation is also demonstrated by the style of leadership adopted by managers and other senior people.

- Recognizing and respecting our clients, Service Users and employees through the promotion of this policy creates a cohesive team and equips Crescent Community Care Services Ltd to meet business and competitive challenges full on.

Learning about ourselves and each other

- **Crescent Community Care Services Ltd**'s philosophy embodies a culture which values people. In recognising the importance of practices that ensure fairness of access, diversity and inclusion, it is equally important to recognise that all individuals carry their own learning and experiences that have shaped the way that they understand and respond to others.

- Cultures which are based upon blame and persecution, as well as the misconstrued and excessive use of the term 'political correctness', are far from suited to supporting ongoing learning. Learning about ourselves and others depends upon a culture which values reflection, positive comment, transparency and constructive feedback as essential, albeit at times challenging.

- Challenging each other and our own attitudes and behaviour, without fear of condemnation, and learning from those experiences, helps everyone to ensure that people are valued and understood; this will promote a better understanding of what is necessary to ensure probity, accountability, inclusiveness and fairness within **Crescent Community Care Services Ltd** services and workplaces.

Listening and consulting

- The culture of **Crescent Community Care Services Ltd** is also demonstrated in the way it responds to the needs and expectations of people who use, or who may use **Crescent Community Care Services Ltd** services. It follows that consulting and listening is a vital part of promoting inclusion and fair access to all. Developing a culture that is responsive to external factors makes it possible to understand, plan and deliver services that are sensitive to the needs of the widest possible range of need in the community. By the same token, information about **Crescent Community Care Services Ltd** services must be presented in a way that is informative, helpful, timely, credible and user friendly.

Workplace / team value statements

- **Crescent Community Care Services Ltd** has adopted a culture whereby work teams are expected to adopt its particular value statements, in addition to those of **Crescent Community Care Services Ltd** as a whole. It is important, in order to promote consistency, that when setting and reviewing value statements this policy is considered as an intrinsic part of that process.

Service Users

- In responding to the needs of Service Users, **Crescent Community Care Services Ltd** will ensure fair access to its services. **Crescent Community Care Services Ltd** will make sure that everything it does is transparent, regularly reviewed and updated in order to meet the changing needs of current and future Service Users.

- Information, with staff support as necessary, will be made available to help to explain the Equal Opportunity Policy and any other key policy or procedure. This information shall be clear, concise and 'fit for purpose'.

- We will take into consideration any communication or language needs of clients, Service Users, and other stakeholders in all future service planning and development.

- **Crescent Community Care Services Ltd's** complaints policy will be made available to all Service Users, parents/carers, advocates, friends and befrienders and the public as we welcome complaints in order to maintain our standards and to improve service delivery.

- Service Users will be encouraged by staff at **Crescent Community Care Services Ltd** to self-advocate by making a complaint themselves if required or will be supported to access Advocacy Services to do so.

- Service Users are asked to respect the rights of others. There is a Safeguarding Policy (CR03, CR04) in place to protect them from abuse.

- **Crescent Community Care Services Ltd** will ensure that all buildings for which it is responsible will be accessible, safe and welcoming and will ensure that the environment is conducive for the purposes for which it is intended.

- **Crescent Community Care Services Ltd** will work and consult with other organisations and groups in a fair open and professional manner at all times.

Service User inclusion

- Service User inclusion is pivotal to the planning and delivery of all services so that they are relevant, of high quality, and sensitive to the needs of people who use them. Crescent Community Care Services Ltd therefore acknowledges that Service Users are 'experts by their experience'.

Suppliers

- **Crescent Community Care Services Ltd** will ensure that procurement and supplier selection includes an assessment of their ability to act consistently with this policy.

- **Crescent Community Care Services Ltd** recognises that the people who use its services have different needs but the right to the same quality of service. All people who use our services have the right to benefit from them without being subjected to direct or indirect discrimination or abuse from employees, contractors or other persons.

- The process of selection and procuring of suppliers will be undertaken in a way that is fair and non-discriminatory and shall be subject to monitoring, audit and regular review.

Employment (includes volunteers)

- **Crescent Community Care Services Ltd** provides equal pay without discrimination as defined under the Equal Pay Act 1970.

- **Crescent Community Care Services Ltd** encourages flexible work practices to reflect work life balance or ways of working to allow a diverse range of people into employment, and to help employees manage disabilities, illnesses or religious observances that occur during their employment.

- Guidelines will be issued to managers on the promotion & monitoring of standards. Any contraventions of Equal Opportunities standards will be fairly dealt with under robust and established systems in place.

- In order to review the effectiveness of the policy and to progress our diversity strategy, **Crescent Community Care Services Ltd** will maintain an up-to-date employee database that provides information which can be used to review and monitor equal opportunities within the workforce. This information will be kept separate from any other employee files or information stored and will be kept according to data protection rules and regulations.

Recruitment (includes volunteers)

All new posts will be thoroughly evaluated for the opportunity of taking positive action, involving the use of statutory provisions of the Equality Act 2010 and the Disability Discrimination Act 2005 to deal with the under-representation of groups in **Crescent Community Care Services Ltd**'s workforce.

- The selection process for the recruitment of new staff and volunteers will be clear and transparent, based solely on relevant criteria.

- Recruitment procedures and recruitment training will aim to promote equal opportunities and avoid the possibility of discrimination from application through to selection.

- **Crescent Community Care Services Ltd** will aim to ensure that staff and Service Users representing a diversity of people will carry out the interview/selection process to reinforce user involvement.

- Recruitment documentation will state **Crescent Community Care Services Ltd's** commitment to Equal Opportunities and anti- discriminatory practices within its service delivery.

Training

- **Crescent Community Care Services Ltd** has a training policy that affects all employees. **Crescent Community Care Services Ltd's** ability to deliver excellence in its service quality is dependent upon a skilled and competent workforce for which learning and continual improvement is a fundamental principle. It follows that principles of merit and equality are central to such a strategy.

- That policy shall set out the processes for all employees undertaking and benefiting from Learning and Development opportunities.

- Eligibility in accessing learning and development opportunities is dependent upon need and will 'fit' with each individual's job description. Access to any formal or informal learning opportunities is carried out in line with the training plan; **Crescent Community Care Services Ltd's** training policy and the individual's Personal Development Plan. In addition, access shall be based upon the need to address issues of equality among **Crescent Community Care Services Ltd** user groups.

- All Induction and Personal Development Plans shall positively address any instances where inequality may prevent someone's ability to participate fully, and will identify necessary measures to maximise outcomes for each individual.

- All managers will assist new staff in developing their understanding of this policy during their induction period. This will include the standards of expected behaviour and confidentiality in relation to others.

- All members of staff will be given ongoing opportunities to discuss and further develop their understanding and practice in relation to **Crescent Community Care Services Ltd's** Fair Access, Diversity and Inclusion Policy.

- Providers, consultants and participants involved in training activities should adhere and actively engage in implementing the Policy.

Methods of complaint

- **Crescent Community Care Services Ltd** is required to adopt a range of formal procedures which are designed to protect Service Users and employees from discrimination and victimisation. They include:

  - Complaints, Suggestions and Compliments Policy and Procedure – for Service Users/members of the public/staff;
  - Harassment Policy and Procedure – for staff;
  - Grievances Policy and Procedure – for staff and former members of staff;
  - Whistleblowing Policy and Procedure – for staff and volunteers.

Appendix

FAIR ACCESS, INCLUSION & DIVERSITY POLICY
SHORT STATEMENT
Service Users

Crescent Community Care Services Ltd recognises that the people who use its services have different needs but the right to the same quality of service. All people who use our services have the right to benefit from them without being subjected to direct or indirect discrimination or abuse from other persons.

Reviewed by Marcus Kerridge McColl
December 2025

Falls Management Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To apply best practices in the management of falls.

Scope

- All falls.

Policy

- All falls by Service Users, however apparently trivial, will be:

  » Immediately recorded in the Care Plan daily records for that Service User. » Immediately relayed to the Manager.

- The Manager will ensure that:

  » All staff are trained to record and report all falls.

  » All reports of a fall result in an immediate dynamic review of the falls plan of Care for the affected Service User, according to the formats in the Care Plan pack.

  » The Care Plan review is holistic.

Procedure

- The Manager will conduct an immediate dynamic review of the falls plan of Care for the affected Service User, according to the formats in the Care Plan pack.
- The Care Plan review must be holistic, that is, the review must look at other risk factors and assessments in order to identify and assess possible reasons for falls.
- The formats for a comprehensive falls assessment are contained in the Care Plan pack.

Reviewed by Marcus Kerridge McColl
November 2025

Family Leave Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with the Employment Relations Act 1999 and the Employment Act 2000.

Scope

- All employees.

Policy

- The organisation recognises that at various stages in employees' working lives, domestic and family commitments and responsibilities potentially reduce or hinder their ability to fulfil work responsibilities. Where such conflicts cannot be resolved, this could result in the company losing employees. To prevent such loss of skilled experienced workers, and to help reduce anxiety and stress amongst our workforce, this organisation not only acknowledges those potential conflicts but also offers practical help through its family leave.

Procedure

Statutory Paternity Leave

- Fathers and partners of mothers are entitled to a paid paternity leave. Partner means a person who lives with the mother and the child in an enduring family relationship, but is not the mother's parent, grandparent, sister, brother, aunt or uncle. Parental rights extend to partners in same-sex couples.

- Fathers and partners are entitled to two weeks statutory paid paternity leave. This must be taken as a single period of one week or two consecutive weeks, and must be taken within 56 days of the birth or 56 days of the first day of the EWC, whichever is later. Odd days cannot be taken, or two one-week blocks.

- The pay is called paternity pay (birth) to distinguish it from the pay available to adoptive parents. The weekly rate is either £100 or 90% of the employee's normal weekly earnings, whichever is lower.

- To be eligible for statutory paternity leave and pay the employee must satisfy the following conditions:

  - The employee must have had 26 weeks of employment with the employer by the end of the 15th week before the EWC;
  - They must have average weekly earnings of at least £77; and
  - They must be the child's father and have responsibility for the child's upbringing or, if they are not the child's father, they must be married to, or be the partner of, the child's mother and must have the main responsibility (apart from any responsibility of the mother) for the child's upbringing.

- Either before or during the 15th week before the EWC or as soon as is practicable after this, the employee must notify the employer as to how much paternity leave they intend to take, the EWC, and the date when they want the leave to begin. This date can be the date of the actual birth, or a date that is a specified number of days after the birth, or a predetermined date which is later than the first day of the EWC. This notice must be in writing.

Adoptive Parents Leave

- Pay and leave are available for the same period and at the same pay as statutory maternity entitlements for one adoptive parent when a child is matched or placed for adoption, and for the same period and at the same rate as statutory paternity entitlements for the other parent.

- The maternity-equivalent leave and pay are called ordinary (and, if applicable, additional) adoption leave and adoption pay. The paternity-equivalent leave and pay are called paternity leave (adoption) and paternity pay (adoption).

- Where a married couple adopt jointly, they can decide who takes the adoption leave and pay and who takes the paternity leave and pay (adoption). (At the moment, only married couples can adopt jointly). Where there is one adoptive parent,

they take the adoption leave and pay, and their partner, if they have one, takes the paternity leave and pay (adoption). The definition of partner is the same as for paternity leave (see above).

- Adopters and partners must have been employed by their employers for at least 26 weeks ending with the week when an approved adoption agency notifies the employee that they have been matched with a child for adoption, or the week when the child is placed for adoption. Adoption pay and leave are not available when the child is known to the adopter, for example a stepchild or foster child. Special rules apply where a child is adopted from overseas.

- To be entitled to adoption pay or paternity pay (adoption) the employee must have average weekly earnings of at least £77. Employers can recover statutory adoption payments from the Inland Revenue in the same way as SMP.

Parental Leave

- Employees who have at least one year's continuous service will be entitled to 18 weeks parental leave upon the birth of their child. This leave may be taken at any time up to the child's fifth birthday. This right also applies to employees who have acquired formal responsibility for a child, e.g. guardians.

- Employees who have at least one year's continuous service who adopt a child will also be entitled to 18 weeks' leave. The child must be under the age of 18. The right to parental leave lasts for five years from the date on which the child is placed for adoption.

- Parents of disabled children can take parental leave up until the child's eighteenth birthday.

- Employees who work part-time will receive a pro-rata amount of leave.

- An employee may not take parental leave in periods of less than one week (although parents of disabled children are allowed greater flexibility).

- An employee may not take more than four weeks' leave per year per child.

- Applications for parental leave must be made in writing to their manager and employees may be required to provide evidence of their responsibility for the child and the child's age.

- Employees must give at least 21 days' notice of an intention to take parental leave and should specify the dates.

- It may be necessary to postpone parental leave for up to six months (unless it is being taken immediately after a child is born or adopted) where the business could not cope or would be harmed by an employee's absence. Postponement will be confirmed in writing no later than 7 days after the employee's notice to take leave. This will state the reason for the postponement and set out the new dates of parental leave.

- Employees will remain employed during the parental leave period. Certain contractual terms continue to apply. In particular, the employer's obligation of trust and confidence, terms concerning notice, redundancy compensation, disciplinary and grievance procedures. Also the employee's obligation of good faith and any express term prohibiting Disclosure of confidential information or participation in a competing business.

- Payment will not be made for any parental leave taken.

- Where both parents are employed by the company, each will be entitled to leave but both employees may not take parental leave at the same time.

- Employees returning from parental leave of four weeks or less will be reinstated in their previous jobs. In extenuating circumstances parental leave may be extended at the company's discretion. Employees returning from parental leave of more than four weeks will be reinstated in their previous jobs wherever possible or, if not offered similar jobs which have the same or better status, terms and conditions as the previous jobs.

- Note: Since January 2002 there are three major changes in parental leave entitlements:

  - Parental leave is extended to parents of children born or placed for adoption between 15 December 1994 and 14 December 1999. The leave must be taken by 31 March 2005 (or up to the child's 18th birthday, if the child is entitled to disability living allowance);
  - Parental leave is generally available only to parents with at least one year's continuous employment with their current employer. However, for parents of a child born or placed for adoption between 15 December 1994 and 14 December

1999, employment of at least one year with a previous employer can count. The year's continuous employment must have been between 15 December 1998 and 9 January 2002. The employee must give the current employer notice of the period of employment with the previous employer, and must provide such proof as the current employer reasonably requires;

- For all children who were born or placed for adoption on or after 15 December 1994 and are entitled to disability living allowance, the period of parental leave is extended from 13 weeks to 18 weeks, to be taken before the child's 18th birthday.

Compassionate Leave

- Employees will be entitled to 5 days unpaid compassionate leave where a member of their immediate family dies, is seriously ill or in severe distress.
- Additional days of unpaid leave, up to a maximum of 10 days, may be granted at the discretion of the Registered Provider.
- Employees should contact the Registered Provider as soon as possible if they wish to make use of this provision.

Urgent Family Leave

- All employees are entitled to up to 5 days unpaid absence per year as family leave, in addition to annual leave and other absence entitlements.
- Family leave may be granted for pressing and important family reasons such as the complete breakdown of childcare arrangements.
- Requests for family leave must be made to the Registered Provider. Wherever possible, such requests should be made in advance. Where the reason for the request is family illness, a medical certificate may be required. Not more than 5 days' family leave may be taken at a time.

Reviewed by Marcus Kerridge McColl
December 2025

Fire Safety Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards. Š
- The Regulatory Reform (Fire Safety) Order 2005.

Scope

- All establishments owned or controlled by Crescent Community Care Services Ltd. Š
- All Service Users wherever located
- All employees

Policy

INTRODUCTION

- We recognise that fire prevention is an important obligation for all organisations, including ours, and that fire has the potential to present significant risks to our health and safety (see Fire Safety Guidance).
- It is our policy to:
  - Assess the risks of fire at our premises and implement appropriate control measures;
  - Ensure good housekeeping to minimise the risk of fire;
  - Provide means of detection and early-warning in case of fire;
  - Inspect and/or test fire safety equipment at appropriate intervals;
  - Provide and maintain safe means of escaping from the premises in the event of a fire;
  - Maintain all equipment and installations intended for fire-detection and fire-fighting;
  - Implement a procedure for the action to be taken in the event of a fire;
  - Train and instruct staff in fire safety including the carrying out of fire drills;
  - Keep records of all fire safety matters (see Fire Records Section);
  - Ensure that all visitors are made aware of the fire precautions and emergency arrangements;
  - Identify people with any disability or impairment who may require assistance in the event of a fire;
  - Comply with the requirements of the Regulatory Reform (Fire Safety) Order 2005 where one is in force;
  - Train staff in assisting Service Users in moving to a safe place.

Procedure

General

- Carry out a fire risk assessment.
- Repeat the fire risk assessment at least once a year, and always when there are significant changes to:
  - The building;
  - The processes carried out within the building;
  - The use of the building;
  - The number of people accommodated within the building;
  - The dependency level of the people accommodated within the building;
  - The equipment within the building;
  - The level of training and skills, including language skills, of the staff using the building. Š
- Identify persons at risk who may need assistance to evacuate or in moving to a safe place.
- Ensure that there is a method for giving warning of fires and that it can be heard in all areas of the establishment;
  - Ensure that the establishment can be evacuated safely in the dark or in the event of a power failure.
  - Display fire evacuation notices and ensure that all escape routes and exits are adequately signed.
- Ensure that all escape routes and exits are free from obstruction and all firefighting facilities are readily available. Š

- Maintain fire detection, warning systems and installations.
- Train staff, carry out fire drills and keep records.

Actions required prior to Evacuation

- The Registered Manager or Person in charge to assess the need for evacuation ensuring the safety of Service Users, visitors and staff.
- The evacuation plan is reviewed taking into account the evacuation procedures for individuals.
- Staff to be briefed on the reason, method, route and onward plan prior to evacuation.

Actions required during Evacuation

- The Registered Manager or Person in charge should ensure that a list is available at all times as to the people who have been evacuated and those still in the building
- The Service User and staff register to be held by the person in charge.
- Designated personnel should be established to man the external doors in the case of Fire Alarm activation ensuring that no Service Users leave the premises unaccompanied and that no inappropriate persons are allowed to enter the premises.
- In the event the emergency services are involved – the lead will be taken from the senior officer on site.

Actions required after evacuation

- The Registered Manager or Person in Charge should collate all of the information regarding the whereabouts of the Service Users and staff. Should it be necessary to contact relatives or next of kin, the Registered Manager or Person in Charge should ensure that all Service Users are in a specified location before any contact is made.
- Relatives of Service Users and staff to be contacted and notified of their whereabouts.
- In the event of a full evacuation of the premises, a designated person is to ensure the security of the building during this time.
- Staff that were not on shift at the time should be notified of the location of the Service Users and what their location of work will be until it is declared safe to return to the building.
- Notify Social Services and other organisations as necessary with the location of Service Users.
- The CQC is to be notified as per on the Statutory Notification form, of the events leading up to, actions taken and current situation with regard to the evacuation, this can be done by telephone but must be followed up with an written notification

Actions required prior to any return to the premises

- The Registered Manager or Person in Charge is to confirm that the premises are safe to return to having taken instruction from the emergency services personnel involved.
- Any transport to return the Service Users and staff to the premises is arranged.
- All areas of the premises to be checked for safety and cleanliness prior to any return.
- Relatives to be contacted and kept informed.
- Contact to be made with Social Services and other organisations advising of the return to premises
- Contact the CQC or use a notification to confirm the premises are safe and Service Users can be returned.

Arson

Arson is a serious threat to homes, shops, offices, storage buildings, and hospitals. All buildings are at risk. Much of the arson is associated with vandalism and burglaries. If small fires have been started on your own or neighbouring premises they could be a warning of worse to come – inform the police and the fire brigade.

- **Security:**
  - Keep the number of entry points to the minimum compatible with safe means of escape in the event that a fire breaks out;
  - Perimeter fences, walls and gates need to be strong and high enough to keep out intruders;
  - Doors and windows must be in good repair and locked when not in use;
  - Locks and padlocks must be of good quality;
  - Keys must be distributed only to a restricted number of people;
  - Gaps under doors must be kept small;
  - Letter boxes should have metal containers fitted on the inside;
  - Stored material of any kind should be kept away from perimeter walls or fences where it could be set alight.
- **Employees:**
  - Warn staff about the threat from arson;
  - They should challenge anyone who should not be on the premises and report any suspicious activities;
  - Vet new employees;
  - Keep an eye on contractors.
- **Visitors:**
  - Control the access and movement of visitors.
- **Fire protection:**
  - Fixed and portable fire-fighting equipment must be regularly maintained and protected against sabotage attempts.
- **End-of-day (onset of darkness or night shift) checks:**
  - Ensure that:
    - The building is secured by a named individual at the end of each working day; Œ
    - **Doors and windows** are secure;
    - No combustible material is left lying around;
    - No unauthorised people are on the premises;
    - Alarms are switched on;
    - External lighting is switched on;
    - Flammable liquids are locked in the proper store.

Common causes of fire and how to guard against them

Electricity

Neglect and misuse of electrical wiring, fittings and equipment can easily cause fires in the workplace. Overheating of electrical circuits, poor wiring connections, use of unauthorised electrical appliances, multi-point adaptors and problems with the use of extension leads are all hazards which frequently result in fires starting. Therefore:

- Ensure that all electrical circuits and equipment in the workplace are inspected and tested on a regular basis. (Remember, there is a requirement to do this under the Electricity at Work Regulations 1989).
- Have any faults in wiring or fittings repaired promptly by a qualified electrician.
- Switch off electrical equipment when it is not in use and particularly at the end of the day's work.

Rubbish and Waste Materials

Rubbish and waste materials that are left to accumulate can easily contribute to the spread of fire; they are also a place for malicious fires to be started. Make sure that you remove all waste materials from the workplace on a regular basis and place them in a suitable container located in a safe position outside the building. Ideally this container will be of metal construction and fitted with a lockable lid. Arrange for the container to be emptied regularly.

Do not burn rubbish on bonfires, even if it is thought safe to do so. They can easily get out of control and spread fire to nearby buildings or structures.

Smoking

Smoking is still a major cause of fires in buildings. You should:

- Consider having a 'No Smoking' policy in your establishment. But remember to make provision for those Service Users who like to smoke. This will be a designated area or room where sufficient glass or metal ashtrays are provided. A fire extinguisher should be provided in or near the room and any seating should have only small amounts of padded upholstery.
- Prohibit smoking in storage areas. If the public visit your premises ensure that they understand that this is a no- smoking area.
- Check designated rooms at the end of the working day to ensure that no smoking material has been left burning.

Cooking

Many small businesses have kitchens where staff may prepare food themselves. These facilities are similar to domestic kitchens and cooking hazards may still arise:

- Avoid undertaking deep fat frying unless a thermostatically controlled pan is provided. Even then it would be wise not to leave the pan unattended.
- Ensure that combustible materials such as cloths, towels and loose fitting clothing (especially sleeves) are kept well clear of hobs.
- Toasters and microwave ovens should not be sited in office areas; they should only be available in kitchens.

Heating Appliances

Portable heaters can often be hazardous in the work environment, especially if placed too close to combustible furniture, fittings or materials. Convector heaters are safer than radiant fires. If you do need to use heaters:

- Ensure that they are securely guarded and properly fixed to prevent them from being knocked over.
- Place them well away from any materials which could easily ignite.
- Never stand papers or books on them or drape clothing over them.
- Do not allow ventilation grilles to become obstructed.

- Clean portable heaters on a regular basis.

Combustible Materials

If combustible items, such as packing materials, glues, solvents, flammable liquids or gases, are used or stored in the workplace, it is recommended that:

- The amounts brought into the premises should be kept to a minimum and sufficient for the day's work only.
- The bulk supplies of such materials should be locked in a secure store, preferably outside the main premises.

Hazardous Materials

If you use paints, solvents, adhesives, chemicals or gas cylinders, keep them in separate storage areas and well away from any sources of ignition. Gas cylinders, even when empty, can explode when exposed to heat. Remember that the Highly Flammable Liquids and Liquefied Petroleum Gases Regulations 1972 may apply to your operations.

Fire drills

- To ensure that all employees know how to leave the premises in the event of fire, repeated practice is desirable. Fire drills should be held at regular intervals and preferably every 2 months or whenever there is a substantial change in staff levels or turnover. All staff, from all shifts should participate in a fire drill. Employees should be trained:
    - To recognise the fire alarm when it sounds;
    - To act in accordance with the evacuation plan;
    - To leave the premises quickly by the nearest possible route;
    - To go to the designated assembly point;
    - To assemble for roll call.
- Departmental managers (or their equivalent) should make sure that their departments are completely evacuated.
- Management should evaluate performances during fire drills and in particular should investigate the causes of any delays in evacuation and take steps to make sure delays are eliminated.

Records

- To ensure compliance with the regulations
    - The service is required to maintain accurate and up to date records of all Fire drills undertaken.
    - The training statistics must be updated for all staff to ensure fire safety training has been undertaken and a plan of ongoing training and drills formulated.

Reviewed by Marcus Kerridge McColl
December 2025

Food and Nutrition Policy and Procedure
Crescent Community Care Services Limited

Purpose

To support the values of Crescent Community Care Services Ltd.

Scope

All Service Users.

Policy

- All Service Users will be assessed for their food preferences and their wishes will be documented in the Care Plan.
- All Service Users who have a particular nutritional need will have a full assessment of nutrition and all information regarding the Service User's nutrition will be documented in the Care Plan.

Procedure

- Line Managers/Area Coordinators/Review & Assessment Officers will be trained in nutritional assessments and reviews.
- Service User nutritional assessments will be carried out, and where any special arrangements or needs are identified, a Care Plan will be prepared.
- Where a Service User is unable to eat appropriately for their correct nutritional balance, a referral to the GP will be made.
- Specialist advice will be obtained from appropriate professionals such as GPs, community nurses and community dietician.

Reviewed by Marcus Kerridge McColl
November 2025

Food Hygiene Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards. Š

- To prevent cross-contamination.

Scope

- All carers also engaged in food preparation.

Policy

- Crescent Community Care Services Ltd will follow good practices to comply with Food Hygiene Regulations and the requirements of the Environmental Health Officer.

- In a residential setting, access to food preparation areas will be controlled to exclude as far as is possible persons who may unnecessarily increase risk.

Procedure

- The kitchen must not be used as a thoroughfare.

- Access must be limited to staff associated with food preparation.

- The kitchen and equipment must be maintained at a high level of cleanliness at all times.
- Smoking is not allowed in food preparation areas.
- Pets are not allowed where food is prepared.
- Staff working in the food service areas must maintain a high standard of hygiene (see Personal Hygiene Policy and Procedure).
- Food waste must be regularly removed from the kitchen for disposal.
- There must be suitable separation between raw and cooked items stored in a refrigerator or any other sterile facility.
- The dishwashing machine should be used whenever possible.
- A lid should not normally cover waste storage containers within the catering area, as this promotes cross contamination by handling. Storage facilities outside of the catering area must have a lid.
- Food placed into temperature-controlled storage must be dated, and similar food rearranged so that the oldest is presented for use first.
- Rice and pasta must not be reheated. Other reheated food must be checked with a temperature probe to confirm that the body of the food has reached normal cooking temperatures.
- On removal of stored food, a check should be made to ensure that the oldest dated item is used first, checking that it is still "in date" from both food hygiene and food quality viewpoints.

Reviewed by Marcus Kerridge McColl
December 2025

Grievances Policy and Procedure
Crescent Community Care Services

Purpose

- The Grievance Policy is a framework for resolving a formal grievance which an employee has concerning any aspect of his/her employment.
- The basic aim of the grievance procedure is to try and arrive at a mutually satisfactory solution of the grievance as quickly as possible. Employees are therefore encouraged to deal with the problem informally in the first instance with their manager. If it is not possible to resolve a grievance informally, employees should then formally raise the matter without unreasonable delay with a manager who is not the subject of the grievance. This should be done in writing and should set out the nature of the grievance.
- The objective of a grievance procedure is to provide a recognised channel through which a grievance can be brought to the attention of management by providing the right for an employee to have their grievance heard, investigated and, if proved justified, remedied.

Scope

- All employees.

Policy

- The organisation recognises that from time-to-time employees may wish to seek redress for grievances relating to their employment.

- In this respect, the organisation's policy is to encourage free communication between employees and their managers in order to ensure that questions and problems arising during the course of employment can be aired and, where possible, resolved quickly and to the satisfaction of all concerned.

Procedure

This procedure is not part of your contract of employment; however, we will not depart from it without good reason.

- If there is any matter relating to your work, or the people with whom you work, with which you are unhappy, you should use the following grievance procedure.

- If your grievance relates to an equal opportunities matter, it may be easier to deal with all complaints under one procedure rather than using both the grievance procedure and our equal opportunities complaints procedure. We will advise you of which procedure is most appropriate.

- At any formal meeting, you have the right to be accompanied by a single companion who is either

  - A work colleague; or,
  - A full-time official employed by a trade union; or,
  - A lay official, so long as they have been certified in writing by their union as having experience of, or as having received training in, acting as a worker's companion at disciplinary or grievance hearings.

- Your companion has the right to explain and sum up your case, and to respond to any views expressed at the hearing. They may not answer questions on your behalf. If your companion cannot attend on the date we have set for the interview, we will always postpone the interview for up to five days and may (at our discretion) postpone it for longer should it become necessary.

Informal Stage

- Where possible, the employee should first discuss on an informal basis, their grievance with their manager who is usually in the best position to help

- On hearing the employee's grievance, the manager shall reach a decision within five working days or as soon as reasonably practical. If a delay to a decision is unavoidable, the manager will provide the employee with the reason why and the anticipated timescale expected to reach a decision

Formal Stage:

- If it is not possible to resolve a grievance informally the employee should raise the matter formally and without unreasonable delay with a manager who is not the subject of the grievance. The grievance should clearly set out the facts in writing of the exact nature of the complaint or problem and the redress being sought. You should state that this comes under the formal stage of the grievance procedure.

- The grievance will be investigated and the manager will arrange as soon as possible a mutually agreed date for a meeting where the employee will be given an opportunity to state their grievance. During the meeting the employee will be allowed to explain their grievance and how they think it should be resolved.

- During this meeting, the employee may also be accompanied by a companion if they wish. To exercise this right, the employee must first make a reasonable request in advance to the manager. What is reasonable will depend on the circumstances of each case however, it would not normally be reasonable for employees to insist on being accompanied by a companion whose presence would prejudice the hearing or where it is not sensible logistically for a companion to attend for geographical reasons.

- If the meeting is likely to take longer to arrange then this will be communicated to the employee giving the reason for the delay.

- Consideration may also be given to adjourning the meeting for any further investigations as it may be necessary for us to make further enquiries with other people about your grievance, and possibly to have a second meeting with you.

- The grievance meeting will be recorded in writing and the signature of the employee will be obtained as verification to the accuracy of the statement. After the meeting, we will write to you with the outcome of your grievance. The letter will include a right to appeal if you are not satisfied with the outcome.

Appeal:

- If after receiving the grievance decision the employee is still aggrieved, they may inform a more senior manager in writing that they wish to appeal stating the grounds on which the grievance should be reviewed. This appeal must be submitted in writing within five working days of receiving the last decision. After this period, the right to appeal will be lost.

- Following receipt of the employee's notification of their wish to exercise their right to appeal, the manager will arrange the appeal hearing for as soon as possible on a mutually agreed date. The employee again will have the right to request a companion at the meeting.

- The decision of the Appeal will be conveyed in writing within ten working days. Where this is not possible, the employee will be given an explanation for the delay and notified when a response can be expected.

- The decision of the Appeal shall be final and binding.

Overlapping Grievance and Disciplinary Cases:

- Where an employee raises a grievance during a disciplinary process then depending on the circumstances, the disciplinary process may be temporarily suspended in order to deal with the grievance. Where the grievance and disciplinary cases are related, it may be appropriate to deal with both issues concurrently.

Reviewed by Marcus Kerridge McColl
December 2025

Harassment Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with the following:

  - Equality Act 2010.
  - Best employment practice.
  - See also the Disabled Workers Policy and Procedure for further information on the effect of the Equality Act
  - 2010.
  - See also the Equality and Diversity Policy and Procedure for further information on the effect of equality and diversity regulations.
  - Note: In particular that detailed procedures and requirements in relation to all aspects of equality, diversity, discrimination and harassment are embedded throughout all of the relevant policies in this management system, and that all procedures are designed to lead users of the system away from discriminatory behaviour and towards best practice behaviour.

Scope

- All employees.

Policy

- This organisation deplores all forms of **harassment, including sexual or racial harassment and harassment based on disability, age, gender, gender reassignment, religion or belief and sexuality and** seeks to ensure that the working environment is conducive to all its employees.

- The following procedure informs employees of the type of behaviour that is unacceptable and provides employees who are the victims of any form of harassment with a means of redress.

- Implementation of the policy is the duty of all staff, particularly managers and supervisors. All employees must comply with this policy. (The organisation encourages and welcomes the support of the recognised trade union(s) in seeking to eradicate harassment of all forms from the workplace).

- **Harassment is unwanted conduct related to a relevant protected characteristic, which has the purpose or effect of violating an individual's dignity or creating an intimidating, hostile, degrading, humiliating or offensive environment for that individual (Equality Act 2010**)

- The relevant protected characteristics are:

  - Race, including colour, nationality, ethnic or national origin;
  - Sex;
  - Disability;
  - Age;
  - Religion or belief;
  - Sexual orientation;
  - Gender re-assignment.

Procedure

- Sexual harassment as in other forms of harassment at work is unlawful. Both the organisation and the perpetrator may be held liable for such unlawful actions, and may be required to pay damages. Sexual harassment can reduce the effectiveness of the organisation by creating a threatening environment, and increasing stress, sickness absence and high staff turnover. Employees have the right to work in an environment free from sexual intimidation and harassment.

- Racial harassment at work is also unlawful and, as with sexual harassment, both the organisation and the perpetrator may be held liable. It also creates a threatening environment and all staff members have the right to work in an atmosphere free from racial intimidation.

- Intentional racial or sexual harassment is also a criminal offence punishable by imprisonment or a fine.

- Incitement to racial hatred is also a criminal offence punishable by imprisonment or a fine.

- It is important for employees to note that the other types of harassment covered in this policy are also taken very seriously by the organisation and will be dealt with in the same manner. Harassment may consist of a single act or a series of acts.

EXAMPLES OF HARASSMENT

- Sexual harassment takes many forms, from relatively mild sexual banter to actual physical violence. Employees may not always realise that their behaviour constitutes sexual harassment but they must recognise that what is acceptable to one person may not be acceptable to another. Sexual harassment is unwanted behaviour of a sexual nature by one employee towards another. Examples of harassment include:

  a. Insensitive jokes and pranks;
  b. Lewd comments about appearance;
  c. Unnecessary body contact;
  d. Displays of sexually offensive material, e.g. pin-ups;
  e. Requests for sexual favours;
  f. Speculation about a person's private life and sexual activities;
  g. Threatened or actual sexual violence;
  h. Threat of dismissal, loss of promotion, etc. for refusal of sexual favours.

- Racial harassment and incitement to racial hatred can also take many forms; from relatively minor abuse to actual physical violence. Examples of harassment include:

  a. Insensitive jokes related to race;
  b. Pranks;
  c. Deliberate exclusion from conversations;
  d. Abusive, threatening or insulting words and behaviour;
  e. Displaying abusive writing and pictures.
  f. Displaying or disseminating material, or acting in a manner, which is likely to influence others to engage in racial discrimination or hatred.

- The examples above are not exhaustive. Items 1(g), 1(h) and 2(d) could well amount to gross misconduct, and lead to summary dismissal, but other items may also constitute gross misconduct depending on the circumstances of the case in question.

- Harassment related to other protected characteristics may also take similar forms to those outlined above with a similar impact on the organisation and employees as stated.

THE ENVIRONMENT

- The organisation prohibits the display of sexually offensive material, e.g. pin-ups and posters or by emails or attachments, and will if necessary ensure that work-places are inspected and offending material removed.

- The organisation expects all managers and supervisors to ensure that this policy and procedure is adhered to at all times.

- The organisation recognises the sensitive nature of complaints of sexual harassment. Employees who wish to discuss such complaints in confidence should contact the Registered Manager.

TAKING ACTION AGAINST HARASSMENT

- If an employee believes he or she has been the subject of harassment they should act promptly.

- Write down a brief description of the unwanted behaviour or keep a diary of incidents as they occur.

INFORMAL REMEDY

- Employees who believe they have been subjected to sexual or racial harassment are advised, if appropriate to speak informally to the person whom the employee thinks is responsible for the behaviour to make it clear to their harasser that the behaviour is unacceptable and must stop.

- The employee should make notes summarising the date and content of the discussion. If an employee is unable to do this verbally then a written request (explaining the distress that the behaviour is causing) handed to the harasser may be effective. The person named above can assist employees in taking such action.

FORMAL PROCEDURE

- Where informal methods fail and the behaviour persists or concerns a specific incident, the employee must report the matter to their line manager.

- If the employees' line manager is the alleged perpetrator/harasser, the member of staff should report the matter to their line manager's manager.

- In some cases, the employee may feel more comfortable speaking to another line manager. A formal grievance should be raised and the employee is advised to seek assistance, as above, in doing so. The complaint should be made in writing, and where possible, state:

  - The name of the alleged perpetrator/harasser;
  - The nature of the harassment;
  - Date(s) and time(s) when harassment occurred;
  - Name(s) of witness(es) to any incidents of harassment;
  - Any action already taken by the complainant to stop the harassment.
  - The complaint/grievance should be sent, in confidence, to the Registered Provider.

- Immediately after a complaint of harassment has been received, action will be taken to separate the harasser from the complainant; this may involve temporary transfer of the alleged harasser to another department, or suspension with pay until the complaint has been resolved.

- The senior manager handling the complaint will carry out a thorough investigation as quickly as possible to establish whether or not there is a case to answer, maintaining confidentiality at all times. All employees involved in the investigation are expected to respect the need for confidentiality. Failure to do so will be considered a disciplinary offence and may lead to disciplinary proceedings under the organisation's Disciplinary and Grievance procedures.

- Copies of statements made by witnesses will be made available to both the alleged perpetrator/harasser and the complainant. Witnesses will be encouraged to appear at the complaint hearing if requested by either party. It is acknowledged that some witnesses may be reluctant to do so. In these circumstances the manager will, if necessary, adjourn the hearing in order to ask supplementary questions of witnesses in private.

- The complainant may, if he or she wishes, be supported throughout the procedure and hearing by a work colleague. Alternatively, he or she may be supported by certified Trade Union Representative or employed Trade Union Official.

- The employee accused of harassment will also have the equivalent right to be accompanied at the hearing. Where the manager concludes that harassment has taken place, he or she will ensure that the perpetrator has every opportunity to defend or explain his or her actions, in accordance with the organisation's disciplinary procedure.

- The severity of the penalty imposed upon an employee against whom an allegation of harassment has been proven will be consistent with those detailed in the disciplinary procedure (e.g. gross sexual harassment will normally result in summary dismissal).

- Where a lesser penalty is appropriate (e.g. a written warning) this may be coupled with action to ensure that the victim is able to continue working without embarrassment or anxiety. After discussion with the victim, the manager may order the transfer of the harasser to a different work area, or arrange for the amendment of working schedules or rotas to minimise contact between the two employees. If the alleged victim so wishes, his or her own transfer may be arranged, subject to practical limitations. The result of the hearing will be confirmed in writing to both employees.

- If the complainant is not satisfied about the way in which his or her complaint has been handled, he or she may ask for it to be reconsidered by the Registered Provider under the relevant stage of the organisation's grievance procedure.

- Requests for reconsideration of the complaint should be made within five working days of the first hearing. The decision of this second hearing will be sent, in writing, to both parties and will be final.

- An employee who receives a warning or is dismissed for harassment may appeal against the penalty in accordance with the organisation's appeals procedure.

- An employee who brings a complaint of harassment will not suffer victimisation for having brought the complaint. However if the complaint is found to be vexatious, frivolous and untrue and has been made in bad faith, then disciplinary action may be taken against the complainant.

- For further details of procedures, please refer to the organisation's Disciplinary and Grievance procedures.

Reviewed by Marcus Kerridge McColl
December 2025

Hate Crime Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To comply with the Criminal Justice Act 2003 & Crime & Disorder Act 1998

Scope

- All Employees
- All Service Users

- Any person who has any forms of dealing with the Service
- Any person targeted criminal offence that is potentially a hate crime

Policy

- The definition of a fate crime is a crime whereby the perpetrator targets the victim due to his / her racial group, religion, sexual orientation. Disability, ethnicity, class, nationality, age, gender, gender identity or political affiliation.
- The Act compels a Court to consider certain issues when considering a sentence
- These issues are
    - That at the time of committing the offence, or immediately before or after doing so the offender demonstrated towards the victim of the offence hostility based on
        - The sexual orientation (or presumed sexual orientation) of the victim
        - A disability (or presumed disability) of the victim
    - That the offence is motivated (wholly or partly)
        - By hostility towards persons who are of a particular sexual orientation
        - By hostility towards a person who has a disability or a particular disability
- Crescent Community Care Services Limited will follow Policies and Procedures that are intended to identify hate crimes and report them when they occur, and will train all workers to avoid and identify these incidents

Procedure

- Policies to be followed are
    - Anti Bullying Policy & Procedure
    - Harassment Policy & Procedure
- In any incident where bullying or harassment is observed which involves verbal abuse, physical abuse, damage to property or any combination of these, then the incident must be reported to the Registered Manager or Directors immediately
- If the incident has any features of a hate crime then this behaviour must be clearly documented in a report
- The Registered Manager or Directors will review the information and decide within the guidelines of the Polices & Procedures and consider the reporting of the case to the Authorities for consideration of prosecution

Reviewed Marcus Kerridge-McColl
October 2025

Hazard Reporting Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards.

Scope

- All establishments.

Policy
Introduction

- We recognise that staff members have an invaluable monitoring role within the workplace in helping to identify hazards (see Hazard Reporting and Recording Guidance) before they cause an injury or accident. In addition, staff members also have a legal duty to report conditions that might cause harm.

It is our policy to:

- Have an effective system for the reporting of hazards found by staff in their workplace. Š
- Ensure all reported hazards are dealt with expediently and efficiently.
- Check that action has been taken following receipt of a hazard report.
- Train staff to verbally report the following circumstances immediately:

    - Discovery of a fire;

- Ineffective, defective or missing guards;
- Damaged or ineffective personal protective equipment or clothing;
- Faulty equipment that cannot be operated safely;
- Insufficient training or information to carry out your work safely;
- Insufficient information on the use and handling of a hazardous substance;
- Spillage of a hazardous substance;
- Potential incident or dangerous occurrence;
- Complete check-lists for hazard spotting at prescribed intervals.

Procedure

- Locate Hazard Record forms in prominent accessible areas.
- Inform employees of the Hazard Records locations and how to complete them.
- Employee identifies hazard.

  - Employee contacts Supervisor immediately, and employee completes form.
  - Supervisor decides on action to take.
  - Senior Manager monitors reports and actions.

Reviewed by Marcus Kerridge McColl
November 2025

Health & Safety Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards.

- The purpose of this policy is to ensure that the organisation, its employees and others experience a safe environment, and that statutory obligations are met.

Scope

- This policy applies to all employees, all Service Users and all visitors to the premises of Crescent Community Care Services Ltd, and any premises in which their employees work.

Policy

- **Crescent Community Care Services Ltd** recognises that they have a responsibility to ensure that reasonable precautions are taken to provide and maintain working conditions which are safe, healthy and comply with all statutory requirements and codes of practice relating to the organisation's particular activities.

- **Crescent Community Care Services Ltd** will, so far as is reasonably practicable, pay particular attention to:

  - The provision and maintenance of plans and systems of work that are safe and healthy.

  - Arrangements for ensuring safety and the absence of risks to health in connection with the use, handling, storage and transport of articles and substances.

  - The provision of information, instruction, training and supervision such as to ensure the health and safety at work of employees and others.

  - The control of the place of work maintaining it in a safe condition.

  - The provision of a safe means of access to and egress from the place of work.

- This policy will be reviewed at least annually.

Procedure

Organisation and Responsibilities

- **Executive**
  - The Registered Provider is responsible for safety in Crescent Community Care Services Ltd and will monitor the safety policy on a regular basis.
  - The Registered Provider will be sufficiently apprised of health and safety matters to ensure that sufficient resources are available to provide any health and safety equipment, clothing, information and training for employees in order (as far as is reasonably practicable) to achieve and maintain a high standard of safety proficiency.

- **Safety Officer**
  - The Safety Officer is the Registered Manager, unless indicated otherwise by a notice on the main staff notice board of the establishment.
  - The responsibilities of the Safety Officer are to:
    - Maintain safety records; Œ
    - Investigate accidents;
    - Provide accident statistics;
    - Keep a watching brief on changing safety legislation.
  - The Safety Officer reports directly to the Registered Provider.
  - Full investigations of accidents will be carried out by the Safety Officer with a view to the prevention of future occurrences.
  - The Safety Officer is responsible for ensuring that the organisation's obligations with respect to assessment, control and monitoring of hazardous substances are met.
  - The Safety Officer is responsible for recording of accidents in accordance with RIDDOR (Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013), by:
    - Ensuring that the Accident Reporting Policy and Procedure is followed, and that all accidents are recorded, using the form attached to that policy;
    - Ensuring that all accidents which result in absence from work for more than seven days (not including the day of the accident) are reported within 10 days to the Health and Safety Executive.

RIDDOR 2013 reporting requirements

- Reportable incidents under RIDDOR 2013 are:
  - Death and injuries where:
    - The accident is work-related;
    - It results in an injury of a type which is reportable.
- The types of reportable injuries are:
  - Death;
  - Specified injuries, which are defined by the HSE as:
    - Fractures, other than to fingers, thumbs and toes;
    - Amputations;
    - Any injury likely to lead to permanent loss of sight or reduction in sight;
    - Any crush injury to the head or torso causing damage to the brain or internal organs; Œ
    - Serious burns (including scalding) which:

        - Causes significant damage to the eyes, respiratory system or other vital organs;
        - Covers more than 10% of the body.

    - Any scalping requiring hospital treatment;
    - Any loss of consciousness caused by head injury or asphyxia;
    - Any other injury arising from working in an enclosed space which:
    - Leads to hypothermia or heat-induced illness;
    - Requires resuscitation or admittance to hospital for more than 24 hours.

- The following occupational diseases are reportable:
    - Carpal tunnel syndrome;
    - Severe cramp of the hand or forearm;
    - Occupational dermatitis;
    - Hand-arm vibration syndrome;
    - Occupational asthma;
    - Tendonitis or tenosynovitis of the hand or forearm;
    - Any occupational cancer;
    - Any disease attributed to an occupational exposure to a biological agent.
- Dangerous occurrences require reporting, for example:
    - The collapse, overturning or failure of load-bearing parts of lifts and lifting equipment;
    - Plant or equipment coming into contact with overhead power lines;
    - The accidental release of any substance which could cause injury to any person.
- For full details of reporting requirements, go to the HSE web site at: http://www.hse.gov.uk/riddor/reportable-incidents.htm
- Contacts for the HSE, including for online reporting are at: http://www.hse.gov.uk/contact/contact.htm

Department Manager (where this differs from the Registered Manager).

- Where there are no designated Department Managers, the Registered Manager fulfils this role.
- Department Managers have the responsibility to provide leadership and to promote responsible attitudes towards health and safety.
- Each manager will:
    - Ensure that each new employee is given induction training, including the precautions and procedures appropriate to their specific jobs. All new members of staff will be shown the location of first aid boxes, fire exits and firefighting equipment;
    - Ensure that all subordinates are aware of the health and safety policy (a copy is on the organisation notice boards);
    - Keep up to date with health and safety matters applicable to the operations of the organisation;
    - Investigate all accidents with the assistance of the Safety Officer, with a view to prevention of a further occurrence;
    - Ensure that good housekeeping standards are applied;
    - Review periodically all new and existing equipment with reference to mechanical and operational safety and, in particular, the location of all equipment bearing in mind all health and safety factors;

- Carry out regular safety checks and audits.

Supervisors

- Supervisors have the responsibility to provide leadership and to promote responsible attitudes towards health and safety. Supervisors must ensure that all tasks carried out in their sections are performed with the utmost regard for the health and safety of all those involved.

- Accidents must be reported immediately to the Department Manager or Registered Manager.

- Particular regard will be paid to:

  - Equipment and its usage to ensure that they are safe and do not endanger health;

  - Provision of safety arrangements for the handling, storage and movement of materials, equipment and substances;

  - Supplying sufficient information, instructions, training and supervision such as to enable employees to avoid hazards and contribute positively to their own health and safety at work;

  - Inspecting, on a regular basis, equipment such as lighting, passageways, fire alarms, fire escapes, fire extinguishers, first aid facilities and work practices, in order to ensure their efficiency and maintenance.

Employees

- All employees have a responsibility to do everything they can to prevent injury to themselves, their fellow employees and others affected by their actions or omissions at work.

- They are expected to follow company procedures in particular, to report any incidents which have or may have led to injury or damage. To neglect this responsibility can lead to prosecution by the Health and Safety Executive.

- Any employee who is faced with a conflict between the demands of safety and their job should raise the matter immediately with the Supervisor.

Administrative Arrangements

- The following statements are an overview, and most areas for health and safety management are amplified by further policies and procedures elsewhere in this management system.

- **Risk Assessment**

  - The Safety Officer will regularly review all areas in use by the organisation, or in which its workers work, to:

    - Identify risks;
    - Assess the risk;
    - Evaluate the risk;
    - Eliminate the risk where appropriate;
    - Introduce control measures to reduce risks, to a reasonable level, where appropriate;
    - Develop or locate, and arrange delivery of appropriate training to reduce risk, to a reasonable level, where appropriate.

  - The Safety Officer will carry out a generic risk assessment whenever workers:

    - Begin work in a new area;
    - Begin work in a new building, or building type, in an existing area;
    - Have or raise an issue in an area or building which they are already working in.

  - The Safety Officer will carry out a risk assessment on new equipment brought into the establishment. Š

- **Reporting Accidents**

- In the event of an accident causing injury you must ensure that the injured person is being cared for, and send immediately for a supervisor or first-aider.
- DO NOT MOVE THE INJURED PERSON.
- Report the full details to the Department Manager who will record the incident in the accident book.
- The record will be regularly inspected by the Safety Officer. The accident will be reported to the inspecting authority as and when necessary.
- Any "near miss" incident which occurs should also be reported to your immediate supervisor who will be responsible for making a report to the Department Manager.
- All accidents will be investigated by the Department Manager and the Safety Officer.
- A report will be made to the Registered Provider, via the Management Meeting, who will ensure that necessary action is taken to prevent recurrence.

- **First Aid**
  - During the induction programme employees will be shown the location of the nearest first aid box to their work area.
  - The organisation will ensure that sufficient employees are trained as first aid specialists to provide coverage on all shifts.
  - The identity of designated first aid specialists will be noted by clear notices complying with the recommended format displayed at all work stations and staff areas throughout the establishment.

- **Fire**
  - Fire exits must be kept clear from obstruction.
  - All employees must know their evacuation route and assembly point in case of fire.
  - **IF YOU DISCOVER FIRE:**
    - Immediately operate the nearest fire alarm call point.
    - WITHOUT INCREASING PERSONAL RISK, try to put out the fire, if possible, with the nearest appropriate fire appliance provided, by directing the hose or extinguisher to the base of the flame.
  - **IF YOU HEAR THE FIRE ALARM:**
    - The senior person on duty will be responsible for calling the Fire Services.
    - Report immediately to the staff assembly point, which is by the fire alarm panel in your building.
    - **DO NOT USE THE LIFT.**
    - **DO NOT DELAY FOR PERSONAL BELONGINGS.**
    - Follow the instructions of the person in charge, who is fully in charge of all staff and persons on the premises until the Fire Brigade arrives.
    - If told to leave the building, do not re-enter the building until instructed by your senior supervisor or the Fire Brigade.

Organisation Code of Safe Practice Š

- **Good Housekeeping – General**
  - Undue hurrying and forgetfulness cause many accidents. Do not run down steps. Use hand rails going up or down stairs.
  - Watch out for someone coming round a blind corner or opening doors quickly.

- Never read while walking.
- Ensure that floor areas are well lit and kept clear of obstruction.
- Where floors are wet through spillages or cleaning, the area must be protected using a recognisable wet floor sign until the area has dried. The sign must be removed to storage as soon as possible after the area has dried.

- **Good Housekeeping – Offices**
  - Leaving a lower filing drawer open causes many trips and falls. Please make sure they are closed.
  - Electrical, computer and telephone cords must not be allowed to lie uncovered on the floor and should be taped down, since they are major tripping hazards.
  - Spilled coffee or soft drinks, tracked-in rain, leaves or snow, should be cleaned up immediately.
  - Pointed objects such as pencils, pens, letter openers, files and the like must be used carefully to avoid puncture wounds.
  - Horseplay, including throwing paper clips, shooting rubber bands, tossing objects out of windows, is unacceptable behaviour, and may be the subject of disciplinary procedure.

- **Electrical Equipment**
  - Electrical equipment is normally safe, provided it is properly installed and regularly inspected.
  - Always remember that water and liquids are conductors of electricity, and be aware that their association with faults caused by, for example, damaged cables, flexes, plugs and sockets, the overloading of circuits and fuses, etc. would make the shock more severe.
  - Therefore you should:
    - NEVER touch electrical equipment with wet hands, move any portable electrical equipment without disconnecting it from the mains, make electrical repairs or do other electrical work unless you are an authorised person;
    - KEEP electrical supply cables and flexes away from wet areas or from where they will be damaged by being walked over or knocked when moving equipment about;
    - ALWAYS switch off all equipment when not required, unless continuous operation is necessary;
    - Disconnect electrical equipment at night by removing the plug from the socket, again unless continuous operation is necessary and/or instructed otherwise;
    - Report defective equipment to the Registered Manager.

- **Moving and Handling**
  - Staff must not carry out moving and handling operations unless the operation has been assessed for risk, an opinion has been formed and recorded by an appropriate person, a recommended handling technique identified, and the technique communicated to all staff.
  - Moving and handling form part of the induction training where general guidelines are given on the prevention of back injury and the importance of risk assessment of both individual lifting/handling operations and environmental consideration.
  - Staff who find themselves alone with a Service User should never attempt to lift/move a Service User who has been assessed as requiring two people to perform such an operation. Advice should be sought immediately and the Service User should be made comfortable/safe until assistance arrives.

- **Basic Food Hygiene**
  - All employees who have contact with food in the establishment, or enter food preparation areas, will be suitably trained in basic food hygiene.
  - Basic food hygiene training is incorporated in the induction training for all employees.

- Employees normally working in food preparation will complete a recognised Basic Food Hygiene qualification as soon as practicable after initial employment, or produce proof of a recent qualification.

- Catering supervisors and cooks will complete the Intermediate Food Hygiene Certificate as soon as practicable after initial employment, or produce proof of a recent qualification.

- **Transmittable Diseases**

  - Transmittable diseases form part of the induction training.

  - When performing hands-on personal Care with Service Users, full protective measures (gloves, aprons etc) should be taken in order to eliminate any risk of cross-infection.

- **Hot Water Bottles**

  - Hot water bottles should not be used except in exceptional circumstances and only if the Service User insists.

  - Hot water bottles must never be used on Service Users with dementia.

  - If a bottle must be used, follow the procedure below:

  - Hot water bottles may be used for Service User comfort, but will be covered by a protective cover and filled in a manner which is safe for staff, and eliminates the risk of scalding in the event of failure of the bottle.

  - Check that the hot water bottle has a protective cover over the bare rubber inner.

  - Mix water to the highest temperature which it is possible to bear on bare skin in a jug. Pour that water into the bottle and seal.

  - Press the bottle and check the stopper for leakage.

  - When placing the bottle against the Service User, wait long enough for the Service User to report that the temperature and position is comfortable before leaving.

- **Infection Control**

  - Infection control training is incorporated in induction training.

- **Prevention of cross-infection**

  - To prevent cross-infection, ensure that:

    - Any infection a Service User has does not spread to others;
    - The Service User is not exposed to potential sources of infection in his/her surroundings; Œ
    - Others do not bring infection to The Agency.

- **Notifiable diseases**

  - The Health Services and Public Health Act 1968, the Public Health (infectious Diseases) Regulations 1988 and subsequent amendments require certain infectious diseases to be notified to the 'proper officer' of the Local Authority.

  - The responsibility for the notification of the listed disease(s) rests with the Doctor attending the Service User.

  - The Local Authority has the power to stop work in order to prevent the spread of infection, including food borne infections (Food Hygiene (General) Regulations 1970).

    - Diseases notifiable under the Public Health (Control of Disease) Act 1984: Cholera, Plague, Smallpox, Relapsing Fever, Typhus, and Food Poisoning.

    - Diseases notifiable under the Public Health (Infectious Diseases) Regulations 1988: Acute encephalitis, acute poliomyelitis, anthrax, diphtheria, dysentery, leprosy, leptospirosis, malaria, measles, meningitis, meningococcal,

septicaemia, mumps, ophthalmia, neonatorum, paratyphoid fever, rabies, rubella, scarlet fever, tetanus, tuberculosis, typhoid fever, viral haemorrhagic fever, viral hepatitis, whooping cough and yellow fever.

- Most outbreaks will present non-specific symptoms; serious sepsis or epidemic wound infections.
- Any member of staff suspecting an outbreak of the notifiable disease should make their suspicions know to the Registered Manager who will inform the appropriate Doctor.

- **Rules to prevent the spread of infection**
  - All staff should adhere to the following:
    - Avoid infection by careful control of coughing and sneezing, i.e. use tissues / handkerchief; Œ
    - Appropriate hand washing;
    - Use of disinfecting hand rubs;
    - Avoid wearing jewellery, except for wedding rings;
    - Keep hair short or tied back;
    - Wear clean uniform clothing, and do not travel to and from work in that clothing;
    - Report any signs of infection to the appropriate person;
    - Keep toilets and commodes scrupulously clean using correct disinfectant agents; Œ
    - Correct handling of food to prevent food borne illness;
    - Take Care when dealing with pets. Always hand wash or use the hand rub after contact. Š

- **Staff skin awareness**
  - All cuts and abrasions should be covered with a waterproof plaster (blue coloured if working in food areas). Early detection and prompt reporting of infection is particularly important.
  - Any staff member with a skin infection must take advice from a doctor before continuing to work. All skin infections must be reported to the Registered Manager.

- **Staff sickness**
  - Staff with diarrhoea and vomiting should not attend work but ring to report sick. Should the condition persist it may be necessary to provide a specimen of faeces and not return to work until medical clearance by a GP is given. It cannot be emphasised strongly enough that young children and the elderly are particularly vulnerable to infection, and every attempt should be made to minimise any risk of infection.

- **Skin Infections**
  - Report to your manager any Service Users who have a rash or unaccountable marks on his/her body.
  - Where scabies or shingles are suspected:
    - The manager must request a visit from the GP;
    - Staff should wear a plastic apron and wear gloves for any direct contact;
    - All linen must be placed in the appropriate bag and the appropriate laundry procedure followed for contaminated laundry.

- **Blood borne viruses**
  - Any Service User may be a carrier of a blood borne virus. There are blood-borne viruses other than hepatitis B, other hepatitis and HIV/AIDS. Appropriate precautions must therefore be taken with all Service Users and particularly with body fluids.
  - Always assume that blood and other body fluids are infected. All accidents, facial, particularly eye, or wound contact with infected body fluids must be recorded as an incident.

- Accident avoidance measures should include common sense precautions to avoid accidents and injuries, particularly when using sharps, whether the Service User is known to be infected or not. All accidents must be reported.

- Body fluid handling and spillage procedure should be as follows:

  - Use no-touch techniques when dealing with blood or other body fluids. Wear gloves and plastic aprons as appropriate. Masks and goggles are not normally needed;
  - Care staff wearing disposable gloves and plastic aprons should wipe up body fluid spillages immediately;
  - Use appropriate disinfectant agents on carpets;
  - Use no-touch techniques or gloves when disposing of anything contaminated with blood, e.g. dressings.

- Avoid contamination with saliva. If saliva contamination to eyes, a cut or an open wound occurs, wash liberally with water and inform the manager immediately.

- **Outbreak control measures**

  - An outbreak of gastroenteritis is indicated by the occurrence of UNEXPLAINED diarrhoea and/or vomiting in two or more Service Users. (Remember that there are also non-infective causes of diarrhoea and vomiting). The recommended action in such cases is as follows:

    - Staff should inform the manager who should then contact the appropriate GPs;
    - A specimen of faeces should be made available for testing, if required;
    - Wear plastic apron and protective gloves when in contact with excreta;
    - Dispose of faeces carefully and disinfect bedpans/commodes using disinfectant;
    - If possible, place the Service User in a single room, with their own toilet facilities such as a commode;
    - Any Service User with, or suspected of having, gastroenteritis should have their own sink/bowl for washing;
    - All crockery and cutlery should be soaked in a bowl of disinfectant for 30 minutes before being removed from the room of the Service User to the kitchen;
    - Place all contaminated linen into a coloured bag and keep separate from any other linen;
    - Wear a protective apron and gloves when sluicing contaminated linen. To sluice any contaminated linen, leave the linen to soak in disinfectant for 30 minutes before removing to laundry;
    - Wash hands thoroughly after attending the Service User and before going to any other task.

  - The manager should notify the local health authorities when the occurrences are unexplained. Š

- **Emergency Situations**

  - In case of being faced with emergency situations such as relating to gas, electricity, water, fire or medical issues, stay calm, assess the situation, and raise alarm by contacting 999, depending on the emergency. Emergency situations will form part of your induction programme.

- **Major injuries**

  - Fracture of the skull, pelvis and any bone in the arm or leg, but not bones in the hand or foot.

  - Amputation of a hand or foot or of fingers, thumbs or toes where the bone or a joint is completely severed.

  - Loss of sight in an eye or a penetrating injury or a chemical or hot metal burn to an eye.

  - Injury requiring medical treatment or loss of consciousness due to electric shock.

  - Loss of consciousness due to lack of oxygen.

  - Decompression sickness.

  - Acute illness believed to be the result of exposure to a pathogen or infected materials.

  - Any other injury that results in the person being admitted to hospital for more than 24 hours.

  - Any incident in which a dangerous substance being conveyed by road and involved in a fire or where there is an uncontrolled release or escape of dangerous substances.

  - Any incident whereby breathing apparatus malfunctions in such a way as to deprive the wearer of oxygen.

- Any incident in which plant or equipment comes into contact with overhead power lines exceeding 200 volts.
- Prescribed diseases and certain poisoning.
- Some skin diseases including: occupational asthma, farmers lung, pneumoconiosis, asbestosis and mesothelioma.
- The following infections: leptospirosis, hepatitis, tuberculosis, and anthrax, any illness caused by a pathogen.

- **COSHH**
  - COSHH forms part of your induction training and are incorporated into the individual Service User accommodation risk assessment, this forms part of the Service Users Care Plan.
  - For the purpose of COSHH, a substance is considered as hazardous if one or more of the following criteria are met:
    - Substances listed as very toxic, harmful, corrosive or irritant;
    - Substances for which maximum exposure limit (MEL) is specified in the COSHH schedule; Œ
    - A micro-organism hazardous to health;
    - Substances airborne as concentrations of dust;
    - Any other substances, which create comparable hazards.

- **Safety Rules for the use of household cleaning agents**
  - Handle all household cleaning agents with Care. Remember they contain powerful chemicals.
  - Always wear protective clothing (overalls, rubber gloves).
  - Always read the instructions on the label of the product to be used.
  - If unsure of the product or it is thought that the chemical is in the wrong container, DO NOT USE.
  - NEVER MIX chemicals, especially bleach and toilet cleaner.
  - Make sure that the ventilation is adequate. DO NOT use chemicals in a confined space.
  - NEVER SMOKE whilst using chemicals. Smoking is not permitted in the homes of Service Users.
  - Store all chemicals in a cool dry place after use.
  - Store all chemicals out of reach of children but not on high shelves. Keep away from heat.
  - NEVER place chemicals in other containers. If a container is broken, discard it with its contents.
  - AEROSOLS must be:
    - Kept away from heat;
    - Never punctured;
    - Never used near a naked flame or heat;
    - Avoid breathing the vapour - used in a well ventilated room.
  - Be careful when throwing away chemicals. Be sure they are in a safe condition and that no one else will be harmed by them. Never throw away metal scouring pads with discarded batteries – they can smoulder and cause a fire.

- **IF AFTER USING HOUSEHOLD CHEMICALS WITHIN THE WORKPLACE A FEELING OF DROWSINESS OR OF BEING GENERALLY UNWELL DEVELOPS, CONTACT YOUR DOCTOR IMMEDIATELY AND THEN INFORM THE MANAGER/CARE CO-ORDINATOR**.

- **Safe systems of work**
  - To help give a better picture regarding the health and safety of employees in the work place, a list of the common areas where risks and hazards occur is shown on the following forms. It shows the areas/appliances that may present a hazard or risk, the types of accident/injury they may cause and the appropriate action that should be taken by Care staff.

Working at Heights/Reaching etc:

- Avoid working at height where possible.
- Use work equipment or other measures to prevent falls where they cannot avoid working at height.
- Where they cannot eliminate the risk of a fall, use work equipment or other measures to minimise the distance and consequences of a fall, should one occur.
- Risk-assess all situations whereby working at heights is unavoidable.
- Provide suitable training for those working at heights.
- Provide suitable equipment.

Employees and other workers on site will:

- Not work at heights without ensuring that the Registered Manager has authorised the action, after carrying out a risk assessment, and the employee has been trained to work at height and has appropriate equipment for doing so.
- Not attempt to obtain items which are beyond your reach. If you cannot reach – get a ladder or stepping stool. Be sure that the ladder is in a safe condition.
    - Do not use chairs, open drawers, or any makeshift device for climbing.
    - Do not climb up the shelves themselves. Do not overreach on the ladder. It is safer to get down and move the ladder.

Smoking

- Smoking is only allowed in designated areas.

Floors

- Floors must be kept free of obstruction.
- Spillages of fluids must be immediately mopped up, and wet floors clearly marked.
- Damage to floors must be reported immediately.

Stairs

- Stairs must be kept clear of obstruction.
- Inflammable materials will not be stored in a stairwell.
- Damage to stairs must be reported immediately.

Lighting

- Non-functioning lighting must be reported immediately.

Doors

- Doors must not be obstructed from closing.
- Damage to fire doors must be reported immediately.

Lifts

- Damage to, or malfunction of, lifts must be reported immediately.

- Lifts will not be used during a fire alarm.

Gas

- Damage to gas installations, or a smell of gas, must be reported immediately.
- Cases of headache, unusual tiredness and muscular weakness experienced in rooms containing a gas appliance must be reported immediately.

Health and Safety: Grievance Procedure

- This procedure relates only to occupational health and safety problems, disputes or grievances.
- In the event of the above, employees should either orally or in writing refer the matter to the Department Manager.
- If employees are dissatisfied with the outcome or in the event of there being a real danger of death, serious injury or health risk and there is insufficient time to eliminate excessive danger, staff should immediately report to the Registered Provider who will investigate and determine what action should be taken.
- After the investigation, the employee will be informed that either:
  - The organisation has so far as reasonably practicable eliminated the danger and employees must resume normal working; or
  - The organisation does not consider that the matter constitutes a grave risk to health or safety, and employees must resume normal working; or
  - The organisation will undertake further investigation and may, if necessary, obtain expert opinion.
- Employees will then be suspended on full pay or be transferred to alternative work whilst the investigation takes place.
- Refusal to resume normal working when instructed will be a breach of organisation discipline. The matter will then be dealt with under the organisation's normal disciplinary procedure.

Reviewed by Marcus Kerridge McColl
November 2025

Health and Safety Training Policy and Procedure
Crescent Community Care Services

Purpose

- To comply with statutes, regulations and quality standards.

Scope

- All establishments.

Policy

Introduction

- Training is a vital part of our strategy to effectively manage health and safety issues within our business. When carried out effectively, it can change our staff's perceptions of risks and result in significant improvements in health and safety performance. So preparing our staff to work safely and reducing accidents and damage to our premises and equipment is important. It is also a general factor in motivating staff, so that improvements are often found in overall commitment and

work performance, and ensures that staff members are competent and confident when carrying out their work. It is our legal responsibility to provide adequate Health and Safety Training (see Guidance).

- It is our policy to:
  - Identify the health and safety training needs associated with our work activities.
  - Provide the following health and safety training for our staff:
    - Induction training for new starters;
    - Training on our health and safety policies and procedures;
    - Work activity training relevant to the member of staff, including the use of any equipment; Œ
    - Training required by specific legislation;
    - Training on fire and emergency procedures including alarm raising;
    - Training on the recognition, handling and use of hazardous substances;
    - Awareness training for management staff;
    - Refresher training where identified in our training needs analysis.
  - Keep records (see training records) of all staff training and related documents;
  - Ensure staff are aware of their legal obligation to co-operate and to put into practice any new instruction or guidance given.

Procedure

- Establish the work content of a job.
- Specify the level of competence required for each task. Consider the individual characteristics which may be needed.
- Assess current competence of individual staff against this analysis.
- Identify staff training needs.
- Identify and if necessary train trainers for each topic. Š
- Identify external training providers.
- Add trainers to approved trainer list.
- Develop training packages if necessary.
- Arrange training dates.
- Deliver training.
- Assess effectiveness of training.
- Amend training and redeliver if necessary.
- Review training needs to ensure that all areas are covered.

Reviewed by Marcus Kerridge McColl
November 2025

Human Rights Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutory obligations and best practices on human rights.

Scope

- All activities of Crescent Community Care Services Ltd.

Policy

- Crescent Community Care Services Ltd will endeavour to comply with best practices in the protection of the human rights of its Service Users and it employees.

- The QCS compliance management system was originally developed in the early 1990s, when Homes are for Living in was first published, and the whole system was designed to uphold the principles of HAFLI:

  - Privacy;
  - Dignity;
  - Independence;
  - Rights;
  - Fulfilment;
  - Choice.

- These principles were also incorporated into the Care sector QCF Diploma awards also developed at that time.

- A guiding principle of the development of the QCS system, and its subsequent development, was a recognition of research, which indicated that carers who felt that they lacked control over their employed time and actions, due to their rights being infringed by their employer, would attempt to regain that control by denying the rights of the Service Users whom they were supporting. The system was therefore designed to apply the same principles to employees as the employer expected the employees to apply to the Service User.

- Subsequently the Human Rights Act 1998 codified the principles of human rights. This however did not change the importance of the six principles as applied to care and support of vulnerable people, but in some instances extended or defined the principles.

- The QCS system has been developed and extended to include the specific requirements of the Human Rights Act within the Care sector.

Procedure

- The QCS policies which support human rights are (main items only):

  - Service Users' Handbook;
  - Witnessing of Documents Policy and Procedure;
  - Access to NHS Services Policy and Procedure;
  - Deprivation of Liberty Safeguards Policy and Procedure;
  - Mental Capacity Act 2005 Policy and Procedure;
  - Mental Health Regulations 1983 Policy and Procedure;
  - Participation Policy and Procedure;
  - Principles of customer Care / philosophy of care;
  - Privacy Policy and Procedure;
  - Restraint Policy and Procedure;
  - Risk Taking Policy and Procedure;
  - Advocacy Policy and Procedure;
  - Safeguarding Policy and Procedure;
  - Choice Policy and Procedure;
  - Confidentiality Policy and Procedure;
  - Consent to Examination or Treatment Policy and Procedure;
  - Harassment Policy and Procedure;
  - Equality and Diversity Policy and Procedure;
  - Anti-Bullying Policy and Procedure;
  - Complaints, Suggestions and Compliments Policy and Procedure.

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- The Articles of the Human Rights Act are:

  - Article 1: This article is introductory and not included in the Human Rights Act;
  - Article 2: Right to life;

- Article 3: Prohibition of torture;
- Article 4: Prohibition of slavery and forced labour;
- Article 5: Right to liberty and security;
- Article 6: Right to a fair trial;
- Article 7: No punishment without law;
- Article 8: Right to respect for private and family life;
- Article 9: Freedom of thought, conscience and religion;
- Article 10: Freedom of expression;
- Article 11: Freedom of assembly and association;
- Article 12: Right to marry;
- Article 13: This is not included in the Human Rights Act;
- Article 14: Prohibition of discrimination.

- A more detailed explanation of these Rights is available on the website below:

  http://www.equalityhumanrights.com/your-rights

- Of particular interest in a Care setting are:

  - Article 8: Right to respect for private and family life.

    - This Right states that everyone has a right to respect for their private and family life, their home and correspondence. People have the right to live their own life and have personal privacy.

  - Article 9: Freedom of thought, conscience and religion.

    - This Right protects people's rights in relation to their thoughts and beliefs. The state is not permitted to interfere with a person's right to hold a particular belief.

  - Article 10: Freedom of expression.

    - This Right states that everyone has the right to express their views and receive opinions without interference from a public authority.

  - Article 14: Prohibition of discrimination.

    - This Right states that discrimination cannot occur on the grounds of particular attributes such as a person's sex, race, or religion. It also protects people from discrimination on the grounds of, for example, disability or marital status. A public authority should ensure policies and decisions do not involve any form of discrimination on any ground.

Reviewed by Marcus Kerridge McColl
November 2025

Illegal Possession of Drugs on Premises Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To guide employees and Service Users in dealing with illegal drugs, while protecting Service User confidentiality and the obligations of Crescent Community Care Services Ltd**.**

Scope

- All workers, Service Users and visitors.

Policy

- Crescent Community Care Services Ltd will proactively manage issues arising from the use of illegal drugs on clients premises.

Legal Framework

- The principal legislation covering the misuse of Controlled Drugs in the UK is the Misuse of Drugs Act 1971. The Act makes unlawful the production, supply and possession of drugs subject to control, except in certain permitted circumstances, such as when prescribed by a doctor.

- Section 8 of the Act states that: 'a person commits an offence if, being the occupier or concerned in the management of any premises, he knowingly permits or suffers any of the following activities to take place on his premises...'

  - Producing or attempting to produce a Controlled Drug...

  - Supplying or attempting to supply a Controlled Drug...

  - Preparing opium for smoking...

  - Smoking cannabis, cannabis resin or prepared opium...

- The intention is to ensure that occupiers and managers of premises are vigilant to ensure that Controlled Drugs are not misused on their premises. If illegal drug use by Service Users, visitors, or staff is known to be taking place on Crescent Community Care Services Ltd's premises, failure to inform and co-operate with the police could expose managers to prosecution.

Procedure

- **Known or suspected possession or use of illegal drugs by a Service User**

  - Any member of staff who observes or suspects that a Service User is using or in possession of an illegal drug must report their suspicions to the Manager immediately.

  - **Note that a Service User's property cannot be searched unless the Service User is present and gives express permission, and a second member of staff witnesses the search.**

  - The Manager and another employee witness should advise the Service User that Crescent Community Care Services Ltd does not permit or tolerate the use or possession of illegal drugs on premises owned or managed by them. The Service User should be invited to voluntarily hand over any illegal drugs in his/her possession for destruction. A record of this conversation must be made in the Service User's records, with due regard to "need to know" confidentiality.

  - In subsequently reviewing the Service User's care needs it may be appropriate for the clinical team to offer the Service User the opportunity of a referral to the Alcohol and Drug Advisory Service (ADAS).

- **Refusal of a Service User to surrender an illegal drug**

  - **Crescent Community Care Services Ltd has no authority to remove or retain any substance without the Service User's express permission.**

  - In the event that the Service User refuses to surrender an alleged illegal drug, the Manager should consult with the Registered Provider to decide whether, in the wider public interest, the Police should be informed of their suspicions. The Service User must be informed that this action may be taken.

- **Handling of an illegal drug surrendered by a Service User**

  - The alleged illegal drug should be placed in a secure envelope, labelled "suspected illegal drug surrendered to (name of person creating envelope), on (date), (time), and that entry signed and witnessed. The fact of the surrender and storage arrangements should be noted in the Service User's records, with due regard to "need to know" confidentiality.

  - Place the envelope in a Controlled Drug cabinet.

- Inform the Police to arrange removal if the matter has been reported to the Police, or request the Pharmacist to remove and destroy it. Pharmacists are empowered to remove and destroy illegal drugs.

- The Manager should consult with the Registered Provider to decide whether, in the wider public interest, the Police should be informed of the situation. The Service User must be informed that this action may be taken.

- **Supplying or dealing in illegal drugs**

  - Where the quantity or circumstances of the observation, suspicion or surrender of illegal drugs gives rise to the suspicion that there was an intention to supply drugs to others, the Police must be informed.

- **Visitors**

  - There is no duty of confidentiality in respect of visitors. Any visitor suspected of or known to be in possession of illegal drugs, they must be told to leave the premises immediately.

  - Any repeat of the possession on the premises, or reluctance to leave, must immediately lead to reporting to the Police.

- **Staff**

  - Any observation or suspicion of the use of illegal drugs on the premises by staff must be reported to the Manager, who will deal with the matter according to disciplinary procedures and the alcohol and drugs misuse policy.

Reviewed by Marcus Kerridge McColl
November 2025

Immunisation Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that Service Users and employees receive immunisation when a risk assessment indicates that they would benefit.

Scope

- All Service Users and all employees.

Policy

- Each Service User's GP will be contacted to obtain their opinion as to what immunisation is available, and if they recommend that the Service User is immunised.

- If the recommendation is positive, the Manager will consult with the Service User, and if they consent to the immunisation, liaise with the GP to arrange for the immunisation.

- All employees will be risk assessed for each available immunisation and recommended to ask their GP to immunise them if the risk assessment indicates that they would benefit.

Procedure

Service Users

- Arrange with each Service User's GP when to request an annual review of that Service User's immunisation status.

- At the agreed date, contact the GP and arrange for an assessment, followed by immunisation if recommended, and the Service User agrees.

Employees

- Pre-employment:
  - During the pre-employment process, assess the risks of:
    - Protecting the employee and their family from workplace infections;
    - Protecting Service Users, including those who may not respond well to their own immunisation;
    - Protecting other employees;
    - Allowing for the efficient and robust running of the Service.
  - Do not use immunisation as a substitute for good practice - only consider immunisation if the risk analysis indicates that the risk cannot be adequately controlled by safe working practices alone.

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- Government recommendations for all healthcare workers is:
  - That routine vaccinations are up to date for all workers, including:
    - Tetanus;
    - Diphtheria;
    - Polio;
    - MMR.
  - In addition, direct healthcare workers who have close contact with Service Users, or are involved with handling materials emanating from Service Users, are recommended to have the following additional immunisations, unless the individual risk assessment indicates otherwise:

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    - BCG, for those in close contact with medically vulnerable people;
    - Hepatitis B and Hepatitis C for those who work with bodily fluids;
    - Influenza, for those directly working with medically vulnerable people;
    - Varicella for vulnerable workers who are in direct contact with Service Users.
  - Employees should be referred to their GP for the immunisation, with a covering letter detailing the need, if required by the GP.
- For a detailed analysis of the requirements, read the publication:
  - Immunisation of healthcare and laboratory staff - https://www.gov.uk/government/publications/immunisation-of-healthcare-and-laboratory-staff-the-green-book-chapter-12
- For information about how to deal with an exposure incident see:
  - Sharps and Needlestick Policy and Procedure.
  - http://www.hse.gov.uk/biosafety/blood-borne-viruses/how-deal-exposure-incident.htm

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November 2025

Induction Policy and Procedure
Crescent Community Care Services Limited

Purpose

- When new employees are recruited, relocated or transferred, the organisation will aim to help them adjust to the new situation and gain the required knowledge and competencies as quickly and easily as possible so that they become integrated and productive members of the organisation from the earliest possible opportunity.

Scope

- All new, relocated and transferred employees.

Policy

- When a job has been offered, or a relocation or transfer agreed, an induction "programme" will be drawn up for use during the early part of the employee's employment covering local environmental matters, employment information, and job specific knowledge.

- The programme will meet the standards of the **Health and Social Care Act 2008**.

  - All new members of staff, including trainees and all staff under 18, will successfully complete an induction programme, within 12 weeks of appointment. Induction resources can be found on the Skills for Care website.
  - New employees may not begin work until a negative check has been received, and when they begin work they will be supervised at all times by an experienced and checked employee during the period before their criminal record check is received and employment confirmed. During the period before a satisfactory criminal record check is received, new staff will not work alone with access to Service Users.

Procedure

Users of the QCS Online System (only):

- On the first day of employment, new employees will be coached to use the PC linked to the QCS system, and given a personal ID and password.

Organisations using the paper based QCS system:

- The induction programme will consist of the compulsory items listed in Recruitment Pack C – Job Start and Induction, together with any specific skills identified during recruitment as required but not present, or which need aligning with this employment situation.

- Before the new employee begins work, the manager, or their delegate, will ensure that a full induction programme is specified, a venue for the induction arranged, and sufficient uninterrupted time is available for adequate one-to-one training.

- When the new employee first reports for duty, the Manager or a person delegated to the task and given sufficient time and knowledge to carry out the task, will greet them and introduce them to colleagues. An experienced and Police-checked member of staff will be allocated to the new employee as a "buddy" or "shadow", with the intention of providing a point for informal support during the introductory period. Where possible, the staff rota will be organised so that the new employee and "buddy" or "shadow" will be working together as much as possible, and alternative supervision arrangements made where the "buddy" or "shadow" is not on duty.

- On the first day of employment, the Recruitment Pack C – Job Start and Induction Pack will be completed, and the appropriate section of the Induction Checklist used to plan and control induction training.

- For catering staff, the Manager must establish before the recruit begins work that they have appropriate food hygiene qualifications and skills and, if not, require them to complete the "Food Hygiene Workbook", and read the "Industry Guide to good Hygiene Practice: catering Guide" before beginning working

- Items in the programme will be demonstrated to the employee, and when the employee is assessed as having understood each individual item the demonstrator will sign off the demonstration and assessment columns. At the completion of induction, a review of understanding will be carried out and signed off if satisfactory.

Reviewed by Marcus Kerridge McColl
December 2025

Infection Control Policy and Procedure
Crescent Community Care Services Limited
Purpose

To maintain prevention and control of cross-infection.

Scope

All employee and others who enter the premises, such as agency workers, visitors and contractors.

Policy

- Crescent Community Care Services Ltd will arrange for the prevention of infection, toxic conditions or spread of infection in the community.
- All work spaces will be kept clean, hygienic and free from offensive odours throughout.
- All employees will be trained in effective hand washing.
- Infected materials and/or clinical waste will be kept in appropriate (yellow) bags and stored separately from foodstuffs and food preparation areas.
- Employees who report an infectious condition must not be allowed back on duty until medical clearance is given.
- Employees known to have an infectious condition will be reported to the relevant authorities if appropriate.
- A register must be maintained of all known causes of infection, from which likely causes of infection may be traced and/or identified.
- For outbreaks of infectious disease which, in the opinion of the medical practitioner involved, is sufficiently serious to warrant notification of the CQC, such notification will be carried out within 48 hours.
- Staff will wear barrier clothing when dealing with waste materials and when in close physical contact with the Service User. The barrier clothing will be removed and disposed of before leaving the premises. New barrier clothing will always be used for each Service User.
- Staff will carefully wash their hands on entry to and exit from a Service User's premises, following the detailed instructions given.

Procedure

- The carer will summon a doctor if an infectious disease is suspected.
- The Community Health Physician must then be notified.
- If the Service User shares a house with others, they should, if possible, be accommodated in a single room, with separate sanitary facilities, for the duration of the infection.
- Staff will explain to the Service User the reasons for taking precautions, and ask for co-operation and participation.
- Relatives should be kept fully informed of the Service User's condition and progress, and should be advised not to bring in anything valuable or expensive which cannot be disinfected.
- Aseptic techniques must be used at all times.
- Sterile disposable equipment should be used.
- Strict personal hygiene must be observed by all staff.

The following is repeated from the Health and Safety Policy:

Prevention of cross-infection

- To prevent cross-infection, ensure that:

  - Any infection a Service User has does not spread to others.
  - The Service User does not suffer from potential sources of infection in their surroundings.

- Others do not bring infection onto the premises.

Notifiable diseases

- Certain cases of infectious disease, whether confirmed or suspected, must be notified by the resident's GP to the proper officer of the local authority (who may be part of the local HPU team) under the Health Protection (Notification) Regulations 2010.

- The responsibility for the notification of the listed disease(s) rests with the doctor attending the Service User.

- Notification of infectious diseases' is the term used to refer to the statutory duties for reporting notifiable diseases in the Public Health (Control of Disease) Act 1984 and the Health Protection (Notification) Regulations 2010.

- Diseases notifiable to local authority proper officers under the Health Protection (Notification) Regulations 2010 include:

» Acute encephalitis;
» Acute infectious hepatitis;
» Acute meningitis;
» Acute poliomyelitis;
» Anthrax;
» Botulism;
» Brucellosis;
» Cholera;
» Diphtheria;
» Enteric fever (typhoid or paratyphoid fever); » Food poisoning;
» Haemolytic uraemic syndrome (HUS);
» Infectious bloody diarrhoea;
» Invasive group A streptococcal disease;
» Legionnaires' disease;
» Leprosy;

» Malaria;
» Measles;
» Meningococcal septicaemia;
» Mumps;
» Plague;
» Rabies;
» Rubella;
» Severe Acute Respiratory Syndrome (SARS); » Scarlet fever;
» Smallpox;
» Tetanus;
» Tuberculosis;
» Typhus;
» Viral haemorrhagic fever (VHF);
» Whooping cough;
» Yellow fever.

- Most outbreaks will present non-specific symptoms; serious sepsis or epidemic wound infections.
  Any member of staff suspecting an outbreak of the notifiable disease should make their suspicions know to the
- Registered Manager who will inform the appropriate doctor.

Records on outbreak

- A record should be kept of the following information on residents, with suspected or confirmed infections or infectious disease:

» Name, age/date of birth;
» GP's name and address;
» Date of onset of symptoms and cessation of symptoms; » Type of symptoms;
» Samples taken and sent;
» Diagnosis;

» Source of infection, if known;
» Contacts - family, staff, visitors;
» Outcome;
» Whether the case was notified/ reported to the Proper Officer/HPU and date of reporting. Š

- Information should also be kept for any staff members that develop similar symptoms.

Rules to prevent the spread of infection

- All staff should adhere to the following:

»Avoid infection by careful control of coughing and sneezing, i.e. use tissues / handkerchief;

»Appropriate hand washing;

»Use of disinfecting hand rubs;

»Avoid wearing jewellery, except for wedding rings;

»Keep hair short or tied back;

»Wear clean uniform clothing, and cover that clothing with barrier clothing while dealing with each Service User, disposing of it before leaving the premises;

»Hand wash on entry and exit of the premises, and as appropriate during stay on premises;

»Report any signs of infection to the appropriate person;

»Keep toilets and commodes scrupulously clean using correct disinfectant agents;

»Correct handling of food to prevent food borne illness;

»Take care when dealing with pets. Always hand wash or use the hand rub after contact.

Staff skin awareness

- All cuts and abrasions should be covered with a waterproof plaster (blue coloured if working in food areas). Early detection and prompt reporting of infection is particularly important.
- Any staff member with a skin infection must take advice from a doctor before continuing to work. All skin infections must be reported to the Registered Manager.

Staff sickness

- Staff with diarrhoea and vomiting should not attend work but ring to report sick. Should the condition persist it may be necessary to provide a specimen of faeces and not return to work until medical clearance by a GP is given. It cannot be emphasised too strongly that young children and the elderly are particularly vulnerable to infection, and every attempt should be made to minimise any risk of infection.

Skin Infections

- Report to your manager any Service Users who have a rash or unaccountable marks on their body.
- Where scabies or shingles are suspected:

» The manager must request a visit from the GP.
» Staff should wear a plastic apron and wear gloves for any direct contact. Face masks and eye protection may be required if there is a risk of blood and body fluid splash into the eyes, nose and mouth.
» All linen must be placed in the appropriate bag, and the appropriate laundry procedure followed for contaminated laundry.

Blood borne viruses

- Any Service User may be a carrier of a blood borne virus. There are blood-borne viruses other than hepatitis B, other hepatitis and HIV/AIDS. Appropriate precautions must therefore be taken with all Service Users and particularly with body fluids.
- Always assume that blood and other body fluids are infected. All accidents, facial, particularly eye, or wound contact with infected body fluids must be recorded as an incident.
- All accidents must be reported.
- Body fluid handling and spillage procedure should be as follows:

  »Use no-touch techniques when dealing with blood or body fluid. Wear gloves and plastic aprons as appropriate. Masks and goggles are not normally needed.

  »Care staff should clean body fluid spillages immediately, wearing disposable gloves and plastic aprons. Face masks and eye protection may be required if there is a risk of blood and body fluid splash into eyes, nose and mouth.

  »Use appropriate disinfectant agents on carpets.

  »Use no-touch techniques or gloves when disposing of anything contaminated with blood, e.g. dressings.

- Avoid contamination with saliva. If saliva contamination to eyes, a cut or an open wound occurs, wash liberally with water and inform the manager immediately.

Outbreak control measures

- An outbreak of gastroenteritis is indicated by the occurrence of UNEXPLAINED diarrhoea and/or vomiting in two or more Service Users. (Remember that there are also non-infective causes of diarrhoea and vomiting). The recommended action in such cases is as follows:

  »Staff should inform the manager who should then contact the appropriate GPs.

  »A specimen of faeces should be made available for testing, if required.

  »Wear plastic apron and protective gloves when in contact with excreta.

  »Dispose of faeces carefully and disinfect bedpans/commodes using disinfectant.

  »In a shared house, place the Service User in a single room, where possible, and with their own toilet facilities, such as a commode.

  »Any Service User with, or suspected of having, gastroenteritis should have their own sink/bowl for washing.

  »All crockery and cutlery should be soaked in a bowl of disinfectant for 30 minutes before being removed from the room of the Service User to the kitchen.

  »Place all contaminated linen into a coloured bag and keep separate from any other linen.

  »Wear a protective apron and gloves when sluicing contaminated linen. To sluice any contaminated linen, leave the linen to soak in disinfectant for 30 minutes before removing to laundry. Face masks and eye protection may be required if there is a risk of blood and body fluid splash into eyes, nose and mouth.

  »Wash hands thoroughly after attending the Service User and before going to any other task.

- The manager should notify the local health authorities when the occurrences are unexplained.

National Colour Coding Scheme

- National Colour Coding for cleaning materials and equipment ensures that these items are not used in multiple areas, therefore reducing the risk of cross-infection.

  » Red: Bathrooms, washrooms, showers, toilets, basins and bathroom floors;
  » Blue: General areas including wards, departments, offices and basins in public areas;
  » Green: Catering departments, ward kitchen areas and patient food service at ward level;
  » Yellow: Isolation areas.

Waste Management

- Any waste that is deemed to be infectious or hazardous is considered to be hazardous waste and should be consigned for treatment/disposal at suitably licensed/permitted facilities.

Segregation of Waste

Clinical waste

- All clinical waste should be secured in an approved way and identified with a coded tie or label to indicate source of waste (see table below). Bags should not be closed by an overhand knot. Good practice is to 'swan neck' the bags by twisting the top and then turning it over on itself. The bag should then be secured with tape and tie. Bags should not be more than 3/4 full.
- Areas where clinical/hazardous waste is produced should have foot-operated bins for waste stored in bags.

Non-clinical Waste or Domestic Waste

- Other general waste (food waste, non-contaminated household materials) unsuitable for recycling, should be disposed of in black refuse bags.

Reviewed by Marcus Kerridge McColl
November 2025

Lifts and Hoists Policy and Procedure
Crescent Community Care Services

Purpose

- To comply with statutes, regulations and quality standards.

Scope

- All establishments with lifts and hoists owned or used by Crescent Community Care Services Ltd, or installed in Service User's accommodation.

Policy

INTRODUCTION

- Various types of lifts and hoists are used by The Agency, ranging from passenger lifts to fixed and mobile hoists. All share a common feature in that if they are not properly maintained and subjected to a thorough inspection by a competent person, failure of the lift/hoist may result in serious injury and even death.

- It is our policy to:

  - If appropriate, provide lifts and hoists that are suitable for their intended use, or complete a referral to the
  - Local Authority's Community Equipment team to provide suitable lifts and hoists;

  - Ensure that all lifts and hoists, including slings and fastenings, are regularly inspected and maintained;

- Ensure that all lifts and hoists are thoroughly examined by a competent person in accordance with statutory requirements;
- Train members of staff in how to use the lifts and hoists.

Procedure

- Prepare an inventory of lifting appliances and equipment.
- Arrange maintenance contracts; these will be the responsibility of The Agency if the lift or hoist is owned by them, or the responsibility of the owner if not owned by them e.g. Service User's own equipment. It is the responsibility of The Agency to exercise due Care and attention even if the equipment is not owned by them.
- Arrange statutory insurance inspections.
- Arrange and deliver training for all members of staff who may use the equipment.
- Carry out visual in-house checks.
- Check safety devices e.g. passenger alarms, interlocks, brakes, slings.
- Set up reporting system for defects.
- Monitor and review.

Reviewed by Marcus Kerridge McColl
December 2025

Management and Prevention of Violence at Work Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards.
- **Crescent Community Care Services Ltd** will promote awareness and preventative measures to reduce the risk of violence towards workers.
- Employees who deal directly with others in the course of their work may increasingly face aggressive or violent behaviour. They may be sworn at, threatened, or even attacked; serious acts of violence are however relatively infrequent. The Health and Safety Executive's definition of work related violence is:
  - "Any incident in which a person is abused, threatened or assaulted in circumstances relating to their work".

Scope

- All workers.

Policy

- Acts of violence can cause pain, distress and even disability or death. Physical attacks are obviously dangerous but serious or persistent verbal abuse or threats can also damage employees' health through anxiety or stress. The organisation considers violence towards employees and the risk of violence to be a serious matter and recognises and accepts that it has both a moral and legal responsibility to prevent or reduce the risk of violence at work through:
  - The introduction of measures to reduce the risk of violence at work;

- Full support for any employee who has been subject to abuse, threats or assault in the course of their work;
- The reporting of all incidents of violence through the Accidents Reporting Procedure;
- The full investigation of all reported incidents and the reporting of all assaults to the Police;
- Serious incidents of violence by employees or students involving assault on another person being dealt with in accordance with the Disciplinary Procedure.

- Persistent or isolated examples of behaviour amounting to racial or sexual harassment will be dealt with in accordance with the Harassment Procedure and/or Disciplinary Procedures.

- In accordance with the Health and Safety Policy, Managers are responsible for the implementation of any measures introduced in accordance with the spirit of this policy within their area of control. The policy should be referred to in conjunction with other Policies, in particular:

  Working Alone Policy and Procedure
  De-Escalation Policy and Procedure

- Managers must ensure that a review of all working activities is undertaken and that an assessment is made of those work activities that might result in members of staff and others being exposed to violence e.g. working practices and procedures and/or physical aspects of premises. Appropriate controls should be introduced.

- **Assessment**

  - Effective measures for dealing with violence at work need to be based on proper analysis of the problems and careful consideration of the alternatives that are available. Issues to be considered include: Œ

    - Physical aspects of premises;
    - Working practices and patterns; Œ
    - Staff training;
    - Contact with the police.

- **Physical aspects**

  - The general design and physical environment of buildings can sometimes be improved to reduce the likelihood of violent incidents, and can help reduce the numbers of unauthorised visitors wandering through the building.

- **Working practices and patterns**

  - Particular activities or jobs can create the potential for violence. Sometimes activities can be altered so that they are carried out in a way which reduces the risk e.g. working in pairs or providing a means of summoning help. Where activities or jobs cannot be altered or adapted, specific precautions or techniques may help reduce risks. Specific precautions to be considered include dealing with angry Service Users or the public, training in interpersonal skills, and precautions for home visits and off-premises activities.

- **Staff training**

  - Employees who may be exposed to violence need to be aware of the risks they face and be capable of recognising the potential for danger, either to themselves or to others, through staff training.

Procedure

- **Contact with the Police:**

  - Where circumstances require it, assistance may be summoned from the Police by dialling 999.

- Risk Assessments must be reviewed at intervals to ensure the effectiveness of the measures introduced for the control of violence.

- **Training:**

  - Appropriate training for employees will be included in the Staff Development and Training Programme arranged by the Head of Personnel Services.

- **Reporting of Incidents:**

  - Any incidents of violence, as defined in paragraph 1, must be reported to the Health and Safety Officer using the Accident (Personal Injury) Report in accordance with the Accidents and Hazards Investigation and Reporting Procedure. Certain acts of physical violence must be reported to the Health and Safety Executive in accordance with the Reporting of Injuries, Diseases and Dangerous Occurrences Regulations and this will be undertaken by the Health and Safety Officer.

Reviewed by Marcus Kerridge McColl
December 2025

Maternity Leave Policy and Procedure
Crescent Community Care Services Limited

Purpose

To comply with the:

- Employment Relations Act 1999;
- Employment Act 2002;
- Equality Act 2010;
- Statutory Maternity Pay (General) Regulations 1986;
- Statutory Maternity Pay (Medical Evidence) Regulations 1987; Š
- Maternity and Parental Leave Regulations 1999.

Scope

- All employees.

Policy

- The organisation's policy is to comply with both the letter and spirit of the law on maternity rights. To this end, its aim is to inform all female employees, regardless of marital status, of your entitlement to statutory maternity rights and to ensure that those rights are understood by employees who qualify. "You" in this policy applies to you if you are a female employee.
- This policy and procedure covers the basic (sometimes called statutory) rights that employers must provide for new and pregnant mothers who work for them.
- This policy also covers the basic allowance that relates to antenatal rights for fathers and partners.

Summary of employment rights if you are pregnant.

- You are entitled to:

  - Paid time off for ante-natal care;
  - 52 weeks' maternity leave, regardless of how long you have been with us;
  - Statutory maternity pay for 39 weeks, provided you have worked for us for 26 weeks by the 15th week before your baby is due. You may qualify for maternity allowance if you do not qualify for SMP;
  - Continuation of the contract of employment, except for pay;

- Return to the same employment after ordinary maternity leave. After additional maternity leave you are entitled to return to the same job unless that is not reasonably practicable, in which case the right is to return to an appropriate similar job;
- Protection from detrimental treatment or dismissal, including redundancy, on any grounds partly or wholly related to pregnancy;
- 18 weeks' unpaid parental leave until the child's 18th birthday;
- Dependants' leave, where appropriate.

- Partners of pregnant women and adopting parents may be entitled to paternity leave.

Maternity leave and your employment contract

- The first 26 weeks of maternity leave are deemed ordinary maternity leave. Your employment contract continues during ordinary (and additional) maternity leave.

- The only element of your contractual rights that does not continue is your wages or salary. You are entitled to receive the same basic statutory rights such as paid annual leave – which you can continue to accrue while on maternity leave.

- Additionally, as your employer, we must not discriminate against you while you are on any part of the maternity leave.

Starting maternity leave

- The earliest that you can start the maternity leave is 11 weeks before the expected week of childbirth, which is when you are around 29 weeks pregnant. To ensure that your rights are upheld, you should use the due-date given on the MAT B1 pregnancy certificate that your midwife or GP will give to you.

- In order to work out when the 11th week before the expected week of childbirth falls, you should find the Sunday before the date which the certificate states the baby is due, or the due date if it is a Sunday, and count back 11 Sundays from that date. The earliest date on which you can start the leave is that Sunday. As long as it does not fall before the 11 weeks before the baby is due, it is generally up to you as to when your maternity leave begins.

- You can even choose to work right up to the date that the baby is born.

- You should note that the start of the maternity leave might be triggered automatically if:

  - You have a pregnancy-related illness or absence in the last four weeks of your pregnancy. In this case you must inform us that you have an illness that relates to the pregnancy and we can insist that you start you maternity leave even if you are off sick for only one day. However, we may allow you to carry on working until you had planned to start your leave, especially if you have only been away for a short time.

  - Your baby is born before the day on which you were planning to start the maternity leave. In this situation, your leave starts automatically on the day after the day of the birth. You must notify us as soon as is reasonably practicable that you have given birth, as well as noting the date of the birth.

- Having been notified of your intention to take maternity leave and the date on which it will begin, we will give you notice of the date that your maternity leave period shall end. We will provide you with this information within 28 days of you informing us of the date on which the maternity leave period will start, or on which it has started.

Additional maternity leave

- The second 26 weeks of maternity leave are deemed additional maternity leave. This leave starts on the day after the ordinary maternity leave period finishes. Your employment contract continues during additional maternity leave.

- You are entitled to return to the same job on the same terms and conditions. If, exceptionally, this is not possible (for example, if the job no longer exists) you are entitled to suitable alternative employment on terms that are not substantially less favourable.

- The contractual rights of women on additional maternity leave are maintained, except those rights related to pay, as they are during ordinary maternity leave.

- The only difference between ordinary maternity leave and additional maternity leave is in relation to pension rights. We will maintain your contributions to the pension scheme, if such applies, throughout ordinary maternity leave and

throughout a period of paid additional maternity leave (based on the salary you would have received if you weren't on leave). We are not obliged to maintain our contributions for any period of unpaid maternity leave. This means if you are paid for 39 weeks of maternity leave, and you take the full 52 week entitlement to leave, our employer contributions may not be made for the final 13 weeks. If this is any different it will be shown in your contract.

- Employee contributions to the pension scheme, if applicable, will be based on your actual pay for the period of the maternity pay period (39 weeks).

Compulsory maternity leave

- It is compulsory for you to take 2 weeks of maternity leave, following the birth of your baby. This is for health and safety reasons.

- All employment entitlements and conditions that apply during the maternity leave period will also apply during the period of compulsory maternity leave.

Notice of maternity leave

- You must notify us of your intention to take ordinary maternity leave by the end of the 15th week before the child is due and you must give us the information in writing if we request it. You can notify us earlier than this, and may find it helpful to do so in order to ensure that your health and safety rights are observed, and that you have time off for antenatal care.

- Your notice must state:

  - That you are pregnant;
  - The expected week of childbirth;
  - The date on which you intend the ordinary maternity leave to start.

- We can request that you give us a copy of your MAT B1 certificate which you will be given by either the GP or midwife when you are around 20 weeks pregnant and which will state the week that the child is due. We can request that all the notice that you give is in writing.

- Once you have given notice of the date on which you intend the leave to begin, you can subsequently change your mind and vary the date. In this case, you must notify us of the variation at least 28 days before the new date begins, or if this is not reasonably practicable, as soon as is reasonably practicable.

Notice of return to work

- You do not need to give any notice that you are returning to work at the end of the maternity leave. However, you might find it helpful to contact us just to make sure that we know you are coming back and the contact would be appreciated.

- On the other hand, if you wish to return to work earlier than the 52 weeks entitlement, you must give us 56 days/8 weeks' notice of the intention to return early. If you fail to do this, and arrive at work regardless, we can refuse to pay you.

Job role on return

- After returning from ordinary maternity leave (OML), you have the right to go back to the same job that you were in before you went on leave.

- After returning from additional maternity leave (AML) you also have the right to return to the same job unless we can demonstrate that it is not reasonably practicable to have kept that job open for you. In this case, you are still entitled to an alternative job that is both suitable and appropriate for you.

Flexible hours

- All employees who have worked for us for 26 weeks have the statutory right to request flexible working.

Entitlement to Statutory Maternity Pay

- You must meet the following three conditions in order to be entitled to SMP:

- You must have worked for us for at least 26 weeks by the end of the qualifying week (i.e. by the end of the 15th week before the week the baby is due);
- You must still be in the job in the qualifying week;
- You must have been paid at least the lower earnings limit on average in the 8 weeks (if you are paid weekly) or the two months (if you are paid monthly) up to the last before the end of the qualifying week.
- To receive SMP you must give us 28 days' notice of the date on which you want the pay to start and you cannot change your mind about this date.

- Your SMP can start on any day of the week that you choose and in most cases will start on the same day as the maternity leave period starts.
- To claim you SMP, you must give us a copy of the MAT B1 form – the maternity certificate - which states the expected week of childbirth which the midwife or GP will give you when you are around 20 weeks pregnant. You may find it easier to give the notice for the leave and pay together in the 15th week before the baby is due. However, if you decide to do this then you must remember that you cannot change the date on which the pay starts, even if you decide to vary the date on which the leave begins.

Workers versus employees

- If you are a worker rather than an employee, for example, because you obtained work through an agency, or are freelance, you will not get the full set of maternity rights, but you will still receive protection, such as health and safety risk assessments that we must carry out, and the right to claim sex discrimination if you should suffer detrimental treatment as a result of your pregnancy.
- You may qualify for maternity allowance, which is paid by the Government through your local Jobcentre Plus office, rather than through us.

Benefits for those not entitled to SMP

- If you are not entitled to receive SMP because you are a worker or for other reasons, you may qualify for maternity allowance (MA) and you should seek advice from the local Jobcentre Plus office or Citizens Advice Bureau about your eligibility.

Sickness during pregnancy, maternity leave, or when supposed to return to work

If you are absent through sickness during pregnancy, this will not automatically trigger your maternity leave and you will be treated as every other member of staff who is off sick. You will usually be paid sick pay in the same way as other staff. However, if you are sick during the last four weeks of the pregnancy then we can insist that the maternity leave starts, although we may overlook occasional days of absence.

Once the maternity leave has started, you cannot claim sick pay from us if you become ill. If you are sick when the maternity leave is due to end, our usual sickness procedures will apply to you.

Time off work to attend antenatal classes

- An antenatal appointment is an appointment you make on the advice of your doctor, midwife or health visitor.
- You have the right to reasonable paid time off for antenatal appointments, and this includes the time spent travelling to an appointment and waiting. It does not include entitlement to pay for time where you could reasonably attend work beforehand or return to work afterwards.
- You cannot be refused time off for the first appointment, but for subsequent appointments we can ask you for written proof of the appointment and a certificate or note from your doctor or midwife, stating that you are pregnant.
- Antenatal care may include not only medical examinations, but also relaxation and parent craft classes.
- The right to paid time off for employees begins on the day the job starts and there is no minimum qualifying service.
- You are not under any obligation to make up the 'lost' hours at a later date.

- This right only applies if you are classed as an employee rather than a worker.

Antenatal rights for fathers and partners

- Fathers and partners can take unpaid time off work to accompany a pregnant woman to see a midwife or obstetrician.
- The right is to time off on up to two occasions for a maximum of 6.5 hours each.
- You should complete the antenatal appointment request form if you are an expectant father or the partner of the expectant mother and plan to accompany the mother to an antenatal appointment.
- Time off to attend an antenatal appointments is time off without pay.

Breastfeeding

- The law does not specifically allow you to delay the return to work from maternity leave or to take special leave once back at work in order to continue breastfeeding.
- You should ensure that we have written notification that you are breastfeeding so that in the health and safety 'risk assessment' we can take account of the particular risks to you and the baby while you are breastfeeding.

Job ceasing and SMP

- As long as you are employed in the 'qualifying week', then you are still entitled to receive SMP provided you meet all of the other conditions outlined above. It does not matter if you are off sick or on holiday in the qualifying week.
- Once you have qualified for SMP you are entitled to receive it for the full 39 weeks. This is the case even if you are made redundant, you leave the job, or a fixed-term contract comes to an end at any time after the 15th week before the baby is due or during the maternity leave.

Part-time working

- The maternity rights of part-time employees are the same as those of a full-time employee.

If you become pregnant during maternity leave

- The maternity leave does not break the continuity of employment, so the right to maternity leave for this pregnancy will be based on your total service with us. You will also qualify for SMP, as long as they meet the normal conditions.
- You must meet the same notice requirements as you did the first time you went on leave.
- You must physically go back into work for a period – even just one day – in order to maintain all of your rights. You will, however, have the same rights as you would have received on returning from additional maternity leave i.e. the right to return to the same job, or if that is not reasonably practicable, a suitable alternative job on similar terms and conditions.
- You should also note that SMP is calculated on average earnings of 8 weeks before the 15th week before the baby is due. Therefore, if you received statutory maternity pay or maternity allowance in this period, then the next maternity leave will be calculated using the actual amount of money you received in this time.
- Therefore returning to work between the two leave periods may increase the amount of SMP you receive, particularly if you can return 8 weeks before the end of the 15th week before the baby is due.

More than one part-time job and SMP

- If you work for a second employer, you are allowed to exercise your rights regarding maternity pay and leave with both of us, on a pro rata basis for each job. However, you must ensure that you have worked for each of us for 26 weeks by the qualifying week (15th week before the baby is due), and that you give the correct notification of when the maternity leave will start to both of us.
- In total you are entitled to receive the same amount of maternity pay as someone who works in a full-time job. However, if you have not worked for us for 26 weeks by the qualifying week, then you will not be entitled to receive maternity pay from us.

Working for us during maternity leave

- After the first 2 weeks after the baby's birth, you may carry out up to 10 days' work for us (or for each employer who pays you SMP, if you have more than one) without losing any maternity pay or leave. These 'Keeping in Touch' (KiT) days could include attending a training day or staff meeting or actually doing a full day's work. Anything you do on any day will count as though you had worked a full day (even if you only went into work for an hour), but KiT days cannot be used to extend your maternity leave period.

- Keeping in Touch days have to be an agreement between you and us. If you choose to work on a KiT day, we do not have to pay you normal wages, but your maternity pay is not affected by working such a day.

Reviewed by Marcus Kerridge McColl
December 2025

Medical Emergency Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To support and train employees to deal effectively with medical emergencies.

Scope

- All medical emergencies.

Policy

- The competent Carer should contact the GP and if necessary the Emergency Services, before returning to the Service User to ensure their safety and wellbeing.

Procedure

- Care staff training in medical emergencies and basic first aid will be included in the Training Plan.
- Care staff will contact the office for advice and guidance in an Emergency situation.
- Care staff may be requested to summon the Service User's GP or emergency services as instructed by the Care supervisor/Area Co-ordinator/Manager.
- Care staff will inform next of kin/advocate/family members.
- **ALL EMERGENCY ACTION MUST BE DOCUMENTED IN THE Service User Care Plan IMMEDIATELY FOLLOWING THE INCIDENT.**

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To comply with current legislation and best practice in the administration of medicines, including**:**
  - Relevant evidence-based guidance and alerts about medicines management and good practice.
  - Published by appropriate expert and professional bodies, including:
    - National Patient Safety Agency
    - National Institute for Health and Clinical Excellence
    - Medicines and Healthcare products Regulatory Agency
    - Department of Health
    - Royal Pharmaceutical Society of Great Britain (RPSGB)
    - Social Care Institute for Excellence
    - Medical and other clinical royal colleges, faculties and professional associations
  - The safe and secure handling of medicines: a team approach (RPSGB, 2005)
  - Safer management of controlled drugs: Guidance on strengthened governance arrangements (DH, 2007)
  - Safer management of controlled drugs: Guidance on standard operating procedures for controlled drugs (DH, 2007)
  - The handling of medicines in social care (RPSGB, 2007)
  - Research governance framework for health and social care: Second edition (DH, 2005)
  - The Mental Capacity Act 2005
  - Mental Capacity Act Code of Practice 2007
  - Misuse of Drugs Act 1971
  - Health and Social Care Act 2008
  - NICE Guidelines – Managing Medicines in Care Homes 2014

Scope

- All medications, drugs.
- All persons delivering Care or support, and persons ordering, receiving, managing, administering, disposing and recording medicines.

Policy

- This policy must be read in conjunction with Royal Pharmaceutical Society - Handling Medicines in Social Care, which takes precedence over this document, and any specific requirements of the Registration Authority. These publications can be found in the Useful Documents section of the QCS web site.

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COMMON POLICY

1. Roles, Responsibilities and arrangements for supervision

- Management and supervisors are responsible for:
  - The training and assessment of Care workers in the skills and techniques of medications management consistent with their role;
  - Instruction of the Care workers in the specific requirements in respect of medications management;
  - Ensuring that Care workers are at all times provided with administration systems and resources to support their role;
  - Supervision and auditing of individual Care workers and systems in order to ensure compliance with this policy, as well as with legal requirements and best practice, and to ensure that at all times Care workers are meeting the needs of Service Users;
  - Regular audit of medications records in order to confirm that the standards of practice and record keeping in respect of medications are as specified to the Care worker. Shortcomings will be reported into the organisation's quality assurance system, and in addition the Care worker(s) involved will be formally supervised and remedial action taken.

- Care workers are:
  - Permitted to assist the Service User in taking prescribed medication, within the limits of the Care worker's training and experience;
  - Care workers will be instructed, through induction and continued training, in the skills, requirements, and level of competence which are required in order to carry out their role. In particular, Care workers who are not a Registered Nurse must have completed a Medications Administration course approved by the Registration Authority before administering medicines, and must also have completed a Medications Management course approved by the Registration Authority if they manage the system through which other Care workers and themselves administer medications;
  - Care workers may purchase, on behalf of the Service User, non-prescribed medicines, and assist in the taking of those homely remedies, only in accordance with homely remedies policy and procedure contained in this policy. Specifically, the Care worker may not purchase or administer homely remedies which have not been specified for use by the Service User's GP.

2. Medications management in a Service User's own home:

- General:

- Wherever possible, Service Users will manage their medications and self-administer them. If an assessment determines that this is not possible, potential Care Plans may include:
    - A Care worker with appropriate training encouraging the Service User to self-medicate, confirming that the medication has been taken or refused, recording this result, including the time, and taking the action described elsewhere in this policy in the event of a refusal;
    - A Care worker trained and certified as competent may administer the medication, taking responsibility for correct administration according to this policy and procedure.

- Doses can be left out for the Service User to take at a later time. This should be used as a last resort and other options should be discussed before deciding on this action, such as an additional visit for medication prompt. If no other suitable option can be found, then it should be agreed with the Service User, included in the Care Plan, and risk assessed to protect both the Service User and Carers.

- Refusal of medication:
    - Identify any special medication administration preferences of the Service User by reading the medication Care Plan.
    - If a Service User refuses to take their prescribed medication then the reasons for refusal should, if possible, be identified with the Service User, and a second attempt made to offer the medication having addressed the Service User's concerns and requirements.
    - A Service User MUST NEVER BE FORCED TO TAKE MEDICATION; this act would constitute abuse.
    - When a Service User refuses medication advice should be obtained from the prescribing GP in order to minimise medication risks.
    - Medication refusal and consequent action taken by the Care worker must be documented on the medication Care Plan and the medication administration record, and the information communicated in the on-site attendance documentation to the incoming person responsible for medication.

THE REMAINDER OF THIS POLICY DOCUMENT WILL ONLY APPLY IN THE EVENT THAT Care STAFF BECOME INVOLVED IN EITHER SUPERVISING OR ADMINISTERING MEDICATIONS, AND SHOULD BE APPLIED APPROPRIATELY. IT IS RECOMMENDED THAT IN SERVICES WHICH MAY BECOME INVOLVED IN MEDICATIONS SUPERVISION AND/OR ADMINISTRATION, ALL Care STAFF ARE REQUIRED TO READ AND UNDERSTAND THIS POLICY, TO AVOID THE POSSIBILITY THAT A Service User CEASES TO SELF-ADMINISTER BUT THEIR Carer IS NOT APPROPRIATELY TRAINED TO DEAL WITH THAT EVENT.

Mental Capacity Act

- Assessments in accordance with the Mental Capacity Act 2005 policy must be carried out in order to determine if there any issues regarding the Service User's mental capacity and understanding e.g. where the Service User may lack the capacity to make a decision regarding taking medication. In certain circumstances section 6 of the Mental Capacity Act allows for the use of reasonable restraint in administering medication, and advice should be sought from health care professionals where restraint may be necessary to prevent harm.

3. Deprivation of Liberty Safeguards

- This policy should be read in conjunction with the Deprivation of Liberty Safeguards policy, in order to determine where a Service User lacks capacity to make decisions about taking medication, where administration of medication might constitute a deprivation of the person's liberty such as where the medication constitutes a sedative.

4. General

- Medicines are managed and administered safely to Service Users that need them at a time they need them and by staff who have been trained and assessed as competent to do so.

- Service Users' rights to choice, independence, privacy and dignity are promoted by staff in the medication processes and procedures in the Care service and Service Users' values and beliefs are respected by staff who are involved with the management and administration of their medication.

- All legal requirements for the management and administration of Service Users' medication will be adhered to at all times and designated staff will be responsible for the sharing of information regarding medication with the individual Service Users and, where appropriate, with their advocates.

- Medication will be used to prevent and manage disease and alleviate discomfort.

- Service Users will be fully involved with the management and administration of their medication.

- They will be fully involved by the staff about their individual medication and its purpose and will have the medication patient leaflet information provided by the dispensing pharmacist shared with them in a method which promotes their understanding.

- Before medication is administered to any Service User, **FORMAL CONSENT** must be obtained. Where a Service User is unable to give valid consent due to mental incapacity, best interest meetings will take place and where it is agreed that it is the best interest of the person, including their medical interests, that medication is administered then **FORMAL AUTHORISATION** for medication administration will be obtained and evidenced in the Service User's Care Plan and medication records.

5. Training

- Care workers must be trained in the handling and use of medication and have their competence assessed:

  - By supervisor's observation of practice during the first medication handling following initial training completion.
  - By supervisor's observation of practice at 3 months following initial training completion.
  - Reviewed at formal supervisions.
  - By supervisor's observation of practice annually.

- Each staff member will have an individual record of medication training, competence assessment, and practice skills supervision monitoring.

- Training must include:

  - The supply, storage and disposal of medicines
  - Safe administration of medicines
  - Quality assurance and record keeping
  - Accountability, responsibility and confidentiality

Induction Training

- CERTIFICATION of previous medication training must be supported by a competence assessment of the staff member's knowledge and ability to manage and administer medication before any such staff member is permitted to undertake medication handling.

- Staff must be given time to read and understand all of the provider policies and procedures related to medication management and administration.

- **Basic medication training** prepares Care staff:

  - To give medicines via the mouth (tablets, capsules and liquids); » Drops into the ears, nose and mouth;
  - Medication by inhalers;
  - Medicines applied to the skin

- **Further training is needed for:**

  - Any invasive techniques such as giving suppositories, enemas and injections;
  - Medication competence is assessed formally by the Care manager or Provider where qualifications and skills are appropriate. This is carried out by observation of practice against the medication policies and procedures.

- **The following key tasks of Care workers are to be observed:**

  - Checking what medication the Service User takes on the medication administration record and on the medication labels;
  - Checking it is the right person;
  - Asking whether the person wants the medication;
  - Checking that the medication has not already been given;
  - Preparing the correct dose for the time of day;
  - Giving a suitable drink, with the medication, to the Service User;
  - Signing the medication record entering the relevant data (for example time, where specific time is required or for a medication which is taken only when needed e.g. for pain management).

Further details of the skills and knowledge required for medicines handling can be found at the Skills for Care website www.skillsforcare.org.uk.

- Care workers are permitted, if trained, to:
  - Give capsules tablets and oral medicines;
  - Apply external creams and lotions;
  - Insert drops to ears nose or eyes;
  - Administer inhaled medication.
- Specialist additional training is needed in order to enable Care staff to carry out the following:
  - Rectal administration. Suppositories;
  - Injectable drugs such as insulin;
  - Administration via gastrostomy tubes;
  - Administration of oxygen.
- **Timing of medication** administration can be crucial and **adherence to medication prescription instructions must be followed**. This must be clearly indicated in the medication Care Plan and in the medication administration record.
- **Medicines must be given only to the person for whom they are prescribed,** following the prescription instructions.
- **Medicines must be given only from the container in which they are supplied by the Pharmacy or GP**. DOSES OF MEDICATION MUST NOT BE PUT OUT IN ADVANCE OF ADMINISTRATION; this can lead to errors and accidents.

Ongoing updating

- Health and social care practitioners should be able to access reliable and up to date information about medicines. Resources may include the patient information leaflet supplied with the medicine and the following websites:
  - Medicines and Healthcare Products Regulatory Agency
  - NHS choices
  - NICE Evidence
  - Patient.co.uk
- Health professionals may also use the:
- British National Formulary (BNF)
- British National Formulary for Children (BNFC)
- Clinical Knowledge Summaries Electronic Medicines Compendium

Particular staff training issues for services delivered in the Service User's own home:

- Where service staff only supervise Service Users' self-medication:
  - There may be occasions when the Care service needs to offer support with medication due to Service Users' mental health issues;
  - Training must include:
    - How to support Service User to take responsibility for their own medicines;
    - Action to be taken if a Service User becomes unwell and unable to take full responsibility for medicines;
    - Obtaining Service User formal consent if Care workers give medication;
    - Which medication the Care worker can give, having been trained to do so;
    - By what method the Care worker keeps records;
    - Providing medication storage on an individual Service User basis.
    - Policies and procedures to guide homecare staff in the processes of medication handling in the person's own home including what to do if a person decides not to take their medication.
    - What to do if the Service User's mental or physical health state changes significantly.
    - How to communicate between Care workers and other agencies.
    - How medicines that are no longer in use are disposed of.

- Where service staff administer medications to Service Users:
  - Homecare providers must ensure that their insurance policy provides suitable indemnification for the policies and procedures that are followed.
  - Homecare workers who work alone must be appropriately trained and regularly assessed for their competence in the safe handling of Service Users' medication.
  - The Care provider must have clear plans on the individualised support for Service Users in their own homes who use medication. This must be detailed in their Care Plan and actively reviewed and monitored by the designated Care staff, who will have the appropriate skills and knowledge to do so safely and effectively.
  - Communications between Care workers, supervisors, and prescribers must be effective in order to ensure that the Care staff are kept up to date with Service User medication changes and associated support needs.
  - Important points for domiciliary Care staff:
    - The amount of help and support the Service User needs with their medications;
    - Which medications the Care worker may administer after being trained to do so;
    - The way that the Care worker keeps records.
    - What the Care worker MUST NOT DO e.g. OFFER ADVICE ON TREATMENT OF MINOR AILMENTS.
  - It is not appropriate for a Care worker to influence:
    - How a person chooses to obtain medicines;
    - How and where the person chooses to keep medicines in The Agency;
    - How medicines that are no longer in use are disposed of;
    - The choice of over the counter medicines the person/parent of the child Service User may wish to buy;

6. Confidentiality

- All medication records for the Service User must be treated as confidential, and permission obtained from the Service User (or their advocate where appropriate) for the sharing of medication records, when this is needed to contribute to their health and wellbeing.

- Where Service Users wish to be given their medication in private this must be accommodated to support their personal dignity and promote their autonomy.

7. Self- administration

- All Service Users that have the ability and mental capacity to self-medicate should be given the opportunity and support to do so.

- In order to protect the safety of the Service User and others it is essential to assess the Service User's ability to manage their medications independently and safely. This assessment should include:
  - Whether the Service User wishes to self-medicate;
  - Identify that the Service User knows the medication they are taking, what it is for, and how and when to take it;
  - Understand how important it is, not to leave the medicines lying around where someone else may take it accidentally.

- A full assessment of their ability and mental capacity to self-medicate must be carried out and documented.

- In a Service User's own home, the assumption is that the GP supports self-medication. However, if there is any doubt then self-medication by the Service User should be authorised by the prescribing GP, following a risk assessment.

- The assessment and documentation will be stored in the individual Service User Care Plan, and a copy held with the Service User's medication record when Service Users are self-medicating. This will help to remind staff of the need to monitor any associated risks.

- The assessment will be reviewed if there is a change in the Service User's mental or physical health state, and on a routine basis timed according to self-medication Care Plan instructions (as the timing of review will depend on individual needs and risks).

- The Agency will have an individual medication record for Service Users who self-medicate and their medication supply will be checked in and out, as for other Service Users.

- Any Service Users who share their home with other people and self-medicate will have a locked facility in their room for the safe storage of medicines. Where the Service User is taking controlled drugs they are also kept in their personal locked storage area together with other medications.

- Consent must be obtained from the Service User to enable designated Care staff to monitor the medication supply being used by the Service User and to monitor the Service User's continuing ability to manage their medication safely.

- A full audit of the medications used in self-medication will need to support the tracking of the medication from the point of ordering, receipt into The Agency, the date given to the self-medicating Service User, the usage of the medication by the Service User and the destruction or disposal of any medications that are not required.

- When Care staff have to prompt self-medicating Service Users to take their medication this should be documented and closely monitored by all staff involved in medication management and administration. If this situation is reoccurring a review of the risk assessment for self-medication for the individual should be carried out and appropriate action taken and documented in the Service User's Care Plan and medication records.

8. Cultural requirements and medication

- Vegetarians and people from certain religious groups will not want to take gelatin capsules, because they are made from animal products.

- Some people may prefer to have medications given to them by people of the same gender.

- Some religious festivals include fasting and some people prefer not to have medicines given at certain times.

- Followers of the Jewish and Islamic faiths may be concerned about medicines containing substances which are unclean according to the tenets of their faith.

9. Obtaining medication supplies

- Care staff should either support the Service User in ordering their own repeat prescriptions, or take responsibility for doing it themselves, in which case the responsibility must be clearly noted in the Care Plan.

- Amounts to be ordered must be checked against the supply required in consideration of any residual medications which are prescribed for intermittent or irregular administration.

- Prescriptions received from GP practices should be checked against the requested medications before submitting the prescriptions to the pharmacy.

- Any unexpected medication changes should be checked with the GP before use.

- A formal process for the arrangements with the dispensing pharmacist should be organised and documented in order to specify the internal medication training and competence assessment programme, and to be available for staff reference.

- On receipt of the medication the designated staff member must check the dispensed and received medication against the list of prescribed medications and the order, checking the amounts in each individual medication container against that stated on the contained label. Boxed medication should be opened and checked for the amounts contained to minimise error potential and support stock audit.

- The prescriptions must always be ordered by the person designated for that purpose.

- The pharmacy should be notified of a person's medication when:

  - The service begins;
  - The Service User returns from a period of hospitalisation;
  - There is any change to the medication.

- The Care service must also have a formal process for the obtaining of medication prescribed to deal with acute health problems. Medication prescribed as a one-off should be obtained quickly and efficiently and new medication should be supplied as soon as possible, with no delay beyond 24 hours at the latest.

- Staff ordering medicines must be provided with protected time sufficient to carry out the task accurately.

10. Administration of medication

- Medication must be given safely to maximise the benefits to Service Users' health and wellbeing.
- Medication must be managed and administered by procedures and processes which promote Service User independence, choice, privacy, and dignity.
- Medication management and administration procedures and processes must take account of Service Users' cultural and religious values and beliefs.
- Service Users must be given an opportunity to self-medicate and be given the range of support and Care needed to enable them to self-medicate when they have been assessed and proved to be able to self-medicate.
- Where Service Users have consented to their medication to be administered by staff, they should be assured that the staff responsible have been trained and assessed as competent to do so.
- All Care staff, including those who are not directly concerned with the administration of medicines, should be trained in the understanding of medications, the main types of medications in use, their administration procedures, and how to look for and report possible adverse reactions, including changes which may require review of the Service User's medication prescription.
- Only staff who have had certificated medication management and administration training and who have been assessed by the manager as competent should be involved in the administration of medication.
- Medication must be checked, administered and signed for on the medication record by one person who is accountable for all stages of medication with respect to individual Service Users.
- MEDICATION MUST NOT BE TAKEN FROM ITS ORIGINAL CONTAINER AND GIVEN TO ANOTHER MEMBER OF STAFF TO GIVE THE MEDICATION TO THE Service User as the person checking the right dose for the right person must also witness the person taking the medication, and must be sure that the medication has been taken properly by the Service User.
- Carers must not administer insulin injections.
- Staff administering medicines must be provided with protected time sufficient to carry out the task accurately.

11. Storage of medication

- Medications must be stored in safe place.
- Where medicines requiring refrigeration are in use, the accommodation must have a fridge and the daily temperature recordings will monitor the temperature requiring MIN 2°C and MAX 8°C. The fridge will be large enough to store all the medications requiring refrigeration.
- External medications such as creams and lotions should be stored separately from other medications.
- Dressings, catheters, and ostomy products will be stored separately from other medications.
- There will be adequate storage facilities to ensure all medications are stored correctly and that no medications will be stored on the floor.
- If required, there will be, on the premises where they are used, a suitable waste disposal unit for paper waste, and a sharps clinical waste disposal bin for any used or waste needles and sharps.
- Individual locked medication cupboards in Service User rooms may be used for self-medication and for administration by Care staff.

12. Record keeping

Record what you do when you do it. As medicines are given they should be recorded immediately and signed for by the person responsible immediately following the administration, and before moving on to the next
Service User.

- Record any medications not taken, with reason.

- Correct mistakes with a single line through the text, accompanied by a signature and date and time. Never use correction fluid.

- Record medicines given by other professionals, such as a visiting health professional.

- Records will be clear, complete, legible, written in black ink dated and signed to say who has made the record.

- The intention of the medications records is to allow a full stock quantity reconciliation to be carried out without warning at any time. Therefore records must include:

  - Receipt;
  - Dispensing;
  - Destruction or disposal for whatever reason;
  - Transfer to another person or facility.

- The medication administration record for an individual Service User will include the name of the Service User, date of birth, weight, name of the drug, the dose, and time to be given, and any special requirements, e.g. with food only.

- Hand written medication administration records should be produced only in exceptional circumstances, and created by a member of staff with the appropriate medicines administration training for the setting. The hand-written record must be checked and verified by a second member of staff with the same training before first use.

- For self-medicating Service Users the date and the name of the Care staff who gave the Service User their supply of medication must be recorded in their medication administration record, as well as in the Care Plan, and must be accounted for by their signature.

- Where records are held on computer these must comply with data protection regulations, must be tamper-proof, and must give clear indications of who has made the record entry.

- ALL STAFF INVOLVED IN MEDICATION MANAGEMENT AND ADMINISTRATION ARE RESPONSIBLE FOR ACCURATE RECORD KEEPING.

- Medication administration records must record:

  - The medications are prescribed for the person;
  - Where, when and the quantity of any medications received;
  - The time they must be given;
  - The dose of the medication;
  - Any special administration requirements;
  - The name and designation of the person making the record;
  - Copies of emails, texts, faxes, and transcriptions of phone messages must be kept and stored with the medications Care Plan.

- Disposal of medication records must record:

  - Date of disposal/return to the pharmacy;
  - Name and strength of medication;
  - Quantity being disposed of;
  - Person for whom medication was prescribed or purchased;
  - Signature of the person disposing the medication and a witness signature for the disposal.

- **Hand-over.** Staff must ensure at shift handover that all information regarding all changes to medications administration during that shift is communicated to the incoming staff, and they have received and understood the message.

13. External medication application

- Medication applied to the Service User's skin should be applied by a staff member wearing disposable gloves. Creams and lotions must be applied according to the prescription instructions.

- Instructions for creams and lotions must be clear.

- Before Care staff members apply creams and lotions they must be trained and assessed for their knowledge and competence related to the application of external medication.

- Clear information must be available to inform Care staff as to what the Service User's cream is for how much to apply, where precisely to apply the cream, the frequency of application and for how long the application is to continue.

- There must be a Care Plan for the application of the cream and a daily account of its application.

- Creams can be stored in a locked cabinet in the Service User's room unless the cream requires refrigeration.

14. Controlled drugs

- There are special legal requirements for the ordering, storage and disposal of all controlled drugs.

- Controlled drugs are prescribed differently.

- Controlled drugs if collected from a pharmacy must be accounted for in the pharmacy controlled drugs register.

- Anyone collecting controlled drugs from a pharmacy will need to produce personal identification and sign for receipt of the medication.

- Separate records for receipt administration and disposal of controlled drugs must be kept.

- All controlled drugs must be accounted for on the Service User's medication administration record for receipt, times of administration and destruction or disposal. They must also be fully accounted for in the hardback controlled drug register.

- The register must have numbered pages and each controlled drug for each person must have a separate page to account for receipt, administration, disposal, and stock audit, which must be accounted for each time the medication is given.

- Where a Service User is self-medicating and taking prescribed controlled drugs these can be kept in the Service User's locked medication facility. Receipt of, and the date that, the controlled drugs are given to the Service User should be accounted for in the controlled drugs register and in the Service User's medication record.

Disposal of controlled drugs

- In a Care Agency without nursing the controlled drugs to be returned to the pharmacy must be accounted for in the controlled drug register, the receiving pharmacist must account in the register for the receipt of the drug to be disposed. This must also take place with a dispensing GP where appropriate.

- In a Care Agency with nursing arrangements for controlled drugs disposal should be discussed with the community pharmacist and an authorised member of the Clinical Commissioning Group contacted regarding disposal.

Storage of Temazepam and Fentanyl

- Both of these drugs require storage within the Controlled Drugs storage facility, and accounting for in the same way as Controlled Drugs, even though they are not Controlled Drugs. This is a requirement of the Registration Authority, which overrides any otherwise legally compliant advice given by a Community Pharmacist.

15. Non-prescribed controlled drugs

- If a Service User is found to have non-prescribed drugs in their possession then the local police should be contacted for their advice.

16. Covert medication

- This is the term used when medications are hidden and given without the consent of the Service User.

- Covert medication must not be given to a Service User who has the capacity to give their consent or refusal with respect to receiving medical treatment.

- GIVING MEDICATION BY DECEPTION IS POTENTIALLY AN ASSAULT.

- The covert administration of medication should only take place within the context of existing legal and best practice frameworks to protect the person receiving the medication and the Care staff involved in giving the medication.

- Covert medication must not be used without, firstly, a formal assessment of the Service User's capacity and, where they lack or may lack capacity, that administration of medication is in the person's best interest. Where covert administration of medication may constitute a deprivation of the person's liberty, a referral to the supervisory body responsible for authorizing Deprivation of Liberty Safeguards should be sought. Authorisation for covert medication must be obtained before covert administration is used.

- Before medication can be given covertly, the Service User's doctor and pharmacist must be consulted and must give their written authority for the medication prescription and method of administration to be used.

- Where covert medication is formally agreed there must be a clear assessment and Care Plan to demonstrate the need for covert medication and the individual review must be stipulated by the responsible doctor and professional authorisation teams. Reviews must be carried out following any change in the Service User's capacity.

- A useful publication is "Covert Medication", published by the Mental Welfare Commission for Scotland.

17. Adverse drug reactions

- If a Service User becomes unwell after taking a new medication then the prescribing GP should be notified immediately, or it may be necessary to contact the emergency ambulance service if the reaction is severe.

- An incident report must be completed and submitted to the Registered Manager for review.

18. Inappropriate use of medication

- MEDICATION MUST NOT BE USED AS A FORM OF RESTRAINT TO SEDATE PEOPLE FOR THE CONVENIENCE OF THE CARE SERVICE STAFF. THIS IS ABUSE.

- Medication must only be administered to the person who has been prescribed that medication.

- Partly used medication dispensed for an individual, and no longer required, must not be used for any other person.

- Where several Service Users have the same medication, the medication must only be administered from the container marked with the Service User's name. This must be clearly accounted for in the drug stock audit.

- An incident report must be completed and submitted to the Registered Manager for review.

- Administration of medicines inappropriately, whether by design or accident, is potentially abuse and must be reported immediately to the Registered Manager, and the formal safeguarding process initiated.

19. Medication errors

- Any medication error must be immediately reported to the GP of the Service User affected by the medication error and any advice given acted upon to protect the safety of the Service User. If a GP cannot be contacted immediately then NHS Direct and the local pharmacy should be contacted for advice.

- The incident and all action taken must be documented in the Service User's Care Plan, medication record, and an appropriate incident form completed by the person responsible.

- The Service User and their personal advocate must be fully informed by the service manager of the incident and the actions taken in order to minimise the risk to the Service User.

- An incident report should be made to the regulatory authorities regarding the incident and actions taken by the service management following the medication incident to investigate causal factors and future medication error prevention strategies to be acted upon.

- All staff involved in medication error incidents should have an immediate formal supervision by their line manager to offer support and to support investigation of the incident.

- All medication errors should be documented on an incident report, reported to the manager and investigated to prevent further errors occurring.

- Any mistake with medication management or administration should be treated as an incident or error and recorded as such, and brought to the attention of the person in charge of the service at the time, and to the attention of the service manager.

- Examples of medication errors:

  - Medication given to the wrong person;
  - The wrong dose is given, i.e. too much or too little;
  - Medication is not given.

- All staff should be encouraged to promptly report medication errors

- All medication errors should be investigated, with any necessary changes to training and procedures being made immediately.

- All actions taken should be recorded.

- All serious incidents should be reported to the professional regulatory body.

20. Nutritional supplements

- There must be a Care Plan for any nutritional supplements used for the Service User. These must be supported by an assessment of need and a monitoring and review record to document when the supplements are given, by whom and the measured quantity taken by the Service User accurately recorded and accounted for.

- The medication record must be signed when the supplement has been taken by the Service User and accounted for by the person who is responsible for giving and witnessing the Service User taking the supplement. A daily record of nutritional supplements taken or refused by the Service User must be recorded in the Care Plan daily record.

- Refusal to accept nutritional supplements must be treated as other medication refusal and reported to the prescribing professionals and recorded as refused on the medication record by the accountable staff member.

- A regular review of the use of the nutritional supplements should be requested by the Care service and this will be made by the prescribing GP or the dietician.

- All medications given must be recorded at the time they are given and also when a person refuses their medication.

- If a person cannot swallow then their medication advice must be obtained from a health Care professional and alternative liquid medication could be prescribed. MEDICATION MUST NOT BE CRUSHED OR CAPSULES SPLIT TO GIVE TO Service Users as this may affect the way medicines work and can be potentially harmful to the Service User.

21. Homely remedies

- Where a Service User wishes to take homely remedies the GP should be contacted for advice and to arrange their agreement and consent in consideration of other medicines currently being taken.

- When a provider decides to use homely remedies for minor ailments this must be formalised in a policy about which non prescribed medications will be used and which minor ailments are to be treated by these. This policy must be formally shared with all staff responsible for medication management and administration. QCS are unable to provide this policy because it can only be locally determined, therefore a blank form is provided with this Policy for local completion; the completed document can be uploaded to the QCS system for publication within the provider organisation.

- Relatives who buy homely remedies for Service Users should be encouraged to contact the GP or pharmacist for their advice regarding proposed homely remedy medication.

- Advice regarding the use of homely remedies must be obtained from a doctor, pharmacist, or nurse.

- Homely remedy policy for the Care agency must be shared with GPs, Service Users and their personal representatives.

- Records must be kept of the purchase, administration and disposal of medication.

22. Advice from a pharmacist

- All services should have an arrangement with a local Pharmacy or dispensing GP to provide advice to staff concerning medication management and administration.

- Medication supply from a community pharmacist should be of appropriate quality and suitably labelled for its intended purpose.

- Dispensing services should be:

  - Accurate;
  - Accessible;
  - Prompt;
  - Reliable.

23. Changing a pharmacy supplier

- Agree the change of date with the new pharmacy.

- Agree the procedure for requesting repeat prescription orders and any documentation by the pharmacy or the GP practice.

- Arrange for prescription repeats a few weeks prior to commencement of the new supply.

- Ensure that current medication stocks are used before reordering.

- Dispose of any medication from the previous supplier that is no longer required by the Service User before the changeover date. Service User consent to medication disposal must be gained before doing so.

24. Monitored dosage systems and compliance aids

- MDS do improve some procedures including:

  - The system of organising repeat prescriptions for Service Users;
  - A visual check whether medicines have been prepared and given to the Service User.

- Medication NOT to be packaged in a monitored dosage system:

  - Medicines that are sensitive to moisture, e.g. effervescent tablet;
  - Light-sensitive medicines, e.g. chlorpromazine;
  - Medicines that should only be dispensed in glass bottles, e.g. glyceryl trinitrate (GTN);
  - Medicines that may be harmful when handled, e.g. cytotoxic products like methotrexate;
  - Medicines that should only be taken when required, e.g. painkillers;
  - Medicines whose dose may vary depending on test results, e.g. warfarin.

- Liquid medicines, creams, eye drops, inhalers must be supplied in traditional containers. Therefore, any Care agency that uses MDS will have two different systems operating.

- Some Care providers who have been unable to get medicines in MDS have allowed Care workers to re-package medicines in compliance aids. This is also known as 'secondary dispensing'.

- Repackaging of medicines by Care workers should NOT take place. The risk of making a mistake is too great.

25. Ordering, storage and administration of “when required” (PRN) medication

Medication with a “when required” (PRN) dose is usually prescribed to treat short term or intermittent medical conditions, i.e. it is **not** to be taken regularly. Therefore, it should be approached differently to regular repeat medication.

1. **Recording PRN Medication in residents' Care Plans**

Staff administering PRN medication must ensure the medication is given as intended by recording a specific plan in the residents' Care Plan which should be kept with the MAR chart. The details of which should include:

- The details of the medication, i.e. name of drug and strength.
- Signs and symptoms – specific to the individual
- The precise reason for dispensing, for instance 'in the event of lower back pain'.
- Why it has been prescribed and how to give it?
- How long for? If advised by the prescriber.
- When the medication should be given, e.g. before, with or after food.
- The maximum daily amount or the time to leave between doses **must** be recorded. E.g. Paracetamol – 2 tabs every 4-6hrs when necessary (**max 4 doses in 24hrs)**.

2. **Ordering PRN Medication**

PRN medication by its nature should not need to be included in the monthly repeat medication, this will help to minimise waste.

- PRN medication should always be supplied in an original box rather that a monitored dosage system (MDS). This allows for a check on the expiry date and reduces waste.
- PRN medication that is still in use and in date should be carried over from one month to the next and **not** disposed of.
- A record of the quantity carried over must be recorded on the new MAR chart so there is an accurate record of the quantity in stock.

3. **Storing PRN Medication**

Please follow The Agencys' medication policy for storage of medicines.

4. **Administration of PRN Medication**

Staff administering medication should have the appropriate training and follow the procedures set out in The Agencys' medication policy. However, when administering PRN medication the following points need to be considered:-

- PRN medication should not be offered or given only at the times listed on the MAR chart or at specific medication rounds. As it is for occasional use the person should be offered the medication at the times they are experiencing the signs and symptoms either by telling a member of staff or by staff identifying the residents' need as **outlined in the Care Plan.**
- The exact time the medication was given and the amount given (e.g. 1 or 2 tablets if prescribed as 1-2 tablets prn,) should be recorded on the MAR.
- A record does not have to be made at each medical round to show the person has been offered the medication. However, the Care Plan should demonstrate that staff know what the medication is for and have made an assessment on whether the person requires the medication.
- Consideration should be given to the residents' capacity to refuse the medication.
- The Care Plan should show the needs of the person e.g. If signs of pain are expressed in a non-verbal way.

5. **Other areas to be considered**

- If a PRN medicine is administered on a regular basis a referral to the prescriber should be considered for a review of the resident's medication. This action must be clearly recorded in the resident's Care Plan.
- Should the PRN medication not have the expected effects the prescriber should be contacted. This action must be clearly recorded in the residents' Care Plan.
- Paracetamol: If body weight is <39kgs consider giving 1 or 2 tablets up to four times a day, in order to stay within safe toxicity limits of a maximum dose of 100mg/kg bodyweight per 24 hours.

6. **The return of unused/ unwanted PRN Medication**

- Please refer to The Agency's policy for the control of medicines for homes.

26. Essential Oils

Use of essential oils.

- Always check the safety of every essential oil on each Service User with that Service User's General Practitioner before use.
- Essential oils should be used only by a practitioner qualified in their use.
- Never take essential oils internally.
- Keep essential oils in locked storage, separate from any other medications.
- Always dilute essential oils with suitable carrier oil before applying it to the skin.
- If you are using a new, untried essential oil, first do a skin patch test before using extensively.
- If irritation occurs with a specific oil or formula, discontinue the use of such oil or formula.
- Take care not to get any essential oils into eyes or on to any mucus membranes.
- A wise practice is to always wash hands after handling pure undiluted essential oils.
- When using essential oils that cause sun sensitivity people should take Care to avoid expose to the sun.
- People with a sensitive skin should always perform a skin patch test before using untried essential oils.

MEDICATION FOLLOWING SERVICE COMMENCEMENT

27. Medication assessment

- Pre-commencement assessments of medication taken by the Service User should be made to include medication name, dose, time of taking, reason for taking and Service User understanding regarding their medication.
- On commencement, the Service User's formal consent will be sought, and recorded, in order to allow the designated Care staff to support the Service User with the management of their medication supply.
- Medication assessments and associated Care Plans will be carried out by staff who have been trained and assessed as competent for the management and administration of Service Users' medication.
- All Service Users will have a formal, documented assessment for self-medication.

- All Service Users will have a formal, documented assessment of their individual needs and specialist requirements in relation to their medications.

- The assessment will include any potential side effects or known allergies associated with medication for the individual Service User.

- On commencement day all medications will be recorded on the Service User's medication assessment.

- Following the assessment the information will be used to formulate a medication Care Plan which will stipulate medication names, doses, times of administration, route of medication, and any special requirements the Service User has in relation to individual medications and their safe management.

- Any changes made by the GP will be noted and appropriate changes made in the records and in the medication supply. This will be discussed with the Service User where appropriate, and with their consent any medication no longer required by the Service User will be returned to the pharmacy.

- Service User will be advised that they should seek the advice of an independent advocate if they have concerns regarding their medication.

28. Medication review

- Medication review can be made by the prescribing GP, community pharmacist, or prescribing specialist community nurse.

- Medication review should take place:

  - As soon as possible following commencement;
  - When medication side effects are noted;
  - Where the Service User's health state changes;
  - When there are any recommendations by the Department of Health regarding particular medications taken by the Service User;
  - When the Service User is discharged from hospital and there have been changes in their medication.

29. Medication on temporary transfer or absence

- The supplying pharmacist will be able to advise the Care service and be able to offer support by providing a special container for medication at the time when the Service User is likely to be on leave.

- If a Service User is likely to be out of their home for one or more medication administration times then the Service User should be given the correct medication, clearly named and with clear written instructions stipulating the medication dose and time to be taken:

  - These medications should be entered onto the medication record as given to the Service User but not administered by the staff;
  - A risk assessment of the Service User's ability to manage the medication should be documented in the Care Plan by the staff responsible for the medication arrangements at the time the Service User leaves the building;
  - Where a Service User is unable to self-medicate the staff member escort will be trained and competent in the administration of medication, will take responsibility for the medication, and will complete the medication records on return to The Agency.

- Ensure that any person responsible for administering or assisting with self-administration of medicines while off the premises is aware of:

  - What medicines the Service User needs to take;
  - Clear directions on how, when and how much of each medicine to take;
  - What time the last dose of each was taken;
  - Contact numbers for queries, such as The Agency, pharmacy, or GP.

- On return to The Agency the senior designated staff should check with the Service User to ensure that their medication has been taken correctly and record the answer on the medication administration record.

30. Medication on permanent transfer

- On permanent transfer to another care facility a copy of the current assessment, most recent review and the current medication administration chart must be transferred together with the Service User.

- Record the transfer of information in the Service User's documentation.

- Original copies of all documents relating to medications for that Service User must be retained in accordance with normal practice for storage and retention.

31. Sharing of information on medications

- Medications information regarding a Service User is confidential and must only be shared with persons or organisation who may have need to temporarily administer medicines to the Service User, or is responsible for reviewing the medications, such as a hospital.

- The Service User must be informed of the proposed information sharing and authorise it.

32. Disposal of medication

- Medicines must be disposed of as soon as they cease to be currently prescribed, or the person for whom they are prescribed is no longer present.

- Where the Service User has left the service or died, see the 'Medication on discharge' or 'Medication on death' sections of this policy.

- Where medications for existing Service Users require disposal:

  - A complete written record of all medicines to be disposed of should be made;
  - Services providing Care in Service Users' own homes are able to return medication to the dispensing service which must have an appropriate arrangement for medication disposal in order to meet current regulatory requirements for waste management;
  - All unused medications will be returned to the pharmacy, covered by a written record of the returns on the form provided in this policy, with signatures against each item.

33. Verbal telephoned, video link or online medication instructions

- Changes of drug doses are sometimes ordered by GPs over the telephone. This must be by exception only. There must be a formal procedure for this and all staff must be made aware of the process by internal training and supervision.

- The prescriber must establish the capacity and consent of the Service User, and the training and capacity of the staff before prescribing when not present.

- Records of telephone medication messages should include:

  - Who took the call;
  - The time of the call;
  - The name of the person who called;
  - Record of changes made.

- Before ending the call:

  - Read back the information that has been written down to reduce the chance of misunderstandings;
  - Spell out the name of the medicine(s);
  - Request written confirmation by fax, letter or by issue of a new prescription, without delay, and before the next administration of the medicine.

- Staff must change the Service User's medications records immediately on receipt of a "not present" prescription.

- Staff must record the time and date of the change, and ensure that a second administration of that medicine does not take place before written confirmation is received.

PROCEDURE FOR MEDICATION ADMINISTRATION Remember the 5 x R's:

1. **Right Medication**
2. **Right Dose**
3. **Right Way**
4. **Right Person**
5. **Right Time**

- **These are the main five rights, some publications may list more, such as 'Right Documentation' or 'Right Reason' which you may choose to incorporate into your procedure.**

- Check you are giving the medication to the right person. Ask them for their name, check recent photograph, and cross reference name and, if appropriate, their room number on the medication record. Make sure that you know the person by name. If in any doubt check with another member of the Care team or contact the administration office who will know the Service User well.

- Select all of the correct medication for the time of day for the individual Service User. ALWAYS CHECK THE MEDICATION RECORD. DO NOT RELY ON MEMORY OR ADMINISTER ONLY WHAT IS IN FRONT OF YOU.

- Ask the person if they are experiencing the condition for which the medication has been prescribed.

- Give the medication with a tumbler of water (or milk if required by the prescription) and encourage the person to sit in an upright position whilst swallowing the medication.

- If the tablets or capsules are in a monitored dosage pack, open the appropriate section and empty the tablets/capsules into a medicine pot and hand it directly to the Service User.

- Transfer the medication from the bottle or pack into a medication pot and give this directly to the Service User.

- Liquid medication must be measured into a clearly graduated and marked medication pot or by using an appropriate sized syringe which clearly identifies individual millilitre markings.

- Medication must not be handled but transferred to the medication pot in a non-handling clean method.

- Some medications may be harmful if handled by the Care worker and disposable plastic gloves must be available and worn where there is an identified risk. This may also apply where the Service User is unable to handle medication and they require extra support, although the handling of drugs should be avoided and medication spoons used to aid administration where difficulties are identified.

- Medication doses affected by the latest blood results should have a copy of the latest results kept with the medication administration record.

- Ask if they want to take their medication before removing them from the pack. If they refuse try again a little later. Refusal must be documented and the GP or pharmacist telephoned for advice. NEVER FORCE ANYONE TO TAKE MEDICATION AND IT MUST NOT BE HIDDEN IN FOOD OR DRINK.

- Medicines to be taken when required (PRN) should be offered to people experiencing pain, and the time and dosage noted in the medication administration record.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To meet the provisions of the Mental Capacity Act 2005.

Scope

- All Service Users who lack mental capacity as defined under the Act.
- The Mental Capacity Act 2005 protects both the Service User and the Care worker provided the law is adhered to.

Policy

- All Service Users assessed as lacking mental capacity will be subject to the provisions of the Act. Š
- The organisation will have access to:
  - The Act;
  - The Code of Practice.
- Each of these documents is available in the Useful Documents section of your QCS management system.

Mental Capacity Act 2005 – Summary

- The Mental Capacity Act came into force in April 2007 and was implemented in October 2007.
- The Mental Capacity Act was substantially amended by the Mental Health Act 2007 to provide for a procedure to authorise the deprivation of the liberty of a mentally incapacitated person. This part of the Act is called the Deprivation of Liberty Safeguards (DOLS). The policy and procedure regarding this part of the Act is contained in the Deprivation of Liberty Safeguards Policy and Procedure.
- The Mental Capacity Act 2005 provides a statutory framework to empower and protect vulnerable people who are not able to make their own decisions. It makes it clear who can take decisions, in which situations, and how they should go about this. It enables people to plan ahead for a time when they may lose capacity.
- Guidance on the Act is provided in a Code of Practice. Everyone caring for, or working with an adult who may lack capacity to make specific decisions must comply with this Act. There is a Code of Practice which provides guidance and information as to how the Act works.
- The whole Act is underpinned by a set of five key principles:
  - A presumption of capacity – Every adult has the right to make his or her own decisions and must be assumed to have the capacity to do so unless it is proved otherwise;
  - The right for individuals to be supported to make their own decisions – People must be given all appropriate help before anyone concludes that they cannot make their own decisions;
  - That individuals must retain the right to make what might be seen as eccentric or unwise decisions;
  - Best interests – Anything done for or on behalf of people without capacity must be in their best interests; and
  - Least restrictive intervention – Anything done for or on behalf of people without capacity should be the least restrictive of their basic rights and freedoms.

What does the Act do?

- The Act enshrines in statute best practice and common law principles concerning people who lack mental capacity and those who take decisions on their behalf. It replaced previous statutory schemes for enduring powers of attorney and Court of Protection receivers with reformed and updated schemes.
- **Assessing lack of capacity -** The Act sets out a single clear test for assessing whether a person lacks capacity to take a particular decision at a particular time. It is a 'decision-specific' test. No one can be labelled ‘incapable’ as a result of a particular medical condition or diagnosis. Section 2 of the Act makes it clear that a lack of capacity cannot be established merely by reference to a person’s age, appearance, or any condition or aspect of a person’s behaviour which might lead

others to make unjustified assumptions about capacity. A form to be used in conjunction with the assessment test for capacity is included towards the end of this policy. The test for capacity is a two stage test. Firstly, is there an impairment of or disturbance in the functioning of a person's mind or brain? Secondly, is the impairment or disturbance sufficient that the person lacks the capacity to make a particular decision? In deciding this you should establish if the person can do all of the following. If not then we should say the person lacks capacity to make that particular decision.

- Understand information about the decision to be made?
- Retain that information in their mind?
- Weigh that information as part of the decision-making process?
- Communicate their decision?

- **Best Interests -** Everything that is done for or on behalf of a person who lacks capacity must be in that person's best interests. The Mental Capacity Act does not define what it means to make a decision in someone's best interests. In considering what is in someone's best interest we would consider the factors outlined in the section of this policy headed 'Best interest'. The Act provides a checklist of factors that decision-makers must work through in deciding what is in a person's best interests. A person can put his/her wishes and feelings into a written statement if they so wish, which the person making the determination must consider. This statement is an expression of wishes, and is not the same as an advance decision to refuse treatment which is referred to later in the policy.

- Acts in connection with Care or treatment: Section 5 clarifies that, where a person is providing Care or treatment for someone who lacks capacity, then the person can provide the Care without incurring legal liability. The key will be proper assessment of capacity and best interests. This will cover actions that would otherwise result in a civil wrong or crime if someone has to interfere with the person's body or property in the ordinary course of caring, for example, by giving an injection or by using the person's money to buy items for them.

- **Restraint/deprivation of liberty -** Section 6 of the Act defines restraint as the use or threat of force where an incapacitated person resists, and any restriction of liberty or movement whether or not the person resists. Restraint is only permitted if the person using it reasonably believes it is necessary to prevent harm to the incapacitated person, and if the restraint used is proportionate to the likelihood and seriousness of the harm.

- Section 4A prohibits the deprivation of the liberty of the person unless deprivation is authorised by the Deprivation of Liberty Safeguards in Part 2 of the Mental Capacity Act. Where a person may be being accommodated in circumstances that amount to a deprivation of liberty, authorisation under the DOLS arrangements should be sought. The authorisation process is described in the Deprivation of Liberty Safeguards Policy and Procedure.

- The Act deals with two situations whereby a designated decision-maker can act on behalf of someone who lacks capacity:

  - **Lasting powers of attorney (LPAs)** – The Act allows a person to appoint an attorney to act on their behalf if they should lose capacity in the future. There are two types of LPA, one to make and health and welfare decisions, and the other to make finance and property decisions. The provision replaces the previous role of Enduring Power of Attorney (EPA). An LPA can only be appointed by someone who has the capacity to do this. Staff should be aware of any LPA in place for service users in their care.
  - **Court appointed deputies** – The Act provides for a system of court appointed deputies to replace the previous system of receivership in the Court of Protection. Deputies are able to take decisions on welfare, healthcare and financial matters as authorised by the Court but are not able to refuse consent to life- sustaining treatment. They are only appointed if the Court cannot make a one-off decision to resolve the issues, and if the person has already lost capacity to make these decisions. Staff should be aware of any Court appointed deputies in place for service users in their care.

- The Act created two new public bodies to support the statutory framework, both of which are designed around the needs of those who lack capacity:

  - **A new Court of Protection** – The new Court will have jurisdiction relating to the whole Act and is the final arbiter for capacity matters. It has its own procedures and nominated judges.
  - **A new Public Guardian** – The Public Guardian and his/her staff are the registering authority for LPAs and deputies. They supervise deputies appointed by the Court and provide information to help the Court make decisions. They also work together with other agencies, such as the police and social services, to respond to any concerns raised about the way in which an attorney or deputy is operating.

- The Act also includes three further key provisions to protect vulnerable people:

  - **Independent Mental Capacity Advocate (IMCA)** - An IMCA is someone appointed to support a person who lacks capacity but has no one to speak for them. The IMCA makes representations about the person's wishes, feelings, beliefs and values, at the same time as bringing to the attention of the decision-maker all factors that are relevant to

the decision. Staff should be aware of how to refer to an IMCA. This information can be found out from the Local Authority or from the Citizens' Advice Bureau in the area.

- **Advance decisions to refuse treatment** - Statutory rules with clear safeguards confirm that people may make a decision in advance to refuse treatment if they should lose capacity in the future. It is made clear in the Act that an advance decision will have no application to any treatment which a doctor considers necessary to sustain life unless strict formalities have been complied with. These formalities are that the decision must be in writing, signed and witnessed. In addition, there must be an express statement that the decision stands "even if life is at risk". An advance decision to refuse treatment that is not life sustaining does not have to be recorded in writing.
- **A criminal offence** - The Act introduces a new criminal offence of ill treatment or neglect of a person who lacks capacity. A person found guilty of such an offence may be liable to imprisonment for a term of up to five years.

Best Interests

- It is important not to make assumptions about someone's best interests merely on the basis of the person's age or appearance, condition or any aspect of their behaviour.
- The decision-maker must consider all the relevant circumstances relating to the decision in question.
- The decision-maker must consider whether the person is likely to regain capacity (e.g. after receiving medical treatment). If so, can the decision that is being made on their behalf wait?
- If the decision concerns the provision or withdrawal of life-sustaining treatment, the decision-maker must not be motivated by a desire to bring about the person's death.
- The decision maker must in particular consider:
  - The person's past and present wishes and feelings (in particular if they have been written down).
  - Any beliefs and values (e.g. religious, cultural or moral) that would be likely to influence the decision in question and any other relevant factors.
  - As far as possible, the decision-maker must consult other people if it is appropriate to do so and take into account their views as to what would be in the best interests of the person lacking capacity, especially:
- Anyone previously named by the person lacking capacity as someone to be consulted.
- Carers, close relatives or close friends or anyone else interested in the person's welfare. Œ
- Any attorney appointed under a Lasting Power of Attorney.
- Any deputy appointed by the Court of Protection to make decisions for the person.

Procedure

- Maintain awareness of the Mental Capacity Act principles and the DOLS.
- If the registration of the service may allow for admission of persons assessed as lacking mental capacity, train key staff using the DOH-provided training resources which can be accessed at: http://www.scie.org.uk/publications/guides/guide44/workingbettertogether/training.asp.
- Seek advice from the QCS Customer Care Team (**0333 405 33 33 option 2**) in the event of any questions arising.
- Documents and resources about the Act, including training resources, are available under Useful Documents section of your QCS management system.

Four Steps to remember when assessing a person's mental capacity

- Bear in mind the key principles which emphasise the fundamental concepts and core values of the Act (noted previously in this document under summary of Mental Capacity Act 2005).
- Complete the attached MCA Assessment with the Service User. Having a member of staff or a family member sit with the Service User whilst you complete the assessment is intended to help them relax and feel comfortable with you.

- Make sure the form is completed fully and that it is signed by the assessor and that it is dated. This form should be kept with the Care Plan so it is readily available.

- If it is determined that the Service User does not have the mental capacity to make a particular decision, any action taken or any decision made must be in his or her best interest.

What do I do if there is a dispute about best interests?

- Family and friends will not always agree about what is in the best interests of an individual.

- If you are the decision-maker you will need to clearly demonstrate in your record keeping that you have made a decision based on all available evidence and taken into account all conflicting views.

- If there is a dispute, the following things might assist you in determining what is in the person's best interests:

  - Involve an advocate who is independent of all the parties involved;
  - Get a second opinion;
  - Hold a formal or informal case conference;
  - Go to mediation;
  - An application could be made to the Court of Protection for a ruling;
  - You should ensure that all documents you complete are both signed and dated.

Reviewed by Marcus Kerridge McColl
November 2025

Minimum Wage Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with legislation.

Scope

- All workers above school-leaving age.

Policy

- The organisation will comply with the minimum wage regulations.

Guidance to the Service Provider

General

Entitlement to the NMW

- Most workers in the UK who are over the compulsory school-leaving age are legally entitled to be paid at least the NMW, and all employers have to pay it if the employee is entitled to it. It makes no difference:
  - If the employee is paid weekly or monthly, by cheque, in cash or in another way;
  - If the employee works full time, part time or any other working pattern;
  - If the employee works at the employer's own premises or elsewhere;
  - What size the employer is;
  - Where the employee works in the UK.
- The employee is entitled to the NMW even if they sign a contract agreeing to be paid at a lower rate, either of the employee's own free will or because the employer persuades or makes the employee sign. The contract will have no legal effect and the employee must still be paid the proper rate.

How to check pay

- The average hourly pay has to be at least the NMW, worked out over a period called the 'pay reference period'.
- To find out whether an employee is receiving the NMW there is a need to know what counts towards it and what does not. There is also a need to know which hours are entitled to be paid for. This depends on which of the four types of work under

the NMW rules the employee is doing.

Time Work

- If the employee is paid according to the number of hours they are at work, they are doing time work – so anybody whose pay goes up or down depending on the actual hours they work is likely to be doing time work. This includes workers who are paid piece rates but have to be at a factory for a set numbers of hours a day to produce their work.
- Time workers must be paid the NMW for hours spent:
  - At work and required to be working or on standby near the workplace (but not on rest breaks);
  - When kept at the workplace but unable to work because of machine breakdown;
  - Travelling on business during normal working hours;
  - Training or travelling to training during normal working hours;
  - Awake and working, *and* during 'sleeping time' if they are required to remain on the premises either by their employment contract or to fulfill other regulations.
- Sleeping time means any time when the employee is allowed to sleep, if the employer arranges for the employee to sleep at or near where the employee works and provides them with suitable facilities for doing so.
- Time workers do not need to be paid the NMW for hours spent:
  - Travelling between home and work;
  - Away from work on rest breaks, holidays (Different arrangements apply, which may nonetheless amount to the NMW.), sick leave (Statutory Sick Pay may be payable) or maternity/paternity/adoption leave (Statutory Maternity / Paternity / Adoption Pay may apply);
  - Away from work because of industrial action.

Salaried Hours Work

- If the employee is paid under the contract for a set basic number of hours a year, and receives an annual salary paid in equal weekly or monthly instalments, the employee is a salaried hours worker. The contract does not have to state the hours as an annual figure (e.g., 2,000 hours a year), but it must be possible to work out from the contract what the basic annual hours are.
- Salaried hours workers must be paid the NMW for hours spent:
  - At work and required to be working;
  - On standby, or on call, at or near the place of work;
  - When kept at the workplace but unable to work because of machine breakdown;
  - Travelling on business during normal working hours;
  - Training or travelling to training during normal working hours;
  - Awake, and working, during 'sleeping time' if they are required to remain on the premises either by their employment contract or to fulfill other regulations;
  - Absences such as rest breaks, lunch breaks, holidays, sick and maternity leave will need to be counted *if* they are included in the basic minimum hours under the contract. (So, if the contract says the employee is employed for 35 hours per week and that includes their lunch break, then you must count that time into the number of hours they work every week or month for calculating the minimum wage).
- Salaried hours workers do not need to be paid the NMW for hours spent:
  - When paid less than the normal pay because the worker is absent from work, for instance if the employee only receives half-pay while on sick leave;
  - On any unpaid leave which the employer allows the employee to take;
  - Away from work because of industrial action.

The pay reference period

- This is usually the period of time that the employee is actually paid for. For example, if employees are paid weekly the pay reference period is one week; if they are paid monthly it is one month. Employees must be paid, on average, at least the NMW during each pay reference period.
- Under the NMW rules, the pay reference period cannot be longer than a month. If the employer pays at longer intervals (e.g. once a quarter) employees must still get the NMW on average each month during that quarter.

- When working out whether the employee is receiving the NMW, the pay that counts may not just be the pay the employee receives during the pay reference period, it also includes pay which the employee earns during that period, but does not receive until the next one. For example, if the employee is paid monthly and does some overtime near the end of July they might not be paid for it until August. The overtime pay will still count towards the July pay.

Pay that counts towards the NMW

- The pay the employee receives that can be used to calculate the NMW is known as the NMW pay.
- NMW pay is calculated on gross pay (before tax and National Insurance have been taken off). The gross pay includes the basic pay for the work done and other types of pay which count towards the NMW. For example, sales commission, performance-related pay or other payments based on how well the employee does the job.
- Some payments do not count towards NMW pay. These should be deducted these from the total pay before working out whether the employee is receiving the NMW. Payments which do not count include:
    - Loans;
    - Advances of wages;
    - Pension payments;
    - Retirement lump sums;
    - Redundancy payments;
    - Rewards under staff suggestion schemes;
    - Any premium element (in other words, anything the employee is paid on top of basic pay) for working at special times, e.g. overtime or on bank holidays;
    - Allowances on top of basic pay, for example for working unsocial hours, in a particular area (e.g. London weighting), in dangerous conditions, 'on call', or performing special duties over and above normal one;
    - Tips, gratuities, service charges and cover charges (until 1 October 2009 these did count towards NMW pay as long as the employer paid them through the employee's payroll, however they no longer count, regardless of how the employee receives them).

Deductions from the employee's pay and payments to the employer

- Some deductions and payments reduce NMW pay while others do not. However, the employer cannot make any deductions from the employee's pay, or take payments from the employee, unless certain conditions are met. The Wage Deductions Agreement in the Recruitment Pack C – Induction and Job Start provides the basis for most deductions, if signed by the employee.

Deductions and payments which do not reduce NMW pay

- Some deductions from pay, or payments the employee make to the employer, are ignored in working out NMW pay. Therefore, if the employee is paid £7.00 an hour and deductions or payments for any of the reasons listed below reduce this to £6.00, the question is whether £7 an hour, not £6, meets the NMW in the employee's case.
- Deductions and payments which do not reduce NMW pay are:
    - Penalties for misconduct – as long as these penalties are in the employment contract;
    - Paying back an advance of wages;
    - Paying back an accidental overpayment of wages;
    - The costs of any shares or securities which employees have chosen to buy in the firm;
    - Money the employee chooses to have deducted from pay (e.g. a pension contribution or trade union subscription), as long as the deduction is not required as part of the employee's work and is not for the employer's own use and benefit;
    - Payments the employee chooses to make to the employer to buy goods or services from them, for example, if the employee spends some of their wages on meals in the staff canteen.

Deductions and payments which reduce NMW pay

- Some deductions from pay, or payments made to the employer, are taken into account in working out NMW pay. Therefore, if the employee is paid £7 an hour and deductions or payments for any of the reasons listed below reduce this to £5, the question is whether £5 an hour, not £7, meets the NMW in that case.
- Deductions and payments which reduce NMW pay are:
    - Refunds of any money that the employee spent in connection with the work, for example the cost of purchasing tools or uniform;

  - Refunds of any expenses incurred doing the employee's job, for example travel costs;
  - Deductions to cover the cost of items the employer supplied to employees that are needed for the job, for example, tools or uniform;
  - Deductions for the employer's own use and benefit for goods and services, for example transport provided to and from work, regardless of whether the employee has the option of using the goods or services.
- The employer does not have to be making a profit from a deduction for it to be 'for their own use and benefit'. For example, if the employer provides transport at a loss, any deductions the employer makes for providing it help to reduce their loss. The reduction is part of the 'use and benefit' they will have had from the deductions.

Do 'benefits in kind' count towards the NMW?

- Benefits in kind are anything the employer provides for employees' benefit apart from pay. There is nothing to stop employers offering employees benefits in kind. However, an equivalent value in money for a benefit in kind cannot be counted towards NMW pay, except for a set amount for accommodation.
- Examples of benefits in kind which do not count towards NMW pay are:
  - Meals;
  - Fuel;
  - A car;
  - The employer's contribution to the employee's pension fund;
  - Help with removals;
  - Medical insurance;
  - Lunch vouchers;
  - Child care vouchers.

Current Rates

- Please note that rates change on an annual basis, usually every October. (See http://www.hmrc.gov.uk/paye/rates-thresholds.htm#2 for confirmation of the most recent figures).
- From 1st October 2025 the rates will be:
  - For workers 21 and over - **£6.70** per hour (previously £6.50).
  - For workers from 18 to 20 - **£5.30** per hour (previously £5.13).
  - For workers from 16 to 17 - **£3.87** per hour (previously £3.79).
  - For apprentices under 19 or 19 and over who are in the first year of apprenticeship - **£3.30** per hour (previously £2.73).

Living Accommodation

- The amount that can be taken into account for living accommodation is **£5.35** per day from 1st October 2025 (previously £5.08 per day).

On-Call

- Being on-call at the business premises is 'work', and is subject to the minimum wage regulations and working time regulations.
- Time on-call away from the business premises, where the worker is free to pursue other activities, is not 'work', and not subject to the regulations.

Volunteers

- The minimum wage does not apply to volunteers who:
  - Receive only reimbursement for genuine or reasonable out-of-pocket expenses, and;
  - Receive no benefits other than training necessary for them to carry out the work.
- It also does not apply to volunteers who:
  - Receive accommodation and/or meals because it is necessary in order for them to carry out their work, or;
  - Receive a payment towards subsistence. However, this only stands if (a) the volunteer is working for a charity, voluntary organisation, associated fundraising body or statutory body, and (b) the volunteer has been placed with that body by a charity acting in pursuance of its charitable objects, or;

  - Are residential members of a charitable religious community (provided the community is not an independent school and does not provide further or higher education courses).

- The minimum wage is likely to apply to volunteers who receive payment above genuine reimbursement. This could include, for example, lump sum expenses, sessional fees, one-off payments, 'honoraria' etc. It is also likely to apply where volunteers receive training beyond what is needed for them to do their work, or where they receive other benefits.

National Living Wage

- This is not to be confused with the National Minimum Wage, but is a minimum "living wage" to be paid to workers aged 25 years or over with effect from 1st April 2016. The rate is £7.20 per hour.

Information given here is guidance only and cannot be taken as an authoritative interpretation of the law. Specific circumstances can vary; the law is complex and is continually changing.

Reviewed Marcus Kerridge-McColl

December 2025

Missing Service User Policy & Procedure
Crescent community Care Services Limited

Purpose

- To comply with regulations and statutes and quality standards
- Crescent Community Care Services Limited will respond in an effective and structured manner to missing Service users

Scope

- All Employees
- All service users and families and other professionals involved in the service

Policy

- In the event that a service user is deemed missing as they are not answering the door, the telephone and no family member or neighbour has no good understanding or reason for them not to be answering or if there is an expression of concern for the wellbeing of a service user. The following procedure will take effect immediately
- All service users have the choice to leave their home whenever they wish, however it is necessary that members of staff respond to concerns about the whereabouts of a Service User in the event that contact can not be made
- All staff should be aware that NO service user can be held at their own premises against their will unless they are subject to a Mental Health Act or a DOLS (Deprivation of Liberty) restraint order. However Safety and Risk management will always be our primary concern
- In the event that a Service User is deemed missing the following procedure must be adhered to and that all staff recognise their own responsibilities for the safety and security of all service users in their care.
- Should a Service User wish to leave their home and the care plan states that there is either a Mental Health reason or Deprivation of Liberty in place then intervention must take place either by management, professionals and / or family members

Procedure

- If the care worker should find that a service user is not where they are expected to be at the visit or that entry to a property is not available then they should contact the office or the On Call immediately and wait for further instruction
- The office and On Call will ask the care worker to try neighbours and look through windows and doors to ascertain that the service user has not fallen or is unwell and unable to answer the door
- The office or On Call will try telephoning the service user and then the Next of Kin to ascertain if there is a reason that entry can not be made
- Other professionals and hospitals will be called to ascertain if the service user has been either admitted to hospital or to another site due to illness or concern
- Social Services will be informed and worked with to ensure that all parties are aware of the situation and the actions taken
- In the event that there is no clue as to the whereabouts of the service user the Police may be called to gain entry to the service user's home to ensure that they are not incapacitated for reasons of illness etc

- In the event that the service user is still not found then the Police will take over with a missing persons investigation
- All actions must be recorded so that the information can be passed over to other professionals to assist the finding of the service user
- CQC have to be notified under a statutory notice
- When the service user has been found a professionals meeting must be called to ensure that this does not happen again and if there requires intervention

Reviewed Marcus Kerridge-McColl
October 2025

Moonlighting Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To control potential conflicts of interest from moonlighting, and to maintain the health and safety of employees.

Scope

- All employees.

Policy

- The organisation recognises that from time-to-time employees may take up separate employment with another employer or pursue outside business interests whilst still remaining in the organisation's employ.

- Although the organisation has no desire to unreasonably restrict an employee's external activities, it must seek to protect its own interests and those of all its employees.

- To this end, the organisation's policy is that employees will not be permitted to undertake business activities or other work where the organisation considers that this is incompatible with its interests and, in any event, unless employees have obtained the prior written authorisation of the department manager.

- Private work for Service Users:

  - Private work for Service Users is strongly discouraged, as it conflicts with our contract of employment, and will very probably be regarded as financial abuse. In exceptional circumstances the organisation may authorise such work where it is clearly for the benefit of the Service User, subject to the approval of the organisation funding the support to that individual Service User, and subject to controls on charging and quality.

Procedure

- When an employee proposes taking up additional employment or external business interests, the employee:

  - Must request an interview with the department manager with a view to establishing the likely impact of these activities on the organisation;
  - The employee will be asked to provide full details of the proposed work and specific consideration should be given to the following areas:

- Working hours: is the employee proposing to conduct their affairs entirely outside their contractual hours of work, or is there likely to be some overlap?

- Competition: is the employee intending to work in competition with the organisation, either in their own right or for a competing organisation? If so, is there a real risk of a conflict of interest and/or confidential information being used to the organisation's detriment?

- Health, safety and welfare: is the employee proposing to carry out work which is inherently hazardous and where the risk of injury is high (should the employee become injured or fall sick as a result of other work, organisation sick-pay entitlement will be affected) and is the extra work likely to cause undue fatigue, stress, etc. which will affect job performance with the organisation?
    - If, after investigation, the department manager considers that the proposed activities are incompatible with the individual's obligations to the organisation, permission will be refused.
- This should be notified to the employee in writing detailing the reasons for refusal.
- Employees should be advised that if they are dissatisfied with the decision they should make use of the organisation's grievance procedure.
- Where employees' proposed work does not significantly affect their employment with the organisation, permission should be given in writing to the employee.
- This, though, should include provisions reminding employees of their obligations to the organisation and stating that should their external activities subsequently have an effect on their employment, permission may be withdrawn.
- Additionally, employees should be advised that any material changes to the circumstances of their outside interests must be brought to the attention of the organisation.

Reviewed by Marcus Kerridge McColl
December 2025

Moving & Handling Policy & Procedure
Crescent Community Care Services limited

Purpose

- To ensure that a standardised, practical and knowledge-based approach is taken by workers ("workers" means anyone working for the employer, including employees, agency staff and contractors) on moving and handling in line with current legislation and company standards. This will facilitate a safe system of work.
- This policy has been produced in line with requirements specified in the Manual Handling Regulations 1992 (revised in 2004) and The Guide to the Handling of People (current revised 6th edition).

Scope

- All workers ("workers" means anyone working for the employer, including employees, agency staff and contractors).

Policy

- There are many situations in which staff may be required to move or handle Service Users or loads during their working day. During this activity there is potential for injury for both the worker and the Service User.
- The Moving and Handling Operations Regulations 1992 (amended in 2004) require employers to:
    - AVOID the need for their workers to carry out moving and handling operations whenever possible.
    - ASSESS if moving and handling is unavoidable a **SUITABLE AND SUFFICIENT** assessment must be undertaken.
    - REDUCE risk to workers and Service Users, as far as is reasonably practicable, to the lowest possible level.
    - REVIEW assessments every time the situation/condition of the Service User changes.
- Initial appraisals of risk and risk assessments will be carried out by qualified Senior Carers for each individual in order to ensure that workplace assessments are undertaken before service delivery.
- Crescent Community Care Services Ltd is required to nominate an appropriate number of risk assessors, who will be trained to carry out risk assessments within the workplace or a Service User's home environment as part of the Care Plan.
- "The Guide to the Handling of People", 6th Edition, published by BackCare in collaboration with the Royal College of Nursing, will be integral to the training process and should only be used in conjunction with the approved basic moving and handling

training.

- An initial appraisal of risk must be undertaken and reported to the appropriate person.
- A Risk Assessment must be carried out **on any** moving and handling task whereby a risk has been identified. This risk must be eliminated, or if that is not possible, reduced to the lowest level as is reasonably practicable.
- Crescent Community Care Services Ltd's Moving & Handling risk assessment form will be the only outcome-accepted evaluation of level of risk**.**
- Moving and handling of Service Users must be carried out as part of the Care planning process, using the documentation contained in the QCS Care Plan format. Where Service Users lack mental capacity to consent to this, decisions must be made in the person's best interest.
- Where workers may move or handle Service Users, those workers must have initial training which has a theoretical and practical assessment of competence. This training is to be delivered by a Moving and Handling trainer. Staff must also have an annual assessment of their competence conducted by a Moving and Handling trainer, and be given remedial training and be reassessed if any assessment indicates that this is required.
- Staff should be aware of mental capacity legislation in relation to decisions made in a person's interest, including where those decisions may involve restraint. Where the level of restraint necessary constitutes a deprivation of someone's liberty, this would be contrary to Article 5 of the Human Rights Act, unless the deprivation was lawfully authorised by the Court of Protection.
- This policy applies to all, inclusive of employees, volunteers, agency workers, students, and relief staff and should be implemented by Line Managers, both during induction and at regular intervals.
- Special consideration should be given to vulnerable workers:
    - For pregnant staff, a specific risk assessment should be completed, this should include a re-assessment upon return to work following maternity leave.
    - Any other workers with an identified medical condition, should have a specific risk assessment supported in writing by the organisation's occupational health provider and signed off by the Registered Manager.
- Induction will include moving and handling instruction, practice and assessment as an essential component before any moving and handling task is undertaken in the workplace, and this will be subject to regular review.

Procedure

- It is recognised that Managers require a broad understanding and working knowledge of risk assessments and moving and handling principles and they should establish working practices in consultation with the Health & Safety Adviser.
- It is the responsibility of the organisation, delegated to the Registered Manager, to identify risks and subsequently manage them. Managers must not rely solely on risk analysis forms, which may not recognise risks particular to one situation or location.
- The Risk Assessment should be a collaborative process between all parties involved in any daily working task. In any moving and handling task - inclusive of people moving - it is recognised that the worker requires adequate training and the Assessors require detailed knowledge of the following:
    - **L.I.T.E.E**;
        - Load - Typically the Service User.
        - Individual Capability - Are they suitably trained?
        - Task - What is the activity?
        - Environment - Where will the activity take place?
        - Equipment - What will be required to reduce the risks?
- Final decisions on the risk assessment outcome will be made by the respective Manager in conjunction with the Health & Safety requirements.
- The employer may be liable if it can be proved that a risk assessment of the task or the standard and regularity of training has not been upheld, in order for the worker to carry out their duties related to moving and handling of inanimate loads and Service Users (i.e. litigation from injured workers).
- The worker could also be liable if found to be negligent in the undertaking of their duties, i.e. failure to observe Lifting Operations and Lifting Equipment Regulations 1998 (LOLER) which requires the carer to check that a hoist has been serviced within the last six months and upon visual inspection is safe to use prior to using it. The employer has vicarious liability for actions taken by their workers on their behalf i.e. risk assessments.
- It should be remembered that Service User Care is paramount and the resulting impact of the risk assessment must be

sensitively managed by all involved.

- Where there are disputes or conflicts between risk assessors, agencies, Service Users, professionals and any others, the senior manager (in consultation with the HSA) would make a decision in line with the respective current practice and legislation.
- The policy will be reviewed, expanded or modified periodically, in accordance with changing conditions or legislation, on an annual basis.
- All people involved in Moving & Handling tasks as part of their daily work must be appropriately clothed in either uniform (if provided) or clothing which facilitates the principles of safe handling. Refer to the following policy and procedure – Employee Appearance Policy and Procedure, in particular:
    - Care staff will wear the appropriate organisation uniform, not wear jewellery of any kind, have long hair tied back, and wear appropriate footwear.
    - Staff with Service User contact:
        - Staff members are expected to wear the uniform appropriate to their role; where a uniform is not specified casual clothing is acceptable, but it must be neat and clean and appropriate to the work which is undertaken.
    - Uniform clothing must be clean, odour-free, in good repair and well pressed.
    - Hair must be kept clean and tidy.
    - Staff must keep their hair secure in such a way that it does not pose a safety hazard.
    - Jewellery, including ear-rings and studs which may come into contact with a Service User should not be worn.
    - Nails must be clean and short, and not pose a scratching risk to Service Users, especially those with frail skin.
    - Footwear must be appropriate to the situation, providing protection from dropped objects, slip protection, not be a scratch or other danger to Service Users. Open toe or sling back shoes are not acceptable.
- The results of the risk assessment must be adhered to, except in instances whereby any of the following emergency situations apply:
    - A person is in imminent danger of drowning;
    - A person is in an area that is on fire or filling with smoke;
    - A person is in danger of a life threatening situation;
    - A person is in danger of a collapsing building or other structure.
- In any other emergency situation time must be taken to assess the situation, plan the manoeuvre and obtain equipment if necessary.
- Prospective workers will be made aware that care and consideration must be given to the tasks to be undertaken, and they must be suitable for the chosen place of work, i.e. Service User bedroom or bathroom.
- The risk assessments should be reviewed by senior managers if tasks alter at any time. Modification of any job description must also be undertaken only with the express permission of senior management.
- After completing a written risk assessment the assessor must make relevant workers and other relevant staff aware of its content and any identified remedial actions should be dealt with, e.g. obtaining equipment, identifying additional training, etc. Staff should follow the risk assessment guidance and instructions given; failure to do so could result in disciplinary action.
- Managers and all users must be conversant with safe use of moving and handling equipment and must ensure that maintenance and use of equipment complies with the Provision and Use of Work Equipment Regulations 1992 (PUWER as amended in 2002), the manufacturer's instructions, and safety precautions. Selection of equipment must be undertaken by a qualified Occupational Therapist (OT) or Physiotherapist.
- All privately purchased equipment must be accompanied by the supplier or manufacturers documentation on safe use, and all relevant workers must receive training on the safe use of the equipment before it is first used.
- All equipment used in moving and handling must be stored in a safe manner and must not pose a hazard in itself and be protected against malicious damage.
- Accidents to staff during moving and handling must be reported and documented as per Crescent Community Care Services Ltd accident reporting and investigation procedure. Those accidents, which cause a worker to be incapacitated for over 7 days not including the day on which the accident happened, are reported under RIDDOR. Remedial action should be taken to limit a recurrence following investigation by senior managers.
- The employer is insured for personal risks undertaken by the worker as described in their job description.
- Safety disclaimers are not legally binding and cannot be upheld by courts.

- The successful implementation of this policy requires total commitment from all employees. Each individual has a legal obligation to take reasonable care of their own health and safety, and for the safety of other people who may be affected by their actions or omissions. No carer should manually handle people or objects until they have completed training and been assessed as competent to do so.

Risk Assessments - General

- A risk assessment format is supplied as part of this policy. Users must note that it is the responsibility of the organisation, delegated to the departmental manager, to identify risks and subsequently manage them. Managers must not rely solely on risk assessment forms, which may not recognise risks particular to one situation or location.
- For general guidance on moving and handling, and charts to assess some generic risks, see the Health and Safety Executive publication available at: http://www.hse.gov.uk/pubns/indg383.pdf

Risk Assessments - Lifts and Hoists

- Where risks associated with lifts and hoists are to be undertaken, recognition of environmental factors is important. The Royal Institute of British Architects publish guidance on room design relevant to safe manual handling. These recommendations are usually incorporated within the lift or hoist manufacturers' safe usage guidelines. The guidelines may preclude some rooms for use with lifts and hoists due to lack of room for safe handling.

Other Relevant Documents:

- Crescent Community Care Services Ltd Health & Safety Policy and Procedure.
- Manual Handling Operations Regulations 1992 revised 1998 and updated in 2004 (MHOR).
- Management of Health and Safety at Work Regulations 1999
- The Guide to the Handling of People", 6th Edition, published by BackCare (current revised 6th edition 2011).
- HSE A Brief Guide to Manual Handling at Work, http://www.backcare.org.uk/wp-content/uploads/2025/02/HSE-Manual-Handling-at-Work.pdf.
- Work Equipment Regulations 1992.
- RIDDOR - Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013.
- LOLER - Lifting Operations and Lifting Equipment Regulations 1998, as amended in 2002.
- PUWER - Provision and Use of Work Equipment Regulations 1998, as amended in 2002.
- Human Rights Act 1998.
- Health & Safety at Work Act 1974.
- The Mental Capacity Act 2005 incorporating the Deprivation of Liberty Safeguards.

Reviewed Marcus Kerridge-McColl
November 2025

Notice Periods Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To manage the termination of an employee in an orderly fashion.
- To comply with the Employment Rights Act 1996

Scope

- All employees.

Policy

- The company acknowledges that the notice period leading up to termination of employment can be a difficult time for the departing employee, management and other staff. In view of this, the accompanying procedure is designed to ensure that, so far as is possible, the working environment is not disrupted by the impending departure of an employee and that termination of employment takes place in an orderly fashion.

Procedure

Notice periods of Crescent Community Care Services Ltd to you

- Under 1 month& 39 days service - 1 week
- 1 month but less than 2 year's service - 2 weeks
- 2 year's service or more - 1 week for each completed year of service to a maximum of 12 weeks after 12 year's service.

Notice periods from you to us

- Under 4 years' service the notice from you to us must be the same as from us to you. Š
- With 4 years' service or more the notice from you to us must be 4 weeks.

Resignation

- Notice of resignation should be given in writing to your manager, stating the final date of employment with the company. The manager will check that the appropriate period of contractual notice has been given.
- Exceptionally, oral notice may be accepted, but will be confirmed in writing by us.
- A copy of the resignation letter is to be forwarded to administration immediately. In the case of oral notice, managers must inform administration as soon as possible so details can be confirmed.
- Where you give shorter notice than that required under the contract of employment, we might advise you that this action is in breach of contract and could require that you submit proper notice.
- An early leaving date may be mutually agreed with you, but at our complete discretion.
- In the event that you leave without working your contractual notice you will pay us liquidated damages of £250 in compensation for the typical additional costs incurred in covering your work. Such a sum may be recovered by deduction from your wages if appropriate or, where this does not cover the amount or where no deduction has been agreed prior to you leaving, paid by cheque or cash from you to us.
- We will not apply such a deduction or require payment where you work your full notice, or have prior written agreement from us for you to waive your notice period, or do not have the term covered in your written contract of employment.
- A manager who is unsure whether or not an employee has resigned will discuss the matter with the Registered Provider without delay. No action will be taken regarding your employment until this is done.
- A formal acknowledgement of resignation, confirming your final day with the company and providing administrative details of final payments, etc will be issued to you.
- We may invite you to attend an exit interview with a member of management at which you will be encouraged to discuss all aspects of your job and your reasons for leaving. Information gained at such interviews will be used to monitor staff turnover and to identify potential problem areas.
- In certain circumstances you may be asked to leave immediately and receive pay in lieu of notice. This may be an express term of your contract of employment.
- If you cease to attend work without giving notice the Registered Manager will investigate the situation thoroughly before deciding on appropriate action.
- If you leave without giving the required notice you will receive statutory holiday pay accrued to the date of termination only. Any additional contractual component accrued will depend on the terms in your written contract.

Dismissal

- Before dismissal for any reason the company will conduct a thorough investigation of the circumstances and follow relevant procedures (e.g. disciplinary, capability, redundancy).

- When a decision is taken to dismiss, notice will be given to you in writing, specifying the effective date of termination. Managers are reminded that notice cannot start to take effect until it is received by the individual concerned and that the period commences the day after notice is received.

- In certain circumstances you may be asked to leave immediately and will receive pay in lieu of notice. This may be an express term of your contract of employment.

- If you commit an act of gross misconduct you will still be entitled to receive statutory holiday pay accrued to the date of termination only. Any additional contractual component accrued will depend on the terms in your written contract.

- We recognise that, particularly in the case of dismissal for redundancy, employees under notice are likely to feel demotivated and unsettled. Management will make every effort to provide appropriate counselling during this period. In addition, management will permit reasonable time off with pay, if you are under notice of redundancy, so you can seek alternative work or make arrangements for training, in accordance with legal requirements.

General Provisions

- In all circumstances, including redundancy, you are expected to work normally in conformity with your contract of employment while in the notice period. Any misconduct will be treated in accordance with the disciplinary procedure.

- As a general rule, you will be required to carry out your contractual duties while under notice. In cases whereby your contract expressly allows it, you may be moved to alternative work without loss of pay or benefits for the notice period where this is deemed necessary by management.

- Where previously agreed annual leave falls within your notice period, we will normally honour this arrangement, subject to overriding operational requirements.

- In cases that are not covered by the preceding clause, holidays may only be taken in the notice period with the express authority of your manager.

- Provided notification rules and procedures have been complied with, then should you be absent through sickness during the notice period you will receive normal pay in accordance with section 88 of the Employment Rights Act 1996, unless your contract of employment provides for contractual notice that exceeds statutory notice by one week (when section 87(4) of the ERA takes effect).

- If there is appropriate provision in your contract we may reserve the right not to provide you with work during any period of notice and to require you to remain away from the premises or require you to undertake non-confidential work (garden leave). During the existence of the notice period you are bound by the contract of employment and you will not work for any other organisation without first obtaining the permission of the [?]. Such consent will not be withheld unreasonably.

- NB. During any period of garden leave you will continue to receive your full salary and any other contractual benefits.

- Amounts paid in lieu of notice will compensate you for all pay and benefits which you would have accrued during the notice period. If you have a company vehicle then it must be returned promptly by the date of termination and in a clean and well-maintained condition.

- If you are paid in lieu of notice and instructed to return a company vehicle on leaving then you will receive reasonable financial compensation for loss of the vehicle for personal use during the equivalent of the notice period.

- It is your responsibility to return all company property to the administration department before you leave, including uniforms, security badges and keys, mobile phone, equipment, records, correspondence, documents, files and other information (whether originals, copies, extracts and whether on disc, computer, hard copy or otherwise) belonging to us or relating to the business which are in your possession or under your control and not retain any copies.

- If there is appropriate provision in your contract then failure to return such items will result in the cost of the items being deducted from any monies outstanding to you. Alternatively we may pursue you at law.

- Outstanding loans and advances of expenses must be repaid to the company before you leave or as soon as is practicable in the case of you not working out your notice. Where the contract or prior written agreement allows, deductions will be

made from final payments, in settlement. A cheque will be required from you in respect of any balance still outstanding. We will forward a cheque in respect of legitimate expense claims up to the date employment ends.

- Whenever possible final payments will be made up and a P45 issued for you on the date of leaving. Where this is not feasible they will be sent by first class post as soon as they have been prepared. Queries over payments should be addressed to the company.

Resignation Acknowledgement Letter

Dear (enter staff members name),

Your resignation from the position of (job title and site/department as applicable), effective from (date of effective resignation which should be confirmed with HR department or equivalent), is acknowledged.

OR

May I remind you that you are required to give full contractual notice of ..... which means that your leaving date will be.... Failure to give contractual notice would mean you leaving in breach of contract [and liable to pay us liquidated damages of £250 in compensation for the typical additional costs incurred in covering your work].

OR

We are willing to waive your notice period in line with your request, which means that your leaving date will be....

AND [if appropriate and if there is an appropriate provision in the contract]]

We are exercising our right not to provide you with work during your period of notice and we require you to remain away from the premises or to undertake non-confidential work. You will continue to receive your salary and contractual benefits. During the existence of the notice period you are bound by the contract of employment and you will not work for any other organisation without first obtaining the permission of the [?]. Such consent will not be withheld unreasonably.

May I remind you to return all company property to the administration department before you leave, including uniforms, security badges and keys, mobile phone, equipment, records, correspondence, documents, files and other information (whether originals, copies, extracts and whether on disc, computer, hard copy or otherwise) belonging to us, or relating to the business, which are in your possession or under your control and that you are not to retain any copies.

Failure to return such items could result in the cost of the items being deducted from any monies outstanding to you or to us pursuing you at law.

Outstanding loans and advances of expenses must be repaid to the company before you leave or as soon as is practicable in the case of you not working out your notice. Where the contract or prior written agreement allows, deductions will be made from final payments, in settlement. A cheque will be required from you in respect of any balance still outstanding. We will forward a cheque in respect of legitimate expense claims up to the date employment ends.

(Insert details of exit interview date, time, venue and interviewer, if applicable.)

Whenever possible final payments will be made up and a P45 issued to you on the date of leaving. Where this is not feasible they will be sent by first class post as soon as they have been prepared. Queries over payments should be addressed to the company.

It has been a pleasure working with you, and I wish you all the best in the future. If I can provide a reference, please do not hesitate to ask.

Yours sincerely,

(First Name Last Name)
(Job Title)

Crescent Community Care Services Ltd

Reviewed by Marcus Kerridge McColl
December 2025

Purpose

- To protect the interests of the employee when the organisation requires medical information regarding the employee.

Scope

- All employees.

Policy

- It is the organisation's policy to seek details from medical practitioners about the state of health of employees only when that information is considered essential for employment purposes.

- Where applicable, employees will be fully informed of their rights of access to any such reports.

Procedure

- The decision to request a medical report about an employee is to be made by the manager after consultation with the Registered Provider.

- The Registered Provider will establish whether or not the provisions of the Access to Medical Reports Act apply to the particular request.

- Permission from the employee to approach the medical practitioner must be obtained in writing on the appropriate form. The manager will explain fully to the employee the reasons for the request.

- At the time permission is sought the individual must be informed in detail of their rights under the Access to Medical Reports Act (if appropriate). This information is contained on a separate sheet which is available from the Registered Provider, and which should be attached to the form requesting permission for the organisation to ask for a medical report. Managers will ensure that the employee's attention is clearly drawn to the information on access rights.

- If individuals have any queries on their rights, they are to be referred to the Registered Provider.

- An employee who refuses to give permission for the organisation to obtain a report will be interviewed by the manager and their reasons will be explored. If the individual persists in the refusal, the manager will explain that decisions made about the person's employment may be affected by the organisation's inability to obtain a report.

- An employee who consents to the company applying for a medical report must complete the relevant form (retaining a copy) and also indicate on that form whether or not they wish to see the report before it is supplied.

- Where necessary, the name and address of the GP or consultant must be provided.

- The form granting permission is to be returned to the relevant manager who will pass it to the Registered Provider for action.

- When applying to the medical practitioner for a report, the Registered Provider will set out in as much detail as necessary the main features of the individual's work and explain the reasons why the application is being made.

- Arrangements for access (where applicable) must be made by the individual directly with the medical practitioner concerned. This should be done within 21 days of the application being made. If no arrangement is made within the time the practitioner may supply the report to the organisation without the consent of the employee. Any request for access by the individual once the report has been received by the organisation should be made to the medical practitioner. The organisation is not required to disclose the contents of its own copy of the report.

- Once the employee has seen the report, the organisation expects the individual to give consent to it being supplied (and/or make representations on its contents to the practitioner) as soon as reasonably practicable.

- When the report has been received by the company, further consultation will take place with the employee concerned before any decisions are made regarding the individual's employment with the organisation.

- When an employee refuses, having seen the report, to give consent to it being supplied to the organisation, the manager will interview the individual concerned. The manager will point out that the organisation would prefer to base its decision on up-to-date medical evidence but that, if it is withheld, a decision will be made on the basis of such information as is available at the time.

- The same procedure will be followed when an employee delays unacceptably in giving consent for the employer to see the report.

- Information acquired by the organisation on employees' health is to be treated in the strictest confidence at all times.

Reviewed by Marcus Kerridge McColl
December 2025

Part-Time Employees Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that part-time employees are valued and not discriminated against.

Scope

- All employees working less than 30 hours per week, averaged over the previous 13 weeks.

Policy

- Crescent Community Care Services Ltd recognises that the role that part-time employees can play in the workforce is equal to the contribution from full time staff.

- There will be no discrimination between full and part-time employees.

- It will employ part-time employees in jobs which can be done satisfactorily on a part-time basis, including those which can be done on a job-share basis.

- It will always consider requests from employees wishing to transfer from full-time to part-time work or vice versa.

Procedure

- Part-time employees are defined for company purposes as those who work less than 30 hours per week.

- Part-time employees may only be employed where:

  - There is insufficient work for the job to be done on a full-time basis or the nature of the work or working hours are best suited to part-time work;
  - The job can be split sensibly and effectively between part-time job sharers;
  - It is difficult or impossible to find suitable full-time employees;
  - Where the employment of a part-time employee adds to the flexibility of the team.

- Managers wishing to recruit a part-time employee must:

  - Carefully assess the viability and desirability of having the job done on a part-time basis and the likelihood of finding suitable part-time applicants;
  - Identify the hours to be worked – whether the job holder needs to attend every day (e.g. mornings only) or whether two or three full days a week are more appropriate – and the potential cost of employment;

- Obtain prior approval from the Registered Manager before filling the post.

- Managers considering splitting a full-time job into part-time jobs to be done by two job sharers, or faced with such a request, must:

  - Discuss the request fully with the employees concerned, where appropriate. Ensure that they are fully aware of all the implications of moving to part-time work, in particular the effect on their terms and conditions of employment;
  - Always consider carefully whether the job can be done satisfactorily on such a basis, taking into account such factors as continuity, an easily divisible workload, etc (an unjustifiable requirement for a job to be done on a full-time basis can be unlawfully discriminatory on grounds of sex);
  - Assess the additional cost (if any) of employing two people to do the job;
  - Decide how the job could be split in terms of hours or days worked, the work to be done etc, and how feasible it would be to fill the vacancy should one job-sharer leave. Compare this with the employee's request and requirements as discussed with him or her;
  - Consider arranging for the job to be shared for a trial period to allow for a review and assessment of the points raised above. Ensure that the situation is reviewed at the end of the trial period and that, prior to its commencement, all the parties concerned know what will happen if the trial is unsuccessful. All details of the arrangement must be set down in writing;
  - Obtain prior approval from the registered provider before splitting a full-time job;
  - Confirm any decision made, in writing, to any employees involved. Where agreement is to be given to an employee's request, ensure that the change to the employee's contract of employment is put in writing, along with details of any changes to terms and conditions of employment. The employee must sign his agreement to the changes.

- Part-time workers are employees of the company and are subject to the usual rules on deductions for tax and national insurance contributions.

- Part-time employees have the same statutory rights as full-time employees and are paid and accrue benefits on a pro-rata basis.

- All part-time employees, regardless of the number of hours they are to work, must be given, on their appointment, details of the terms and conditions governing their employment. This is particularly important in respect of the terms which differ from those enjoyed by full-time employees.

- Full-time employees, including those returning from maternity leave, who wish to transfer to part-time work or who wish to share their job with another part-timer, must put their request in writing to their Department Manager setting out the details. For example they should outline the number of hours wanted, whether this constitutes two or three full days a week or half days, the type of work they wish to do, if not their current work, and any other information relevant to their request.

- Employees who are dissatisfied with any decision made in respect of a request to do part-time work or job-sharing should use the organisation's formal grievance procedure.

Reviewed by Marcus Kerridge McColl
December 2025

Paternity Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with the:

  - Employment Relations Act 1999;
  - Employment Act 2002;
  - Paternity and Adoption Leave Regulations 2002;
  - Statutory Paternity Pay and Statutory Adoption Pay (General) Regulations 2002.

Scope

- All employees.

Policy

- The organisation's policy is to comply with both the letter and spirit of the law on paternity rights. To this end its aim is to inform all employees, regardless of marital status, of their entitlement to statutory paternity rights and to ensure that those rights are understood by employees who qualify.

Statutory Paternity Leave

- Paternity leave can be taken for either one week or two consecutive weeks after the child is born or placed for adoption (starting any day) and has to finish within 56 days of the baby's birth.

Qualifying conditions

- You are eligible for paternity leave if you:
  - Have or expect to have responsibility for the baby's upbringing and are either the:
    - Biological father of the baby;
    - Mother's husband or partner (including same-sex relationships);
    - Child's adopter;
    - Husband or partner (including same-sex relationships) of the child's adopter.
- Are taking the time off to care for the child.
- Make a declaration that you are taking leave for the statutory purpose and that you meet these qualifying conditions.
- Have 26 weeks continuous service, as an employee, ending with the 15th week before the baby is due (the qualifying week) and are employed from the 15th week before the baby is due up to the date of birth, or have 26 weeks continuous service by the end of the week you are notified you are matched with your child. The qualifying week remains the same if the child is born early.
- If you are the husband or partner to the mother, but are not in an enduring family relationship with the mother and baby then you are not entitled to paternity leave.

Notification requirements

- You must notify us by the end of the 15th week before the expected week of childbirth (EWC), or within 7 days of being told by the adoption agency that you have been matched with a child, or as soon as reasonably practicable, following:
  - Your intention to take paternity leave.
  - The expected week of the baby's birth.
  - Whether you wish to take one week or two consecutive weeks' leave.
  - When you want to start your leave (if subsequently, you wish to change this date you must give 28 days notice in writing of the date you intend to start, where reasonably practicable).
- You can choose when the leave period will begin, but it must take place within 56 days of the child's birth and it must start on one of the following dates:
  - The day the child is born, or;
  - A day which falls a certain number of days after the child is born, which you must specify before the child is born, or;
  - A pre-determined date that falls after the first day of the week that the child is due, you specify;

- If the child is not born on or before the date your leave is due to start, you can vary the leave date to begin:
  - With a later pre-determined date, or;
  - On a specified number of days after the child is born, or; Œ
  - On the date that the child is born.

- Once the child has been born and as soon as is reasonably practicable, you must give us notice of the date on which the child was born.
- It is particularly important that you comply with the notification requirements because if you fail to do so you can lose your rights.
- The paternity leave period will begin on the date that you specified in the final notice that you give. If you stated that the leave period would start on the date that the child is born, and you are at work on that date, then the paternity leave period will begin the next day.

Ordinary Paternity pay

- To be entitled to Statutory Paternity Pay (SPP) you must satisfy the above qualifying conditions and the notification requirements.
- Ordinary Paternity Pay is payable at the SPP rate OR 90% of your average weekly earnings (whichever is lower).
- Ordinary Paternity pay will be for the length of leave that you have decided to take and is subject to the maximum of 2 weeks. The rate of SPP is determined by the Department for Work and Pensions.
- To claim SSP, you must complete form SC3 'Becoming a Parent', confirming that you are entitled to take the leave. (You can download a pdf file of this form from the HMRC website).

Start of SPP

- You can choose when the pay period will begin, but it must take place within 56 days of the child's birth and it must start on one of the following dates:
  - The day the child is born, or;
  - A day which falls a certain number of days after the child is born, which you must specify before the child is born, or;
  - A pre-determined date that falls after the first day of the week that the child is due, specified by you;
  - If the child is not born on or before the date your pay is due to start, you can vary the date the pay is to begin:
    - With a later pre-determined date, or;
    - On a specified number of days after the child is born, or; Œ
    - On the date that the child is born.

SPP payment

- You can claim SPP in blocks of one week or two weeks – up to a maximum of two weeks. You will be paid either a flat rate per week or 90% of the average weekly earnings if you earn less than the flat rate per week. The average weekly earnings are calculated using the gross pay, which includes the pay that forms part of the NI contributions. See the ***HMRC*** website for the current amounts.

Paternity rights if not married

- 'Statutory Paternity Leave' and 'Statutory Paternity Pay' applies to all couples, whether heterosexual or same-sex and whether married, in a civil partnership or co-habiting, where one person in the couple is giving birth to, or adopting, the child.

Working while receiving SPP

- You are not entitled to receive any other payments for employment while they are receiving Statutory Paternity Pay (SPP) from an employer. If you do any paid work for anyone other than us while receiving SPP from us, then we will cease paying you SPP for the remainder of the pay period.

Not eligible for SPP

- If you are sick when the paternity leave period is due to begin you cannot claim both Statutory Sick Pay and Statutory Paternity Pay. If the sickness begins before the leave period starts, you can postpone the leave period to begin at a later date and give notice to us of this change. However, you must take the paternity leave within the 56 days following the child's birth or placement, and you cannot extend the leave or pay period beyond that time.
    - If you are taken into legal custody or sentenced to prison (and the sentence is not suspended), we can suspend the SPP payments immediately.
    - If you die while claiming SPP, we are entitled to stop payments of the SPP for the week following the death.

Avoidance of payment of Statutory Paternity Pay

- We will not terminate the contract of employment simply to avoid paying you Statutory Paternity Pay (SPP).

Dismissal and paternity leave

- We will not penalise you in any way for asking to take paternity leave, or seeking Statutory Paternity Pay, irrespective of how long you have worked for us.

Entitlement if child dies or is stillborn

- If the child is stillborn after 24 weeks of the pregnancy, or is born alive but dies after this time, you are still entitled to take paternity leave if you have worked with us for 26 weeks by the qualifying week.

Multiple births

- The number of babies born at the same birth does not affect the amount of leave you are entitled to take, so whether your partner gives birth to one baby or has a multiple birth, the leave and pay entitlement remain the same.

Additional Paternity Leave

- Additional paternity leave and pay is no longer be available for babies due after 5 April 2025. This has been replaced by shared parental leave. See the policy and procedure on shared parental leave for details.

Terms and conditions while on paternity leave

- If you return to work after a period of paternity leave (ordinary or additional) and have taken no other leave, or you have taken paternity leave followed by annual leave or other statutory leave, then you are entitled to benefit from the same terms and conditions as if you had not been away – except for those terms and conditions relating to remuneration (i.e. wages, salary).

Same job on return from paternity leave

- You are entitled to return to the same job that you were employed in before you went on leave, and benefit from the same conditions as if you had never been on leave, (unless a redundancy situation has arisen).

- You are entitled to benefit from any general improvements to the rate of pay or other terms and conditions, which may have been introduced for the grade or class of work while the employee was away. This is the case if you took the paternity leave on its own, or linked it with other statutory leave such as annual leave or less than 4 weeks of parental leave.

- However, if you chose to link the paternity leave period with, for example, a period of parental leave of more than 4 weeks then you are entitled to return to the job that you were employed in before taking the leave, or, if that is not reasonably practicable, another job which is suitable and appropriate for you, and which has similar – and certainly not less favourable – terms and conditions attached.

- This is not an exhaustive statement of paternity rights. If there are errors or omissions in your rights as stated here, then your statutory rights will apply.

- There is more information here.

- As an organisation we will use this guide.

Reviewed Marcus Kerridge-McColl
December 2025

Personal Hygiene Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards. Š

- To prevent cross-contamination.

Scope

- Carers also engaged in food preparation.

Policy

- Crescent Community Care Services Ltd will protect Service Users' food from contamination.

- All food handlers must be appropriately informed, skilled and trained to be able to understand their part in the control and prevention of contamination.

Procedure

- Inform your supervisor, before commencing work, of any skin, nose, throat, stomach or bowel trouble, or infected wound. It is breaking the law not to do so.

- All food handlers have a duty to keep themselves scrupulously clean; these measures include regular bathing, keeping head and facial hair clean.

- Nails must be short, clean, unbitten and not coated with nail varnish. False nails may not be worn.

- Regular hand washing must take place, especially when entering a food room, after handling raw foods, handling refuse, using toilet facilities and blowing nose.

- Hand washing must take place at the specified sink, using soap, and hands dried on a fresh disposable towel or if in situ, using a warm air dryer.

- Cover all cuts and sores with high visibility, waterproof dressings.

- Do not handle foods unnecessarily.

- Do not smoke in a food preparation area, or cough or sneeze over food.

- Prepare food as near as possible to the time it is to be served.

- Clean as you go – keep all surfaces and equipment clean at all times.
- Hair must be either short or tied back, always clean, and always covered.
- Only non-scented deodorants are to be worn.
- No perfumes or after-shaves are to be worn.

Reviewed by Marcus Kerridge McColl
December 2025

Photographs Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To comply with privacy standards

Scope

- All Workers

Policy

- Photographs of individuals may only be taken for specified purposes and with the express permission of the subject and only by a person authorised by the Registered manager or Directors
- Photographs will be stored in a manner which does not allow them the be accessed by unauthorised persons or for unauthorised purposes

Procedure

Photographs of Service users

- May usually only be taken for the following purposes
    - Placing on a care plan or medication administration sheet in order to identify definitively a Service User. The photograph for these purposes will only be of the service users face
    - Placing in a care plan to provide definitive proof of the appearance of such things like a wound or lesion. In this case the photograph must be of the area only and not of the whole body
    - For use of moving and handling and positioning of a service user as advised by Occupational Therapist. This photograph may be of the whole body of the service user and must be placed only in the Service Users logbook or maybe on the wall above the bed if the Service user wishes
- Photographs of Service users in any other context or any other setting or occasion, will be as far as possible taken to avoid identification of individuals and where they can be identified and the photograph should only be taken with the express permission of the Service user and all other people that are identifiable in the photograph.

Photographs of Employees

- These will be taken for the employees personnel file and badge and will only be of the face
- Photographs may be taken of the employee while they are working to assist in determining if there is a case to be answered in a disciplinary and if it is thought to be proved that the employee is bringing the Company into Disrepute

Storage of Photographs

- Following the printing or other use of an authorised photograph, the original will be stored electronically where it can only be accessed by authorised staff. Any printed photographs will be stored in a locked facility

Reviewed Marcus Kerridge-McColl
September 2025

Poor Performance Policy and Procedure
Crescent Community Care Services Limited

Purpose

- The organisation will at all times endeavour to ensure that employees achieve and maintain a high standard of performance in their work.

Scope

- All employees.

Policy

- The organisation will ensure that standards are established, performance is monitored and employees are given appropriate training and support to meet these standards.

Procedure

- Where the manager/supervisor first establishes that an employee's performance is unacceptable, an informal discussion will be held with the employee to try to establish the reason. Should this discussion result in a decision that the established standards are not reasonably attainable, the standards will be reviewed. Should the interview establish that the performance problems are related to the employee's personal life, counselling/support may be provided. However, if it becomes apparent that the poor performance constitutes misconduct, the disciplinary procedure will be invoked.

- The Carer Supervision Record form will in all cases be used to record this process, planned action and outcomes.

- If it is decided that the poor performance is a result of a change in the organisation's standards, those standards will be explained to the employee and help will be offered to achieve conformity with the standards.
- Š
- Should the employee show no (or insufficient) improvement over the next four weeks, a formal interview will be arranged between the employee (together with a representative if so desired) and the department manager. The aims of this interview will be to:

  - Identify the cause(s) of the poor performance and to determine what – if any – remedial treatment (e.g. training, retraining, support, etc) can be given;
  - Explain clearly the shortfall between the employee's performance and the required performance;
  - Obtain the employee's commitment to reaching that standard and set a reasonable period for the employee to reach the standard;
  - Tell the employee what will happen if that standard is not met.

- The outcome of this interview will be recorded in writing and a copy will be given to the employee.

- At the end of the review period, a further formal interview will be held, at which time:

  - If the required improvement has been made, the employee will be told of this and encouraged to maintain the improvement;
  - If some improvement has been made but the standard has not yet been met, the review period will be extended;
  - If there has been no discernible improvement, the department manager will explain to the employee that they have failed to improve. Consideration will be given to whether there are alternative vacancies which the employee would be competent to fill. If there are, the employee will be given the option of accepting such a vacancy or being dismissed;

- If such vacancies are available, the employee will be given full details of such vacancies, in writing, before being required to make a decision;
- In the absence of suitable alternative work, the employee will be told that the employer has no alternative but to dismiss. The employee will be invited to give their view on this before the final decision is taken.

- Employees may appeal against their dismissal by writing to the Registered Provider, stating the reasons for their appeal. The appeal must be made within seven working days.

  Reviewed by Marcus Kerridge McColl - December 2025

Principles of customer care / philosophy of care
Crescent Community Care Services Limited

Principles and values underpinning our Service

- Crescent Community Care Services Ltd is committed to supporting vulnerable people so that they can continue their lives with dignity and independence and be participating members of their own communities. Because of this, in the first instance, we are committed to meeting the needs of those people for whom we provide care or support. Usually we see no conflict between meeting the needs of Service Users and those of workers. Where such conflict exists, the needs of Service Users must take precedence. The basic principles underlying our support to vulnerable people include:

- **Personal Choice:**

  - The care or support worker will assist each individual Service User to exercise, to their fullest potential, personal choice in opportunities and lifestyle. The care or support worker will ensure that the person they provide care or support for is fully involved in decisions about the provision, extent and timing of any care or support, and also over the withdrawal of any Service. The duty of the care or support worker is, together with the Service User, to assess situations and needs and identify practical methods of providing services and support to meet those needs, together with the advantages and disadvantages of each method. Those alternatives will be explained to the Service User, including the risks and advantages, the Service User will then be supported to make their own choice, and that choice will be honoured.

  - Where, for reasons of mental frailty, the person who is being cared for is not able to participate fully in making a choice, due consideration will nevertheless be given to their wishes, so far as these are expressed and practical to implement. We welcome designated advocates in this context. Any information which the service obtains about the Service User's past wishes will be incorporated into the current Care Plan. Where a Service User is not able in any way to participate in controlling their care or support, referral must be made to the Deprivation of Liberty Policy and Procedure.

  - The rights of the Service User not to accept care or support workers with whom they are not compatible will be upheld.

- **Consultation:**

  - Service Users will be consulted about daily living arrangements and enabled to participate in discussions about any proposed changes to those arrangements and be fully involved in and fully informed about the individual assessment of their care or support needs. Service Users have a right to be involved in a careful and thorough assessment of their needs and wishes, and to be included in the development of the
    outcome. Crescent Community Care Services Ltd.'s commitment will be to find the best and most cost effective way of meeting the Service User's needs and aspirations. Service Users will be supported to make informed choices about their future, which should be incorporated into their personal Service User Care Plans.

- **Privacy of Service Users:**

  - The care or support worker recognises the right of Service Users to be left alone, undisturbed and free from intrusion and public attention. The Service User also has a right to privacy with regard to both his/her personal affairs and belongings.

- **Confidentiality of information:**

- The Service User's rights to confidentiality must be safeguarded. The care or support worker will not disclose any personal information about Service Users to a third party unless this has been agreed with the Service User concerned. Agreement to disclose information should only be sought if this is for the benefit of the Service User, e.g. for the purpose of assisting in their Care.

- **Access to Information:**
  - Every Service User has a right to information about the objectives of their Care and a detailed explanation of the service being offered, and a right to see all records relating to them.

- **Discrimination:**
  - The Care worker will not discriminate against Service Users on the grounds of race, nationality, language, gender, religion and beliefs, age, sexual orientation, or social standing. The Care team will not discriminate between Service Users who pay directly for their Service and those who do not.

- **Personal Dignity, Independence and Individuality:**
  - Irrespective of the severity of their physical difficulties or mental infirmity the Service User's dignity, independence and individuality will be respected and maintained. The Care worker will recognise and respect, regardless of circumstances, the uniqueness of each Service User and their intrinsic value as an individual. Dignity in social Care is maintained when each person is valued and treated with respect in all aspects of their daily life, irrespective of their circumstances or level of dependency, and when they have skilled, sensitive Care such that enables them to achieve the highest possible quality of life.

- **Fulfilment of aspirations:**
  - Every Service User has the right to have their social, emotional, spiritual, cultural, political and sexual needs accepted and respected. Service Users will be enabled to achieve their potential capacity – physical, intellectual, emotional and social. Individuals will be given support and freedom to realise their personal aspirations and abilities in all respects of daily life.

- **Review:**
  - The Service User will have a regular review of their individual circumstances at which they have a right to be present and fully involved.

- **Services Information:**
  - The Service User will be fully informed about the range of Services provided by the Sharon Haran.

- **Legal Rights:**
  - The Service User will be fully informed about their legal rights, including their rights under the Deprivation of Liberty Safeguards regulations of the Mental Capacity Act 2005.

- **Medication:**
  - The Service User will be fully informed in the assessment of their medication needs and to make decisions about their medical treatment whenever possible.

- **Family and Friends:**
  - The Service User will be supported to maintain continued access to family, friends, facilities and the community.

- **Complaints:**
  - The Service User will be supported to make informal complaints and have them dealt with, and have access to a formal and effective complaint procedure and be supported to be represented by a friend or adviser if they so wish.

- **Supporting independence of Service User**

- The Service User will be supported to take risks to the extent of their own informed opinion. Service Users will have the opportunity to think, act and make decisions without reference to another person or unreasonable restriction. This will include the willingness to incur a degree of calculated risk.

THE PRINCIPLES OUTLINED ABOVE MUST BE GUIDED BY PRIOR COMMITMENTS IMPOSED BY HEALTH AND SAFETY OR STATUTORY REQUIREMENTS

Our aims & objectives and philosophy of care:

- We aim to offer skilled Care to enable people to achieve their optimum state of health and wellbeing.
- All people who receive services from Sharon Haran and all people who visit the Service User will be treated with respect at all times.
- We uphold the human and citizenship rights of all people for whom we provide care and support.
- Individual choice and personal decision-making are the right of all Service Users and these rights will be supported by all the people who work for Sharon Haran.
- The right of independence will be respected and encouraged for all Service Users.
- The individual uniqueness of Service Users will be recognised and all people will be treated with dignity and respect at all times.
- The individual's requirement for privacy will be respected at all times and all information relating to individuals will be treated in a confidential manner.
- We recognise the individual's need for personal fulfilment and aim to support Service Users to participate in their choice of individualised programmes of meaningful activity to satisfy their needs.

Reviewed by Marcus Kerridge McColl
November 2025

Privacy Policy and Procedure
Crescent Community Care Services Limited

Purpose
- To support the human rights of the Service User.

Scope
- All workers.

Policy
- The organisation recognises the right of Service Users to be left alone, undisturbed and free from intrusion and public attention. The Service User also has a right to privacy with regard to both his/her personal affairs and belongings.

Procedure
- The individual requirement for privacy will be respected at all times and all information relating to individuals will be treated in a confidential manner.
- The organisation recognises the right of Service Users to be left alone, undisturbed and free from intrusion and public attention. The Service User also has a right to privacy with regard to both his/her personal affairs and belongings.
- All Service Users have the right to be alone or undisturbed and to be free from public attention or intrusion into their private affairs.
- Any building or equipment fault which reduces the privacy of any Service User must be reported to the landlord.

- Staff will not discuss Service Users or their affairs within earshot of anyone not directly concerned with their Care. Discussion of Service Users and their affairs will be for the purposes of managing and improving their care, and not as entertainment, e.g. gossip.

- Records will be designed, used and stored in a manner which ensures privacy.

- Records will be made available to the Service User's principal Carer and family according to the wishes of the Service User.

- In shared accommodation, where the Service User lives with someone who is not their partner e.g. in a shared household:

  - In shared accommodation, each Service User's personal rooms will have a lock fitted which is appropriate to their needs, and the Service User will be provided with a key unless a documented risk assessment indicates that this is inappropriate. Decisions in this respect will be recorded in the Service User's Care Plan and signed as agreed by the Service User or their Advocate.
  - Particular attention will be given to preserving privacy in the use of bathrooms, toilets and when dressing and undressing. At the same time, health and safety and personal risk management will be considered and discussed.

Reviewed by Marcus Kerridge McColl
November 2025

Professional Relationships Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To illustrate good practice in professional relationships between staff, and staff and Service Users.

Scope

- All workers.

Policy

- Interpersonal relationships have a significant effect on the outcomes of actions. This is true of inter-staff relationships, and relationships between staff and Service Users.

- Crescent Community Care Services Ltd, through this Policy, will inform, coach and train all staff in best practice in relationships between themselves and other staff, and themselves and Service Users. The keynote at all times is Professionalism.

- The quality of professional relationships will be monitored by supervisors and managers and any concerns will be dealt with by informal discussions, or in serious or persistent cases, during formal supervision sessions.

Procedure

- Being a professional as a care worker means 2 things:

  - Having a fund of skill and knowledge to draw on in your work;
  - Keeping your feelings under control when dealing with people you care for and work with.

- The first is relatively straight forward and will be built through continuous training and work experience. There are numerous documents within the QCS system which will assist in the rapid gaining of the necessary skills and knowledge, and employees should bear in mind at all times that if they have a perceived skill or knowledge gap, then the first place to look for an answer is the QCS system. In particular there are numerous documents specifically designed to impart knowledge about professional standards of behaviour, such as the Employee's Handbook, the Care Planning Policy and Procedure, and the Service User's Handbook amongst others.

- The second point can be difficult. Working with older people, seriously ill children or people with learning difficulties, mental health need etc. can be upsetting and exhausting. In addition, working with colleagues in a potential challenging

workplace may also lead to the same effects. To cope with it carers (in the widest sense of the word) need to find a way of keeping feelings at arm length. If they don't they may form emotional attachments which could be harmful to all concerned. They could also get in the way of caring. Some things, helping people to be independent, advising them for their own safety what they may or may not be able to do, taking control of emergencies, will demand clear thinking of the sort that is difficult for someone feeling distressed or 'sympathetic'. Therefore carers need to learn to put such feelings to one side so that they can concentrate on caring in a professional and efficient way:

- However, in practice, carers will become emotionally involved and will have to find ways of dealing with their emotions. When caring for someone who is ill, or dies, or moves away there will be feelings to work through. Carers need to be aware of this and think how it is going to affect them. There are no easy answers here but the best approach is being able to talk about the situation with others in a similar situation; express the feelings and move on to whatever comes next. The supervisor, manager or other people are important here. Carers should talk to them about their feelings when things become difficult. Even if talking is draining and upsetting its better than keeping feelings bottled up as this can lead to clinical problems like depression and anxiety. More experienced colleagues will have had the same reactions in the past to the business of caring; they'll be able to help others cope.

- Being a care worker can be exhausting, both physically and emotionally. Often staff work long hours for little perceived reward. People outside the care setting will not understand what carers do. There may be no answer to some of the problems that have to deal with. The people being cared for may be unkind and unpleasant. Their relatives may criticise the approach. Work colleagues may be unpleasant or critical. Being aware that these issues will occasionally happen, but will be transient, will help in maintaining a positive attitude to the work, which in itself will help to overcome the negative aspects and allow recognition of the rewards of helping vulnerable people to enjoy their life to the fullest extent they are able to.

- Tensions can be particularly difficult between staff members themselves, because everyone is under pressure in contemporary care settings, and this can lead to uncharacteristic outbursts. If the incident is out of character, either ignore it, or make it clear in a calm, assertive but unaggressive manner that such behaviour is unacceptable. If the behaviour of another person is seriously disruptive, unpleasant or unprofessional, the issue must be raised with the supervisor or manager. Do not allow tensions to fester, as they will always become worse and affect the workplace even more seriously. Tensions between staff are very easily communicated to Service Users, whose quality of life will be reduced by the effects. Never complain about other staff within the hearing of a Service User.

- Bear in mind that in the workplace, relationships which have their basis outside of the workplace such as partnerships, marriage, friendships, likes and dislikes, have no place whatsoever within the workplace. In the workplace all workers have only their work status as denoted by their job title. They do not carry into the workplace their other relationships, such as marriage, partnerships, family relationships, friendships, or disputes because these have no place within the work system of the workplace, and are in almost all cases disruptive to professional standards of work and behaviour. Colleagues have to trust each other, and the use of outside relationships to form groups which are not based on work requirements is unprofessional, disruptive, and leads to lack of trust and transparency.

  - There are two old anecdotes of some use here:

    - Be friendly, but not friends.
    - Leave the outside world at the door.

- The employer has a responsibility, and procedures, for minimising the possibilities of disruptive relationships leaking into the workplace. If recruitment is carried out to professional standards, whereby the job to be offered is fully specified before recruitment starts, and the best person for that specification is always employed using objective measures, then it is less likely that person with outside relationships, such as husband and wife, partners, siblings etc. will be employed. The existence of family relationships between staff members should always cause recruitment practices to be questioned and investigated, and give cause for supervisors and managers to be vigilant for unfair favour or disfavour between family members.

Reviewed by Marcus Kerridge McColl
November 2025

Public Holidays Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To manage the special circumstances of a 24 hour, 7 day a week business against the desire to observe Public Holidays

Scope

- All employees

Policy

- Crescent Community Care Services Limited recognises all Public Holidays which are included as part of your 5.6 weeks holiday entitlement
- If in the normal operation of the duty roster you would work on the Public Holiday, then you will attend on the holiday and be paid as specified in the Terms & Conditions of Employment within your contract.
- If you are not scheduled to attend on the Public Holiday you are not affected by these arrangements and are not affected by the Public Holiday
- No holiday pay will be given over the Christmas and New Year holiday period as separate arrangements will be organised for this period.

Procedure

- The organisation recognises the following Public Holidays
    - Good Friday
    - Easter Monday
    - May Day Bank Holiday
    - Spring Bank Holiday
    - Summer Bank Holiday
    - Christmas Day
    - Boxing Day
    - New Years Day
- Where a public holiday falls on a weekend, the day that would normally be a Public Holiday will be regarded as the holiday for payment and other organisational purposes, eg. Christmas Day being on a Saturday and Boxing Day being on a Sunday – this will lead for the following Monday & Tuesday being declared as Public Holidays for National Purposes. This will be disregarded and the Public Holidays for the organisations purposes will be the Saturday & Sunday.
- You are required to work if you are
    - A Care Worker
    - A Senior Care Worker
    - An On Call Supervisor
    - An On Call Manager

Reviewed Marcus Kerridge-McColl
October 2025

Purpose

- To identify that which the organisation defines as "quality".
- To ensure that the policies and processes which will promote that definition are in place, and to ensure that those processes are carried out in accordance with the definition.
- To comply with statutes, regulations and quality standards.

Scope

- All processes herein defined.

Policy

- The organisation defines quality service as:
  - The provision of Care and support to Service Users and their families which is in accordance with the clearly identified individual wishes of the Service User (or their appointed advocate where it is clearly identified that the Service User is unable or unwilling to express their wishes), fully supporting their human rights, within the confines of legal and fiscal constraints.
- The organisation believes that the management style, policies and day-to-day practices within the organisation should support the human rights of its Service Users by promoting open communications, a culture of problem resolution rather than blame, and the involvement of all participants in the services provided.
- The organisation believes that a culture that is supportive of continuous improvement must be maintained by way of regular auditing and reviewing of the standards of performance in all aspects of the organisation and its personnel, followed by open discussion of strengths and weaknesses and action planning to resolve weaknesses.
- The key processes which must be followed in order to provide a quality service in this organisation are identified as follows:

Admission

- Processes to identify the physical, psychological and social needs of the Service User prior to service commencement.
- Processes to match the Service User to The Agency, which ensure that the stated needs of the Service User are within the services that The Agency is able and willing to offer, and provide a negotiation path to an agreed compromise in situations where The Agency is unable to provide the requested service but the Service User is willing to modify their requirements.
- Processes on service commencement to ensure that previously identified requirements of the Service User are made known to all Care staff who will be concerned with the Care of the Service User; ensure that the reception of the Service User is personalised and welcoming; encourage and support further information gathering and recording in order to guide the actions of staff.

Care Planning

- Processes to ensure that Care and support for each Service User is informed by a Care Plan which records sufficient static and dynamic information to provide an individualised and planned service to each Service User.
- Processes to ensure that the Care Plan supports and accommodates the regular reviewing of Service Users' needs and services, revision of the plan of care, communication of those changes to all persons concerned with the Care of the individual, review of the effect of the changes, followed by further review.
- Individually named members of staff are identified as primarily responsible for the organisation's interaction with each respective Service User.

Discharge

- Processes on discharge to other Care facilities/agencies which ensure that Service Users who are transferring are accompanied by sufficient information in order to minimise the disruption caused by the move and to help the receiving organisation to begin providing quality Care services as soon as possible.

- Processes for discharge on death are to ensure that the family and friends of the Service User, and the employees who knew the Service User, are sensitively supported through their grieving process.

Human Resources

- Processes which identify the number and skill-level of employees that is required to deliver quality services to the specified number of Service Users.

- Processes which recruit employees according to the identified skill specification, and whose values match those identified as important by the organisation, particularly with respect to the preservation of the human rights of Service Users.

- Processes which inform the employee about the management framework within which they work, the standards of performance required of them, and the opportunities for development which are open to them.

- Processes to support the personal development of the employee through induction and throughout their employment with The Agency, ensuring that the needs of the organisation and the skills of the employees are regularly reviewed and matched via the development programme.

- Processes which ensure that employees are managed in accordance with best practice and statutes, signifying the value of the employee to the organisation.

Administration

- Processes to ensure the financial stability of the organisation.

- Processes which identify and accurately record all value transactions between the organisation, the Service User, and the Service User's representatives, including any sponsoring organisations.

- Processes to ensure the organisation's compliance with its statutory responsibilities.

- Communication processes to ensure that all employees are fully and accountably informed of information required to carry out their duties.

- A complaints receipt and recording process which promotes the ability of the Service User to control their living environment.

- Processes to control the quality of goods and services brought into the organisation.

- Processes which inform the organisation, its employees and customers of the legal, financial and administrative framework within which they interact.

- Processes which check that all audit and review processes are carried out as specified, action planning takes place, planned actions are carried though, and the effects of planned action are fed back into the management process.

Ancillary services

- If appropriate, processes to ensure that Service Users' individual and collective requirements for food are delivered.

- If appropriate, processes to ensure that Service Users' clothing is adequately cared for.

- Processes to enable the Service User to maintain their normal life patterns, and their normal connections with their communities.

Procedure

- Quality will be assured by the existence and adherence to the relevant policies and procedures.

Auditing

- Audits.
- Each area to be audited will be supported by a full policy statement (purpose/scope/policy/procedure).
- The purpose of audit is to verify the implementation of stated and documented policy, procedure and standards.
- Audit reports will identify action required to achieve compliance.
- Employees will be involved in the audit process, and results will be reported to them.
- Specific communication strategies will be implemented to support audit and the carrying out of remedial action and improvements, such as Quality Meetings, Team Meetings, Staff meetings and Management Meetings.

Reviewed by Marcus Kerridge McColl
December 2025

Record Keeping Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To comply with good practice on record keeping

Scope

- All Employees of Crescent community Care Services Limited

Policy

Records will be generated and kept on all activities which may affect the Quality of Care and Support given to it's Service Users, the continuity of the care and any other business that may may affect the integrity of the Service or the safety of the Service Users

Procedure

Why record keeping is important and what to record

Record keeping will provide a permanent record of events
You may forget information that is not written in a permanent record
Information may be dangerous or illegal if it is remembered incorrectly for example in giving of medication

Accountability – this may protect you and the company from later complaints that something improper had occurred

> In the event of questions over the behaviour or the competence of staff members or the organisation, for example a complaint, a Coroner's report, any investigation by CQC – all documents become formal evidence. Due to this if something is not written down then the legal view is that it never took place so remembering something after the event will be immiscible as evidence. This is why it is a necessity to ensure that all documents are existing, can be found, are legible, understandable, truthful, relevant and signed

For Example records can be looked at if there is a problem and this will not matter if the person is off sick or has even left Crescent Community Care Services Limited and these can be held account to if they are not concise and true.

These records may require to be shared amongst other professionals and may even be read by someone that you or the company have ever met and so they need to be understandable.

These records can mean better care for the Service Users and colleagues can read back on them to establish how the Service User has been over previous visits in the case of a long term illness

Records are of no use if no one reads them and they can be of no use at all if they are not kept up to date and are classed as legitimate accounts of actions that have taken place. Daily Record sheets must be read at the beginning of your shift to ensure that the correct care is given and that you are current in your knowledge of the Service User along with reading the Care Plan at the beginning of the Service Users Log Book.

Objective and Subjective reporting

Objective reporting is to report what actually was seen, heard, smelt or even felt while at the Service users property. This includes if a Service user complains of an illness or dizziness or something is wrong. You are required to report exactly what is reported to you. This is the best form of reporting when completing Daily Record Sheets

Subjective reporting is used to report when you can not sense or measure and this should be avoided as far as possible, however, if you think something is wrong or it is bothering you then you must report it to the office or the On Call Supervisor.

Monitoring and obtaining information

Talking and listening carefully to your Service User is the most effective way of finding out the needs or concerns. Observation will also do the same

Why it is important to be a skilled observer

Observe & Record

- Being a skilled observer detects problems at an early stage and could help to prevent a serious problem
- Being alert to people and the environment whether it be a change in the surroundings or the Service Users condition can help to reduce accidents or incidents and assist in the well-being of the Service user
- Careful observation can also increase your awareness of the Service Users physical, emotional and social needs
- When working in the community Setting it is vital that you accurately record and report information in the Daily Record Sheets after every visit that you make and for all the Service Users that you provide care for. Failure to adhere to this policy will lead to disciplinary action taken against you
- Records should not be made until after the event – do not write at the beginning of your shift what you are expecting to do as something may happen that need reporting
- Many changes will happen and these not only need recording in the Daily Record Sheets but also reported to the office or On Call Supervisor

Guidelines for receiving and transmitting messages

- Write down messages clearly and concisely
- Listen carefully and check for the accuracy with the person transmitting the message
- Work out the scale of the emergency with the message and act accordingly
- If you leave a message with someone then make sure the correct person will receive the message
- When writing down a message make sure that you put who it is to, who it is from, the date and time of the message, clear details of the message, an indication of the urgency

Reviewed Marcus Kerridge-McColl
September 2025

Recruitment Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that personnel are recruited when necessary, that the best person for each vacancy is selected, and no discrimination takes place.

- To comply with statutes, regulations and quality standards.

Scope

- All recruitment.

Policy

- People will only be recruited when there is a long-term permanent vacancy, and the best person for each vacancy will be recruited, regardless of age, gender, sexual orientation, race, creed or disability.

- Wherever possible, existing employees will be invited to apply for promotion opportunities.

- A decision to interview, shortlist or offer employment will take no account of an applicant's trade union membership or non-membership.

- Interviewers who are not completely sure that they can comply with the highest standards of interviewing will seek and take the advice of the Registered Provider prior to engaging in interviews. It is a requirement that for each interview a strategy is implemented to test the applicant against previously agreed standards, and that these tests are carried out and recorded. Specific attention will be given to avoiding discrimination of any kind. Care will be taken to ensure that the interviewee has all of the information they require in order to fully understand the job on offer, their responsibilities and, conversely, that no false impressions are conveyed.

- All recruited employees will be required to provide a National Insurance number, with evidence that it is valid (P45, NI Card).

- All applicants will be required to establish their identity, using photographic evidence (photographic drivers licence, passport, etc) for criminal record checks, and all candidates will be required to produce documentation of any change of name (e.g. marriage licence), or sign a declaration stating the reason for the name change.

- Candidates will be informed that, because the job requires them to have access to vulnerable people, if offered the post they will be subject, on recruitment, to a request for their criminal records to DBS, and that their employment will be conditional on that information being satisfactory. They will also be informed that the employer may make repeat checks at any time during their employment and disciplinary action taken if the information received indicates that the employee is no longer suitable, by reason of relevant conviction or listing on the register of unsuitable persons.

- Carers will be aged at least 16.

- **Blacklists**:
  - In order to comply with The Employment Relations Act 1999 (Blacklists) Regulations 2010, Crescent Community Care Services Ltd will not use blacklists as defined by the Regulation.
  - Lists of Trade Union members or of those who have taken part in Trade Union activities are known as blacklists if their intention is that they be used for discrimination in relation to selection and in employment of workers. The Employment Relations Act 1999 (Blacklists) Regulations 2010 make it unlawful to compile, use, sell, or supply such a list, or to discriminate on its basis. The remedies available include compensation to any worker affected, which normally will not be less than £5,000 and may be substantially more.

- Recruitment will be carried out by use of Recruitment Packs for Recruit Specification, Individual Candidate Application and Job Start and Induction.

- The recruitment system embodied in the Recruitment Packs includes the following processes:

  - **Recruit Specification Pack:**

    - Gaining authorisation for the recruitment from the budget holder;

    - Specifying the job description and the person specification for the recruitment in order that the process is planned and non-discriminatory;

    - Control of advertising to ensure that the correct facts are published, and that the image of the organisation is either maintained or enhanced;

    - An interview schedule to ensure that interviews are adequately planned and organised;

- Applicant score summary to ensure that the selection is objective, in line with the original job and person specifications, and non-discriminatory.

- **Individual Applicant Pack:**

  - Telephone screening forms, in order to balance the time savings made by excluding applicants who do not meet basic job and person specifications from the interview process, with the need to carry out and record a non-discriminatory process;

  - A letter offering interview, specifying the process and those documents required, in order to minimise the interviewer's wasted time;

  - A Carer standards form to place on record the views of the applicant about the post, in order to begin the standard setting process for performance management in the future;

  - The application form, including:

    - Personal details;
    - Educational details;
    - Skills details;
    - Employment history;
    - Interests and hobbies, which may contribute to Service Users' life quality;
    - Health details;
    - Identity details;
    - Capacity to work in the UK;
    - Referees;
    - Specific health questionnaires (non-specific health questionnaires have been held to be discriminatory);
    - Criminal records statements.

  - Form for checking employment continuity and flagging up matters for further investigation;

  - Identity-establishing checklist;

  - Interview preparation forms to enhance the interview effectiveness, and help avoid discrimination claims;

  - Standard questions for the interview, in order to help structure the interview process and ensure that certain essential matters are discussed;

  - Candidate risk assessment, for use when any matter within the process gives rise to a concern, for instance the lack of a recent employer reference;

  - Candidate score sheet to promote the consistent assessment of candidates, the score given to be transferred to the score summary in Recruit Specification Pack;

  - Telephone reference request forms to record any telephone references taken, with a form to ensure that a copy of the record is signed by the referee for the files;

  - Standard reference requests where these are to be used in preference to telephone requests;

  - Letters to the applicant:

    - Rejecting their application because they did not meet the job and person specification, or too many applicants of the required standards were interviewed;

    - Placing their application on file, where they met the job and person specifications, but no post was currently available;

    - Offering them the post, ensuring that relevant details of the post are recorded;

    - Equal opportunities monitoring form, to be given out anonymously for anonymous return, which will contribute to the database which will inform the organisation's equal opportunities reviews.

- **Job Start and Induction Pack:**

- Form for informing administration and payroll functions of the details required for a new employee;
- Authority to make deductions form for signature by the new employee, to authorise the employer to make wages deductions other than income tax and National Insurance;
- Induction control system;
- Training records;
- Statement of Main Terms and Conditions, for issue on the first day of employment;
- Personnel file checklist;
- Three-month employment review pro-forma;
- Personal copy of the Codes of Practice for Social Service Workers and Employers, to be given to the employee on the first day of employment.

Procedure

- Complete one Recruit Specification Pack, including:
- Reason for vacancy.
- Copy the relevant job description; place the copy in the Pack.
- Complete the person specification, taking into account the current team members and any special requirements for the Service User(s) to be supported by this recruit.
- Write out the proposed advertisement, take three quotations.
- Check advert against checklist (look on the advert as a sales advert for The Agency; avoid specifying matters which may unnecessarily put off applicants e.g. a strict no smoking rule (no smoking on the premises is normal, none at all will exclude about 60% of otherwise suitable applicants), reference to restricted Service User groups, where this does not affect employees e.g. a "Christian Agency". Avoid any potentially discriminatory statements, such as those referring to race, gender, age or sexual orientation, or which set conditions which place any group at a disadvantage to another due to their race, gender, age or sexual orientation.
- Refer pack to the Registered Provider for:
  - Approval of recruitment;
  - Approval of advertisement, media and cost.
- Place advertisement.
- Place copy of advert on employee notice board.
- Arrange for sufficient telephone answering resources to be available during expected telephone application period.
- Ensure that interview times are diary booked; enter times in the Recruit Specification Pack. Ensure that times of interviews include time for preparation before each interview, interview time, and time to record the results of that interview. Spaces of one hour are recommended.
- Ensure that a sufficient supply of the Individual Candidate Application Pack is available, by the telephone that is to be used for receiving the calls, together with the Recruit Specification Pack open at Interview Schedule.
- For each telephone enquiry, use one Individual Candidate Application Pack.
- Ensure that all questions on the Telephone Screening form are asked, and that answers are entered on the sheet.
- Do not ask health or disability related questions.

- Decide, on the basis of the telephone screening answers, while the enquirer is still on the line, if they are suitable for interview, i.e. meet the basic job and person specification.

» If not, inform them immediately, state the reason, and record the reason on the form.
» If yes, offer interview time, agree it, and record the candidate's name on the Interview Schedule against the agreed time.
» Ensure that the candidate is informed that the full interview process will take about one hour, that they will be given an application form to complete on arrival, and that a short written test of their understanding of Care issues will be part of the process.

Complete to this stage for each telephone applicant.

Before the interview day, set time aside for a review of all Individual Candidate Application Packs, listing any questions arising from telephone screening on the Interview Preparation sheet.

Arrange for two people to conduct the interviews, one of whom should be the immediate supervisor or manager of the post being advertised.

If the interview is carried out by only one person by reason of staff resources on the day, the interviewer will be the immediate supervisor or manager of the post advertised, and the interview will be preceded by a tour of The Agency accompanied by a member of the staff team on duty. That team member will report their personal impressions of the applicant to the interviewer before the interview, and will be consulted about the employment decision.

On interview day:

» Prepare interview room, ensuring that interviews will be private and uninterrupted, making arrangements as necessary;
» If possible arrange for someone to receive candidates at the door, with instructions to deliver them to a waiting area, making them feel welcome;
» If possible, have the reception person give each candidate their own Individual Application Pack as they arrive, with instructions to complete the Carer standards and application form pages;
» If this is not possible, ensure that all members of staff are aware that interviews are taking place, so that candidates are welcomed appropriately and the interviewer is informed of their arrival.

Before each interview:

» Take the completed Individual Application Pack for that candidate;
» Review telephone screening, Carer standards and the application form, and note any issues arising on Interview Preparation sheet;
» Check that educational qualifications are appropriate and adequate;
» Check work history, note and investigate all periods of no work;
» Note any declared requirements for adjustments for disability;
» Check that references are complete and appropriate;
» Check the applicant's Disclosure status;
» Check that identity has been clearly satisfied, and recent photographs supplied;
» Check that all claimed relevant qualifications are accompanied by documentary evidence;
» Review the whole against the job and person specifications.

- At the start of the interview, give a brief outline of The Agency and the company and, in particular, the terms and conditions, ensuring that the terms given are noted on the interview log.

- During interview, ask all required questions, note answers on sheets provided.

- Inform candidate that, if they are successful in their application, the offer will be subject to the receipt of two satisfactory references, of which one must be from the previous employer, and that they cannot start work until those references have been received in writing. Also inform them that their appointment will be subject to satisfactory information being received from the DBS, to which they must be referred under the Health and Social Care Act 2008, and that although they can start work before the reply is received, the offer of employment will be conditional on the content of the reply, and cannot be confirmed until a satisfactory reply has been received.

- Give the applicant a copy of the Disclosure application form, with instructions to complete it and return it.

- On completion of interview questions, ask candidate if they have any questions. Answer any questions posed.

- Thank candidate, tell them when the decision will be made, and how they will be informed.

- After the candidate leaves, complete the Score Sheet.

- In the case of internal candidates, the full procedure will be carried out, but the completion of forms and interview questions will be adjusted to take account of prior employment with the company.

- On completion of all interviews, transfer the score for each candidate onto the Applicant Score Summary in the Recruit Specification Pack. Decide on the successful candidate(s), noting reasons for rejection and acceptance against every candidate. Make particularly comprehensive notes if the person(s) offered the post(s) are not the highest scoring candidates. Crossing out of notes is not permitted, and any alteration must be scored through allowing the original to be legible, and signed by the person making the alteration.

- When the shortlist is complete, review every candidate file to critically question all reasons for rejection and acceptance, bearing in mind that the company does not condone in any manner choices made for any reason other than the ability of the candidate to carry out the job as previously specified, with no discrimination due to race, sex, sexual orientation, creed or disability. If the interviewer is unsure of their proposed decision in this area, they MUST consult the Registered Provider for further guidance.

- Telephone each reference for each successful candidate, completing Reference Request 1 to 3 for each candidate.

- If a candidate is contra-indicated due to a reference, decide if the reference is to be accepted and the candidate rejected, or investigate further (for instance, by recall of the candidate and further questioning).

- In the case of Registered Nurses, telephone the NMC to test the validity of the given NMC PIN number.

- **NB – IN NO CIRCUMSTANCES PROCEED BEYOND THIS POINT TO OFFER A POST TO A CANDIDATE UNLESS:**

    1. At least two satisfactory written employer references have been received for that candidate, including one from the last employer who has also been verbally spoken with to confirm employment.

    2. A Disclosure form has been completed and despatched.

    3. The DBS check is satisfactory (with no exceptions).

    4. If there are any issues with 1 or 2 above, complete an assessment using the form supplied in the Individual Applicant Pack, and make an assessment as to the possible implications of employing this person and exposing vulnerable people to them. Discuss the matter with the registration authority if in doubt. Note that no exceptions may be made if a person is listed on the DBS register.

- For all accepted candidates, telephone them to offer them the post on the terms and conditions noted in the interview log. If they accept, agree a start date, and tell them the appointment will be confirmed by letter. The letter will state that the appointment is subject to a satisfactory report from the DBS, the receipt of which will enable the employment to be confirmed, but that until that time they must work under the constant supervision of a fully DBS- and DBS-checked member of staff.

- On completion of all offer contacts, complete the offer letter in each Individual Candidate Application Pack for each successful candidate; post a copy of it to the candidates.

- Complete reference request letters for each successful candidate, ensuring that a copy of the telephone reference is attached, and post to referees.

- Complete either a rejection or a hold letter for every unsuccessful candidate, as appropriate, and post a copy to the rejected candidates.

- File the Recruit Specification and all Individual Candidate Application Packs for unsuccessful candidates together, in archive.

- Start an employee file with the Individual Candidate Application for each successful candidate.

- Before the start day, complete a Job Start and Induction Pack for each new employee, particularly the front sheet; complete a Terms and Conditions form.

- On the start day, complete the Job Start and Induction Pack, copy starter details from the pack and send that to administration, guide the employee to the location of the Employee Handbook, show them the main elements of the Handbook, and allow them time to read through it.

- Ensure that the employee hands over evidence of the validity of their National Insurance number. Investigate further to a satisfactory conclusion if they do not.

- Hand the employee an Equal Opportunities form, ask them to complete it voluntarily.

- Carry out the Induction Procedure.

- Diarise the Three-month interview.

- Carry out the interview, and if the employee is performing satisfactorily, confirm their appointment in writing.

Reviewed by Marcus Kerridge McColl
December 2025

Redundancy Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with employment legislation.

Scope

- All employees.

Policy

- Whilst it is the organisation's intention to develop and expand its business activities, and thus provide a stable work environment and reasonable security of employment for its employees, it must ensure the economic viability of the enterprise in an increasingly competitive business environment.

Procedure

- In this respect, circumstances may arise where changes in the market, technology, organisational requirements, and the like, necessitate the need for redundancies to be made. In order to minimise the impact of such reductions in the workforce, the following procedure will be adopted wherever possible.

- It must, however, be recognised that where the needs of the business so dictate, the procedure will be adapted to the particular circumstances which prevail. Where the possibility of a reduction in the size of the workforce arises, management will enter into consultations with employees with a view to establishing whether the proposed job losses can be achieved by means other than compulsory redundancies.

- Consideration will be given specifically to the following alternative options, subject to the organisation's immediate business considerations:

  - Imposing an immediate ban on further recruitment of new staff, other than where this is essential, and considering redeployment and/or retraining of surplus personnel;
  - Restricting the use of sub-contracted labour, temporary and casual staff;
  - Reducing the amount of overtime working in the organisation;
  - The implementation of temporary layoff, short-time working, job-sharing etc, where appropriate;
  - Inviting applications for consideration for early retirement and/or voluntary redundancy.

- It is important that the employer consults with employees before making any redundancies. This allows the potentially affected employees to have an opportunity to influence any redundancy plan, so provision must be made to enable this to happen.

- Consultation can take place in two ways: Individually or collectively:

- **Individual consultation:** Here, the employer needs to advise the individuals selected to be "at risk" of redundancy about the reasons why they have been placed at risk. The employer will then need to look at alternatives to redundancy where possible.
- **Collective Consultation:** If the employer is thinking about making 20 or more employees redundant at one establishment, then they must consult with employee representatives. Who the employer will consult with will depend on whether the employees are represented by a trade union or not. If the employer consults with the trade union, then they are not required to consult with any other representatives. Should no trade represent the employees, then the employer must make arrangements to allow the employees to elect representatives. The representatives will then act on behalf of the employees. The representatives must be elected, and must be fit for the task.

- The **collective consultation process** must be conducted with the aim of reaching a meaningful agreement. It is required to cover:

  - Ways to avoid redundancies;
  - The reasons for the proposed redundancies;
  - How to keep the number of dismissals to a minimum;
  - How to limit the effects on those dismissed, for example, by offering training etc.

- During the collective consultation process, written information must be provided by the employer in good time to enable the representatives to make constructive proposals. This must include:

  - The reasons for the proposed redundancies;
  - The number of employees that have been proposed for redundancy;
  - The different types of employees that have been proposed for redundancy; » The number and type of employees;
  - How employees, if any, will be selected for redundancy;
  - The time period over which any redundancies may be carried out;
  - How any redundancy payments will be calculated.

- The notice of termination, including confirmation of the final day of an employee's employment, cannot be issued until after the consultation has been completed.

- The consultation must begin in good time and continue for as long as required. There is no actual time limit in place for how long the consultation should last, but there is a minimum time frame in place from when the consultation begins to the actual termination of employment. This length of time depends on the number of proposed redundancies:

  - 20-99 proposed redundancies – consultation must be started at least 30 days prior to the actual dismissal date;
  - 100+ redundancies – consultation must begin at least 90 days before the actual dismissal date.

- If you are planning on making 20 or more employees redundant, then you are required to notify the appropriate Redundancy Payments Office of the proposals before any termination notices are sent to the employees. The notification must be made at least 30 days before the termination date, or 90 days in the case of 100+ employees being made redundant. Failure to do this will lead to a fine. This is part of the consultation process, and a copy of the notification must be provided to the representatives.

- As far in advance of the proposed termination date as possible, management will, therefore, notify all employees that compulsory redundancies are proposed, and that provisional selection has been made. It should be stressed that the selection is provisional only and may be subject to change. Management will then enter into immediate consultation on an individual basis with those employees provisionally selected for redundancy. In the course of consultation employees will be informed of the basis of their provisional selection and invited to make representations on their proposed terminations.

- Management will take due note of, and will give full consideration to, any such representations before confirmation of those selected for redundancy is made by the Registered Provider, in consultation with the Registered Manager.

- Where selection has been confirmed, those selected for redundancy will be given notice of termination in accordance with statutory and contractual entitlements.

- **Methods of Selection**: If a method for selection has been agreed with either the trade union or the employee representatives, then it must be followed. Otherwise, there are common approaches which can be followed. Occasionally there will not be a need to do this where the selection is very clear, e.g. a particular operation within a business is being closed down.

- At the employer should consider "selection pools", which will cover pretty most other situations, when making a decision about the employees to select for redundancy. This involves identifying which group or section of the workforce you

propose to draw the redundancies from. Once the pool has been identified, the employer needs to apply selection criteria to narrow down the employees. Objective criteria must be set, which can then be applied equally and fairly to all potentially affected employees. Examples of the selection criteria include:

- Disciplinary records;
- Experience;
- Capability;
- Relevant skills and competence.

- **Under no circumstance should employees be selected for redundancy because of the following reasons:**

  - Gender;
  - Marital Status;
  - Sexual Orientation;
  - Race;
  - Disability;
  - Religion or belief;
  - Age;
  - Trade Union membership;
  - Health and safety activities;
  - Working patterns (e.g. part-time or fixed-term employees).

- If employers select employees to be made redundant based on any of the above grounds, then this could lead to a claim for unfair dismissal or discrimination.

- **Last in first out**: An employer can look at an employee's length of service with a company when considering them for potential redundancy, and therefore select those with the shortest term of service. What must be kept in mind is that an employer can be accused of indirect discrimination on the grounds of age if this method affects one age group more than another, unless the employer can objectively justify it.

- **Unfair selection**: An employer must not use unfair reasons for making a selection for redundancy. The following reasons are considered to be "unfair", and can therefore lead to a claim for unfair dismissal or discrimination at an Employment Tribunal:

  - Any reason relating to maternity leave, birth or pregnancy or any other family leave, paternity leave, parental or dependents' leave;
  - An employee's disability;
  - The employee has transferred employers and is protected under the Transfer of Undertakings (Protection of Employment) Regulations 2006 (TUPE)
  - The employee has membership or non-membership of a trade union;
  - The employee is exercising their statutory rights (for example, asking for a written statement of employment particulars);
  - Whistle-blowing (that is, making disclosures about the employer's wrongdoing);
  - The employee is/has taking/taken part in lawful industrial action lasting 12 weeks or less;
  - The taking of action on health and safety grounds;
  - The employee is doing jury service;
  - The employee is the trusteeship of a company pension scheme.

- **Redundancy Notice Periods**: If an employee has been selected for redundancy, then a redundancy notice must be provided to the employee before their employment ends, outlining the notice period. The statutory minimum notice periods are:

  - At least one week's notice if the employee has been employed between one month and two years;
  - One week's notice for each year, if employed between 2 and 12 years;
  - 12 week's notice if employed for more than 12 years.

- When advising of the above, it is important that you also check the contract of employment, in case you have set out higher amounts than the statutory minimum. You may also, as a business, make a decision to provide greater amounts of notice than those provided on a statutory level.

- **Redundancy Pay**: If the employee is entitled to statutory redundancy pay, then the calculation is based on:

  - How long they have been continuously employed;
  - Their age;

- Their weekly pay (this is capped at £475 for dismissals after 6 April 2025).

- Taking into account the above, the employee is entitled to:

  - 0.5 weeks' pay for each full year of service if they are 22 or under;
  - 1 week's pay for each full year of service if they are above 22, but under 41;
  - 1.5 weeks' pay for each full year of service if they are 41 or above.

- For example: If the employee is aged 45, her weekly pay is £475 per week and she has completed 15 years' full service. She will receive £8,075 as statutory redundancy pay.

  - Step one: 1.5 weeks x 4 years full service for when she was age 41 or above = 6 weeks.
  - Step two: 1 week x 11 years service for when she was aged under 41 = 11 weeks.
  - Step three: 6 weeks + 11 weeks = 17 weeks x £475 = £8,075 statutory redundancy pay.

- You should check each employee's employment contract to see if the employer offers a more generous redundancy package.

- **Taxation**: Redundancy pay under £30,000 is not taxable. More information on whether elements of the payment, such as pay in lieu of notice (PILON), is taxable is available from HM Revenue and Customs. Pay in lieu of notice is money paid to the employee by the employer as an alternative to being asked to work their full notice period.

- **Offers of alternative employment**: When making employees redundant, you should, wherever possible, look to offering them "suitable alternative employment" within the organisation or at an associated company. Whether an offer of suitable alternative employment is, indeed, suitable, depends of a number of factors, such as:

  - How close the work is to the employee's current role;
  - The terms of the job being offered;
  - The employee's skills, abilities and circumstances in relation to the job;
  - The pay (including benefits), status, hours, and location of the job.

- Details of what the new role entails must be made available to the employee. The employee must also be made aware of any vacancies within the company during the redundancy period.

- If the employer has suitable alternative employment available, but does not offer it to the employee, then the redundancy could be considered to amount to an unfair dismissal. If an employee has been selected for redundancy during her period of maternity leave, an employer has a duty to offer any suitable alternative employment to that employee, without requiring her to apply for that position. Failure to offer suitable alternative employment, if available, to an employee on maternity leave could amount to discrimination.

- Where, after due consideration of these alternatives to redundancy, management considers that the need for redundancies still remains, management will give to employee representatives written details of their proposals. A provisional selection for redundancy will then be made by the Registered Provider in consultation with the Registered Manager based on the above criteria.

- Management will make every effort to seek alternative employment within the organisation and, where this exists, to consider redundant employees for suitable vacancies. All redundant employees will be interviewed individually by the Registered Provider to inform them of any available vacancies, to establish individual requirements, and to consider employees suitability for particular jobs. In the course of individual consultation, employees will also be informed of any entitlements they may have to redundancy compensation in accordance with the statutory redundancy payments scheme.

- Where alternative vacancies are offered to redundant employees, the terms and conditions which will apply:

  - Will be in accordance with the standard terms for the job in question, or;
  - Will be in accordance with those which applied to the employee in the redundant job.
  - Redundant employees who accept alternative employment with the organisation will be entitled to a trial period in accordance with prevailing legislation.

- **Trial Periods**: If you offer suitable alternative employment, then the employee has the right to a four-week trial period. The aim is for the employee to discover if the new employment is, indeed, suitable, and for the employer to likewise decide if the employee is suitable for the position. If an offer of suitable alternative employment has been made and the employee unreasonably refuses it, then the employee may lose their entitlement to statutory redundancy pay. Conversely, should an

employee accept it, but then decide that the role is not suitable for them during the trial period, then they will continue to qualify for statutory redundancy pay.

- **Time Off for Job Hunting**: If an employee has been employed continuously for at least 2 years by the date that the notice of dismissal by redundancy expires, then they are allowed a reasonable amount of time off to look for alternative employment or to arrange training to help them look for another job. Reasonable is defined by the circumstances, but the employer is only obliged to pay the employee for up to two fifths of a week's pay for any time taken off, so if an employee who works a standard 5 day week takes 4 days off to look for employment, then the employer need only pay them for the first 2 days.

Reviewed by Marcus Kerridge McColl
December 2025

Restraint Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To manage Service Users who may harm themselves or maybe vulnerable to harm and for whom restraint may be a Care option.

- To give guidance to staff in relation to legal protection offered by Mental Capacity Act 2005.

Scope

- All Service Users.

Policy

- Respecting people's rights to dignity, freedom and respect underpin good quality social Care. People may need support in managing their care and making decisions but they have the right to make choices about their lives and to take risks. Decisions regarding restraint need to be taken as part of the process of managing risk.

- People using Careservices are free to do what they want, and go where they want unless limited by law.

- Where people in Careservices have capacity, restraint may only take place with their consent or in an emergency to prevent harm to themselves or others, or to prevent a crime being committed.

- The following guidelines should be adhered to when it can be demonstrated that, for an individual in particular circumstances, not being restrained would conflict with the duty of Care of the service and that the outcome for the individual would be harm to themselves or to others otherwise restraint may not be used and any acts of restraint could be unlawful.

- Staff will receive basic training and certification on restraint techniques, which will be re-certified on an annual basis.

DEFINITIONS

- Restraint is the act of restraining a person's liberty, preventing them from doing something they wish to do. The definition included within *Showing restraint: challenging the use of restraint in care homes* (Counsel and Care UK, 2002), restraint is defined as 'the intentional restriction of a person's voluntary movement or behaviour.'

- The Mental Capacity Act 2005 describes restraint as the use or threat of force to help do an act which the person resists, or the restriction of the person's liberty of movement, whether or not they resist. Restraint may only be used where it is necessary to protect the person from harm and is proportionate to the risk of harm. This policy should be read in conjunction with Mental Capacity Act 2005 Policy and Procedure.

- Restraint that amounts to a deprivation of liberty contravenes Article 5 of the Human Rights Act 1998 and would only be lawful where a Deprivation of Liberty Safeguards authorisation had been sought and obtained. Therefore anyone who applies any form of restraint must be prepared to justify the restraint. This policy should be read in conjunction with Deprivation of Liberty Safeguards Policy and Procedure

- The Commission for Social Care Inspection (now part of Care Quality Commission) offers descriptions of the following types of restraint (*Rights, risks and restraints: An exploration into the use of restraint in the care of older people* 2007) These definitions should guide you to thinking whether any of the interventions you use could be considered a constraint, whether any of their use could be avoided, whether there is a less restrictive alternative, or whether the continued use of one or more of these constitutes a deprivation of the person's liberty:

- **Physical restraint:**

  - **Physical restraint** can be defined as stopping an individual's movement by the use of equipment that is not specifically designed for that purpose. This could be through the use of bed restraints, belts, tables or chairs etc.
  - **Mechanical restraint** is the use of belts, arm cuffs, splints or helmets to limit movement to prevent self-injurious behaviour (SIB) or harm to others.
  - **Physical intervention** is direct action by one or more members of staff holding or moving the person, or blocking their movement to stop them going where they wish. This should not be confused with interventions such as guiding and prompting that are intended to support the person.

- **Environmental restraint** is designing the environment to limit people's ability to move as they might wish. This could be through locking doors, using coded electronic keypads, complicated door handles, narrow doorways, not providing corridor rails, steps or stairs, poor lighting or heating etc.

- **Chemical restraint** is the use of drugs and prescriptions to change or moderate people's behaviour. This is also known as covert medication.

- **Forced care** is the act of 'forcing' someone to receive Care. This could be food, medication, clothing etc.

- **Threatening or verbal intimidation** could be used to make a person subservient or scared of doing what they want to do. It may also be acts calculated to lead people to believe that they have no option but to remain in a particular Care setting, or make them fear repercussions should they choose to resist or leave.

- **Electronic surveillance** – examples include the use of electronic tags, exit alarms, CCTV and pressure pads to monitor or restrict movement.

- **Cultural restraint** can be the result of constantly telling people not to do something, or that doing what they want to do is not allowed, is illegal, or is too dangerous. It could also include being got up or put to bed at unwanted times, or having meals at a time to suit the staff. It could also be seclusion in bedrooms because of their behaviour resulting in deprivation of activities and other stimulation.

- **Medical restraint** is the fixing of medical interventions such as catheters in order to deliberately restrict movement or being positioned to prevent their removal.

- Section 5 of the Mental Capacity Act 2005 offers protection for staff against civil or criminal actions, where decisions about care and treatment are made in the best interest of someone who lacks the capacity to make those decisions themselves. In addition there is similar legal protection for staff, who in the process of making best interests decisions, restrain someone, as long as two further conditions are met, namely,

  - It is reasonable to believe that it is necessary to restrain the person to prevent harm to them; and
  - Any restraint is a proportionate response to the likelihood of the person suffering harm and the seriousness of that harm.

- This policy should be read and applied in conjunction with Mental Capacity Act 2005 Policy and Procedure. In addition the following good practice should be borne in mind in the context of restraint:

  - Anyone can make decisions on behalf of someone who lacks the capacity to do so. You should consider who is the decision maker in each context, for example: *A Care assistant can legitimately decide what clothes a person should wear if they do not have the capacity to make the decision themselves.*
  - The more complicated or serious the implications regarding the capacity issue, the more safeguards there should be put in place to prevent abuse of the decision-making process.
  - The fact that someone does not have capacity does not mean that restraint or other practices to limit a person's freedom can be freely used.
  - If someone does not have capacity then the Mental Capacity Act (and its Code of Practice) defines a clear process that Care services should follow in order to assess and record decisions that are being made on a person's behalf.

- If it has been agreed that someone lacks capacity then the decisions made on their behalf must be clearly defined. This is because it is important that services do not assume someone lacks capacity in all situations as this could result in people being unnecessarily restrained.

Procedure

- Restraint, in all cases, should very much be seen as the 'last resort', with other techniques and strategies always being employed before restraint is considered as an option.

- Staff should consider what is acceptable restraint, unacceptable restraint (where less restrictive alternatives are not considered) and unintentional (where staff do not realize their interventions constitute restraint).

- The Registered Manager, in consultation with staff members, must assess and record in the Service User Care Plan:

  - The problem behaviour which is causing restraint to be considered – does it affect any other persons;
  - Evidence of consent of a person to their own restraint, where they have the capacity to make the decision;
  - Note – a person who does not have capacity (see notes regarding the Mental Capacity Act 2005) cannot give consent.

- Restraint must be time limited and noted as such.

- Families must be aware of the restraint and in full agreement, and that agreement noted. The family consent does not replace the requirement for the restraint subject's consent to their own restraint.

- Restraint must be reviewed as per the stated time limit and certainly at no greater than weekly intervals. The review must be noted.

- Pre-printed "Restraint Register" forms are available from the office and must be used to ensure compliance with this policy.

- All "LIVE" restraint sheets must be kept in the register, and be available for inspection.

- Sheets no longer in use must be filed in the Service User's individual wallet file accompanying the Service User Care Plan.

Reviewed by Marcus Kerridge McColl
November 2025

Retirement Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with current legislation on retirement in the UK.

Scope

- All employees.

Policy

- The company currently has no fixed retirement age, although this will be reviewed from time to time to reflect our business needs. We acknowledge that retirement is a matter of choice for individuals, and will not pressurise employees into resigning because they have reached or are approaching a certain age.

- Employees are free to retire whenever they choose, or to seek alternative roles or working patterns. For further information, see our Flexible Working Policy and Procedure.

- We are proud to employ people of all ages and consider that age diversity is beneficial to the organisation. We are committed to not discriminating against employees because of age and adhere to the principles set out in our Equality and Diversity Policy and Procedure.

Purpose of this policy

- This policy aims to create a framework for workplace discussions, enabling you to express your preferences and expectations with regard to retirement and enabling us to plan for our business.

- This policy does not form part of your contract of employment, and we may amend it from time to time as we consider appropriate.

Discussing your future plans

- You or your manager may want to discuss your short, medium and long-term plans, as the need arises. For example, a promotion opportunity may arise, or, if your circumstances change, you may want a different working pattern or to stop work altogether. We need to plan for the business, and so may indicate to staff from time to time that it would be helpful to know what their plans are. There is no obligation for us or you to hold workplace discussions about your future plans, but it may be mutually beneficial to do so.

- We will not make generalised assumptions that performance will decline with age, whether due to competence or health issues. If we think there are problems with your performance or ill-health, these will be dealt with in the usual way, through the Capability Policy and Procedure or Sickness Absence Policy and Procedure.

- If a workplace discussion takes place we will aim to make it as informal as possible.

- During any workplace discussion:

  - We will not assume that you want to retire just because you are approaching a certain age, such as state pension age, and;
  - We will not make discriminatory comments, suggesting that you should move on due to age.

- If you indicate that you are thinking of retiring, you are free to change your mind at any time until you have actually given notice to terminate your employment.

- Your employment or promotion prospects will not be prejudiced because you have expressed an interest in retiring or changing work patterns.

- If you express an interest in moving to a more flexible working pattern or changing role, we will confirm that this is what you want before any action is taken which could affect your employment, such as a change to your role or responsibilities.

Giving notice of retirement

- If you have decided to retire, we would appreciate as much notice as possible, although you should give the company at least the notice you are obliged to give under your contract of employment.

Reviewed by Marcus Kerridge McColl
December 2025

Risk Taking Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To support the independent ability of Service Users to take responsible risks.

Scope

- All Service Users.

Policy

- Staff will be trained and coached to understand that there is a delicate and difficult balance between Service Users' self-determination in risk-taking, and the responsibility of carers to protect Service Users from self-harm or from unintentionally harming others. This approach can be defined as positive risk taking, that is weighing up the potential benefits on the one hand, and the potential harm on another, of choosing one action instead of another.

- Some Service Users may lack the mental capacity to make decisions involving balancing risk issues. Guidance on making decisions in people's best interests can be found the Mental Capacity Act 2005 Policy and Procedure.

- Service Users will be provided with information about opportunities available to them, and the risks attendant on those opportunities, and will be supported to take an informed decision and manage the risks wherever possible.

- Before commencement of the service, assessments will be carried out which will seek to identify opportunities and risks with respect to the individual Service Users, and examine and identify risk management strategies in full consultation and agreement with the Service User.

- The risk management strategies will be recorded in the Service User's Care Plan, will form a consistent part of the overall service, and will be re-examined regularly. By this process of integration the views of the Service User, principal carer, family members and professional advisors will be fully taken into account, as part of the participative Service User Planning process.

- Attention is drawn to the Missing Client Policy and Procedure.

Procedure

- The following checklist of issues forms the basis of the risk management assessment. Each of these may be a feature of the Service User's daily living routine, and there may be others that you may wish to add to this list. Each issue will be discussed in detail and those discussions will be recorded and will determine the way in which the organisation aims to undertake to meet the Service Users own wishes about their lifestyle, and areas of interest to them:

  - Privacy and use of rooms and bedrooms.
  - Going out alone.
  - Visitors, and visiting outside the home.
  - Attendance at clubs and centres.
  - Going to church and other external activities.
  - Engaging in leisure and recreational pursuits.
  - Carrying identification or contact details in an emergency situation.
  - Access to certain areas in the home, such as the kitchen at certain times.
  - Bathing.
  - Other issues involving a degree of independence.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To ensure that abuse of Service User rights is avoided.
- To comply with the Disclosure Barring Service (DBS) requirements.
- To comply with Regulation 18 of the CQC (Registration) Regulations 2009.
- To comply with the Protection of Freedoms Act 2012.
- To comply with the Mental Capacity Act 2005.
- To comply with the Safeguarding Vulnerable Groups Act 2006.
- To comply with and ensure awareness of safeguarding powers and the safeguarding requirements of the Care Act 2014, and the Care and Support Statutory Guidance that accompanies this. These requirements replace the 'No Secrets' guidance issued in 2000.

Scope

- All workers.
- **Important note -** If your Local Authority area publishes a local set of guidelines on safeguarding arrangements, those local guidelines must take precedence over this policy. You should obtain a copy of the local guidelines and attach a copy to this policy in the manual. The Care and Support Guidance that accompanies the Care Act 2014 requires local authorities to establish adult safeguarding arrangements.
- Where reference is made to the Disclosure Barring Service reporting processes and requirements, be aware that the following policy and procedure is generic, and intended to set out general principles and courses of action where no local guidelines apply.
- The contact details for your local Safeguarding Adults Team is as follows:

0300 555 1386

adult.services@hants.gov.uk

Out of Hours - 0300 555 1373

- You should contact this number if you need to report any actual or suspected case of abuse.

Policy for the prevention of abuse:

- Abuse is a violation of an individual's human and civil rights by any other person or persons.
- The main range of different forms of abuse are described in the statutory guidance accompanying the Care Act 2014 as:
  - Physical abuse;
  - Domestic violence;
  - Sexual abuse;
  - Psychological abuse;
  - Financial or material abuse;
  - Modern slavery;
  - Discriminatory abuse;
  - Organisational abuse;
  - Neglect and acts of omission;
  - Self-neglect.

- The organisation will ensure that Service Users are safeguarded from abuse in all these forms. This duty to safeguard adults applies to Service Users who:

  - Have needs for care and support;
  - Are experiencing, or at risk of, abuse or neglect, and;
  - As a result of those care and support needs are unable to protect themselves from either the risk of, or the experience of abuse or neglect.
  - The following six principles as set out in guidance to the Care Act 2014 should inform practice with Service Users. These are as follows:
  - **Empowerment** – People being supported and encouraged to make their own decisions and informed consent.
  - **Prevention** – It is better to take action before harm occurs.
  - **Proportionality** – The least intrusive response appropriate to the risk presented.
  - **Protection** – Support and representation for those in greatest need.
  - **Partnership** – Local solutions through services working with their communities. Communities have a part to play in preventing, detecting and reporting neglect and abuse.
  - **Accountability** – Accountability and transparency in delivering safeguarding.

- The organisation will seek to ensure that Service Users are not subject to any forms of abuse. The organisation will promote the well-being of its Service Users and a safe and supportive environment for its Service Users. It will ensure this through active care planning that involves the Service User and their family and carers. That care planning process should include the management of risk that is reviewed on a regular basis. This policy should be read in conjunction with QCS assessment and care planning policies.

- In addition, there are a number of other QCS policies and procedures that should be read in conjunction with this policy, and these are listed towards the end of this document. There are a number of other documents listed at the end of this policy which should be read in conjunction with this policy, in particular the Care and Support Statutory Guidance issued under the Care Act 2014.

REFERRAL PROCEDURE

- The first priority should always be to ensure the safety and protection of vulnerable adults and if medical attention is required this must be sought immediately.
- This procedure should be used in conjunction with the decision making tree in Diagram 1b contained in Chapter 14 of the Care and Support Statutory Guidance issued under the Care Act 2014 which refers to the duty to investigate suspicions of abuse.

Report:

- Staff should report suspicion or evidence of abuse to their Registered Manager, who in turn will report to the Adult Social Care Safeguarding Adults Team.

- If staff suspect or have evidence the Registered Manager is involved in abuse, they should report directly to the Safeguarding Adults Team. They should also go directly to the Safeguarding Adults Team if there is an obvious failure by management to respond appropriately to suspicion or evidence of abuse.

- It is the responsibility of everyone to act on suspicion or evidence of abuse or neglect (see Public Interest Disclosure Act 1998 and in-house procedures) and refer to the local Safeguarding Adults Team.

- If after referral to the Safeguarding Adults Team, you do not feel the concerns have been adequately addressed, you may wish to report to the Care Quality Commission (CQC).

Lead Responsibility:

- The local Safeguarding Adults Team will take the lead and be responsible for managing the process by establishing the facts of the case, identify those that need to be involved and coordinate the response.

Consult with the Police:

- When complaints about alleged abuse suggest that a criminal offence may have been committed it is imperative that reference should be made to the police as a matter of urgency. The CQC must also be notified. Early referral or consultation with the police will enable them to establish whether a criminal act has been committed and this will give them the opportunity of determining if, and at what stage they need to become involved. Early involvement of the police will help

ensure that forensic evidence is not lost or contaminated and this may prevent the abused adult being interviewed unnecessarily on subsequent occasions. Notification of the Police may be done either by the referring individual/agency or by the local Adult Social Care Area Team. This will be dependent upon the information received at the initial point of concern and following any subsequent preliminary enquiry, and at what point the lead agency was alerted. To prevent any possibility of failure to alert the police at the proper time the person/agency making the referral should identify whether or not the police have been informed. When Adult Social Care receive the initial referral they will identify and record whether or not the police have been informed. Constabularies have Adult Protection officers whose role it is to investigate allegations of the abuse of Vulnerable Adults where the person responsible is a family member or in a position of Care. Where the person responsible is not a family member or Support Worker or Carer or where the reported incident is one of financial abuse, then the matter will be investigated by local officers. Criminal investigation by the police takes priority over all other lines of enquiry; however, police investigations may proceed alongside those dealing with health and social care issues.

Notes:

- In some areas the Police have determined that Providers must carry out an initial investigation before referral, in order to avoid the Police becoming involved in large numbers of investigations of reports which prove to be groundless. Check your local policy.

- In some areas, the Police demand that their investigation takes precedence over internal disciplinary procedures of the employer. However, the employer is entitled, indeed obliged, to take such action as may be necessary to safeguard the wellbeing of Service Users, wherever they may be being cared for. This may include dismissal and referral to appropriate agencies for banning from working with vulnerable people. It is unlikely that this responsibility is discharged simply by referral of the case to the Police.

Inform relevant inspectorate:

- By regulatory law service providers must notify the Care Quality Commission (CQC) without delay, incidents of abuse and allegations of abuse, as well as any incident which is reported to or investigated by the police.

- Service providers must notify the CQC about abuse or alleged abuse involving a person(s) using the service, whether the person(s) are the victim(s), the abuser(s), or both.

- Service providers must also alert the relevant local safeguarding authority when notification is made to the CQC

- The forms are available on the CQC website at http://www.cqc.org.uk/.

- The execution of the statutory responsibilities of the CQC relies on timely and considered intervention by their officers. It is therefore essential that enquiries into allegations involving residential, nursing or domiciliary Care services are undertaken in collaboration with the appropriate inspectorate. The CQC must satisfy themselves that the local Adult Social Care Safeguarding Adults Team is aware of incidents or allegations of abuse which come to the attention of the CQC from other sources.

Establish Working Arrangements:

- The enquiry must take account of other agencies and identify those who need to be involved. Where there is a joint interest or responsibility an early agreement regarding lead responsibility and on working arrangements must be clarified. Where agreement cannot be reached or where difficulties are being experienced, the Adult Social Care Team should be asked to obtain clarification at a senior level with the agencies involved.

Involve the Alleged Victim:

- The process of the enquiries should be carefully explained to the allegedly abused person and their consent to proceed with the enquiry obtained if possible. Arrangements should be made to have a relative, friend or independent advocate present if the person so desires. The relative, friend or independent advocate should not be a person suspected of being in any way involved or implicated in the abuse.

- A review of a Service Users Care Plan should be undertaken to ensure that they are properly supported following the alleged abuse incident, and they should be supported by the service to take part in the safeguarding process to the extent to which they want or are able to do, or to which the process allows, and they are kept informed of progress.

Contact Advocacy Services:

- The local authority must arrange, where appropriate, for an independent advocate to represent and support an adult who is the subject of a safeguarding enquiry or Safeguarding Adult Review where the adult has 'substantial difficulty' in being involved in contributing to the process and where there is no other appropriate adult to assist. The organisation shall use the QCS Advocacy Policy and Procedure, and the Complaints section of the Service User's Handbook for further information about advocacy contacts. And how to involve advocates.

Bring in Specialist Skills:

- Consideration must be given to enlisting the services or advice or personnel with specific skills or knowledge, particularly where people involved have limited communication skills, or where English is not their first language.

Coordinate:

- The Adult Social Care Adults Safeguarding Team, as the lead agency, coordinating the response, must notify other agencies and identify those who need to be involved and ensure the following processes are addressed:

  - Investigation of the incident using the guidance;
  - Action to ensure immediate safety of the alleged victim;
  - Early involvement of key agencies through a strategy meeting or discussion (by telephone if appropriate);
  - Agreement with other agencies who should take the lead in the investigation;
  - Assessment and Care Planning for the vulnerable person who has been abused;
  - Action with regard to criminal proceedings;
  - Action by employers, such as, suspension, disciplinary proceedings, use of complaints and grievance procedures and action to remove the perpetrator from the professional register;
  - Arrangements for treatment or Care of the abuser, if appropriate;
  - Consideration of implications relating to regulation, inspection and contract monitoring;
  - Appropriate measures to reassure and support carers and in keeping them informed;
  - Development, implementation and monitoring of a Care Plan;
  - Maintain appropriate records.

Investigation:

- For a variety of reasons, agencies other than the Police may need to conduct investigations into incidents of alleged abuse. This is because absence of (or insufficient evidence to substantiate) criminal activity cannot be taken as confirmation that abuse has not taken place. Investigation is a process that focuses on gathering "good evidence" that can be used as a basis for the decision whether or not abuse has occurred. It must be a rigorous process and the evidence must be capable of withstanding close scrutiny, as it may later be required for formal proceedings. Such proceedings may be against the perpetrator of the abuse or against other individuals or organisations in positions of responsibility and accountability. In these circumstances any decision made on the facts that arise from the investigation is made on the civil standards of proof i.e. on the balance of probabilities.

Case Conference:

- Following the investigation or if deemed necessary at any time during the process; it may be necessary to call a case conference involving all relevant agencies and parties. One of the purposes of the case conference would be to make multi agency decisions about future action to address the needs of the individual. An agency involved in the case may ask for a case conference to be held, although the final decision to call a conference rests with the Adult Social Care Safeguarding Adults Team Manager.

Confidentiality:

- Reporting incidences of alleged abuse may involve disclosing confidential personal information. Organisations and their workers will need to make reference to their organisation's information sharing policies to ensure they are aware in what circumstances information sharing with other agencies may be appropriate.

- The Data Protection Act makes important requirements about how information about individuals is stored 'processed' and shared, appropriate measure must be taken by the service to protect personal data. Notifications about individuals must not include their name or other details that a third party could use to identify them. Advice is to use codes instead of names, even where codes are used, they should not easily identify an individual such as room number or date of birth. A record of agreed codes should be established and kept secure, should the CQC require more information regarding an incident.

- The Care Act 2014 requires information to be shared with Safeguarding Adults Boards where they request it from any individual or organisation.

Feedback:

- The accepted good practice concerning recording, minuting and circulation will be observed and case conference chair must ensure that feedback is given to the referring organisation and family as appropriate.

Policy for preventing persons barred from working in the Care sector from being offered employment as a Care worker:

- Definition of a Care worker, from the CQC publication "Scope of Registration"
  - The definition of personal Care is broader than that used in previous registration systems. It covers:
    - (a) Physical assistance given to a person in connection with:
      (i) Eating or drinking (including the administration of parenteral nutrition);
      (ii) Toileting (including in relation to menstruation);
      (iii) Washing or bathing;
      (iv) Dressing;
      (v) Oral care;
      (vi) The care of skin, hair and nails (with the exception of nail care provided by a chiropodist or podiatrist).
    - (b) The prompting and supervision of a person to do any of the types of personal Care listed above, where that person is unable to make a decision for themselves in relation to performing such an activity without such prompting and supervision. “Prompting and supervision” means that services whereby staff prompt and directly supervise a person when they are carrying out the above actions (i)- (vi). Supervision will normally include direct observation of the action as it is carried out or otherwise checking on how it was carried out, but will not normally include merely encouraging someone to perform the activity, or checking at some point afterwards on whether it has been done.

- The flow chart attached to this policy guides employers through the steps required to meet regulatory and best practice requirement of safe and responsible employee recruitment. While this policy is mainly concerned with the definitions of abuse, and the actions which should be taken when abuse is suspected or discovered, the policy and the flow chart in particular are also concerned with the wider issue of ensuring that only appropriately recruited and checked employees come into contact with vulnerable adults. The required actions are embedded in the recruitment system (Recruitment Pack - Induction and Job Start) and must be followed for the recruitment of any Care worker, in order to prevent a person included on the DBS register of individuals barred from gaining employment in a proscribed position.

- The CQC will be notified immediately of any persons who apply for a post as a Care worker who are discovered to be on the DBS Vetting and Barring scheme register.

- Employees who are proposed to be transferred from non-care positions to Care will be DBS Vetting and Barring scheme system-checked before the transfer is confirmed, and may not be transferred if the DBS check is positive.

- All the provisions which apply to employees also apply to volunteers.

Procedure
Procedure for preventing persons barred from working in the Care sector from being offered employment:
Background:

- The DBS Vetting and Barring scheme is designed to identify those people who are considered as ‘unsuitable’ to work with vulnerable adults and are therefore prevented from obtaining employment in such positions. A responsibility is placed upon employers to identify such people and notify the DBS. They will also be required to check the DBS Vetting and Barring scheme list before offering employment to any new staff who are likely to have regular contact with vulnerable adults. This scheme is managed by the Independent Safeguarding Authority.

Checking the list:

- There is a statutory requirement on providers who are registered for a regulated activity including Care Home Services with or without nursing, Domiciliary Care services, Shared Lives (formally known as Adult Placement but not day services) to check the DBS Vetting and Barring scheme list before employing a new member of staff who is likely to have regular contact with a vulnerable adult, or in cases whereby an existing member of staff moves into such a position. The only way of making this check will be as part of a Disclosure Service application.

- Employees may not be offered a post before a satisfactory DBS Vetting and Barring scheme check has been received.

- The DBS Adult First check may only be used in circumstances whereby recruitment is required in order to meet statutory levels of staffing.

Referral to the list:

- There is a statutory requirement for providers of Care to refer workers to the DBS for inclusion on the DBS Vetting and Barring scheme list if they consider that the person is guilty of misconduct such that a vulnerable adult was harmed or placed at risk of harm. This requirement covers both existing employees and those who leave their employment, and whose conduct comes to light at a later date.

- A referrer will need to provide full details of the individual concerned, the nature of the allegation, any investigation which has taken place and any action taken as a result. Referral forms are available on the DBS web site.

- An individual who is included on the DBS Vetting and Barring scheme list as a confirmed (but not provisional) listing would be guilty of a criminal offence if they applied for a position as a Care worker. If a provider discovered that a person they offered employment to was on the list, they must cease to employ that person.

- It will be the duty of the employer to consider when a member of staff is suspended pending an allegation, whether they should be referred to the list (this is not a requirement at this stage). There will be a need to weigh up evidence. In some cases there may be a narrow tightrope between duty of Care to a member of staff and duty of Care to a vulnerable person. Best practices assert that a vulnerable person always has to be safeguarded first. If you do refer and subsequently find no evidence, the person's name will immediately be removed. The details of the referral will be passed on to the person concerned, so you should not pass on any information to the DBS Vetting and Barring scheme which cannot be shared with the person concerned.

- Employers should note that the above advice is challenged by the Chartered Institute of Personnel and Development – the professional body for HR and Personnel practitioners. The CIPD points out that referral of a person to the DBS Vetting and Barring scheme register may breach the legally defined Human Rights of the employee, when the allegation that led to the referral is later found to be incorrect or insufficient to support the employee's inclusion on the register. It will almost certainly constitute a breach of the employment contract, and is also likely to be held as libellous. This would apply even if the disciplinary procedures had been carried out to the letter. Caution and referral to a legal professional before taking any action is strongly advised.

- We would advise clients that information regarding all positions involving regular contact with vulnerable adults should include advice to prospective candidates that they will be checked against the DBS Vetting and Barring scheme list. That a declaration as to any previous incidents in relation to misconduct resulting in harm to a vulnerable adult should be included in the application form for all relevant jobs.

- Those services which regularly use agency staff will need to be particularly vigilant to check that the workers they are using have up to date checks. Although most agencies will in practice undertake this, the responsibility for ensuring that they are not 'employing' a worker without a check rests with the Registered Provider, who may delegate the day to day check to the Registered Manager.

Procedures vary between local authorities, notwithstanding the existence of a national standard. If your local authority has its own procedure, this must be appended to this policy.

Only if there are no local guidelines, use the following: Š

- **Inform the Disclosure and Barring Service (DBS)**

  - Do not assume that referral through to the Police or Safeguarding Adults Team will lead to a referral to the DBS within an acceptable time, or at all. If an employer comes to the conclusion, based on the evidence available to them, that a referral of an employee to the DBS is required, then they must immediately do so.

- Make the referral as soon as the evidence available to you, as the employer, indicates that a referral is required under DBS guidance (see below).

- Suspension pending enquiry may not trigger referral because of presumption of innocence until all evidence is reviewed and a decision as to culpability arrived at. Suspension following a hearing at which culpability has been determined, pending a disciplinary award, may be appropriate if the offence falls within the DBS remit.

- Further information about referral to the DBS, including referral factsheets and appropriate forms is available at: https://www.gov.uk/government/collections/dbs-referrals-guidance--2

- The DBS referral flowchart is attached to this document for information, but must be supplemented by a current check on the DBS website as above.

- **Procedure in the event of allegations of misconduct by staff:**

  - Allegations of misconduct from any source will be taken seriously by the organisation and must be immediately reported to a senior manager for investigation. It will be fully and openly investigated by the organisation. The objectives of the investigation will be to:

    - Establish facts;
    - Assess the needs of the vulnerable adult for protection, support and redress;
    - Make decisions regarding what follow-up actions should be taken with respect to the perpetrator and the service or its management if they have been culpable, ineffective or negligent.

  - Allegations of misconduct against an individual will normally result in the immediate suspension of that person from duty, pending an investigation of the allegations.

  - On the suspension of an individual, the Registered Manager will carry out a thorough internal investigation. On completion of the investigation a decision will be made by the Registered Provider as to whether it is appropriate to report the matter to the appropriate authority. An appropriate authority may be the Multi- Disciplinary Safeguarding Adults Team, who should be contacted via Adult Social Care.

  - Allegations of misconduct resulting in actual or potential harm to a Service User will be notified to the CQC within 48 hours, if substantiated by the investigation.

- **General procedures for the prevention of abuse during employment**

  - The Whistleblowing Policy will be drawn to the attention of all employees during induction.

  - Managers will ensure that they have a clear understanding of the chapter on Safeguarding in the Care and Support Statutory Guidance issued under the Care Act 2014.

  - During induction training, all employees will complete the “Understanding Abuse” workbook, and have an opportunity to read the chapter on Safeguarding in the Care and Support Statutory Guidance issued under the Care Act 2014.

  - The person responsible for training new employees will verify that they have a clear understanding of the possible forms of abuse or neglect based on the Care and Support Statutory Guidance issued under the Care Act 2014, their responsibility to prevent and report such abuse or neglect, and the avenues open to them for reporting and seeking further guidance.

  - Allegations of abuse will be dealt with according to the procedures set out in the chapter on Safeguarding in the Care and Support Statutory Guidance issued under the Care Act 2014.

  - Staff should receive training on the Mental Capacity Act 2005 and in particular being aware of any potential deprivation of a person’s liberty, and how that might be prevented and if not how that can be properly authorised.

  - The Service Users’ Finances Policy and Procedure will be made available to all employees during induction.

  - All Service Users will receive a copy of the Service Users Handbook.

  - Refer to the organisation’s Complaints Policy and Procedure, Whistle-blowing Policy and Procedure and for detailed procedures if allegations of abuse are made.

- Formal supervisions will take place at least every two months, and the results recorded (see Employee Supervision Policy and Procedure). The supervision format will ensure that employees are asked, at every instance of supervision, whether they have received a criminal conviction or warning, or notice of referral to the DBS register, which has not yet been declared to the employer.

- Complete the DBS Referral Form, which can be downloaded from https://www.gov.uk/government/publications/dbs-referrals-form-and-guidance

Reviewed by Marcus Kerridge McColl
November 2025

Serious Incident Notification Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards.

Scope

- All workers, Service Users and visitors to the establishment, and all employees working in the community during working hours on the organisation's business.

Policy

- Serious accidents will be reported to the CQC within 24 hours of occurrence.

- Any current funding contracts will be checked so as to ascertain if notification of serious incidents is required with respect to them under the contract. In the event of doubt, the funding authority or authorities for any Service Users involved in, or whose wellbeing was threatened by the serious incident will be notified.

- The accident must be recorded and submitted to CQC within 7 days (refer to the Registration Authority Notification Policy and Procedure).

- A serious accident is a notifiable accident under Health and Safety legislation.

- If a serious accident or incident involves a Service User, the Duty of Candour Policy and Procedure should also be referred to and, if applicable, followed. This policy can be located in the Administration section of your QCS system.

Procedure

- Complete the appropriate notification form, copy to file, and send original to CQC.

- If appropriate, follow the process detailed in the Duty of Candour Policy and Procedure.

- If appropriate, complete form F2508 for Health and Safety Executive, copy to file, and send original to the local HSE office.

- If appropriate, notify any relevant funding authorities.

Reviewed by Marcus Kerridge McColl
November 2025

Severe Weather Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To reduce risk of disruption to care services from severe weather conditions as far as possible

Scope

- Whenever news outlets or other information indicates impending weather problems.

Policy

- The Registered Manager and Directors will maintain awareness of possible adverse weather conditions by using trusted information sources, such as the Meteorological Office (Met Office).
- On receiving information from any source that severe weather conditions may be imminent or forecast, the Met Office National Severe Weather Warning Service will be accessed to determine the threat level.
- The advice given below, reprinted from the Met Office, will be noted and appropriate action taken.
- Staff are to be reminded of the Unable to Attend Work Policy and Procedure.

Procedure

- Access the National Severe Weather Warning Service on http://www.metoffice.gov.uk/public/weather/warnings/.
- Warning will be issued in the form:

Risk level	Warning colour	Description
Very Low	Green	No severe weather
Low	Yellow	Be Aware
Medium	Amber	Be Prepared
High	Red	Take Action

The Met Office advice for each risk category is as follows:

	Very Low	Low	Medium	High
Impact and advice applying to ALL SEVERE WEATHER:	The weather is not expected to have any noticeable impacts, but there may be some minor issues e.g. when travelling some extra care may be needed on occasions and there may be some disruption to outdoor events.	**BE AWARE** and ensure you access the latest weather forecast for up to date weather information. Expect some minor delays due to slower traffic. Outdoor events may be disrupted or cancelled.	**BE PREPARED**. Take precautions where possible and ensure you access the latest weather forecast. **BE PREPARED** for some disruption to normal daily routines. Travel only if well prepared and **BE PREPARED** for longer journey times.	**TAKE** precautionary **ACTION** and remain extra vigilant. Follow orders and any advice given by authorities under all circumstances. Ensure you access the latest weather forecast. **EXPECT** significant disruption to normal daily routines. Avoid all non-essential journeys. If you must make a journey carry emergency food/ clothing/ blanket etc.
Impact and advice associated with RAIN:	Some flooding of low lying fields, recreational land and car parks but little or no disruption to travel. Wet road surfaces and possibility of ponding water leading to difficult driving conditions. Take extra care when driving in affected areas.	Localised flooding of low lying fields, recreational land and car parks. Flooding of a small number of homes and businesses. Wet road surfaces and possibility of ponding water, especially in known trouble spots. Local disruption to travel — longer journey times. Water on roads — drive according to the conditions encountered.	Some flooding of homes, businesses and transport links possible. Disruption to travel likely. Disruption to gas, electricity, water supplies and telecoms. Some evacuations may be required. Be prepared to protect yourself and your property.	Widespread flooding of property. Severe disruption to travel. Loss of gas, electricity, water supplies. Significant disruption to communities. Evacuation expected. Significant risk to life. Take action to protect yourself and follow the advice of the emergency services.
Impact and advice associated with WIND:	Debris dislodged and some branches removed. Perhaps some very limited travel disruption. Difficulties on some prone routes e.g. cross winds on exposed or high level roads.	Some branches or trees brought down. Localised travel disruption. Localised problems for high-sided vehicles on prone routes. Drive with care, especially on exposed routes. **BE AWARE** of possible debris being blown around.	More widespread tree damage & other debris, slates etc dislodged from roofs. Some minor structural damage possible. Risk of injury from flying debris. **BE PREPARED** for some travel disruption e.g. closed bridges. Potential for some localised interruptions to power.	Widespread structural damage, e.g. roofs blown off, mobile homes overturned, power lines brought down. Risk to personal safety from flying debris. Potentially widespread and/or prolonged interruptions to power. Expect widespread transport disruption e.g. due to roads blocked by fallen trees.
Impact and advice associated with FOG:	Some localised non-persistent fog affecting limited geographical areas.	More widespread, locally dense fog affecting significant areas of the country	Widespread and dense fog affecting large areas of the country including a number of major	N/A

	Take extra care when driving in affected areas. There may be some very limited impact on road transport.	but not persisting beyond 1 - 2 days. Some airports may close for short periods. Take extra care when driving in affected areas. Journeys through affected areas may take longer than usual.	airports and/or ports. The fog is persistent and may last for many days in some areas. **BE PREPARED** for some travel disruption. Take great care if driving and **BE PREPARED** for increased journey times. **BE PREPARED** for delays at affected airports and/or ferry ports.	

Impact and advice associated with SNOW:	Small amounts of snow lying on roads and pavements so some slippery road surfaces possible. Traffic may move generally slower than normal. Take extra care when walking, cycling or driving in affected areas.	More widespread snow lying on roads and pavements but road networks generally open. Care needed with only localised travel disruption. Problems mostly confined to usual prone areas. Take extra care when walking, cycling or driving in affected areas. Journeys through affected areas may take longer than usual.	Widespread snow with a number of road closures, others passable only with care. **BE PREPARED** for some disruption to road, rail and air transport with difficult driving conditions likely and longer journey times.	Widespread deep snow with many roads closed or impassable. Roads likely to become impassable with high risk of drivers becoming stranded. Significant disruption to road, rail and air transport. Risk to personal safety. Expect significant disruption to normal day to day life as a result of transport issues, school closures etc. Avoid making unnecessary journeys.
Impact and advice associated with ICE:	Localised icy stretches on some untreated roads and pavements are possible. Take extra care when walking, cycling or driving in affected areas.	More widespread icy stretches on untreated roads and pavements but road networks generally open. Take extra care when walking, cycling or driving in affected areas. Journeys through affected areas may take longer than usual.	Widespread black ice, some roads passable only with care. Possibility of road collisions and significant increase in slips & falls.	N/A

At all times staff are reminded that under the Unable to Attend Work Policy, Severe weather is not a reason for non attendance to work.

All staff should work together with Crescent Community Care Services Limited and its office staff and Supervisors to ensure that staff are both safe while delivering care to their Service Users during the severe weather. This may be that calls are prioritised to ensure that the most vulnerable are seen first and that Service users with family members living with them will receive a call at some point during the day. This will only be enacted during the most severe weather times.

Reviewed Marcus Kerridge-McColl
September 2025

Purpose

- To ensure employees can exercise their rights under The Shared Parental Leave Regulations 2014.

Scope

- All employees.

Definitions

- **"SPL"** - Shared Parental Leave.
- **"You"** - Mother/main adopter.
- **"Your partner"** - Means your partner who is the other parent/adopter of the child. It does not need to be the child's father, but the person who is in an enduring relationship with you and share primary responsibility for the upbringing of the child.

Policy

- Crescent Community Care Services Limited will comply with both the letter and spirit of the law on SPL and pay. To this end, its aim is to inform all employees of your entitlement to SPL rights and to ensure that those rights are understood by those who qualify.
- In most cases, it provides the opportunity for a mother and partner or adopters to share leave which otherwise would be taken only by the mother/main adopter.
- In many cases they will also be able to share any statutory maternity/adoption pay.
- It must not be confused with statutory parental leave, which is an additional, unpaid, entitlement covered in a separate policy.
- In this policy “you” assumes you are the mother. “Your partner” is the other person in your relationship.
- The rights also apply to those who are adopting, to same-sex relationships, and to surrogacy. Reference to your partner can apply to a woman in a same sex relationship.
- Parental leave can be shared by only two people.
- It is your choice, as the mother/main adopter, as to whether or not such leave is shared.
- SPL provides for the mother to return to work earlier, or to return for specific periods, and for the other parent to have more involvement in bringing up their child.
- This policy is intended for your general guidance. There are many complications that apply in particular circumstances, for example if you are the father (or partner) and the mother is self-employed or if you are entitled to maternity allowance rather than maternity pay. As well as providing the notification required you are advised to also consult your manager. If your particular circumstances do not appear to be covered in this policy then it is especially important that you seek advice from your manager.
- This policy and procedure covers the basic (sometimes called statutory) rights that employers must provide.

Procedure

Will you be eligible?

- Firstly you and your partner must both share the main responsibility for the care of your child at the time of its birth.
- You will also need to meet some further conditions. These include notifying us of your intention to share parental leave. The main conditions are summarised below.
- In addition, you also need to confirm a number of matters in writing. If you complete the ‘Leave notification of intention – birth/adoption’ form you will be providing the correct notification. The form contains a second part for completion by your partner. The form also contains the matters that you and your partner need to confirm. They are shown as declarations at the end of each part of the form. **A declaration is binding and false declarations could lead to disciplinary action and, conceivably, criminal proceedings.**
- Main conditions:
    - You must be/have been entitled to statutory maternity/adoption leave;
    - You have ended or given notice to reduce your maternity/adoption entitlements;
    - You are still working for us at the start of each period of SPL;
    - You have a minimum of 26 weeks' service at the end of the 15th week before your child’s expected due date;

  - You have correctly notified us of your entitlement and provide evidence as required (see notification section below).
- Your partner must satisfy the 'employment and earnings test', i.e. have:
  - Worked for at least 26 weeks in the 66 weeks leading up to the child's expected due date/placement date;
  - Earned the minimum amount required by law (£30 in 2025) per week in any 13 of those weeks.

Your rights to SPL

- In relation to a mother (not a main adopter) you must take your two weeks' compulsory maternity leave. Thereafter you and your partner can share 50 weeks' leave between you. For an adoptive couple you can share the 52 weeks' adoption leave between you.
- In sharing your leave you give clear notice that you will reduce (curtail) your period of leave by the number of weeks you wish your partner to take.
- You and your partner could take the shared leave at the same time. If you are a mother, once you have taken two weeks' leave (compulsory in your case) you could then each take 25 weeks' leave so as to be together in your child's early months. Adoptive parents can take their shared leave together at the outset of the placement.
- Provided it is taken in complete weeks there may be a lot of flexibility available in how you share the leave, but it must end no later than one year after the birth/placement of your child.

You can have different periods of leave

- You apply to have periods of leave that are either continuous or discontinuous.
- A continuous period of leave is an uninterrupted number of weeks.
- A discontinuous period of leave is one which is interrupted, for example, alternate weeks. As your employer we do not have to allow discontinuous periods of leave but, if we don't, you are entitled to a meeting for us to explain our reasons. We will seek to work with you to find a suitable solution or you may take the leave as a continuous period.
- You can apply for up to three periods of leave only.
- Leave must be in whole weeks but can begin on any day of the week.

You can share the pay

- In order to be able to share the 37 weeks' (maternity) or 39 weeks' (adoption) statutory shared parental pay (ShPP) there are further conditions to be met.
- The conditions are that you:
  - Must be/have been entitled to statutory maternity/adoption pay and must have reduced your maternity/adoption pay period by the amount that is to be shared with your partner.
- You or the partner receiving the pay:
  - Must intend to care for the child during any week in which ShPP is payable;
  - Must have an average weekly earnings for the period of eight weeks leading up to and including the 15th week before the child's expected due date are not less than the lower earnings limit in force for national insurance contributions;
  - Must remain in continuous employment until the first week of ShPP has begun;
  - Must give proper notification as explained below.

Notification procedures

- In order to safeguard your rights, you must comply with a number of notification requirements. Please read this section carefully and see your manager if you have any questions.
  - **Notification of ending maternity leave early** - This must be given separately to the form below and needs to state the number of weeks by which you will end your leave early. It must be given at least eight weeks before the point at which your leave is to end.
    - If you give notice before the child is born or matched with the child you may be able to revoke it;
    - If you give notice after the birth or after you have been matched you cannot change it.
  - **Notification of intention** - This should be provided on the 'Leave notification of intention – birth/adoption' form and eight weeks before the first period of leave is to start. We are entitled to ask for evidence to support the information on this form. Misleading information could result in disciplinary action against you.
- **The above notifications alone are not sufficient, you must also apply for each period of leave. For an explanation of the different periods of leave available, please see earlier in this policy.**

- **Applying for a period of leave** - Applications should be made on the appropriate part or parts of the 'Leave notification of Shared Periods' form. You can make up to three applications but any subsequent request to amend a period of leave counts towards that maximum. Requests arising because the child is born early, or because we have agreed a change requested by us, do not count towards the maximum.
- **Amending a period of leave -** If you wish to amend a period of leave then you must provide eight weeks' notice before the period which you want to amend would start. You must make clear the change you want to make and provide a declaration that the change is agreed by you and your partner.
  - If you are taking a single period of continuous leave you have only until the 19th day from the date of the original notification of intention to amend the start date.
  - If you wish to return to work earlier than the final end date notified to you (by us) then eight weeks' notice is required and the request counts as one of your maximum of three requests.
- **Applying to share statutory parental pay (ShPP) -** Applications should be made on the appropriate part or parts of the 'Leave pay birth/adopter' form. Note that information and signed declarations are required on both parts of the form.

Your rights during leave

- Your employment contract continues during all periods of SPL, with the exception of pay. Therefore holiday entitlement and all rights and benefits are unaffected. You are subject to our policies and procedures such as may apply, including the disciplinary procedure.
- You should endeavour to use any holiday entitlement in the appropriate holiday year, for example, before taking maternity leave or upon return.
- We are entitled to keep in reasonable contact with you during your period(s) of leave, for example, to keep you informed, to discuss training or to make arrangements for return to work.

Shared parental leave in touch days

- In addition to reasonable levels of contact, we may be able to mutually agree up to 20 days where you return to work during leave and without affecting your right to shared parental pay (ShPP).
- We are not obliged to offer you such days, nor are you obliged to take any days offered.
- On days on which you work we will top up any ShPP to your full pay or, if no ShPP is due, pay you in full.

Returning to work

- If the total amount of leave taken by you and your partner is 26 weeks or less, you are entitled to return to the same job on the same terms and conditions. You can also take an additional period of unpaid (i.e. not SPL) parental leave of four weeks or less without any effect on your right to return.
- This right to returns also applies if you take more than 26 weeks in total (or if you take four weeks' unpaid parental leave). However, in these two cases if, exceptionally, it is not possible to provide this (for example, if the job no longer exists) you are entitled to suitable alternative employment on terms that are not substantially less favourable.
- Failure to return on the date notified to you could result in disciplinary action. If your return is prevented by sickness, our normal sickness absence reporting procedures apply.

GUIDANCE TO THE SERVICE PROVIDER

Check that the mother/main adopter (and their partner) is eligible

- Forms are provided below for employees to complete and they will assist in checking eligibility.
- There are declarations at the end of each part of the form. While it may seem open to abuse, a declaration is binding and false declarations could justify you taking disciplinary action.

You can ask for further evidence of eligibility

- You have 14 days from receipt of an SPL notification to ask for further information to confirm your employee's eligibility. You can ask for:
  - The name and business address of the partner's employer (where the employee's partner is no longer employed or is self-employed their contact details must be given instead);
  - A copy of the child's birth certificate/matching notification.
- Your employee has 14 days in which to produce this information. If they don't then you can refuse their entitlement. It is therefore important that you have evidence of when you asked for the information.

How much leave can the mother/main adopter share?

- A mother who has given birth must take her two weeks' compulsory maternity leave. Thereafter she and her partner can share 50 weeks' leave between them. Adopters can share the entirety of the 52 weeks' adoption leave.
- In sharing leave the mother or main adopter must give clear notice that they will reduce (curtail) their period of leave by the number of weeks they wish their partner to take.
- Provided it is taken in complete weeks you may need to accept a lot of flexibility in how they share the leave, but the leave must end no later than one year after the birth/placement of their child.

They can have different periods of leave

- This means they can apply to you to have periods of leave that are either continuous or discontinuous.
- A *continuous* period of leave is an uninterrupted number of weeks. You cannot refuse periods of continuous leave *and your employee can ask for three periods of continuous leave. This he or she can do by providing the maximum of three separate notifications.*
- A *discontinuous* period of leave is one which is interrupted, for example, alternate weeks. As the employer you do not have to allow discontinuous periods of leave but, if you don't, then the employee is entitled to a meeting for you to explain your reasons. You should seek to work with the employee to find a suitable solution, but if one cannot be found then the employee may take the leave as a continuous period.
- Leave must be in whole weeks but can begin on any day of the week.

The mother/main adopter and their partner can share the pay

- In order for them to be able to share the 37/39 weeks' statutory shared parental pay (ShPP) there are further conditions to be met. They must meet the conditions shown above. They must also comply with the notification requirements including the notification of ending maternity/adoption leave early. This must be given separately to the form below and needs to state the number of weeks by which the mother/main adopter will end her leave early. It must be given at least eight weeks before the point at which their leave is to end. Note:
    - If they give notice before the child is born/matched they will normally be able to revoke it;
    - If they give you notice after the birth/placement they cannot change it.

Responding to requests

- Once you have a notification from your employee then you must respond quickly. i.e. within 14 days, informing the employee in writing as to whether you will allow the request or not.
- You can only refuse requests for discontinuous leave and, if you do, the employee has the option, as indicated above, of taking shared leave in one continuous period.
- In the latter case there are further rules about when the employee can start the leave. They have until the 19th day from the date of the original notification to give a date that is different from that in the original notification. They also cannot start it sooner than eight weeks from the date of that notification.
- These details (about notifying you of the date they want leave to start) are not in the employee version of this document for the sake of brevity.
- Therefore you are advised to tell the employee of these requirements in writing in the event that you refuse discontinuous leave.

Employees' rights during leave

- Employment contracts continue during all periods of SPL, with the exception of pay. Therefore holiday entitlement and all rights and benefits are unaffected. Employees on leave are subject to your policies and procedures such as may apply, including the disciplinary procedure.
- It is worth encouraging (not obliging) employees to use any holiday entitlement in the appropriate year, for example, before taking shared leave or upon return.
- You are entitled to keep in reasonable contact with employees during their period(s) of leave, for example, to keep them informed, to discuss training or to make arrangements for their return to work.

SPL in touch days

- In addition to reasonable levels of contact you are able to mutually agree up to 20 days where the employee returns to work during leave and without affecting their right to shared parental pay (ShPP).
- You are not obliged to offer such days, nor is any employee obliged to take any days offered.
- On days on which employees do work, you should top up any ShPP to their full pay or, if no ShPP is due, pay them in full.

Returning to work

- If the total amount of leave taken by the mother/main adopter and her partner is 26 weeks or less, they are entitled to return to the same job on the same terms and conditions. They can also take an additional period of unpaid (i.e. not SPL) parental leave of four weeks or less without any effect on their right to return.
- This right to return also applies if they take more than 26 weeks in total (or if they take four weeks' unpaid parental leave). However, in these two cases if, exceptionally, it is not possible to provide this (for example if the job no longer exists) the employee is entitled to suitable alternative employment on terms that are not substantially less favourable.
- Failure of the employee to return on the date you have notified to the employee could justify disciplinary action. If the return is prevented by their sickness, then normal sickness absence reporting procedures should apply.

Reviewed Marcus Kerridge-McColl
November 2025

Sickness Absence Policy

Crescent Community Care Services Limited

Purpose

- The overall objective of this policy is to provide fairness and consistency to employees in the treatment of absence management and to ensure that, while employees feel able to take sickness absence when genuinely ill, they take a responsible attitude towards attending work regularly and reliably.

Scope

- All employees.

Policy

- It is the organisation's policy to offer security of employment during absence periods, where reasonably possible, while operating absence monitoring systems to ensure that unusual patterns of absence or high levels of absence are assessed and discussed with you.

Procedure

- **Organisation Absence Control**
  - If you are away from work due to sickness or accident you must conform to the following procedure:
    - You must telephone your manager at the earliest opportunity and no later than 30 minutes before your shift starts on the first day of absence, giving the reason for the absence and the date that you expect to return to work. You are expected to telephone personally and not to pass a message through a third party. However, if you are unable to telephone personally through illness, you may ask a relative or friend to telephone for you. Text or email is not acceptable.
    - If your absence extends longer than originally anticipated, you must contact your manager to advise him/her of the situation, unless your absence is covered by a medical certificate. If your absence becomes prolonged you must keep in regular contact with your manager, advising on your progress and when you are likely to return to work. For all absences, whatever the length, you must complete a Return to Work Questionnaire on the day of return, submit it to your line manager, and attend a face to face interview to discuss the questionnaire contents.
    - If you return to work after no more than seven calendar days of absence, you must complete a self-certification form.
    - If your absence lasts for longer than seven calendar days, you must forward medical certificates covering all days of absence at weekly intervals, in addition to the self-certificate.
- **Statutory Sickness Pay**
  - You may be entitled to statutory sick pay (SSP) if you are absent because of sickness or injury, provided you meet the criteria in the current SSP regulations. When you are absent for four or more consecutive qualifying days you will be paid SSP by us if you are eligible. This is treated like wages and is subject to normal deductions.
  - Qualifying days are the only days for which you are entitled to SSP. You will be notified of these days. The first three

qualifying days of absence are waiting days for which SSP is not payable. Where a second or subsequent period of incapacity (of four days or more) occurs within 56 days of a previous period of incapacity, waiting days are not served again.

- Where the circumstances of your incapacity are such that you receive, or are awarded, any sum by way of compensation or damages with respect to the incapacity from a third party, then any payments which we may have made to you because of the absence (including SSP) shall be repaid by you to us, up to an amount not exceeding the amount of the compensation or damages paid by the third party.

- **Returning to Work**
  - You should notify your Manager as soon as you know on which day you will be returning to work, if this differs from a date of return previously notified.
  - If you have an infectious or contagious disease or illness such as rubella or hepatitis you must not report for work without clearance from your own doctor. Separate rules relating to infectious diseases and food handlers are to be found later in this handbook and you must familiarise yourself with them.
  - On your return to work after any period of sickness/injury absence (including absence covered by a medical certificate), you are also required to complete a self-certification absence form and hand this to your Manager.
- **Sickness Absence and Performance**
  - Due to the nature of many of the jobs and staffing levels, high levels of absenteeism or repeated spells of sickness/absence cause considerable disruption. Both short-term/spasmodic and long-term sickness absence cause disruption to the services we provide and incur additional costs. They also place an undue burden upon other team members. Therefore, if your attendance record is significantly worse than those of comparable employees, or where it creates a particular operational difficulty, or it has gone on for a considerable length of time your manager will investigate and arrange a meeting to discuss the situation further with you.
  - You may be given a Return to Work or Stress Related Return to Work Questionnaire form to complete prior to the meeting.
  - Dependent upon the outcome of this meeting counselling may be appropriate or disciplinary action taken, or indeed no action taken if none is appropriate.
  - Issues of capability arising from poor levels and/or patterns of absence are distinct from capability issues involving long-term health problems or disability.
  - If you fail to comply with notification or certification procedures or otherwise abuse the organisation's rules on sickness absence you will be dealt with under the disciplinary procedure.
- **Long-term Absence**
  - Should you unfortunately become disabled or suffer from a long term health condition during employment it is our policy to make every effort to help you retain your original position. Your manager will try and balance the needs of the business with their support for you. In the event that the length of your absence causes service delivery problems, which requires either your return or replacement, your manager will determine what action is to be taken.
  - Action may include a personal interview with you, or a request to provide a medical report from your GP, or a request to undergo a medical examination by a practitioner appointed by us who is more knowledgeable about the working environment. The examination will determine your fitness to resume your normal role. Once sufficient medical advice is available, a further meeting with you will usually be arranged to discuss it.
  - All alternative employment options, including transfers, part-time working, reasonable adjustments to work premises, practices or equipment will be fully examined by us and may be discussed with you. You may be suspended from work on full pay, remain on sick pay or be transferred onto maternity leave (where the absence is a pregnancy related sickness and occurs within the four weeks prior to the expected date of birth). Absence due to pregnancy will be dealt with due regard to your statutory maternity rights.
  - If it is advised by the doctor that you are fit to return to work, we will cease to pay sick pay and you will be expected to be at work the following morning.
  - You will accrue holiday entitlement during long-term sickness absence, and you are permitted to take all or some of your accrued holiday during a period of absence.
- **Intermittent Absence**
  - Short-term spasmodic absence is particularly disruptive, as it cannot be planned for and puts more pressure on managers and colleagues.
  - Intermittent non-attendance will be dealt with in accordance with the stages of the disciplinary procedure, as

appropriate. Where the absences are unexplained, or may have an underlying sickness reason, a medical examination will normally be requested. The examination may be carried out by your own GP and/or a doctor appointed by us. Their objective is to provide us with sufficient information to more effectively balance your needs with that of the organisation, or to make a decision regarding your continued employment.

- Irrespective of the cause of the intermittent absences, you will be warned of the consequences of further failure to meet attendance targets at each stage of the procedure. If the procedure is exhausted without sufficient improvement in your level of attendance, you may have your employment terminated with due notice on capability or conduct grounds.
- You may appeal against the decision to dismiss by writing to the Registered Provider giving the reasons for the appeal. This appeal must be made within five working days.

Guidance to the Service Provider

- Where appropriate, and after initial assessment of the problem, the department manager should conduct an interview with the employee concerned who may be accompanied by a colleague. It may be helpful to give the employee one of the attached Return to Work (or Stress Related Return to Work Questionnaire – see below) forms to complete prior to the interview.
  - If the absence involves frequent and persistent short-term absence the interview will aim to:
    - Identify the frequency and reason for the absences and ensure that the employee is aware that the absence record is giving cause for concern;
    - Advise the employee to seek proper medical attention if there is an underlying medical problem. A medical report could be requested, if appropriate;
    - Inform the employee that the persistent short-term absences are unacceptable and put continued employment at risk;
    - Give consideration to the employee's personal problems and possible ways of helping the employee to resolve them;
    - Agree a reasonable period of time over which the employee's attendance can be assessed;
    - Indicate what the next step will be if the employee fails to reach the standards required. These standards or targets should be specified;
    - A letter should be drafted confirming the facts, the action to be taken and specifying what will happen if attendance is not improved. This will be handed to the employee.
    - Take note of the advice given to the employee over intermittent absence. Dismissals by reason of capability are safer where there are health issues. Dismissal for misconduct would be appropriate if the employee continually flouts reporting procedures.
  - If the issues involve long-term ill-health the interview (if appropriate) will:
    - Seek to establish the reasons for absence and its likely duration. The employee should be requested to allow the organisation to contact their doctor in order to establish the likely length of absence and the long-term effect on capacity in relation to job performance and attendance at work. The employee may be asked to see a doctor appointed by the organisation to enable a medical report to be prepared by the employer. The employee has the right to refuse unless they have previously agreed medical reports in their contract. If you receive a refusal in the latter circumstances you should seek advice;
    - Consider offering alternative employment or a shorter working week if this would enable the employee to return to work;
    - Inform the employee that long-term absence due to ill health may put their employment at risk, bearing in mind any qualifying disability recognised within the Equality Act 2010 and the needs of the organisation at that time;
    - The Equality Act does not mean that an employee with a disability cannot be dismissed fairly, but it does mean that reasonably adjustments have to be considered (and applied if appropriate) prior to any decision about dismissal;
    - Set a date at which point dismissal will be considered if the employee is still unable to return to work;
    - A letter should be drafted confirming the facts and the action to be taken. The letter should be handed to the employee where possible or sent by recorded delivery.
  - If the attendance record does not improve, or if the employee's long-term absence continues, a second interview will be arranged. At this point, unless the employer has reasonable grounds to believe that there will be an improvement in the foreseeable future you should consider dismissal. If this action is taken, the decision to dismiss the employee

by reason of capability would need to be explained to the employee.

Reveiwed Marcus Kerridge-McColl
October 2025

Smoking at Work Policy and Procedure
Crescent Community Care Services Limited

Purpose

- It is illegal to smoke in enclosed public spaces. The purpose of this Policy is to uphold the legislation and to avoid the health problems of passive smoking.

Scope

- All premises owned or operated by the organisation.

Policy

- The organisation recognises that its employees are entitled to be protected from the effects of passive smoking, and therefore the establishment is a non-smoking environment.

- Note that smoking in the context of this Policy includes the use of e-cigarettes.

Procedure

- It is strictly forbidden to smoke anywhere on the premises (except in designated areas) and as such, anyone who is found to breach this procedure will be subject to disciplinary action.

- Service Users residing within the community will be advised that smoking is not permitted whilst staff are on the premises. Crescent Community Care Services Ltd will also request that smoking is refrained from 30 minutes before the staff member is scheduled to attend. This information will be clearly communicated to the Service User prior to the service commencing. Staff must ensure this policy is adhered to and communicate and non-compliance to the administration office, where the matter can be dealt with appropriately.

- Any staff member found to be smoking on the premises of a Service Users home whilst working, may be subject to disciplinary action, even if they have gained permission from the Service User and/or homeowner.

- Crescent Community Care Services Ltd will provide employees who wish to smoke with a sheltered facility outside of the building so that they may smoke without affecting the health and safety of others, and the shelter will be fitted with a non-flammable waste bin for cigarette butts.

- This facility may only be used when not on duty, or during recognised and authorised breaks.

- Smoking will not be tolerated in any other part of the site other than the shelter provided, or at any other time than during authorised breaks or while not on duty.

- Cigarette waste will not be left in any part of the site other than the non-flammable waste bin in the designated shelter.

Reviewed by Marcus Kerridge McColl
December 2025

Smoking Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To meet the provisions of the:
    - Smoke free (premises and Enforcements) Regulations 2006
    - Smoke Free (Exemptions and Vehicles) Regulations 2007
    - Smoking, Health & Social Care Act

Scope

- The offices and anywhere that care workers are employed at Crescent Community care Services limited

Policy

- This Policy has been developed to protect all employees, service users, customers and visitors to our offices from exposure to second hand smoke and to assist with compliance to the Health Act 2006
- 2nd hand smoke increases the risk of lung cancer, heart disease and other serious illnesses. Ventilating or separating smokers and non smokers do not completely stop potentially dangerous exposure
- It is the Policy of Crescent community Care Services Limited that our office workplace is a smoke free environment
- While Service Users may smoke in their own home it is totally prohibited for a care worker to smoke in a service users home at any time
- Smoking areas in the outside garden are set aside for any member of staff that requires a cigarette while in the office workspace. It is totally prohibited in any inside workspace of the offices
- If a member of staff breaks this policy then Disciplinary Process will be undertaken against the employee.

Procedure

Building

- Staff
    - The offices are a no smoking area
    - A smoking area is set aside in an outside area for employees who smoke
    - No smoking may take place in any other part of the workplace
    - No smoking may take place in a service users home
    - Staff may not leave Service users homes to take a cigarette break without specific consent of the Registered Manager during their hours of duty

- Service Users
    - Service Users may of course smoke in their own home. However if this causes a problem for the care worker then they must report it to the Registered manager immediately where a Risk Assessment will take place on the issue of smoking

Recruitment

- During the Induction training all members of the staff will be informed of the Policy and also the consequences of this Policy and their employment and the repercussions of breaking this Policy and the subsequent disciplinary process

Reviewed Marcus Kerridge-McColl
September 2025

Social Networking Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To comply with the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014, Regulation 14 - 1, "Persons employed by the registered person must - (a) be of good character".
- To Crescent Community Care Services Limited employees on use of social media, safeguard privacy of Service Users and protect the reputation of The Agency.

Scope

- All social media including Facebook, LinkedIn, Twitter, blogs and similar interactive facilities on the Internet.
- All use of social media whatever facilities are used to access or create material (e.g. PC, Tablet, Smartphone) and whether such material is created inside (e.g. on behalf of the organisation) or outside working time.

Policy

- You are required to follow reasonable standards of behaviour in using social media so that the impact on the workplace can only be positive.
- It is important to recognise that if you can be identified as an employee of Crescent Community Care Services Limited in any postings or profile, then you must be careful that your behaviour does not bring us into disrepute or impact negatively on our organisation. This may also include any comments that are placed on the posting by other members of the public in answer to your posting. If the comments impact negatively on the Company then you may face disciplinary procedure for bringing the Company onto Disrepute and for breaking this Policy.
- Material placed on social networking sites is public and complete privacy should never be assumed whatever the settings, as information may be passed on.
- If you choose to use our systems (outside of working time only) for social networking then such communications should not be regarded as private and may be accessed.
- Should you have already posted questionable material, i.e. material which may bring into question "good character" in the context of employment caring for vulnerable people, before this policy was published, or before you were employed by us, then you are required to declare it to your manager. Failure to do so will render you liable to disciplinary action.
- In all cases, you will be required to remove any material that contravenes this policy, or which we might reasonably request. It is recognised that this will not always be achievable, but not being able to remove material is unlikely to be a mitigating factor in any disciplinary action. Such action may include dismissal. Should it prove not possible to remove material, it is recommended to set the status of the post as “me only”, or the equivalent. Media account users should however be aware that historic posts may have already been copied before being made private. For this reason it is recommended to discuss the matter with the Manager on or before recruitment, to place the information in a hopefully better context, if possible, and allow the manager assess if the engagement meets the Regulation, with the benefit of full information.
- We reserve the right to access any information placed in the public domain, given a reasonable reason for doing so. This includes information that may be relevant to selection, promotion decisions or to discipline.
- Adverse information, identified using online or social media searches, will not be used to make decisions without you first having the opportunity to respond.

Procedure

Behaving Responsibly

- Because of the Regulation requirement placed on Crescent Community Care Services Limited, be aware that judgement of the “good character” of employees is required, and that the judgement is not confined to behaviour in the workplace. Behaviour outside of time employed also contributes to a judgement of good character, in terms of behaviour in the real world and in social media.
- Do show respect to colleagues and Crescent Community Care Services Limited as a whole. Our expectations of your behaviour as a blogger or Tweeter are much the same as our expectations of you in the workplace.
- Cyber-bullying or online harassment will be taken as seriously as any other form of bullying or harassment and render you liable to disciplinary action.
- Offensive, discriminatory or derogatory remarks towards an employee, a Service User, a relative or towards us as your

employer, or a sustained campaign of critical comment towards any of the above could also lead to disciplinary action.

- Social networking may foster good team work but if you want to share photographs, videos or images depicting Service Users on the Internet, then you must read the Photographs Policy and Procedure, which requires that express permission is required from every person in a photograph before publication. The Registered Manager should be asked to review the photographs to check that the images do not in any respect compromise the security of the Service User, or depict them in a manner which shows lack of dignity and respect.
- Similarly, if you want to share photographs, videos or images depicting employees in connection with work (such as a work organised social event), or the work environment, on the Internet, then you must also request and be granted permission from the Registered Manager first, and then every individual in the photo.
- Photographs that include documents or computer screens may contravene our policies on confidentiality.
- If you want to publish commentary on events or other matters appertaining Crescent Community Care Services Limited we ask that you first check your facts with the your manager. If you are asked to correct or amend details we will require that you do so and that you make those changes transparent.
- Never disclose personal information that could put yourself, Service Users or Crescent Community Care Services Limited at risk.
- You must not publish material that may bring the Crescent Community Care Services Limited into disrepute or may reduce respect for you in the consideration of Service Users, their family members, or your colleagues.
- You must safeguard your own personal standing. If you do not do so and, in being identified as an employee of Crescent Community Care Services Limited, such failure brings us into disrepute or impacts negatively on us, then you may be subject to disciplinary action including dismissal.
- You are strongly advised not to befriend Service Users or their relatives on social media sites such as Facebook. Doing so may release information about Service Users which may compromise their safety, for instance by identifying them as vulnerable people, leaving them in turn open to exploitation. Doing so may also contravene your professional standards, such as the maintaining of professional objectivity.
- Employees who have contact with Service Users who exhibit disinhibition, or who have problems understanding social context should be particularly careful not to post information or images which may compromise their professional standing and safety.

Social Networking at Work

- If you are given specific permission to Tweet or blog or otherwise post material on our behalf, or if material can be traced to us, then you must respect copyright legislation and not engage in any form of defamation. As an employee you represent our organisation in these circumstances. Inclusion of this matter in our policy *is not permission* to post on our behalf and doing so without permission could lead to disciplinary action.
- If, with our permission, you engage in social networking in relation to your work, for example on LinkedIn, all of the safeguards and standards of behaviour in this Policy also apply.
- By default we own all the data in accounts created in connection with your work with us and you are not entitled to use such data if you leave our employment. However, we may enter into a separate agreement with you, if appropriate.
- The use of social networking is permitted during lunchtime or at the beginning or end of a shift / the use of social networking is not permitted on our premises or using our facilities at any time.
- You must not engage in social networking during work time unless it is directly related to your work. Any engagement with social networking in connection with work must not be excessive. As applies to phone calls and text, social activities outside work must not intrude into work time other than for emergencies. You must also be open and transparent with Platinum Care at Home Ltd about any networking activity undertaken during the course of your work.

Your Legal Responsibilities

- Irrespective of your obligations as an employee, you should keep in mind that you are legally responsible for commentary that you personally place online. The Internet is the ultimate public place.
- Actions that we may take notwithstanding, other parties such as Service Users or relatives of Service Users can take legal action against you for derogatory comments, breaches of data protection or copyright.

Confidentiality

- You are responsible for confidentiality and data protection as set out elsewhere in your Employee Handbook.
- There are additional clauses in your contract covering intellectual property, trade secrets and other relevant restrictive covenants, and these apply to data contained in social networking performed on our behalf, unless otherwise specified in an agreement.
- Divulging confidential, protected or restricted information on the Internet may be regarded as a disciplinary offence; breaching a covenant may make you liable by law to injunctions or claims for damages.

The Media

- Social media postings can attract attention from traditional media such as the newspapers.
- If your social networking attracts media attention, where you are identified as an employee of Crescent community Care Services Limited, then you must refer enquiries to the Registered Manager.

Guidance to the Service Provider

- Different employers take differing views on the use of social media during work time. Employees in customer services or similar roles need to know what may be being said about the service on the Internet, while for direct care staff it is not advisable for access to social media during working time to be possible, or allowed.
- It is for the service provider to decide whether the use of social networking is to be permitted during lunchtime. There may be technical risks in doing so and if you are concerned about this it is recommended that you discuss security with your IT provider.
- In some cases an absolute ban may be preferable, but where employees are permitted to bring their own devices to work (especially for work purposes) it is unlikely to be practical.
- Where social media is used on your behalf, for example in sales or marketing, it is advisable to be clear about ownership of data and contacts. Employees in these functions may be engaged for their contacts as much as for their skills, knowledge or experience. Therefore, ownership is best established on appointment and enshrined in the contract of employment or in a separate signed covenant (which endures beyond the contract of employment).
- The statements in this open policy indicate the minimum standards likely to be required of you in relation to privacy laws, data protection and recruitment practice.
- It is particularly important that adverse information, identified using online or social media searches, is not used to make decisions without the applicant or employee first having the opportunity to respond.
- If you use social media for the purpose of recruitment vetting or selection decisions your policy should be clear and open to applicants.
- Use of information gleaned from social media, the Internet, or indeed other sources, should be used only where it is relevant to a job or position and can be definitive.
- Personal data, from any source, collected during the recruitment process should not be kept beyond the period for which it is relevant.
- Social media is increasingly pervasive and controversial. If, for example, during the induction process new employees sign to say that they have understood, accept and agree that they will follow the policy, then it brings these matters to their attention in an unambiguous way.

Reviewed Marcus Kerridge-McColl

September 2025

Spot Checks Policy and Procedure

Crescent Community Care Services Limited

Purpose:

- To ensure services are delivered to the specified standard

- To afford the Service User an opportunity to raise concerns with a person other than their direct carer.

Scope

- This policy will cover all services provided by Crescent Community Care Services Ltd in the Service User's own home

Policy

- Crescent Community Care Services Ltd will make arrangements for unannounced spot checks to be made on each of the individual services delivered by its care staff, in the Service User's own home.

- The spot checks frequency will be centrally planned to respond to:

  - The level of risk to the Service User inherent in the individual care package. That risk may arise from:

    - The intensity of the service,
    - The complexity of the service, either in content or arrangements, for example requiring multiple staff
    - and high levels of communication,
    - The level of training and skill required of the carers,
    - The experience of the carers providing the service,
    - Potential issues with the environment within which the Service User lives, including isolation,
    - The Service User not having ready access to family or advocate support,
    - Any other factors relevant to the individual Service User's service, including temporary factors.

and a visit frequency established for each Service User.

- An appropriate person, such as the team leader, area manager or Registered Manager be allocated to each visit.

- The spot checks will be diarised to occur at approximately the intervals determined for each Service User, varying them slightly to ensure that they are unexpected.

- Spot checks will not be announced, unless overriding reasons related to Service User privacy, dignity and security are known. Spot check visits may be announced to the Service User prior to the visit, but never to the carers providing that service.

- The checker/interviewer will be provided with identification and authorisation to carry out the checks, which will be shown to the Service User and the staff member(s) before the check begins.

- On each check visit a standardised data set will be recorded, and the Service User will be asked to comment on the adequacy of the service and its delivery.

- The checker/interviewer will record the topics covered and the information gained during the spot checks.

- The checker/interviewer will report to the person managing the individual service any matters of concern, and any ideas for improving the service to that Service User. **Possible, potential or actual Safeguarding issues will be reported immediately by telephone to the Registered Manager.**

- The checker/interviewer will record any compliments, particularly regarding individual carers, for use during staff supervision.

- The interviewer/checker will make a brief note on the pro forma used of the overall adequacy of the service since the previous check, highlighting any areas of concern or for improvement.

- The Registered Manager will review the final notes on each spot check report at regular intervals:

  - follow through any serious matters or safeguarding issues identified to ensure satisfactory resolution, and
  - identify any trends or service wide issues which may become apparent from multiple reports, and take action.

- Crescent Community Care Services Ltd will inform Service Users of the policy of making spot checks, and the reasons for them, providing that information in a manner which reassures the Service User that the checks are positive management actions taken for the protection of the Service User and the improvement of their service.

- Crescent Community Care Services Ltd will train staff who carry out spot checks and the reasons for the checks, the scope of checks required, and that they are to conduct the checks in a positive manner, and at all times respecting the privacy, dignity and wishes of the Service User.

- Crescent Community Care Services Ltd will train staff who will be subject to spot checks to inform them in a positive manner of the reasons for the checks, and of the administrative arrangements for the checks.

- Spot checks will be carried out at short notice outside of the normal schedule when matters of concern, such as a complaint, are raised about the individual service or the well-being of the Service User.

Procedure

- Carry out a risk analysis for each Service User and establish the recommended spot check frequency.

- Diarise the planned visit schedule.

- Carry out the spot checks according to the schedule by instructing the check staff. Decide if the individual spot checks are to be carried out while the carer is with the Service User (the default check), or if a check is required while the carer is not present, such as where concerns regarding that carer may have been raised, and make the appropriate arrangements.

- Provide the checker with the relevant information which may focus the discussions, such as a complaint detail, and the documentation to manage and record the check.

- The checker will carry out the spot check according to the policy, the training provided, and using the documentation and individual Service User information provided.

- On completion of the spot check the checker will immediately inform the Registered Manager of any concerns, and deliver the check report to the Manager.

- Registered Manager – review the summary of each spot check report and take appropriate action.

- File the spot checks with the Service User data file, with a copy of any record where a concern has been recorded in the incident file organised by incident type, in order to look for trends over time.

- Compliments given to individual staff will be fed back to those staff, and recorded on their personal file.

Reviewed by Marcus Kerridge McColl
December 2025

Staff Identification Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure The Agency staff are adequately identifiable and identified.

- To comply with regulations on safeguarding Service Users.

Scope

- Š All employees of Crescent Community Care Services Ltd.

Policy

- All employees of Crescent Community Care Services Ltd will carry authenticated identification, issued by Crescent Community Care Services Ltd, which will state:

  - The name of the organisation;
  - The employee's name;
  - The employee's job title;
  - A photograph of the employee which is good quality and recent in terms of likeness.

- All employees of Crescent Community Care Services Ltd will, when making face to face contact with Service Users, show their identification.

- All employees of Crescent Community Care Services Ltd will, when making telephone contact with Service Users, clearly state their name, their job title and that they are employed by Crescent Community Care Services Ltd.

- All Service Users will be informed, as a part of the service induction, that they should check the identification of The Agency staff before admitting them into their home, and that they should feel free to contact The Agency office if they have any concerns.

Procedure

- Crescent Community Care Services Ltd will create and issue an identification card, or other media, which:

  - Contains the name of Crescent Community Care Services Ltd;
  - Contains the logo of Crescent Community Care Services Ltd;
  - States the employee's full name;
  - States the employee's job title;
  - Contains a clear and recognisable photograph of the employee;
  - Clearly states an expiry date;
  - Be laminated to make it tamper proof;

- To each new employee, before that employee has contact with Service Users.

- Crescent Community Care Services Ltd will create a register of issued cards, showing the expiry date of each, and one month before the expiry date of an individual card contact the employee to instruct them to attend the office to exchange the current card for a new card.

- When creating the replacement card, the issuer will check that the photograph in use is still current i.e. that the employee is clearly recognisable, and replace the photograph if necessary.

- On termination of an employee's employment contract, for whatever reason, the office will contact the employee and instruct them to attend the office on the last day of work to return company property, including the identification.

- If the employee does not return the identification, the reasonable costs of not returning the identification will be deducted from any wages owed to the employee, having first checked that a signed copy of "Authorisation to make deductions from wages" is on file. That cost may include the cost of contacting all Service Users with whom the employee has previously had contact, or all Service Users in the area serviced by the employee, to inform them that the employee no longer works for The Agency. Failure to prove the existence of a copy of "Authorisation to make deductions from wages" signed by the employee negates any right to make costs deductions.

- In the event of non-return of the identification, Crescent Community Care Services Ltd will contact all Service Users with whom the employee has previously had contact, or all Service Users in the area serviced by the employee, to inform them that the employee no longer works for The Agency.

- The relevant section of the agreement is:

  - "RETURN OF ORGANISATION EQUIPMENT / PROPERTY - If, on leaving the organisation, for whatever reason, you fail to return the organisation's equipment/property as may have been entrusted to you during your employment, at least 2 working days prior to your official leaving date, an amount equal to the cost of the property/equipment will be deducted from any final monies due to you."

Reviewed by Marcus Kerridge McColl
November 2025

Staff Rota Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that staffing levels are planned, adequate and in compliance with regulations. Š
- Enable staff to have one month's notice of their hours to be worked.
- Minimise management time dealing with the rota.

Scope

- All employees.

Policy

- The manager will calculate a clear specification of staffing requirements for each Service User at each time of the day and night, specifying qualifications, experience, numbers, and where Service Users have expressed a requirement, the gender of the staff on duty. The specification will take account of the aggregated needs of the Service User group in cases whereby more than one is provided with services at one location, as well as taking account of security risk assessments, and regulatory requirements.
- Where the calculated specification is in excess of that authorised by a commissioning agency, that agency must be informed and an agreement reached which meets the standards of the provider and the commissioner.
- The specification will be agreed with the Registered Manager, who will sign and date the written specification to signify agreement and authorisation.
- The manager or their delegate will manage the rota from time-to-time in such a way as to achieve, over a minimum 4 week period, a planned staffing of 100% of specification, with adequate reserves in place to cover absences for any reason.
- There will be a standard 4-week (or longer) rota, posted in the staff room and in addition communicated to all staff not working on site or visiting, varied by weekly variation sheets. The standard rota will be consistent with the agreed specification.
- Variation sheets will be completed and posted in the staff room and, in addition, they will be communicated to all members of staff that are not working on-site or visiting a minimum of one week before the week to which they refer.
- Employees with very similar experience and qualifications may swap shifts occasionally on a one-off basis to allow for personal and social requirements, but will in no circumstances "swap" shifts without informing the Manager in writing in advance of the swapped shift and entering the change on the weekly rota sheet. Failure to comply with this policy may lead to disciplinary action.
- Standard rota sheets and weekly variation sheets will be retained on file after use for a minimum of 20 years.
- Support Worker or Carers will be aged a minimum of 16 years. Young people between the ages of 16 and 18 may provide personal care, subject to:
- The informed consent of the Service User who may receive the care; » Being supervised by an experienced person over 18;
- Being enrolled on a recognised care training course.
- Not being left alone in sole charge of a service.

Procedure

- Any gaps created by holidays, requested days off, or sickness will be left as blanks on the weekly rota sheet for staff to volunteer themselves into those gaps.

- The Manager retains the right to resolve any conflicts relating to the working of the rota.

Principles of good rota design

- All staff will equally share unsocial hours, except for staff who have been specifically recruited to a limited range of hours, or have requested and had those limitations agreed by the Registered Manager. Favouring a small number of staff with more favourable hours will lead to inflexibility of the rota, difficult recruitment in the future, and develop dissatisfaction amongst the team.

- In particular, from the period 18th December to 7th January, during which time the Annual Holiday Policy and Procedure states that no holidays may be taken, the normal rotating duty rota will be suspended and the Manager will replace it with a rota which shares out the holiday duties equitably. The Christmas rota will also share the inconvenience from one year to the next i.e. employees who work Christmas Day one year will automatically work Boxing Day the next, and similarly New Year will rotate. Employees are, however, free to make other arrangements if they are able to arrange agreed "swaps" with other staff, but the Manager must approve such arrangements before they are entered onto the rota. This arrangement may be relaxed if Service Users indicate that they do not require services over the holiday period.

- Always confirm that the skill-mix on the standard rota is as authorised and at minimum-cost.

Reviewed by Marcus Kerridge McColl
December 2025

Statutory Time Off Policy and Procedure

Crescent Community Care Services Limited

Purpose

- To comply with legislation.

Scope

- All employees.

Policy

- The organisation's policy is to grant reasonable requests for time off, in addition to holiday and sickness absence, parental leave and maternity leave, subject to the operational needs of the business.

- The organisation will comply fully with legal requirements in respect of statutory time off work.

Procedure

Potential entitlements

- You may be entitled to *reasonable* time off, not necessarily paid, if you hold one or more of the following positions:
    - Representative of a trade union that we recognise for purposes of collective bargaining;
    - Safety representative;
    - Elected employee representative, for certain consultation purposes;
    - A public office;
    - A pension scheme trustee.

- You may also be entitled to reasonable time off, not usually paid, for the following activities:
    - Trade Union activities, such as meetings;
    - Ante-natal care, made on the advice of your doctor, midwife or health visitor (paid);
    - To deal with a domestic emergency yourself or involving a dependent;
    - If you are under notice of redundancy, to seek work or training;
    - If you are age 16 or 17, to train or study for a qualification (paid);
    - Accompanying a colleague to a disciplinary or grievance hearing (paid);
    - Jury service;
    - Court appearances;
    - Hospital appointments and doctors appointments where these cannot be arranged outside of working hours.
- Unless specified by legislation, what constitutes “reasonable time” will be determined by us as your employer, but we will listen to your views first before refusing, or disciplining you for, time off. "Reasonable" may be determined by both the duration and frequency of the time off and the circumstances giving rise to it.
- Appeals against decisions in these matters may be raised through the grievance procedure.

Applying for Leave

- Employees wishing to request time off for any of the above purposes must comply with the following procedure:
    - Requests for time off should be made in writing on the appropriate form (Request for Statutory Time Off Work) at the earliest reasonable opportunity and passed to the departmental manager. The expected period of absence and full details of the purpose for which time off is required must be specified. Where available, supporting documentary evidence should be supplied;
    - The departmental manager must consider the request in conjunction with the HR department, with respect to considering the individual’s eligibility, the relevance of the request and the needs of the business;
    - A copy of the form, marked “approved” or “not approved” and signed by the departmental manager should be returned to the employee;
    - The HR department will retain one copy of the form to an update their absence records for statistical purposes and to effect any necessary pay adjustments.

Other Time Off

- All requests for leave under the following headings should be made on the absence request form which will be processed by the HR manager in liaison with the HR department.
- **Jury service**:
    - The notification document from the court must be produced when making the request;
    - Employees should claim from the court the maximum amount to which they are entitled in respect of loss of earnings;
    - If employees report for jury service but are not called upon to serve on any particular day, they are expected to attend work on that day if it is reasonably practicable to do so in the time available;
    - The HR department will retain a copy of the absence request form and make arrangements for pay adjustments if necessary.
- **Court appearance as a witness:**
    - Where employees are called upon to give evidence as a witness in court they should produce supporting documentation when the absence request form is completed.
- **Special leave:**
    - Time off may be granted at the organisation’s discretion for special leave which covers:
        - Territorial Army training;
        - Study leave.
- In the event of an employee disputing a management decision in relation to time off, the company’s formal grievance procedure should be used.

Guidance to the Service Provider

- **Trade union duties or training:** officials of independent trade unions recognised by you as an employer have the right to take reasonable paid time off to attend to appropriate duties. This may include requests for training.
- **Trade union activities:** members of independent trade unions recognised by the organisation are entitled to take reasonable unpaid time off work to take part in the activities of their union. This means they may attend meetings (for example, to discuss a pay offer). It is wise to agree in advance how much time is to be allowed.
- **Safety representatives:** accredited safety representatives have the right to take paid time off as far as is necessary to carry out their duties and to undergo training, as have other elected safety representatives where there is no trade union representation.
- **Elected employee representatives:** employees who are elected by their colleagues to take part in the consultation process in a collective redundancy or business transfer situation are entitled to paid time off in order to carry out their duties and to undergo training. (This right also extends to candidates in such an election). Obviously these are special circumstances.
- **Public duties:** employees who hold certain public offices are entitled to reasonable unpaid time off. Local councillors, magistrates and tribunal members would be typical examples. The amount of time allowed may be limited (by what is "reasonable"), as may its frequency.
- **To seek work or arrange training:** employees with two years service or more, who are under notice of redundancy are entitled to reasonable paid time off for these purposes. As a guide here, "reasonable" may be up to a couple of days per week. Again, these are special circumstances.
- **Antenatal care:** All pregnant women may take time off with pay for antenatal care. The right does not extend to the partner. The right is for reasonable paid time off for such appointments, and this includes the time spent travelling to an appointment and waiting. It does not include entitlement to pay for time where the employee could reasonably attend work beforehand or return to work afterwards. Recent changes in legislation allows antenatal rights for fathers and partners, which means that fathers and partners can take unpaid time off work to accompany a pregnant woman to see a midwife or obstetrician. See the Maternity Policy and Procedure for details.
    - You cannot refuse time off for the first antenatal care appointment, but for subsequent appointments you can ask the employee for written proof of the appointment and a certificate or note from the employee's doctor or midwife, stating that she is pregnant. It may be possible for the employee to arrange for the appointments to take place outside working hours. You cannot insist on this nor in any way victimise the employee if she refuses.
- **Pension scheme trustees:** employee pension scheme trustees are entitled to take reasonable paid time off to carry out their duties and to undergo training.
- **To study or train:** employees aged 16 or 17 who have not achieved A-C grades in five GCSEs (or certain equivalents – see The Right to Time Off for Study or Training Regulations 2001) will be entitled to reasonable paid time off during working hours to undertake study or training which will lead to a qualification which will improve their future employment prospects. The right extends to 18 year old if they have already started their studies.
- **Request to study or train:** Employees who are employed by an employer with at least 250 employees and who have at least 26 weeks' continuous service have the legal right to make a request in relation to study or training and have their request considered.
- **To deal with an emergency involving a dependent:** employees will be entitled to take a reasonable period of unpaid time off work to deal with an unexpected or sudden problem and make any necessary longer term arrangements. The intention is to provide employees with the right to deal with the emergency. Other than in exceptional circumstances this would be for no more than 2 or 3 days.
- **Disciplinary or grievance hearings:** a worker, who may be a colleague, a full-time or lay trade union officer or a workplace representative, is entitled to take reasonable paid time off work to accompany a colleague to a disciplinary or grievance hearing and to confer with the worker before and after the hearing.

Reviewed Marcus Kerridge-McColl
November 2025

Purpose

- To ensure that standards of work are communicated and maintained.
- To provide opportunity for staff to discuss and reflect on their work practice.

Scope

- All employees.

Policy

- The purpose of supervision is to promote standardised, safe, and best practice working practices by:
  - Assisting employees to develop a full understanding of the standards to which Crescent Community Care Services Ltd works
  - To ensure that each individual employee is clear about their roles and responsibilities
  - To ensure individual employee understands and accepts accountability for their work
  - To identify ways to support the individual employee's personal development
  - To be a primary source of support for the employee by encouraging them to reflect on the achievements and challenges of the job
  - To provide regular and constructive feedback to individual employees on their performance
  - To encourage employees to share any issues or concerns they may have
  - Crescent Community Care Services Ltd believes that all employees should be formally supervised
- A clear difference will be established between employee appraisals and supervision:
  - Employee appraisals are intended to provide employees with a forum to discuss their capabilities, training needs and development plans in relation to the needs of the business
  - Supervision is intended to provide the manager or supervisor with a regular channel for communicating the standards of performance which the organisation requires of each employee, and facilitate a formal discussion about the employee's actual performance against those standards. It is also a time for reflection and discussion around attitudes and behaviours at work.
- Every employee will be invited to a supervision session with their manager or supervisor at least 3 times each year, and more often if a performance problem is under discussion.
- Where the Service is carrying out the QCS recommended Mock Inspection process using the recommended schedule of two monthly focus periods, supervisions during each Key Question Review Period will be focused on the employees' performance in relation to that Key Question. The advantage of this approach will be that all employees will be being assessed on the same skills at the same time, which will help make group discussions, coaching and formal training more meaningful and, therefore, more effective.
- Supervision sessions will take place in private. This does not imply a very formal setting; supervision should be frequent enough that simply using any empty room should be appropriate.
- Supervision records will be kept confidential between the employer and the individual employee.
- Supervision records will be stored in a manner which preserves their confidentiality.
- Sessions should aim to take around 15 minutes, but there may be times when longer discussion is required and it is important both parties should not feel rushed. Supervisors should allow reasonable flexibility to manage these sessions during a working shift.

Procedure

- During the first supervision with an individual supervisee, the Agreement for Supervision attached to this policy will be discussed and completed by the supervisor and supervisee. Additional time must be allowed for completion of the agreement. Good practice is to delay the first supervision until a day or more after the agreement session.

- A new agreement will be discussed and agreed if an employee's supervisor changes.

- The supervision agreement will be reviewed annually, or more frequently, as required.

- The supervisor maintains a diary of when supervision sessions are due, and will ensure that they are carried out within a reasonable period of falling due. Employees are provided with a copy of their supervision planner annually and advised of any changes as soon as is practicable.

- Each formal instance of supervision will be recorded using the forms attached to this policy. Employees should sign to agree the record is accurate and may retain a copy for their own information. Good practice is to also record informal supervisions, such as coaching or informal reprimands, on a supervision record sheet.

- Cancellation of appointments by the supervisor should be avoided as far as is possible, and on cancellation a substitute appointment must be made. Cancellations of appointments by the employee should only be accepted in unavoidable circumstances and frequent incidences of cancellations should be considered as poor performance.

- All personnel required to supervise colleagues must have attended formal training in the theory of supervision and the organisation's procedure, with competency checked and confirmed, prior to undertaking any supervision activity.

Notes

- **Rota:** Check with the employee if they have any issues with the current standard rota. The objective is to ensure that rota reviews meet, as far as is possible, the needs of employees as well as those of the organisation, and to avoid allegations of stress and/or harassment due to the rota deviating from agreed patterns.

- **Grievances**: Check with the employee if they have any grievances, either with the organisation or any individual within it. The objective is early identification of issues which may, if left unresolved, grow into significant issues which may in turn become contractual and/or disciplinary matters.

- **Safeguarding concerns**: discuss with the employee any potential concerns they may have around the safeguarding of the Service Users. This is an opportunity to remind employees of the need to recognise signs of abusive or discriminatory behaviour in the workplace.

- **Health and safety concerns**: It is helpful to explore the employee's safe working practice and encourage discussion as to the potential hazards and risks in the work environment

- **Personal development:** Check with the employee that any training which they have participated in has been put to use within the organisation, with positive results. The objective is to ensure that coaching, training, and other staff development has been usefully targeted, and that the employee is effectively using the investment which the organisation has made in them. Note that training, when carried out, becomes part of that employee's expected standards of work, and that they should be expected to comply with the standards set out in the training. Failure to do so is certainly a matter for discussion during supervision, and may become a matter for discipline. The approach during supervision is in contrast to the annual appraisal, which is focused on future development requirements, especially those required to meet the organisation's future objectives.

- **Timekeeping and attendance:** Take to the meeting records of the employee's timekeeping and attendance, and openly discuss with the employee any matters for concern, such as a pattern of regular late arrival or early leaving, and in particular any short sickness absences which give rise to lack of trust. Confirm the organisation's policy of low tolerance to poor timekeeping, and its policy to investigate all sickness absences with a view to ensuring that all such absences are supported by appropriate evidence, and that employees who cannot commit to regular attendance due to personal reasons may be subject to detailed review of their performance. The objective is to reduce poor timekeeping and eliminate "sickies".

- **Standards of working:** Review the employee's work performance against the relevant standards e.g. QCF Diploma standards for Carers; catering standards and standard of the food service for cooks; cleanliness and lack of odour for cleaners; efficiency of administration for administrators; maintenance standards and lack of breakdowns and breakages for maintenance staff. The objective is to regularly reinforce to all employees that the organisation has standards to which it must work and therefore to which all employees must work, and that employees who cannot, with appropriate training and support, reach those standards, will be subject to disciplinary procedures.

- **Working with colleagues:** Check if the employee has any issues with relationships with other members of staff, and raise any issues which other members of staff may have raised in this regard. The objective is to eliminate tensions between

individual employees, whether they arise for personal reasons or through accusations around work performance. Skilled supervisors will recognise where there are genuine relationship issues and avoid becoming embroiled in minor workplace disagreements.

- **Feedback from other employees and Service Users:** Feed back to the employee any positive or negative observations from either group. The objective is to praise for good work, and draw attention to poor performance. Supervisors should remember to respect and protect the confidentiality of all team members at all times.

- In all cases where an issue is identified and discussed, ensure that the employee agrees to a solution to the issue, and that the action(s) to be carried out are recorded in the Action column. Also ensure that both parties sign the document to denote agreement with the contents and particularly the agreed action plan, thereby allowing the supervisor to bring the record to the next meeting and discuss any progress or lack thereof.

- Both parties are to read the completed document, and sign and date it to denote that they have both agreed the actions specified.

- The employee must be asked to complete the Disclosure declaration at the end of the form at each supervision. This process must not become trivialised.

- Supervisors must remember that supervision is a formative and supportive process and not punitive.

- Sessions should be open and honest and care must be taken to instil confidence in employees of the value of this process, rather than make it an ordeal.

AGREEMENT FOR SUPERVISION

- Between: Supervisor and supervisee.

Crescent Community Care Services Ltd expectations

- Under the provisions of the Health and Social Care Act 2008, all staff must attend formal supervision sessions at least 3 times a year.

- The purpose of supervision is to promote standardised, safe, and best practice working practices by:

  - Assisting employees to develop a full understanding of the standards to which Crescent Community Care Services Ltd works
  - To ensure that each individual employee is clear about their roles and responsibilities
  - To ensure accountability for the work undertaken by each individual employee
  - To assist in the individual employee's personal development
  - To be a primary source of support for the employee
  - To provide regular and constructive feedback to individual employees regarding their performance
  - To encourage employees to regularly raise issues and grievances with respect to their employment

Reviewed by Marcus Kerridge McColl
December 2025

Training Policy and Procedure
Crescent Community Care Services Limited

Purpose

- By providing opportunities, facilities and financial help, the organisation aims to ensure that all employees are in possession of the knowledge, skills and experience necessary to perform their jobs to a satisfactory standard.

Scope

- All employees.

Policy

- The organisation believes that continuous improvement of its services is dependent upon the continuous development of the skills of its workforce. The organisation will therefore review and plan for that continuous development.

- The organisation believes that, as a consequence of the requirement for continuous development, in-house resources and in-house training and development events and support are the most effective way of achieving its aims and will use those strategies wherever possible. Experience has shown that in-house development is normally better received by employees, leads to better performance, achieves organisational goals more closely, and overall produces better results for the organisation and its employees.

- The organisation will ensure that it reviews (at least annually – see Business Plan) its operational plan and objectives to ensure that all objectives are supported by appropriate training and development.

- The organisation will review the needs of its Service Users (at least annually) to ensure that all needs are supported by appropriate training and development.

- The organisation will review all requests for development against its objectives to ensure that the activity supports the achievement of its objectives. Employees requesting development which is not required by organisational goals may be authorised by the Registered Person for individual reasons, such as a reward for exceptional attendance or performance.

- Employees supported by the organisation to attend external courses will prepare a teaching summary of the course content, and an evaluation, within seven days of completion of the course, and deposit the course material or a copy of the same in the library.

- The organisation will expect employees undergoing training to remain in employment for 12 months following the end of the training, otherwise a portion of the training costs will be recovered from the employee on termination.

- *The organisation follows a policy of up-skilling Care staff that are not Registered Nurses in order to compensate for the shortage of Registered Nurses.*

Procedure TRAINING PLAN

- Before the start of each financial year, the person responsible for training will agree an outline training plan and budget which is aligned with organisation goals and objectives for the year. The organisational goals for the year will include the results of a review of the Care needs of the Service Users in aggregate.

- The training plan will ensure that sufficient training is available to satisfy at least the minimum employee entitlement detailed below.

- All training requested or planned must checked to ensure that it is consistent with the organisation training plan, and therefore organisation goals and objectives, using the attached form.

- Focused Mock Inspection, Supervision and Training.

  - If Platinum Care at Home Ltd has adopted the focused and coordinated Mock Inspection, Surveys, Supervision and Training processes, as recommended by QCS, training activity will be scheduled mainly according to the planned Key Question Review Periods. During these Periods, Mock Inspections will take place, and staff supervisions and development activities will be focused on the same topic, in order to maximise the effectiveness of staff development. Refer to the Mock Inspection Toolkit – Introduction for an explanation of the approach.

TRAINING ENTITLEMENT

- Employees are entitled to a minimum of three days paid training per year, including in-house training. Note that focused personal development, such as recorded and focused Supervisions and individual coaching also count towards training time, and should be recorded as such.

- Under the Health and Social Care Act 2008, the organisation has to meet certain standards for the competence of its employees. These are as follows:

  - 50 per cent of all Care staff, excluding Registered Nurses, will be qualified to QCF Diploma Level 2.

- Managers will be qualified to QCF Diploma Level 4, and demonstrate having undertaken periodic and appropriate update training.
- The Registered Provider will undertake training to ensure that they have the appropriate skills for carrying out the running of the establishment or agency. The meaning of "appropriate" should be discussed with the Care Quality Commission to obtain a current definition.

- *In addition, the organisation believes that it is appropriate that:*

  - *All Registered Nurses, supervisors and departmental managers achieve QCF Diploma Level 3 by that date.*

- Trainees, including all staff under 18, should be undertaking an approved training programme in health and social care.

- All members of staff will receive training to NTO standards within six weeks of appointment, and foundation training to NTO standards within six months of appointment.

- Terms and conditions of employment will be varied to include these requirements. While the standards are placed upon the organisation and not the individual employee, the organisation believes that it can only achieve these required standards by requiring employees to achieve the standards individually, reviewing individual performance as part of the Performance Appraisal Procedure, and only agreeing to non-achievement by an individual in exceptional circumstances.

COSTS RECOVERY

- Employees undergoing training will be given the Reimbursement of Training Costs Form, they will have the recovery scheme explained to them, and will be asked to sign the form. If the employee refuses to sign the form, the uptake of the training will be at the direction of the Registered Provider alone.

OFF-SITE TRAINING

- All employees who attend off site courses will be asked to complete a course appraisal form. The evaluation will use a copy of the form attached. The teaching summary, together with all course material (or a copy of the material), will be packaged in a wallet-file and stored in the organisation's library for reference by other employees in accordance with the organisation's 'Specialist' training philosophy.

IN-HOUSE TRAINING

- All employees who attend in-house courses will be asked to complete a course appraisal form. The evaluation will use a copy of the form attached.

- **Best practice:**

  - The organisation believes that all employees must participate in the identification and dissemination of best practice in their work area.
  - Accordingly, employees will be polled to identify those persons with interests in a specific skill area, or specialist skills, who will then be appointed and recognised as 'Specialists' in that area. The Specialists will:

    - Receive prioritised access to all training and development in their specialist area

    - Be funded, within the training plan and training budget, to access such training, and research the specialism

    - Receive training in coaching, training and presentation skills

    - Be responsible for the dissemination of the specialist information to all other employees

    - Be responsible for identifying sources of best practice and publicising that standard within the organisation

- **Coaching:**

  - The most common form of training in the organisation is on-the-job training provided by colleagues and team leaders. All members of staff are expected to cooperate in training newcomers.

- The organisation gives regular tuition on coaching skills and team leaders should ensure that all relevant employees receive such tuition.

- **Work experience:**

  - Due to minimum age restriction on Carers, it is not possible for the organisation to offer work experience to any person under 18 in any area where contact with Service Users may occur, unless:

    - They have completed or are undertaking an approved training programme in health and social care;

    - The registered manager or a delegated person assesses the competence and confidence of the young worker to carry out all the tasks required of them, including where necessary intimate personal care;

    - That appropriate support is offered to the young worker;

    - The consent of the person being supported and/or their advocate has been obtained;

    - Inexperienced practitioners are not left in charge of a care setting or left to work on their own.

  - Where such a guarantee can be made, the following procedure applies:

    - From time-to-time the organisation will arrange for work experience students from local schools to spend a short time working in different departments. The cooperation of managers and other staff is requested in making such visits as useful and pleasant an experience as possible for the student concerned.

    - The Provider will agree in advance with each departmental head a suitable plan of work and the manager will be asked to complete a form detailing the tasks accomplished.

    - A programme of practical and directed training will be agreed with the scheme manager for each trainee, and thereafter they will be placed in appropriate departments by the Provider in consultation with departmental heads.

    - All managers responsible for trainees will be expected to adhere to the agreed programme, and any problems in this respect should be referred to the Provider.

    - Trainees under the scheme have the same protection as employees for the purpose of health and safety legislation and against sex and race discrimination. They are also subject to organisation rules on discipline, and any problems regarding conduct should be reported to the Provider.

  - **Sandwich students:**

    - Placements will be offered to sandwich students where operational circumstances make this possible. Full consultation will take place with the relevant managers.

  - **Induction:**

    - All employees will, on joining and when transferring to a new area of the organisation, participate in the organisation's induction programme. Full details are available from administration.
    - Managers are responsible for ensuring that members of staff under their control complete the initial induction course within six weeks of joining or transferring.

  - **In-house courses:**

    - Periodically the organisation will organise short courses in-house, usually where a group of employees require specific additional knowledge or training (e.g. changes in procedures, the introduction of new equipment, and courses for newly promoted supervisors). A list of organisation courses will be circulated to managers who will be responsible for ensuring that regular updates are provided. Nominations for courses should be sent in writing to the Manager. Suggestions for additional courses are welcomed.

  - **In-house resources:**

- The establishment provides a library of training materials in the form of books, videos, periodicals etc. All employees are requested to make use of these resources, taking care to return them as soon as possible to ensure their availability to all employees.

Reviewed by Marcus Kerridge McColl
December 2025

Training Policy and Procedure
Crescent Community Care Services Limited

Purpose

- By providing opportunities, facilities and financial help, the organisation aims to ensure that all employees are in possession of the knowledge, skills and experience necessary to perform their jobs to a satisfactory standard.

Scope

- All employees.

Policy

- The organisation believes that continuous improvement of its services is dependent upon the continuous development of the skills of its workforce. The organisation will therefore review and plan for that continuous development.

- The organisation believes that, as a consequence of the requirement for continuous development, in-house resources and in-house training and development events and support are the most effective way of achieving its aims and will use those strategies wherever possible. Experience has shown that in-house development is normally better received by employees, leads to better performance, achieves organisational goals more closely, and overall produces better results for the organisation and its employees.

- The organisation will ensure that it reviews (at least annually – see Business Plan) its operational plan and objectives to ensure that all objectives are supported by appropriate training and development.

- The organisation will review the needs of its Service Users (at least annually) to ensure that all needs are supported by appropriate training and development.

- The organisation will review all requests for development against its objectives to ensure that the activity supports the achievement of its objectives. Employees requesting development which is not required by organisational goals may be authorised by the Registered Person for individual reasons, such as a reward for exceptional attendance or performance.

- Employees supported by the organisation to attend external courses will prepare a teaching summary of the course content, and an evaluation, within seven days of completion of the course, and deposit the course material or a copy of the same in the library.

- The organisation will expect employees undergoing training to remain in employment for 12 months following the end of the training, otherwise a portion of the training costs will be recovered from the employee on termination.

- *The organisation follows a policy of up-skilling Care staff that are not Registered Nurses in order to compensate for the shortage of Registered Nurses.*

Procedure TRAINING PLAN

- Before the start of each financial year, the person responsible for training will agree an outline training plan and budget which is aligned with organisation goals and objectives for the year. The organisational goals for the year will include the results of a review of the Care needs of the Service Users in aggregate.

- The training plan will ensure that sufficient training is available to satisfy at least the minimum employee entitlement detailed below.

- All training requested or planned must checked to ensure that it is consistent with the organisation training plan, and therefore organisation goals and objectives, using the attached form.

- Focused Mock Inspection, Supervision and Training.

  - If Platinum Care at Home Ltd has adopted the focused and coordinated Mock Inspection, Surveys, Supervision and Training processes, as recommended by QCS, training activity will be scheduled mainly according to the planned Key Question Review Periods. During these Periods, Mock Inspections will take place, and staff supervisions and development activities will be focused on the same topic, in order to maximise the effectiveness of staff development. Refer to the Mock Inspection Toolkit – Introduction for an explanation of the approach.

TRAINING ENTITLEMENT

- Employees are entitled to a minimum of three days paid training per year, including in-house training. Note that focused personal development, such as recorded and focused Supervisions and individual coaching also count towards training time, and should be recorded as such.

- Under the Health and Social Care Act 2008, the organisation has to meet certain standards for the competence of its employees. These are as follows:

  - 50 per cent of all Care staff, excluding Registered Nurses, will be qualified to QCF Diploma Level 2.
  - Managers will be qualified to QCF Diploma Level 4, and demonstrate having undertaken periodic and appropriate update training.
  - The Registered Provider will undertake training to ensure that they have the appropriate skills for carrying out the running of the establishment or agency. The meaning of "appropriate" should be discussed with the Care Quality Commission to obtain a current definition.

- *In addition, the organisation believes that it is appropriate that:*

  - *All Registered Nurses, supervisors and departmental managers achieve QCF Diploma Level 3 by that date.*

- Trainees, including all staff under 18, should be undertaking an approved training programme in health and social care.

- All members of staff will receive training to NTO standards within six weeks of appointment, and foundation training to NTO standards within six months of appointment.

- Terms and conditions of employment will be varied to include these requirements. While the standards are placed upon the organisation and not the individual employee, the organisation believes that it can only achieve these required standards by requiring employees to achieve the standards individually, reviewing individual performance as part of the Performance Appraisal Procedure, and only agreeing to non-achievement by an individual in exceptional circumstances.

COSTS RECOVERY

- Employees undergoing training will be given the Reimbursement of Training Costs Form, they will have the recovery scheme explained to them, and will be asked to sign the form. If the employee refuses to sign the form, the uptake of the training will be at the direction of the Registered Provider alone.

OFF-SITE TRAINING

- All employees who attend off site courses will be asked to complete a course appraisal form. The evaluation will use a copy of the form attached. The teaching summary, together with all course material (or a copy of the material), will be packaged in a wallet-file and stored in the organisation's library for reference by other employees in accordance with the organisation's 'Specialist' training philosophy.

IN-HOUSE TRAINING

- All employees who attend in-house courses will be asked to complete a course appraisal form. The evaluation will use a copy of the form attached.

- **Best practice:**
  - The organisation believes that all employees must participate in the identification and dissemination of best practice in their work area.
  - Accordingly, employees will be polled to identify those persons with interests in a specific skill area, or specialist skills, who will then be appointed and recognised as 'Specialists' in that area. The Specialists will:
    - Receive prioritised access to all training and development in their specialist area
    - Be funded, within the training plan and training budget, to access such training, and research the specialism
    - Receive training in coaching, training and presentation skills
    - Be responsible for the dissemination of the specialist information to all other employees
    - Be responsible for identifying sources of best practice and publicising that standard within the organisation

- **Coaching:**
  - The most common form of training in the organisation is on-the-job training provided by colleagues and team leaders. All members of staff are expected to cooperate in training newcomers.
  - The organisation gives regular tuition on coaching skills and team leaders should ensure that all relevant employees receive such tuition.

- **Work experience:**
  - Due to minimum age restriction on Carers, it is not possible for the organisation to offer work experience to any person under 18 in any area where contact with Service Users may occur, unless:
    - They have completed or are undertaking an approved training programme in health and social care;
    - The registered manager or a delegated person assesses the competence and confidence of the young worker to carry out all the tasks required of them, including where necessary intimate personal care;
    - That appropriate support is offered to the young worker;
    - The consent of the person being supported and/or their advocate has been obtained;
    - Inexperienced practitioners are not left in charge of a care setting or left to work on their own.
  - Where such a guarantee can be made, the following procedure applies:
    - From time-to-time the organisation will arrange for work experience students from local schools to spend a short time working in different departments. The cooperation of managers and other staff is requested in making such visits as useful and pleasant an experience as possible for the student concerned.
    - The Provider will agree in advance with each departmental head a suitable plan of work and the manager will be asked to complete a form detailing the tasks accomplished.
    - A programme of practical and directed training will be agreed with the scheme manager for each trainee, and thereafter they will be placed in appropriate departments by the Provider in consultation with departmental heads.
    - All managers responsible for trainees will be expected to adhere to the agreed programme, and any problems in this respect should be referred to the Provider.
    - Trainees under the scheme have the same protection as employees for the purpose of health and safety legislation and against sex and race discrimination. They are also subject to organisation rules on discipline, and any problems regarding conduct should be reported to the Provider.

- **Sandwich students:**
  - Placements will be offered to sandwich students where operational circumstances make this possible. Full consultation will take place with the relevant managers.
- **Induction:**
  - All employees will, on joining and when transferring to a new area of the organisation, participate in the organisation's induction programme. Full details are available from administration.
  - Managers are responsible for ensuring that members of staff under their control complete the initial induction course within six weeks of joining or transferring.
- **In-house courses:**
  - Periodically the organisation will organise short courses in-house, usually where a group of employees require specific additional knowledge or training (e.g. changes in procedures, the introduction of new equipment, and courses for newly promoted supervisors). A list of organisation courses will be circulated to managers who will be responsible for ensuring that regular updates are provided. Nominations for courses should be sent in writing to the Manager. Suggestions for additional courses are welcomed.
- **In-house resources:**
  - The establishment provides a library of training materials in the form of books, videos, periodicals etc. All employees are requested to make use of these resources, taking care to return them as soon as possible to ensure their availability to all employees.

Reviewed by Marcus Kerridge McColl
December 2025

Unable to attend Workplace Policy and Procedure

Crescent Community Care Services Limited

Purpose

- To ensure that failure to attend the workplace, or workplace closure, due to adverse conditions is managed effectively.

Scope

- Situations where you are unable to travel to your workplace.
- Where the workplace is closed by us due to adverse conditions or other unforeseen circumstances.

Policy

- During bad weather or when there is severe transport disruption, you may have difficulties getting to work or returning home. You should make every reasonable effort to get into work. However, Crescent Community Care Services Limited is committed to protecting staff health and safety and does not expect staff to put themselves at risk. You should use your judgment, take heed of any travel warnings and not travel if the advice is to only make essential trips.

Procedure

- As the staff member you are legally required to ensure that you can attend work, it is not the responsibility of the employer, so in times of weather disruption the excuse of bad weather and transport is not an acceptable on under Employment Law.
- If you normally work in the community you should notify your manager of your circumstances and the manager will take responsibility for managing you and others in the best possible way.
- In all circumstances, the manager will make a judgment on whether:

  - You can be allowed to come in late, being paid for the hours of attendance, which may be extended to make up the normal paid hours for that day; or
  - You can work effectively from home; or
  - You can work at another location (you are not to report to another location unless that has been arranged); or
  - What work you may be able to do in the community
- If you are unable to attend work, or carry it out at all that day, or work from home, the Manager will agree with you:
  - Whether you wish to take a day's statutory holiday; or
  - Whether you wish to take a day's unpaid leave; or
  - Whether the lost time can be made up at a later date.
- While working in the community the management will gauge and determine the employees safety and work with the Client base to ensure that all calls are covered for the safety of the clients while balancing the safety of the employees in getting to the clients homes.
- Where attendance is possible but weather conditions show signs of worsening, your manager will obtain the best reports on road conditions etc, then reach a decision as to allowing you to leave early. It will be the manager's responsibility to make such a decision with regard to your, and possibly others', journeys to and from work and any other relevant circumstances.

Guidance to the service provider

You are expected to:

- Treat all staff fairly and consistently;
- Make available a number on which a relevant manager can be contacted in the event of adverse weather;
- Ensure that adequate communication takes place with your staff where these circumstances arise;
- Have regard to the Health and Safety of staff, particularly where special conditions apply such as pregnancy or disability;

Reviewed by Marcus Kerridge-McColl

November 2025

Unable to Gain Access Policy and Procedure
Crescent Community Care Services Ltd

Purpose

- To ensure that all care is delivered safely and in accordance with contractual requirements.
- To safeguard all vulnerable Service User living in the community.

Scope

- All Carers.

Policy

- Carers are required to alert Platinum Care at Home Ltd if they are unable to gain access to a Service User property.

Procedure

- If a Carer is unable to gain access to a Service Users home, they must report this to their Line Manager immediately.
- The Carer must not leave the property until they have been told to do so by their Line Manager.
- The Carer must look through the windows to see if they can see the Service User.
- The Carer must knock on the door and window to gain the attention of the Service User.
- The Carer must call out the Service User name through the letterbox and listen for any reply. It is important that the Carer does not shout or scream through the letterbox and potentially startles the Service User.
- The Carer should ask neighbours if they have seen the Service User.

- The Carer should try, if safe to do so, to gain access through the rear of the property.
- The Carer should update their Line Manager on the actions taken and any feedback from neighbours.
- The Line Manager should make extensive enquiries about the whereabouts of the Service User, such as contacting the next of kin or the local hospital to check for a recent admission.
- The Line Manager should keep in contact with the Carer at all times as to the progress of their enquires.
- When making their enquiries, the Line Manager must record the name of every individual and the time they contacted them. This should be recorded on the Service User Care Plan.
- If the Service User cannot be positively located elsewhere within 30 minutes of the Carer reporting the No access, the Line Manager must contact the Local Police on 08450 454545 to advise of the No access and to request a Welfare Check.
- The Police may decide to force entry if necessary.
- All actions must be reported to the local authority.

Reviewed by Marcus Kerridge McColl
November 2025

Use of Hidden Cameras Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To guide Services in complying with the legal requirements of the Data Protection Act 1998 and the Data Commissioner's guidelines on the use of CCTV, while following the CQC guidance on use of hidden cameras.

Scope

- All camera recording equipment.

- Where misconduct and abuse are suspected or reported, and evidence is required.

Policy

- The QCS CCTV Policy and Procedure must be read and understood before considering the use of hidden cameras, because all camera recording activities must comply with the Data Protection Act 1998, which over- rides any other considerations.

- The CCTV Policy and Procedure covers a much wider range of possible camera recording activities than use of a hidden camera, but the probable reason for the use of hidden cameras is included in the range – under the description 'To provide evidence in support of misconduct'. It is therefore essential to follow the analysis of alternative actions, appropriateness and legality set out in the CCTV Policy and Procedure.

- Advice and considerations deriving from the CQC guidelines:

  - **Issues common to the proposed use of hidden cameras by either the Service or the Service User or their representatives.**

    - The CQC advises, in accordance with the Data Commissioner's guidelines, that consideration first be given to other methods of dealing with or producing evidence about the possible problem.

    - The CQC points out that informed consent from the Service User who is to be observed must be obtained and recorded. If the Service User is unable to give informed consent, then the Service must carry out a best interests analysis, and only act in the best interests of the Service User. All conversations and decisions must be recorded.

    - Consideration should be given to giving the Service User who is subject of the investigation personal control over the switching on and off of the hidden camera.

    - If a Service User or representative complains about an issue which may need to be further clarified, this is a complaint, and in the first instance should be dealt with under the QCS Complaints, Suggestions and Compliments Policy and Procedure. Only after exhaustion of the complaints procedure without having gathered the evidence with which to proceed to a resolution should a hidden camera be considered.

- If the CQC is given a recording which shows poor care, it will share that with the inspector of the service, who will progress that matter with the Service. The CQC may, depending on circumstances, opt to share the recording with the police, Adult Abuse Team, Disbarring Service, Health and Safety Executive, or other appropriate bodies.

- **Proposed use of hidden camera by the Service**

  - It is consistent with the requirement for a safe service that the Service responds quickly and effectively when an allegation of poor care is made. In the event that the allegation focusses on the actions of a single individual or group acting outside of the behavioural standards required by the Service, then use of a hidden camera, after exhausting all other avenues for investigation, may be appropriate. The Service will maintain the trust and co-operation of the Service User and/or representative if both parties co-operate in using the camera, jointly consider any evidence of poor practice observed, and agree any further action required.

  - Full co-operation also meets the requirement for an open and transparent approach to care and support.

  - If poor care is identified by the recording, in the first instance this should trigger an immediate disciplinary process, the start point depending on the details of the action. Minor infringements of acceptable behaviour or skill levels might for instance be dealt with by informal coaching, retraining or formal supervision, but short of formal disciplinary action. However, where the nature of the infringement is considered more serious, and particularly if Service User safety or wellbeing is compromised, the formal disciplinary process may be initiated (refer to the Discipline Policy and Procedure for the legally compliant process), and the matter should be reported to the CQC and the appropriate the local Safeguarding Team.

- **Proposed use of hidden camera by the Service User and/or their representatives**

  - The Service welcomes discussions of alleged problems affecting the care and safety of Service Users, as set out in the Complaints, Suggestions and Compliments Policy and Procedure. Where the nature of the problem precludes the gathering of evidence by other means, for instance, where the alleged actions take place behind closed doors and are deliberately concealed, then the Service will co-operate in order to consider if the use of a hidden camera, in line with the common guidelines set out above, is the appropriate response, and will assist that use if it is decided that hidden camera use in the only way of gathering the required evidence.

  - The Service User and their representatives are of course able to unilaterally use a hidden camera if a co-operative approach is not seen to be appropriate, such as when the Service has been obstructive or unresponsive to complaints, or when the alleged problem concerns management personnel themselves.

  - However, Service Users and representatives are warned in the CQC guidance that use of a hidden camera without the express permission of the Service may contravene, and therefore, cancel the contract between the Service and the Service User, possibly resulting in the ending of service provision.

  - If poor care is identified by the recording, in the first instance this should be reported to the Service Provider in line with the Complaints, Suggestions and Compliments Policy and Procedure, and then to the CQC if the response of the Service is not considered to be adequate.

- The QCS Service User Handbook will include guidance regarding use of Hidden Cameras by the Service User and/or their representatives.

Procedure

- Follow the analysis detailed in the CCTV Policy and Procedure, taking into account the specific CQC advice given above.

Reviewed by Marcus Kerridge McColl
December 2025

Purpose

- The organisation believes that compliance with the Public Interest and Disclosure Act 1998 and the Enterprise and Regulatory Reform Act 2013 is consistent with its objective of providing a high-quality service, and that use of an agreed procedure will support that compliance.

Scope

- All employees.

Policy

- This organisation encourages a free and open culture in its dealings with its officers, employees and all people with whom it engages in business and legal relations. In particular, this organisation recognises that effective and honest communication is essential if malpractice is to be effectively dealt with and the organisation's success ensured.

Procedure

- Members of staff are likely to be the first to realise that there may be something seriously wrong within the organisation. However, they may not express their concerns because they feel that speaking up would be disloyal to their colleagues or to their employer. They may also fear harassment or victimisation. In these circumstances, it may be easier to ignore the concern rather than report what may just be a suspicion of malpractice.

- The organisation recognises the need for, and importance of, whistleblowing as a means of ensuring that staff can confidentially raise issues of concern and conscience inside the organisation and therefore members of staff are encouraged to raise genuine concerns about malpractice at the earliest practicable stage. This procedure is, accordingly, intended to provide a safeguard to enable members of staff to raise concerns about one or more of the following that has occurred, is occurring, or is likely to occur. These qualifying disclosures are set out in The Public Interest Disclosure Act 1998:

  - That a criminal offence has been committed, is being committed or is likely to be committed;
  - That a person has failed, is failing or is likely to fail to comply with any legal obligation to which they are subjected;
  - That a miscarriage of justice has occurred, is occurring or is likely to occur;
  - That the health and safety of any individual has been, is being or is likely to be damaged. It must indicate a greater danger than is associated with the normal use of the process/product, or a danger that is not usually associated with it;
  - That information tending to show any matter falling within any one of the preceding examples has been, is being or is likely to be deliberately concealed.
  - More recent changes to the law (June 2013) state that disclosures will now only be a qualifying disclosure if the worker reasonably believes that the disclosure is "in the public interest". This follows amendments made by the Enterprise and Regulatory Reform Act 2013.

- Any person wishing to raise concerns should report the matter to the Registered Manager, in a confidential manner, when such concerns are with regard to matters such as the following: fraud and financial irregularities; serious maladministration arising from deliberate improper conduct; miscarriages of justice; failure to comply with legal obligations; unethical activities that may be of a criminal nature; or dangerous acts or omissions that create a risk to health, safety or the environment within the sphere of activities of the organisation. In the event that the concerns relate to the Registered Manager they should be reported to the Registered Provider.

- If you do not feel that the Registered Manager or Registered Provider will appropriately handle your concerns, you may report your concerns directly to the CQC. The CQC will not disclose your identity without your consent unless there are legal reasons requiring them to do so, e.g. where your information is about a child or vulnerable adult who is at risk.

- It is important to recognise that whistleblowing is not a substitute for the Grievance Procedure that is available to staff who have individual or collective complaints about their management.

- Concerns should normally be raised in writing, but oral reports will also be accepted on the understanding that instances of whistleblowing raised anonymously will not be investigated.

- Where an abuse complaint is received by a member of staff and refers to the actions of the Manager or Deputy Manager, then the referral must in the first instance be made to the Social Services Adult Protection Team. The Adult Protection

Team will take the responsibility of informing other agencies. The contact details for your local Social Services are as follows:

0300 555 1386
adult.services@hants.gov.uk
Out of Hours - 0300 555 1373

- Responsibility for dealing with any concerns reported will lie with the Registered Manager who will have access to the Registered Provider.

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- The Registered Manager, in consultation with the Registered Provider, shall have discretion over the nature of the investigation into concerns raised, including, where it is considered appropriate, the involvement of the Inspection Unit or Auditors. If there is evidence of criminal activity, the Police will be informed.

- A member of staff will never be disciplined for raising a concern, so long as they follow the Whistleblowing Procedure or make disclosures in accordance with the Public Interest Disclosure Act 1998. However, disciplinary action could be taken if someone used the Whistleblowing Procedures to raise false concerns in bad faith. The Registered Provider shall decide whether disciplinary action is to commence.

- The organisation will take any steps necessary to ensure that those who raise concerns are protected from bullying or victimisation by fellow workers.

- The organisation will regard a member of staff's actions as legitimate if they have:

  - Followed the Whistleblowing Procedure;
  - Acted in good faith and not for personal gain or out of personal motives;
  - Had reasonable grounds for believing that the information disclosed indicates the existence of one or more of the following:

    - That a criminal offence has been committed, is being committed, or is likely to be committed;
    - That a person has failed, is failing, or is likely to fail to comply with any legal obligation to which they are subject;
    - That a miscarriage of justice has occurred, is occurring or is likely to occur;
      That the health and safety of any individual has been, is being or is likely to be damaged.
    - That information tending to show any matter falling within any one of the preceding examples has been, is being or is likely to be deliberately concealed.

- There is also, within the organisation's contract of employment, a specific confidentiality clause. The breach of this condition could be regarded as gross misconduct which would justify summary dismissal.

- All matters of concern must be raised internally in the first instance. The purpose of this is to give the organisation every chance to investigate and take appropriate action before Disclosure is in the public domain.

Reviewed by Marcus Kerridge McColl
December 2025

Witnessing of documents Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To comply with regulations and quality standards
- To avoid conflicts of interest

Scope

- All employees

Policy

- To avoid all possible conflicts of interests, no employee of Crescent Community Care Services Limited may witness or sign any documents for a Service User or family member
- All employees are expressly forbidden to act as a witness to the signatures of any documents such as wills and testaments.
- Neither Employees nor the Service may be a beneficiary under a will of any past or present Service user
- Gifts to employees are subject to disclosure to both the Registered Manager and the Directors and may be refused to avoid conflict of interest

Procedure

- Service users and/ or family members must be politely informed of the policy and the reasons for it and will be asked to provide their own witnesses

Reviewed Marcus Kerridge-McColl
September 2025

Working Alone Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To safeguard the health and safety of employees working alone.

Scope

- Any employee working alone.

Policy

- This organisation recognises that solitary workers face particular problems, and will not require employees to work alone where this results in unacceptable risks.

- The organisation will ensure that all solitary workers are thoroughly trained at the commencement of employment, receive regular monitoring and supervision, and are instructed in all matters relating to their health and safety at work.

- The following procedure should be read in conjunction with the organisation's health and safety procedure.

Procedure

- The organisation is anxious to ensure that this procedure for solitary workers ensures safe and effective systems of work at all times.

- All employees are therefore required to draw to the attention of their superior any amendments or additions to the procedure which may become necessary from time-to-time.

- All employees have a responsibility to act in such a way as not to put themselves or their colleagues at risk.

- Where employees work alone the organisation places even greater trust in them to act responsibly and safely.

- Any breaches of that trust will be viewed very seriously and may result in disciplinary action being taken.

- All candidates for solitary worker positions must undergo a full medical examination.

- Each individual's suitability on medical grounds will be related to the specific job applied for and the particular hazards and risks associated with that job.

- Requests for medical reports will be subject to the Access to Medical Reports Act 1988 where appropriate.

- All new appointees to solitary worker positions will receive comprehensive induction training.

- All employees must satisfy their supervisor that they are competent in all aspects included in the training, which will cover:

  - The duties of the particular post;
  - Safety aspects of all premises, machinery / equipment to be used;
  - Emergency procedures: fire, accident, illness, physical attack;
  - Issues regarding how to handle behaviour which may be challenging, how to defuse any difficult situation which may arise as a result of such behaviour, and the reporting procedures to be used when concerns arise in the mind of the lone worker about the behaviour of anyone who they meet or may meet in the performance of their duties;
  - Departmental systems of monitoring, supervision, and employee support;
  - Pay, employment conditions, organisation rules.

- Supervisors are responsible for ensuring that safe working systems are operated throughout their departments.

- These systems should be set down in writing and copies given to all members of staff.

- Supervisors are responsible for setting up and maintaining an effective procedure relating to all premises and equipment/machinery used by their staff, to ensure that:

  - All premises, equipment / machinery are safe; defects are quickly reported and rectified, and regular maintenance is carried out.

- Supervisors will regularly review the training needs of their staff, and ensure that refresher training, or training in new working methods, is provided.

- All solitary workers will have verbal contact with their supervisor at least once every shift.

- In addition each supervisor will hold departmental meetings, which all staff must attend, on a regular basis, ie monthly/quarterly/etc.

- All workers will be informed by their supervisor whom they should contact for help and support in fulfilling their duties.

- Where possible the contact will be an office-based employee, and readily available.

- In the event that the contact is unavailable and advice is urgently required, employees should contact the department manager.

- All solitary workers will have quick and easy access to first aid facilities.

- Mobile workers will be provided with a first aid kit.

- Mobile workers must have, or be provided with, a means of communication with their supervisor or a central point which can receive their communication.

- In circumstances where the organisation accepts that a solitary worker could have difficulty in raising the alarm in an emergency, the organisation will install or provide an alarm system appropriate to the situation.

- Where solitary workers are mobile during their working day, systems will be established whereby the location of individuals at any particular time can be determined:

  - This may include the requirement to report to a central point at the end of a working period;
  - Such systems must be strictly adhered to.

- Temporary or casual workers will not be used to cover absence by solitary workers, unless they are known to be competent and fully trained in all the relevant procedures and systems of working.

Reviewed by Marcus Kerridge McColl
December 2025

ISO 9001:2008
5 - MANAGEMENT RESPONSIBILITY

5.3	**Quality Policy**
Summary of Requirements	The Quality Policy must: a) Be appropriate to the Organisation's business activity b) Include a commitment to comply with the Quality Management System c) Include a commitment to continually improve the Quality Management System d) Provide a framework for establishing and reviewing quality objectives e) Be communicated and understood within the Organisation f) Be regularly reviewed to confirm continuing suitability

	STATEMENT/PROCEDURE
1.	In order to provide evidence of the Organisation's commitment to the Quality Policy, the Policy is regularly reviewed and any changes approved as part of the formal Management Review proceedings. These reviews and all approved changes are recorded in the minutes of the Management Reviews (reference Section 5.6).
2.	Copies of the Quality Policy are made available to all members of staff.
3.	Copies of the minutes of Management Reviews, or extracts thereof, are provided to individual members of staff in accordance with their role and responsibilities as a means of communicating the effectiveness of the Quality Management System.

5.4	**Planning**
5.4.1	**Quality Objectives**
Summary of Requirements	Quality objectives must be established that are measurable, in accord with the Quality Policy and include a commitment to continual improvement. These objectives must also address product requirements.

	STATEMENT/PROCEDURE
1.	Quality objectives are established as part of day-to-day management, are fully defined by the application of the procedures set out in this Quality Procedures Manual at Section 7.1, and confirmed by the application of the procedures and conclusions outlined in Section 5.6 Management Review.
1.	The Organisation considers that implementing the contents of this Quality Procedures Manual to meet ISO 9001 : 2008 requirements constitutes the primary quality objective. Our commitment is to provide all customers with the best possible service, and we recognise the importance of continually evaluating our standards, ensuring we provide a complete service to our customers. Accordingly, regular review and audit is used to continually monitor and measure the effectiveness of our procedures.
2.	Staff are fully trained and supported by revised requirements resulting from changing legislation and service user needs. Individual goals are determined under review at annual appraisal.
3.	Feedback is actively sought. This process measures the level and standard of service that customers can require and expect, and provides management with information upon which continual improvement can be measured and achieved.
4.	To develop and maintain quality consciousness amongst management and employees as stated in the Organisation's Quality Policy.

CUSTOMER SATISFACTION OBJECTIVES

ORGANISATION: CRESCENT COMMUNITY CARE SERVICES LIMITED

Date: September 2013

We have been successfully assessed to the new BS EN ISO 9001 standard. This standard focuses very much on customer satisfaction which includes the setting of Customer Satisfaction Objectives.

These objectives apply to all staff and are intended to complement our organisations activity, which is to provide a first class service to our customers.